

Our data and cloud alliances multiply what we deliver

PARTNER LOGO

Alliance partner A

PREMIER PARTNER

86 52

joint engagements clients served

PARTNER LOGO

Alliance partner B

SELECT PARTNER

61 38

joint engagements clients served

PARTNER LOGO

Alliance partner C

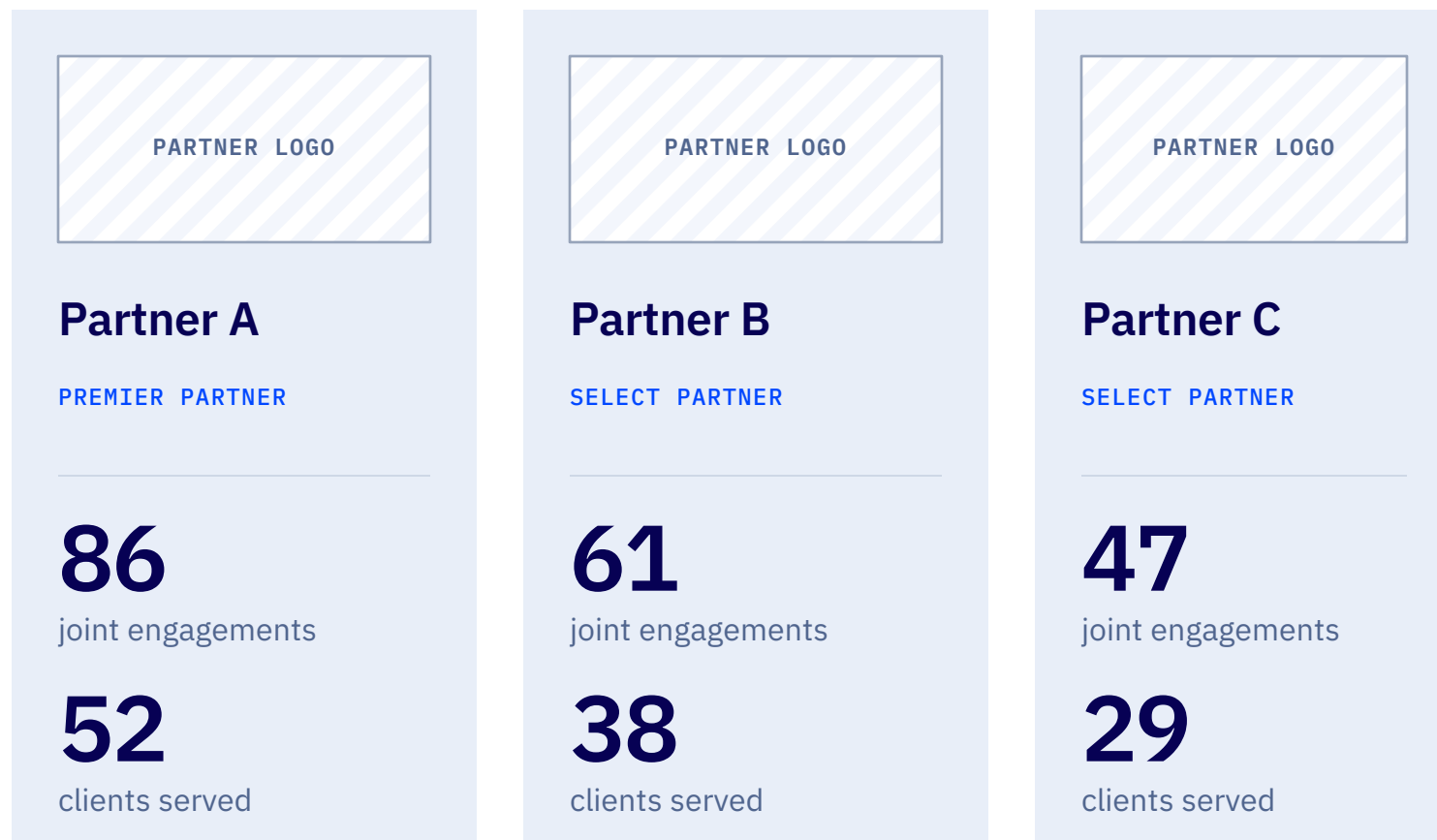
SELECT PARTNER

47 29

joint engagements clients served

Source: Sample figures for layout only; replace with current firm data before use.

Our alliances multiply what we deliver



Source: Sample figures for layout only; replace with current firm data before use.

WHY ALLIANCES MATTER

194

joint engagements with our top three alliance partners

Joint teams bring platform depth to our industry expertise, so clients get one plan and one delivery team.

Our data and cloud alliances multiply what we deliver

PARTNER LOGO

Alliance partner A

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PARTNER LOGO

Alliance partner B

SELECT PARTNER

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joint engagements clients served

PARTNER LOGO

Alliance partner C

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29

joint engagements clients served

WHY IT MATTERS

Joint teams bring platform depth and delivery discipline, so clients adopt faster.

Source: Sample figures for layout only; replace with current firm data before use.

Trusted by leaders across industries and technology

CLIENTS WE SERVE



OUR PARTNER ECOSYSTEM



West Monroe at a glance

We are tech-forward advisors who draw on decades of industry experience to create value quickly and execute with precision.

WHAT WE DO

- Artificial intelligence
- Operations and technology
- Organization and people

WHERE WE DO IT

- Healthcare and life sciences
- Financial services and insurance
- Energy, industrial and high-tech

“

A team that excels at every phase of complex work.

CHIEF DATA OFFICER, REGIONAL
PAYER

People are at the core of everything we do

99%

of clients consider us a
strategic advisor

71%

of employees say their
work has real impact

10K+

pro-bono and volunteer
hours last year

Source: Sample figures for layout only; replace with current firm data before use.

We bring deep experience to eight industries



CONSUMER AND
INDUSTRIAL PRODUCTS

64 **38**
engagements clients



ENERGY AND
UTILITIES

48 **29**
engagements clients



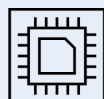
FINANCIAL SERVICES

72 **41**
engagements clients



HEALTHCARE

110 **57**
engagements clients



HIGH-TECH AND
SOFTWARE

58 **34**
engagements clients



INSURANCE

44 **26**
engagements clients



LIFE SCIENCES

36 **22**
engagements clients



PRIVATE EQUITY

52 **31**
engagements clients

Source: Sample figures for layout only; replace with current firm data before use.

WHY IT MATTERS

One finance playbook, tuned to each industry.

Each card shows engagements /
clients over the last five years.

ABOUT US

Eight industries we know well



CONSUMER AND INDUSTRIAL
PRODUCTS

64 / 38



ENERGY AND UTILITIES

48 / 29



FINANCIAL SERVICES

72 / 41



HEALTHCARE

110 / 57



HIGH-TECH AND SOFTWARE

58 / 34



INSURANCE

44 / 26



LIFE SCIENCES

36 / 22



PRIVATE EQUITY

52 / 31

Source: Sample figures for layout only; replace with current firm data before use.

We bring deep experience to eight industries



CONSUMER AND
INDUSTRIAL PRODUCTS

64

engagements

38

clients



ENERGY AND
UTILITIES

48

engagements

29

clients



FINANCIAL SERVICES

72

engagements

41

clients



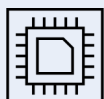
HEALTHCARE

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engagements

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clients



HIGH-TECH AND
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58

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INSURANCE

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engagements

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LIFE SCIENCES

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PRIVATE EQUITY

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ABOUT US

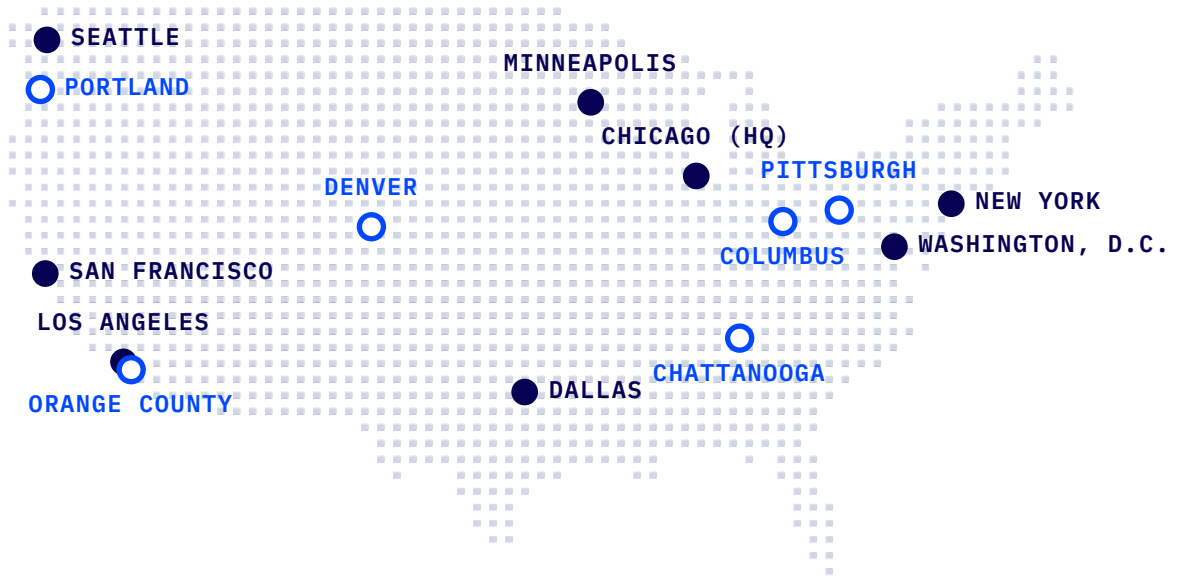
Eight offices and six talent hubs across the US, plus two abroad

INTERNATIONAL OFFICES

- London
- Costa Rica

TALENT HUBS

Teams of consultants who work from a metro area without a local office.



■ OFFICE

□ TALENT HUB

8

US offices

6

talent hubs

1,500+

people in all locations

Source: Sample figures for layout only; replace with current firm data before use.

ABOUT US

Ten offices and six talent hubs across three countries

■ OFFICE
□ TALENT HUB

10

offices in three countries

6

US talent hubs

1,500+

people in all locations



Talent hubs are consultant teams who work from a metro area without a local office.

Source: Sample figures for layout only; replace with current firm data before use.

Each subtrack has defined activities and a deliverable

PHASE	SUBTRACK	KEY ACTIVITIES	DELIVERABLE	WEEKS
Rationalize	Portfolio assessment	Interview owners, scan code, score each application	Scored portfolio	4
	Application rationalization	Hold disposition workshops and agree verdicts	Disposition list	4
	Target architecture	Design the scheduling and billing target state	Reference design	4
	Business case	Model cost, risk and benefit by wave	Funded roadmap	2
Build	Re-platform	Stand up the landing zone, move scheduling	Live platform	12
	Refactor	Split billing into services behind the existing interface	Billing services	10
	Data migration	Map, load, reconcile and sign off patient data	Reconciled data	12
	Cutover	Rehearse three waves, go live, hold rollback	Production cutover	6
Sustain	Run and operate	Stand up monitoring, runbooks and a support rota	Operations handover	12
	Optimize	Review cost, performance and capacity each quarter	Quarterly review	4
	Modernization governance	Set standards, funding rules and decision rights	Governance charter	4
	Continuous modernization	Refresh the backlog of next candidates each release	Backlog	Ongoing

ACTIVITIES BY PHASE

Who leads each activity in each phase

Every activity has a West Monroe lead and a named Northfield owner, so decisions and handovers never wait on a role that is unclear.

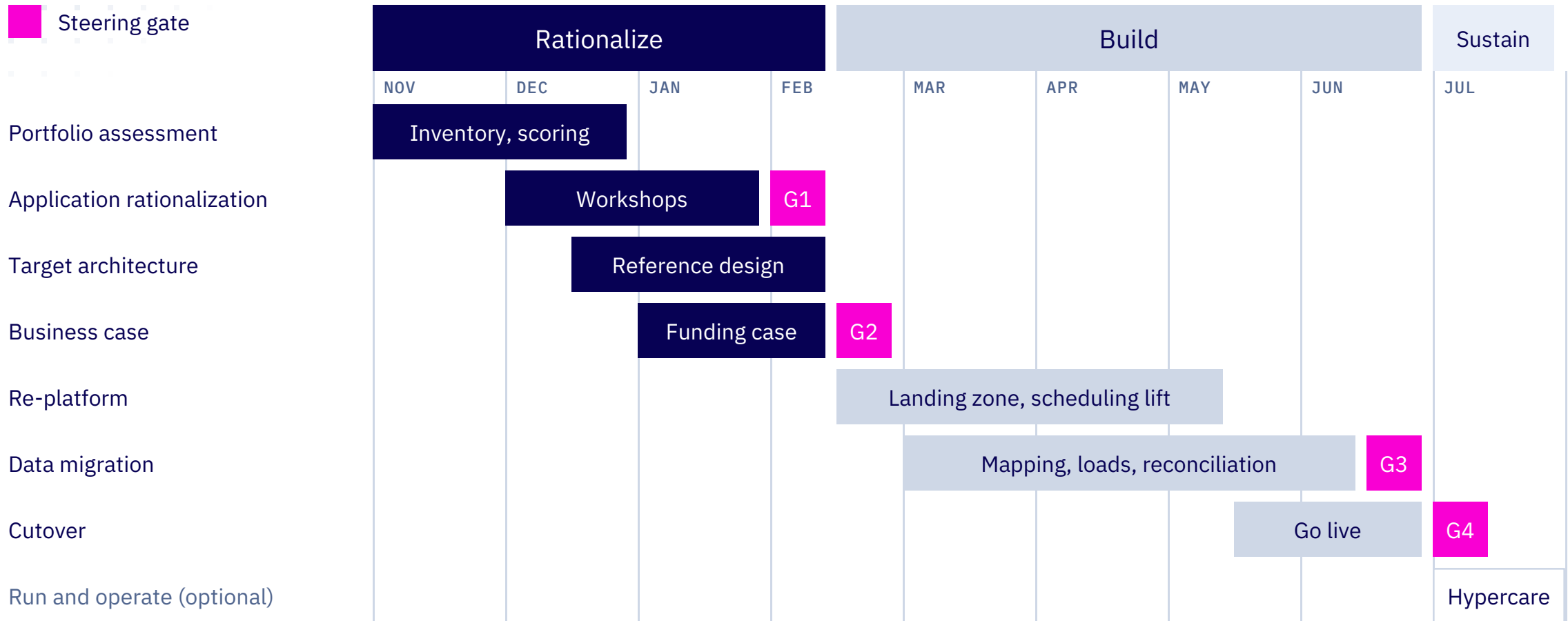
HOW WE WORK

Ownership rule

- One owner on each side
- Owner signs the phase gate
- Escalate within two days

ACTIVITY	PHASE	WEST MONROE	NORTHFIELD	WKS
Application scoring	RATIONALIZE	Portfolio lead	App owners	4
Disposition workshops	RATIONALIZE	Strategy lead	CIO office	4
Target design	RATIONALIZE	Architect	Architecture	4
Funding case	RATIONALIZE	Lead partner	Finance	2
Landing zone	BUILD	Cloud lead	Infrastructure	12
Billing refactor	BUILD	Tech lead	Revenue cycle	10
Data reconciliation	BUILD	Data lead	Data stewards	12
Wave cutover	BUILD	Delivery lead	Patient access	6
Hypercare	SUSTAIN	Support lead	Service desk	12
Quarterly tuning	SUSTAIN	Cloud lead	Infrastructure	4
Governance charter	SUSTAIN	Strategy lead	CIO office	4
Modernization backlog	SUSTAIN	Portfolio lead	Product leads	Ongoing

Nine months from portfolio view to Northfield cutover



Four weeks after go-live, adoption is 14 points behind plan

61%

of bookings in the platform

▼ 14 PTS VS PLAN

1,610

staff active this week

92%

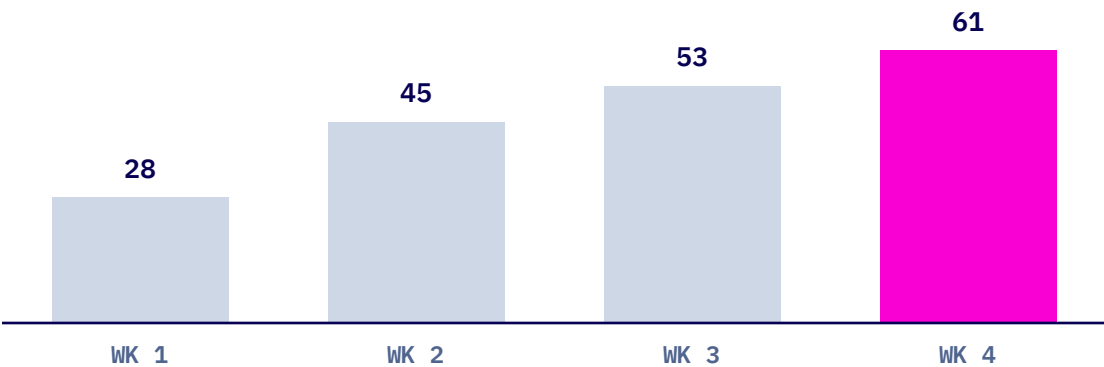
training complete

38

open tickets per day

Adoption by week

PERCENT OF BOOKINGS



Source: Northfield scheduling platform telemetry, week 4; sample figures for layout only.

SITE	ADOPT.	STATUS	SUPPORT
Main campus	91%	On track	
Lakeside	84%	On track	
Eastgate	72%	At risk	
Ridgeview	58%	At risk	
Harbor Clinics	41%	Off track	

Adoption trails plan at three of five sites

61%

of bookings in the platform

92%

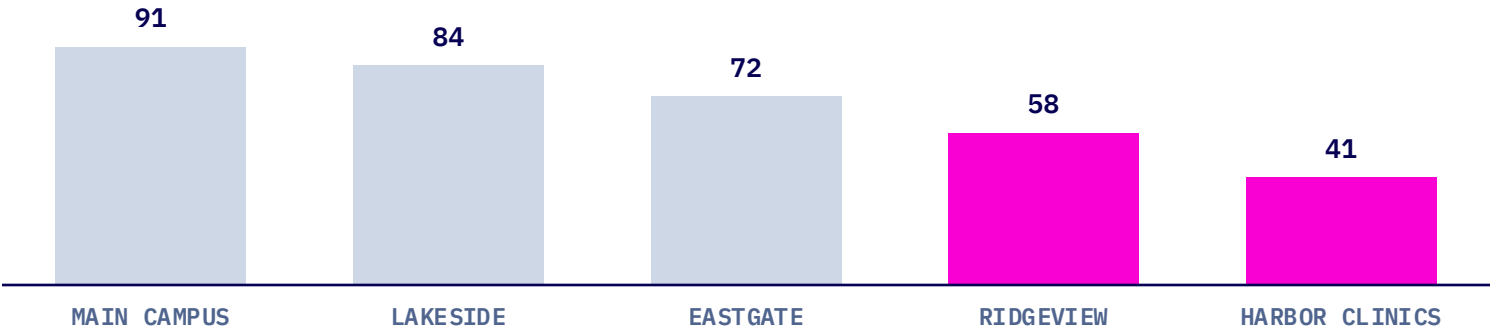
training complete

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Source: Northfield platform telemetry, week 4; sample figures.

Adoption by site
PERCENT OF BOOKINGS



SITE	ADOPT.	STATUS	SUPPORT
Main campus	91%	On track	
Lakeside	84%	On track	
Eastgate	72%	At risk	
Ridgeview	58%	At risk	
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ADR

014

STATUS

ACCEPTED

DATE

12 Sep 2026

DECIDERS

Architecture review board

SUPERSEDES

ADR-009 Point-to-point HL7 feeds

ARCHITECTURE DECISION RECORD

ADR-014: Route clinical events through a shared bus

CONTEXT

Northfield runs 14 clinical and revenue systems, and every pair that exchanges data has its own interface. The 61 feeds fail in different ways, and each new system adds more.

DECISION

Publish clinical and revenue events to one managed event bus. Systems subscribe to the events they need, so a new feed no longer changes the sending system.

POSITIVE CONSEQUENCES

- One place to monitor and replay every event
- New systems connect in weeks, not quarters

NEGATIVE CONSEQUENCES

- The platform team must run a new service around the clock
- Interface teams need training in event design

ARCHITECTURE DECISION RECORD

ADR-014: Route clinical events through a shared event bus

STATUS

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DATE

12 Sep 2026

DECIDERS

Architecture board

SUPERSEDES

ADR-009 Point-to-point HL7 feeds

Review in 12 months or when the platform team changes.

CONTEXT

Northfield runs 14 clinical and revenue systems, and every pair that exchanges data has its own interface. The 61 feeds fail in different ways, and each new system adds more.

DECISION

Publish clinical and revenue events to one managed event bus. Systems subscribe to the events they need, so a new feed no longer changes the sending system.

POSITIVE CONSEQUENCES

- One place to monitor and replay every event
- New systems connect in weeks, not quarters
- Failed deliveries are visible in one dashboard

NEGATIVE CONSEQUENCES

- The platform team must run a new service around the clock
- Interface teams need training in event design
- Short period of running both models

ADR-014: Route clinical events through a shared event bus

STATUS

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- New systems connect in weeks, not quarters

NEGATIVE CONSEQUENCES

- The platform team must run a new service around the clock
- Interface teams need training in event design

Review in September 2027, or sooner if feed incidents rise.

AGENDA

What we will cover today



- | | | |
|----|----------------------------|-------|
| 01 | Where the close loses time | P. 3 |
| 02 | Target operating model | P. 7 |
| 03 | Phased roadmap | P. 12 |
| 04 | Business case | P. 18 |
| 05 | Team and next steps | P. 22 |

Close redesign working session

9:00–9:30 Welcome and goals

9:30–10:30 Where the close loses time

10:30–11:30 Target operating model

11:30–12:15 Wave plan and owners

12:15–1:00 Decisions and next steps

OUTCOME

One agreed wave 1 scope and a named owner for each entity.



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OUTCOME

One agreed wave 1 scope and a named owner for each entity.



Agenda for the design-sprint day

TIME	SESSION	FINANCE AND DATA CONTRIBUTION	SHARED RESULT
8:30–9:30	1. Joint framing	- Finance: bring close calendar and pain points - Data: test feasibility and source availability	One scoped opportunity with a visible path
9:35–11:30	2. Parallel definition	- Finance: define rules, controls, acceptance - Data: profile sources and prove the data flow	Agreed rules and a working data sample
12:15–1:00	3. Handoff	- Finance: explain intent and open questions - Data: challenge ambiguity; propose a slice	Agreed interface, scope and demo slice
1:00–3:15	4. Build and review	- Finance: review each increment against rules - Data: convert feedback into bounded changes	Traceable feedback and acceptance evidence
3:15–4:30	5. Scale-up	- Finance: name operating-model implications - Data: name platform and ownership implications	Next actions and a reusable learning record

Outcome of the day: one agreed wave 1 scope, a tested data slice and a named owner for each decision.

Revenue cycle returns the most value per dollar

WORKSTREAM	INVESTED	SHARE OF INVESTMENT	ANNUAL VALUE	SHARE OF VALUE	PER \$1
Close and reporting platform	\$1.3M	31%	\$1.0M	38%	\$0.77
Revenue cycle	\$1.0M	24%	\$0.8M	31%	\$0.80
Procure-to-pay	\$0.7M	17%	\$0.4M	15%	\$0.57
Data and controls	\$0.6M	14%	\$0.3M	12%	\$0.50
Change, training and risk	\$0.6M	14%	\$0.1M	4%	\$0.17
Total	\$4.2M		\$2.6M		\$0.62

Change and training: 14% of spend, 4% of value, and every other lever depends on it.

Revenue cycle and close automation hold 55% of spend and 69% of value.

Annual value per dollar invested is \$0.62; five-year value per dollar is \$2.40.

Source: Sample figures for layout only; replace with current program data before use.

Revenue cycle holds the most exposure per application

APPLICATION	BUSINESS FIT	TECH FIT	COST	RISK	DEBT	DISPOSITION
CLINICAL						
NorthChart EHR	4	3	4	2	2	RETAIN
OrderSet Manager	2	2	1	2	3	REFACTOR
PharmaTrack	4	3	2	1	1	RETAIN
REVENUE CYCLE						
BillPath	3	2	3	3	3	REFACTOR
Claims Edit Engine	3	3	2	2	1	RETAIN
Statement Print	1	1	2	3	4	RETIRE
CORPORATE						
StaffLink Rostering	3	2	2	2	2	RE-PLATFORM
SupplyPoint	2	1	2	3	3	RE-PLATFORM

FIT Low Some Fair Good Strong EXPOSURE Low Some Mid High Severe

Source: Northfield application inventory, Sep 2026.

Fourteen of 30 apps leave or move

APPLICATION	FIT	COST	RISK	DEBT	DISPOSITION
NorthChart EHR	4	4	2	2	RETAIN
Lumen Scheduler	2	2	3	4	RE-PLATFORM
BillPath	2	3	3	3	REFACTOR
OrderSet Manager	2	1	2	3	REFACTOR
PharmaTrack	4	2	1	1	RETAIN
Legacy Registration	0	3	4	4	RETIRE
StaffLink Rostering	2	2	2	2	RE-PLATFORM
SupplyPoint	2	2	3	3	RE-PLATFORM

FIT Low Some Fair Good Strong
EXPOSURE Low Some Mid High Severe

Source: Northfield application inventory, Sep 2026.

DISPOSITION, ALL 30 APPS

11 Retain
7 Re-platform
5 Refactor
7 Retire

Retiring seven apps removes \$3.2M of annual run cost.

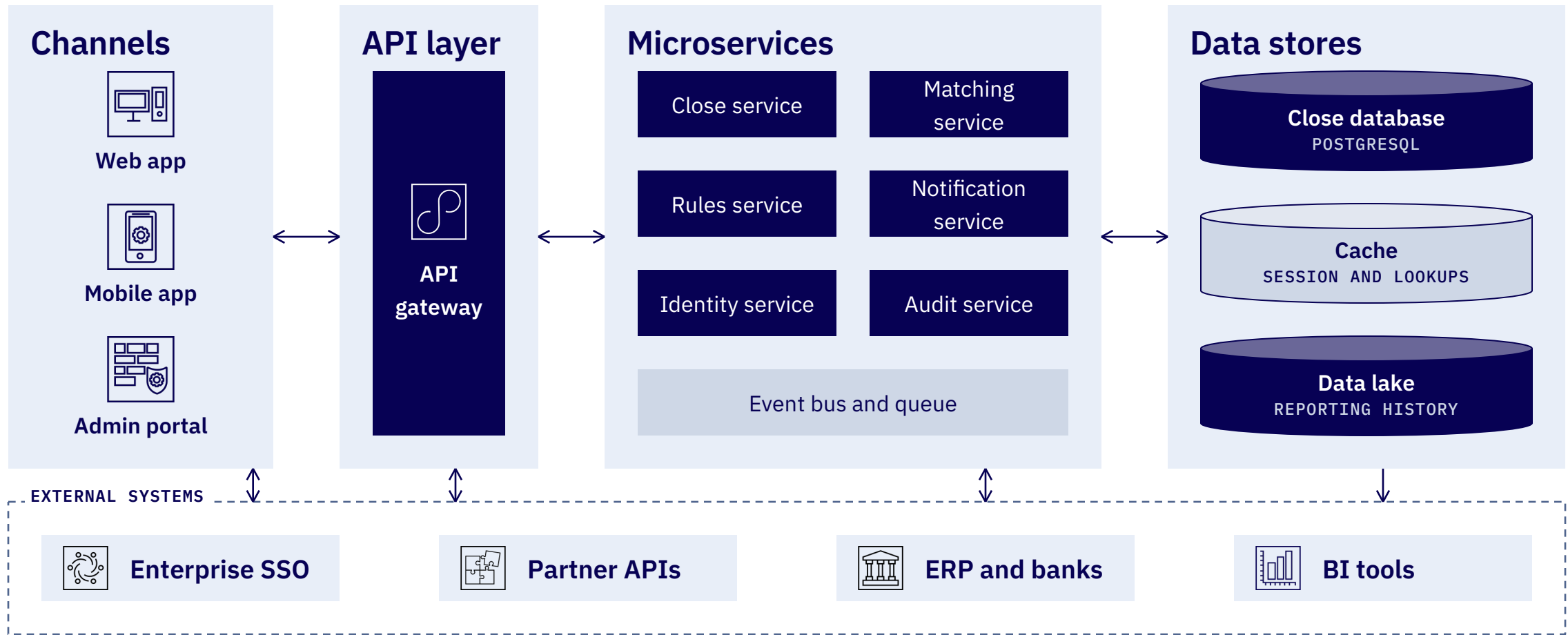
Nine applications, four dispositions

APPLICATION	BUSINESS FIT	TECH FIT	COST	RISK	DEBT	DISPOSITION
NorthChart EHR	4	3	4	2	2	RETAIN
Lumen Scheduler	3	1	2	3	4	RE-PLATFORM
BillPath	3	2	3	3	3	REFACTOR
OrderSet Manager	2	2	1	2	3	REFACTOR
PharmaTrack	4	3	2	1	1	RETAIN
Legacy Registration	1	0	3	4	4	RETIRE
StaffLink Rostering	3	2	2	2	2	RE-PLATFORM
SupplyPoint	2	1	2	3	3	RE-PLATFORM
FaxBridge Referrals	1	0	2	4	4	RETIRE

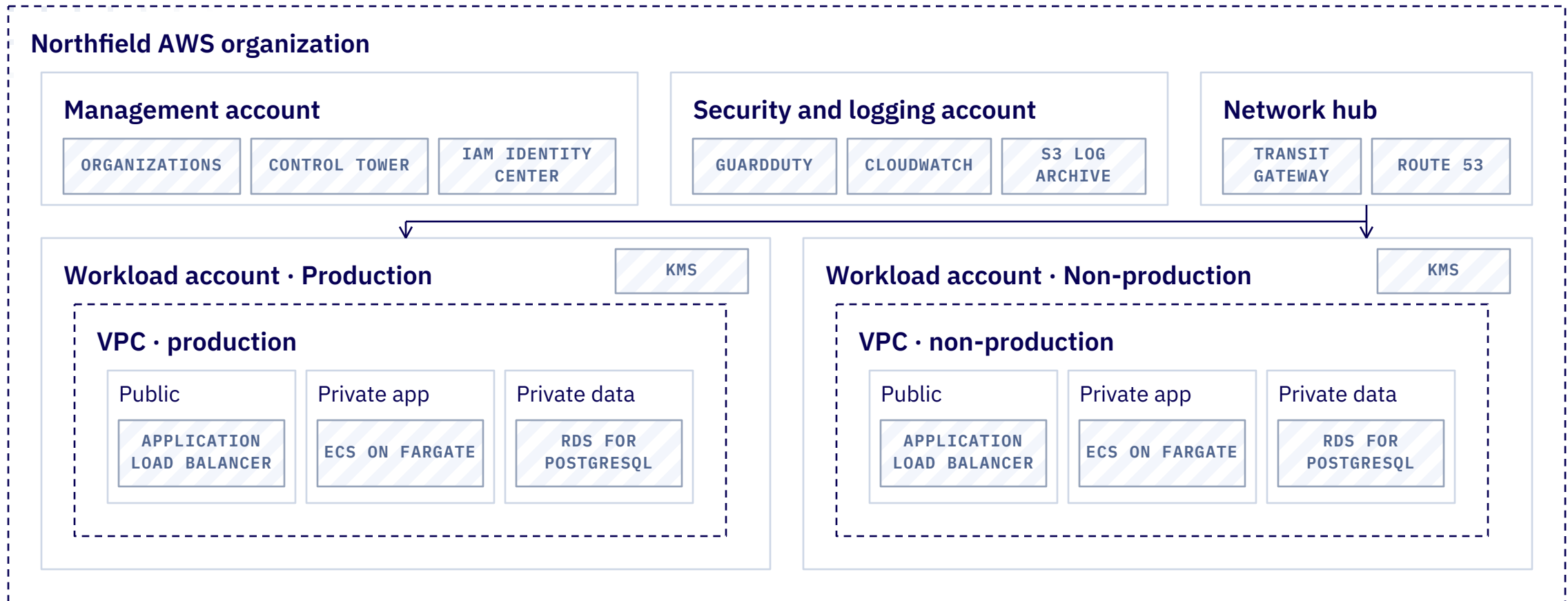
FIT Low Some Fair Good Strong EXPOSURE Low Some Mid High Severe

Source: Northfield application inventory, Sep 2026.

Small services behind one gateway contain change

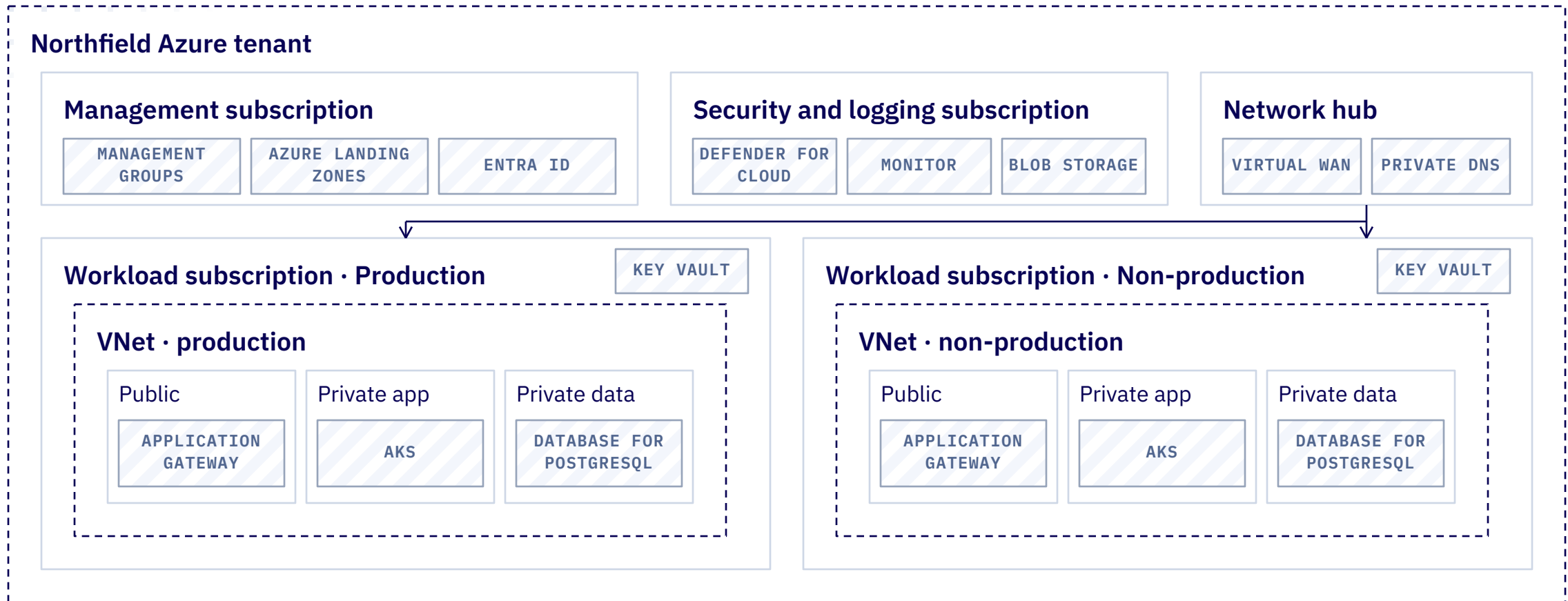


Workloads land in a governed multi-account foundation



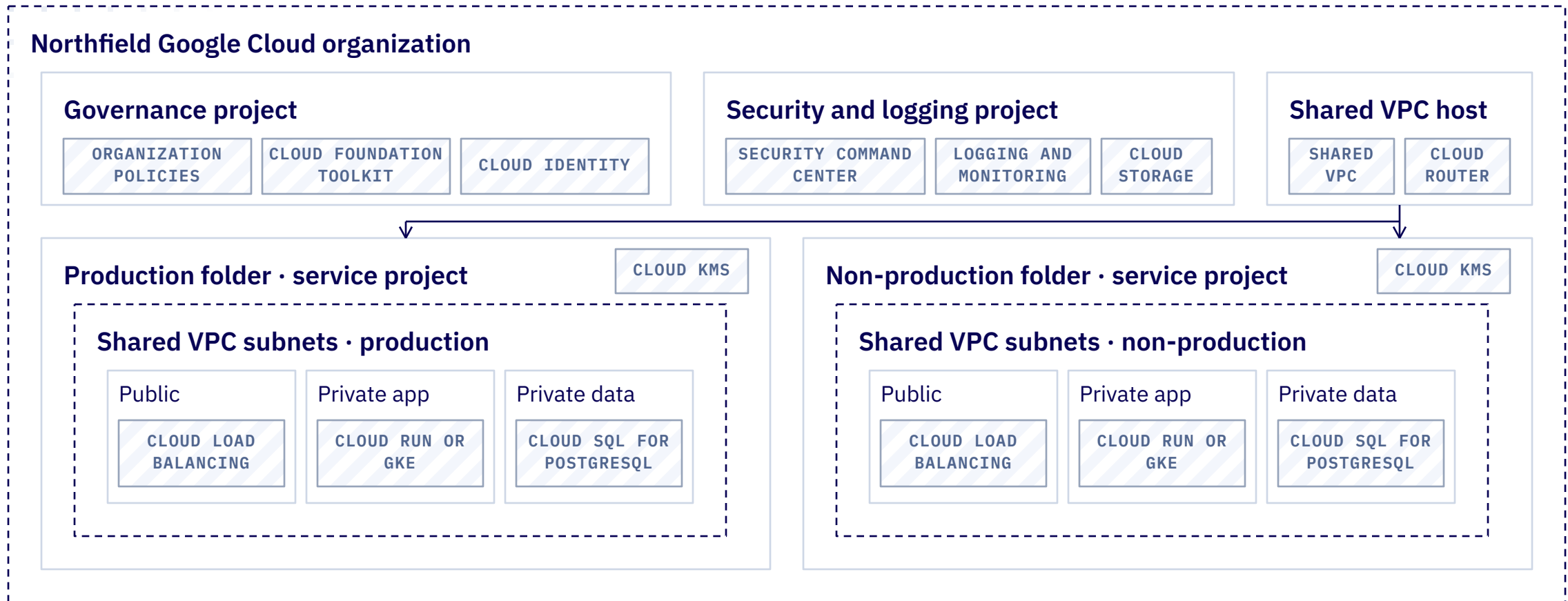
Source: Sample AWS services; logos and services swap per provider, and an Azure or GCP variant keeps the same structure.

Workloads land in a governed subscription foundation



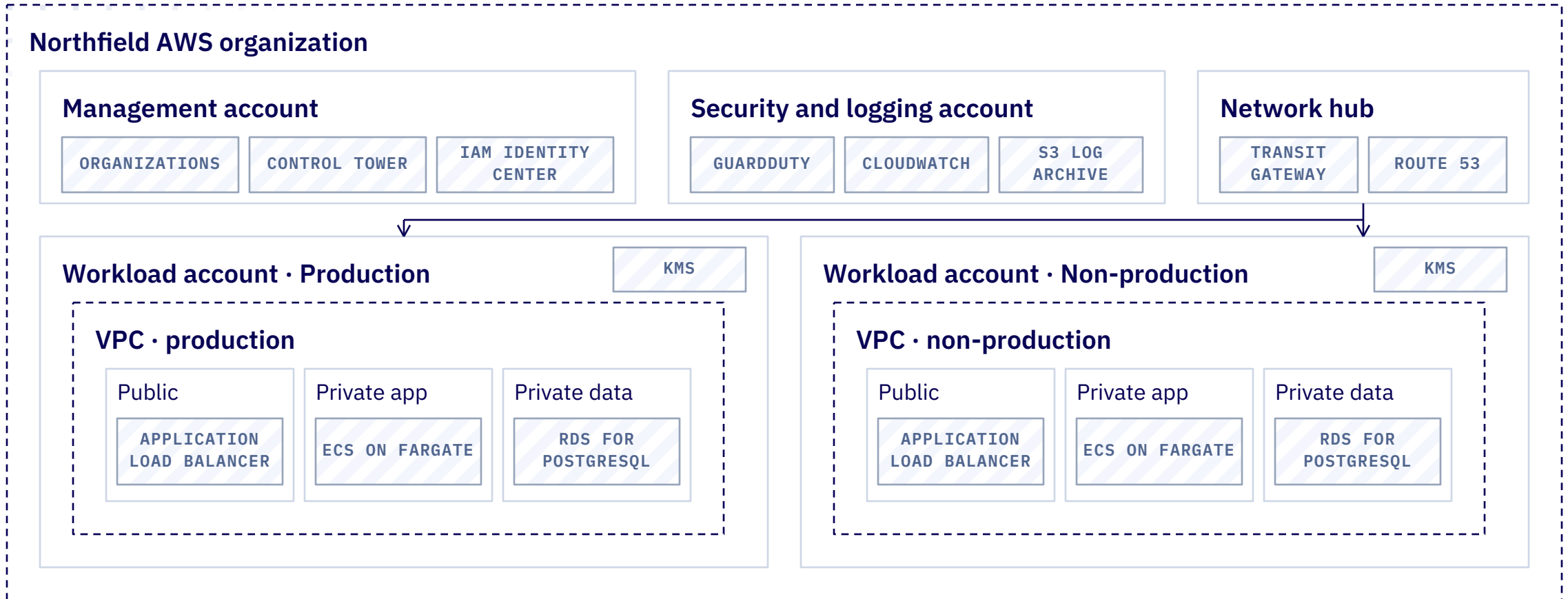
Source: Sample Azure services; logos and services swap per provider, and an AWS or GCP variant keeps the same structure.

Workloads land in a governed project foundation



Source: Sample Google Cloud services; logos and services swap per provider, and the structure stays the same for AWS or Azure.

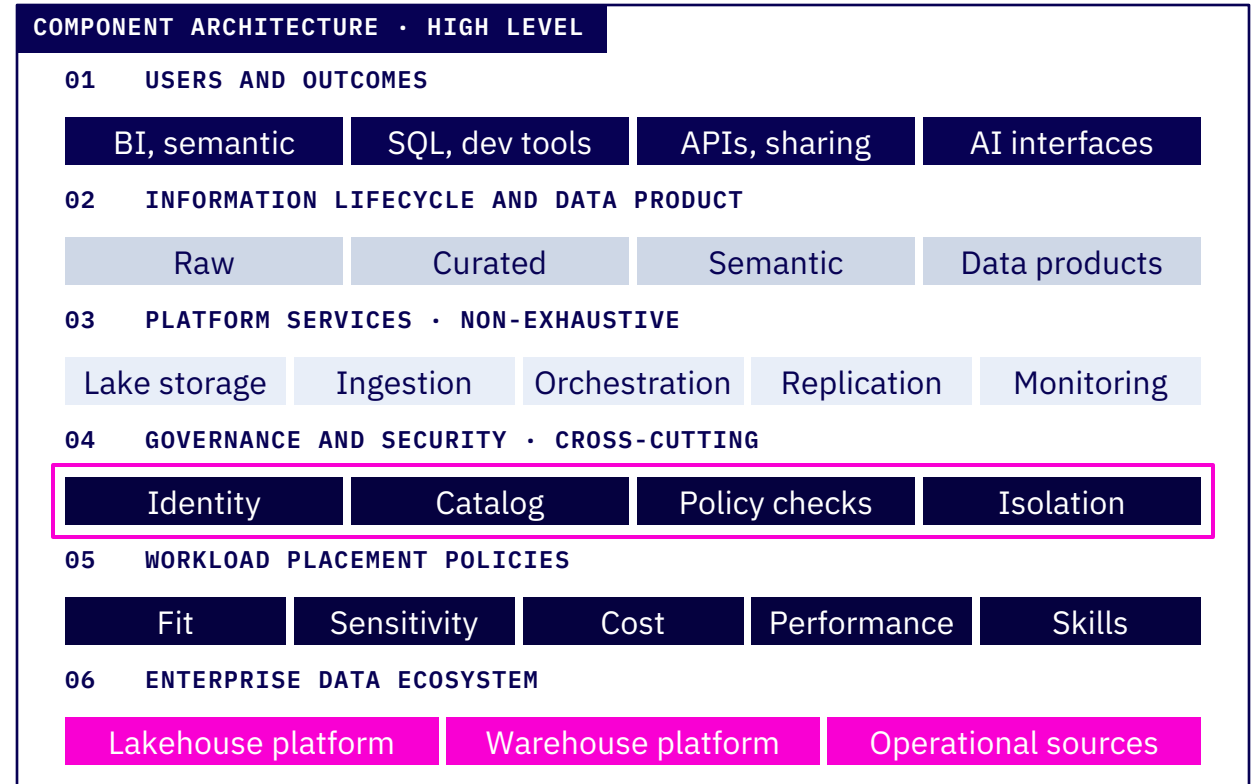
Workloads land in a governed multi-account foundation



Source: Sample AWS services; logos and services swap per provider, and an Azure or GCP variant keeps the same structure.

Six layers connect users to governed data products through reusable services

-  **Users and outcomes**
BI, developer, sharing and AI experiences
-  **Information lifecycle**
Raw to curated, semantic and governed products
-  **Platform services**
Reusable processing, interoperability, operations
-  **Governance and placement**
Embedded controls and clear placement criteria
-  **Enterprise data ecosystem**
Existing platforms, connected not consolidated



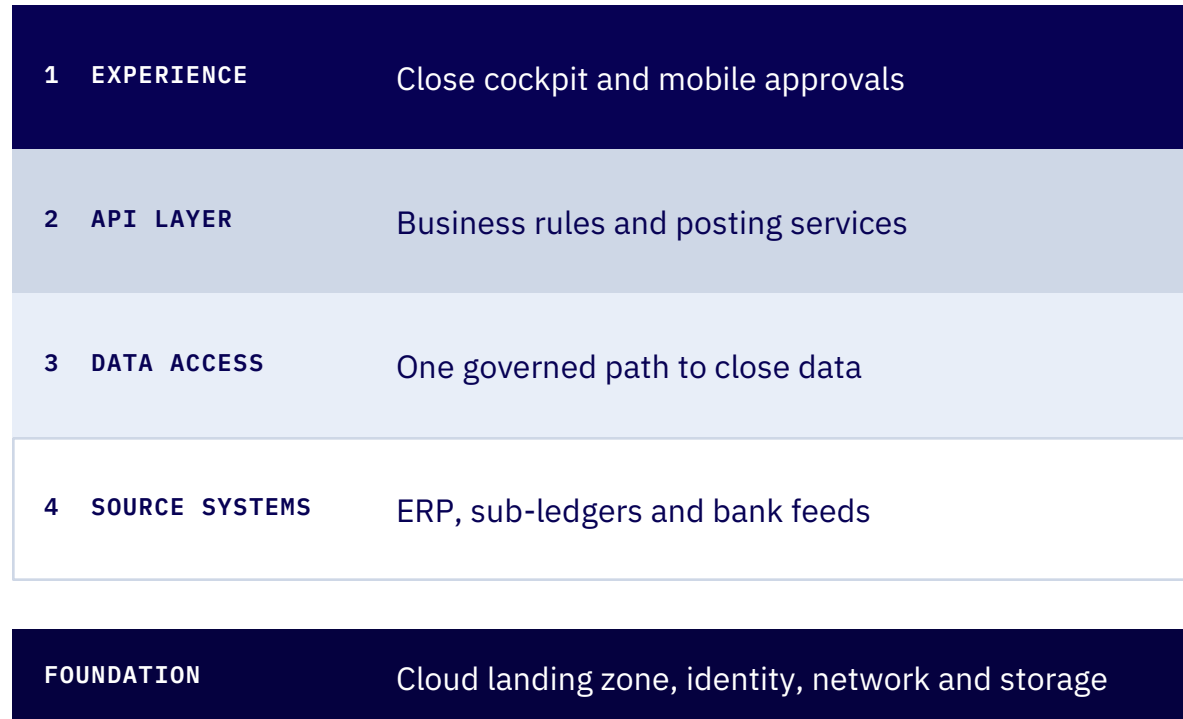
Source: West Monroe reference architecture for Northfield Health System.

One layered platform replaces the old close tools

CLOSE PLATFORM

Build once

Each layer serves every close process, so one moves without a rebuild.



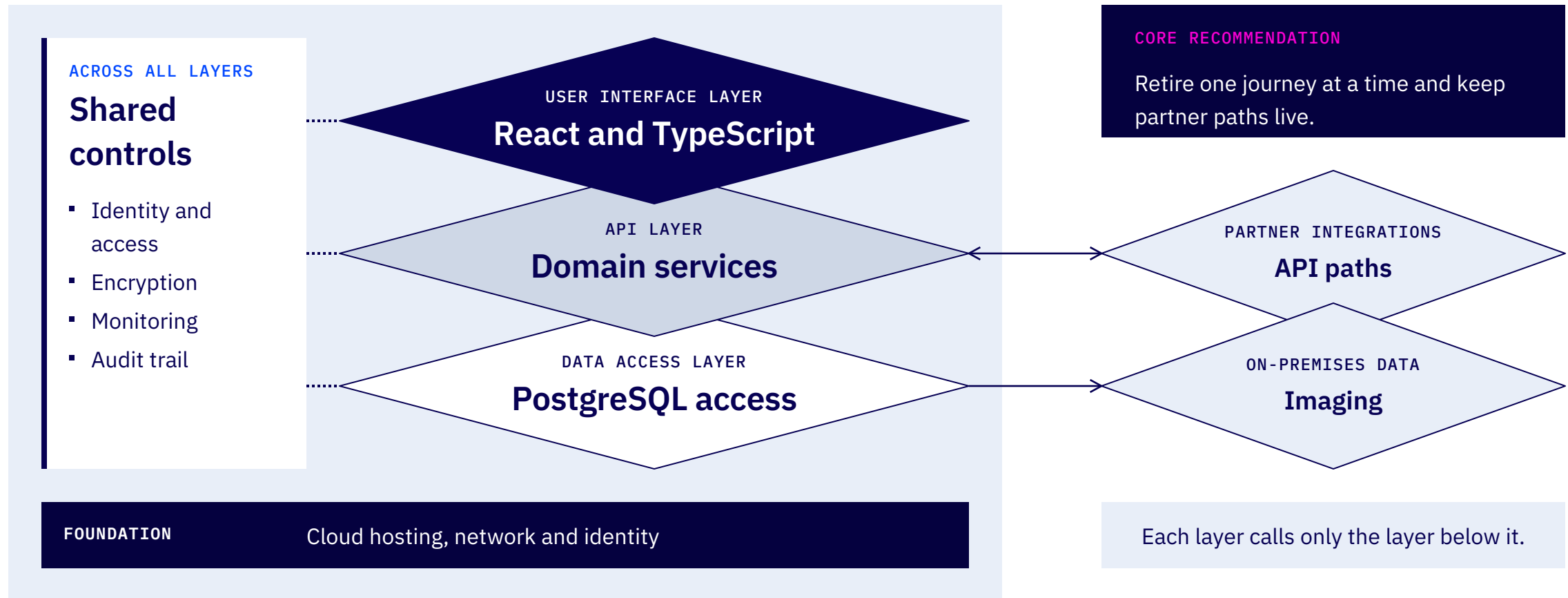
ACROSS LAYERS 1-4

Shared controls

- Security
- Governance
- Observability
- Audit trail

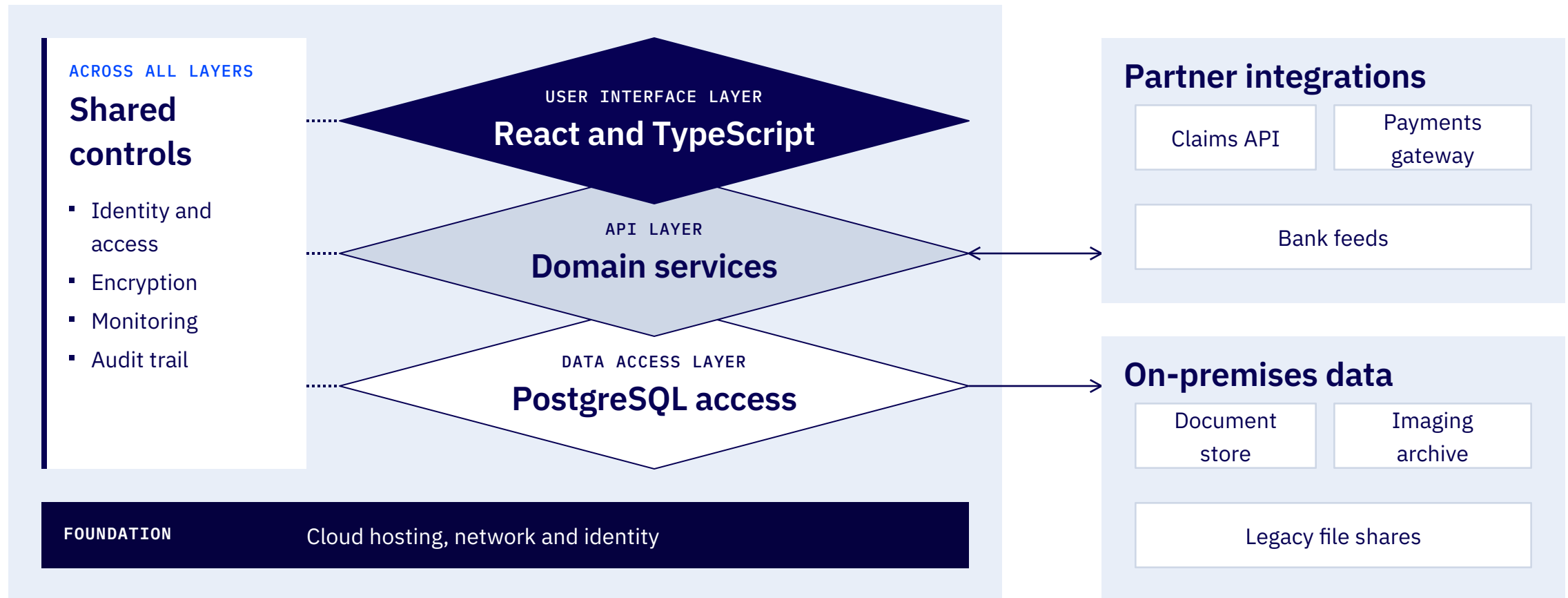
Source: West Monroe reference architecture.

Three tiers let us change one layer in isolation



Source: West Monroe reference architecture.

Three tiers change in isolation and reach other systems



Source: West Monroe reference architecture.

One layered platform replaces the old close tools

CLOSE PLATFORM

Build once

Each layer serves every close process, so one moves without a rebuild.



Experience

Close cockpit and mobile approvals



API layer

Business rules and posting services



Data access

One governed path to close data



Source systems

ERP, sub-ledgers and bank feeds



Foundation: cloud landing zone, identity, network and storage



ACROSS LAYERS 1-4

Shared controls

- Security
- Governance
- Observability
- Audit trail

Source: West Monroe reference architecture.

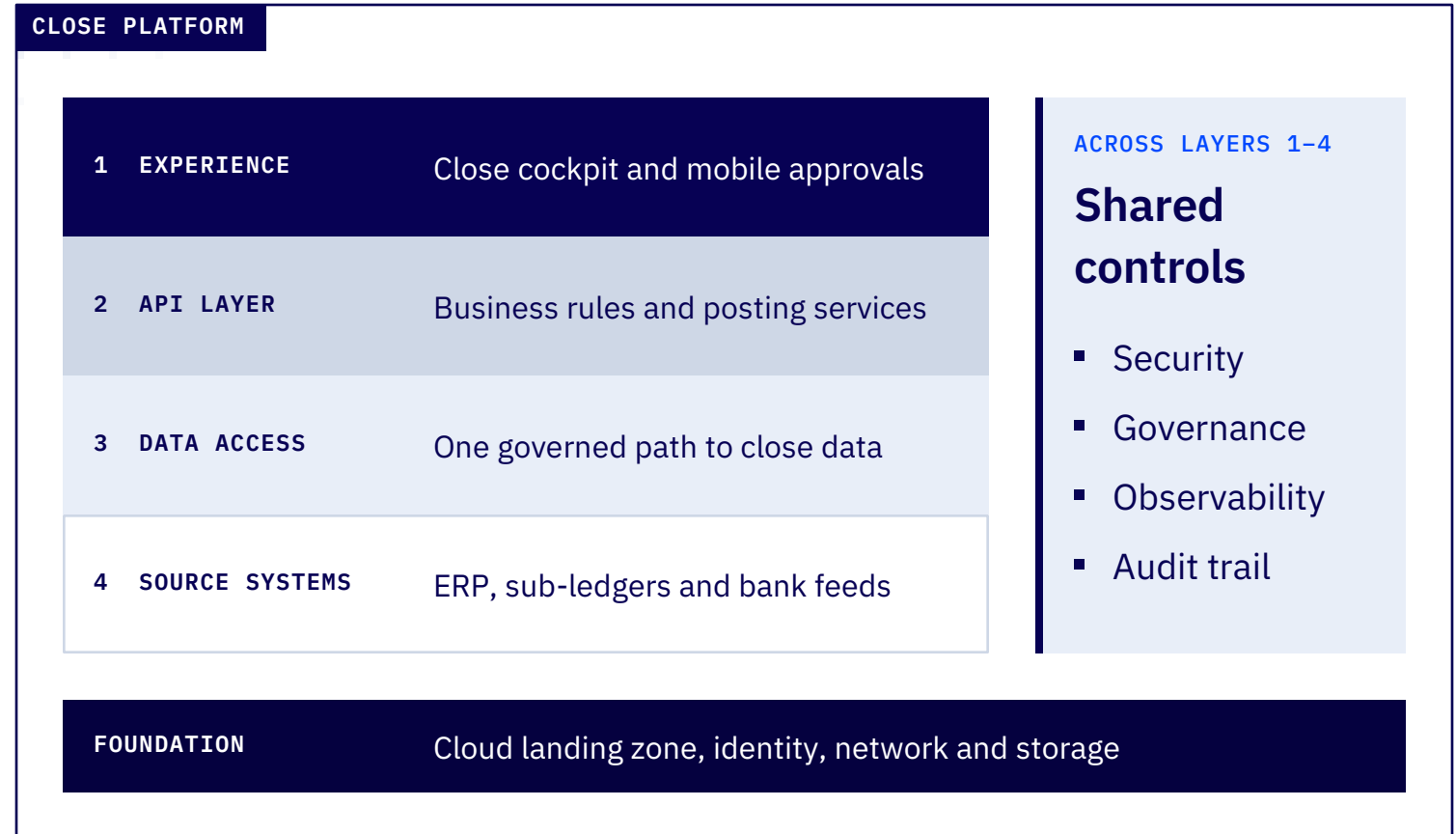
DESIGN PRINCIPLE

Build once

Each layer serves every close process, so one moves without a rebuild.

SOLUTION ARCHITECTURE

One platform replaces the old tools



Source: West Monroe reference architecture.

One layered platform replaces the old close tools

CLOSE PLATFORM

Build once

Each layer serves every close process, so one moves without a rebuild.

1	EXPERIENCE	Close cockpit and mobile approvals
2	API LAYER	Business rules and posting services
3	DATA ACCESS	One governed path to close data
4	SOURCE SYSTEMS	ERP, sub-ledgers and bank feeds

FOUNDATION Cloud landing zone, identity, network and storage

ACROSS LAYERS 1-4

Shared controls

- Security
- Governance
- Observability
- Audit trail

Source: West Monroe reference architecture.

SOLUTION ARCHITECTURE

One layered platform replaces the old close tools

Build once

Each layer serves every close process, so one moves without a rebuild.

ACROSS LAYERS 1-4

Shared controls

- Security
- Governance
- Observability
- Audit trail

Source: West Monroe reference architecture.

CLOSE PLATFORM

1 EXPERIENCE

Close cockpit and mobile approvals

2 API LAYER

Business rules and posting services

3 DATA ACCESS

One governed path to close data

4 SOURCE SYSTEMS

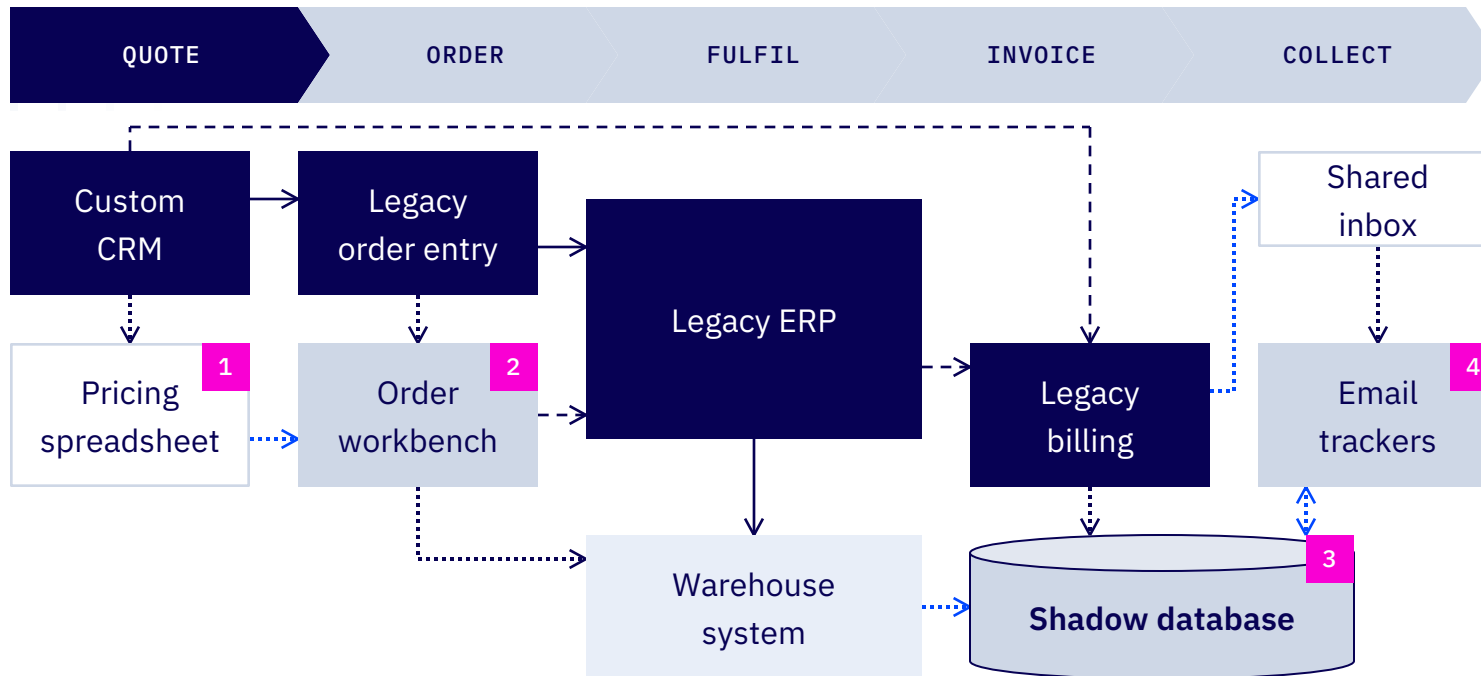
ERP, sub-ledgers and bank feeds

FOUNDATION

Cloud landing zone, identity, network and storage

CURRENT STATE

Four hotspots drive most of the rework



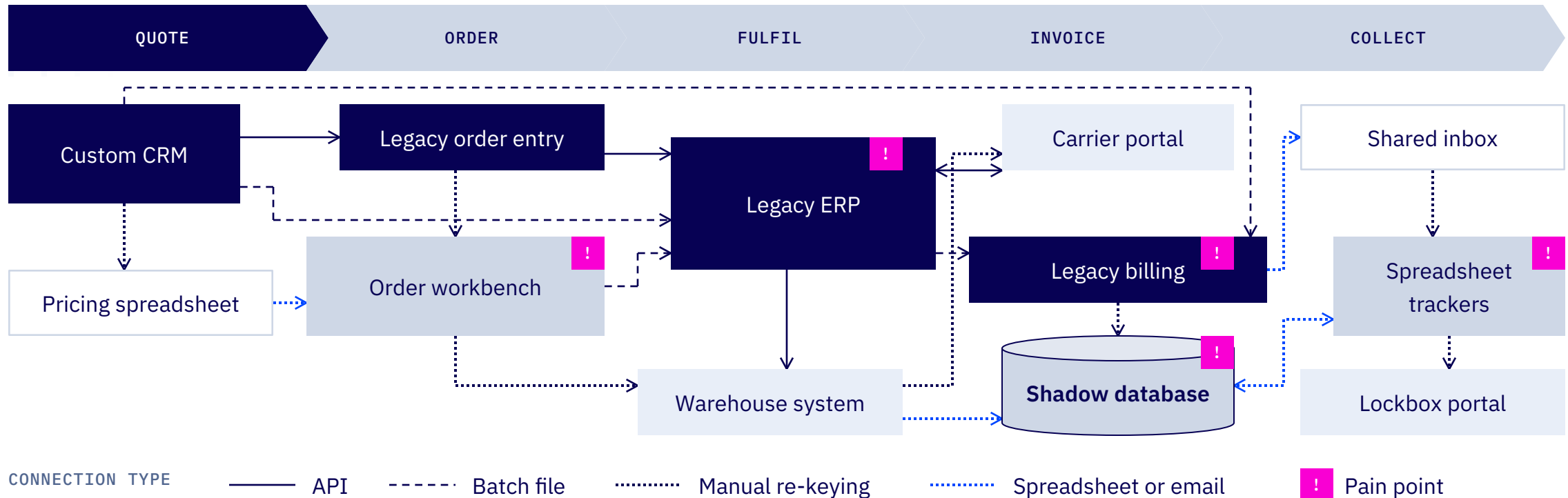
CONNECTION TYPE

- API
- - - - - Batch file
- Manual re-keying
- Spreadsheet or email
- ! Pain point

- 1 Prices re-keyed three times**
Quote prices live in a spreadsheet, the CRM and the ERP, and drift apart.
- 2 Orders repaired by hand**
The workbench exists to fix failed orders before they reach the ERP.
- 3 Shadow database**
Billing and trackers sync through an unmanaged copy nobody owns.
- 4 Collections run on email**
Cash is chased from inboxes and trackers, with no single status.

CURRENT STATE

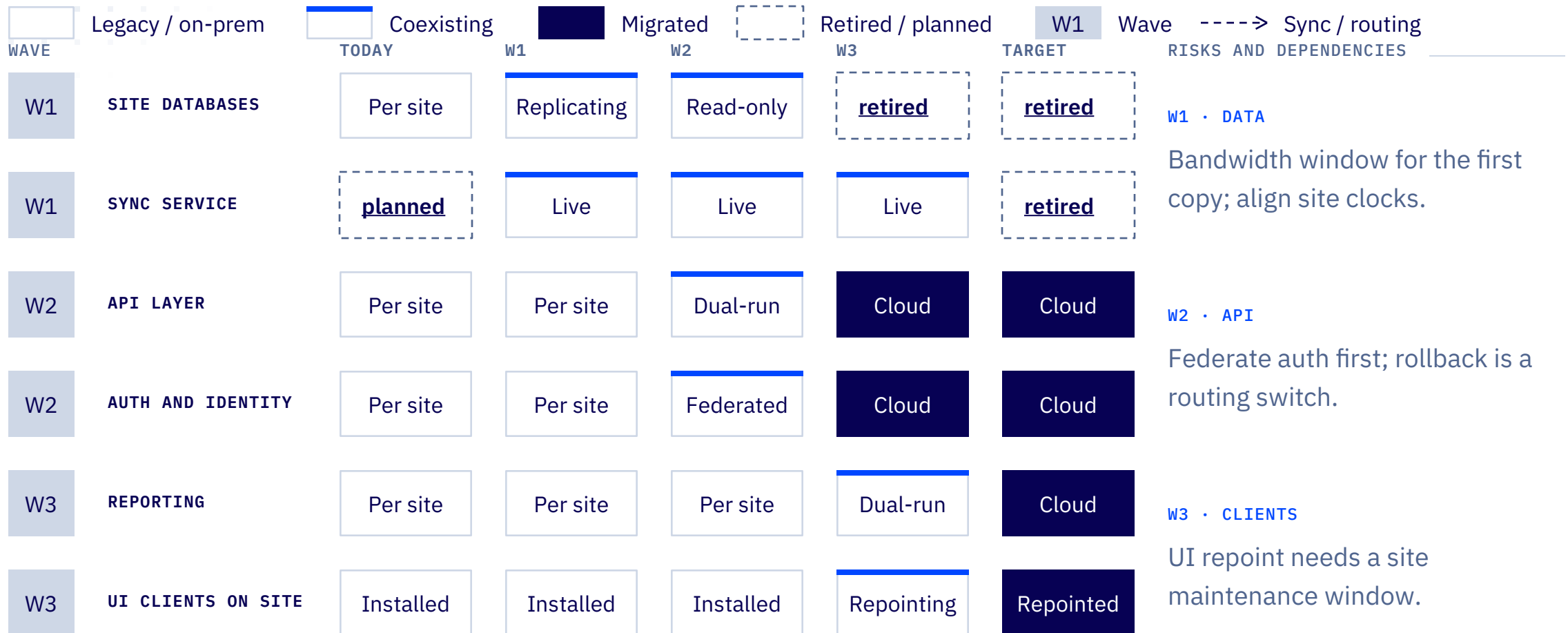
Quote to cash today crosses 12 systems



COST OF THE MESS	9 manual hand-offs per order	6 days from shipment to invoice	3 versions of customer data
------------------	------------------------------	---------------------------------	-----------------------------

Source: Illustrative current state. Sample figures for layout only; replace with current firm data before use.

Three waves move data, then API, then clients



Three waves move data, then API, then clients

WAVE		TODAY	W1	W2	W3	TARGET	RISKS AND DEPENDENCIES
W1	SITE DATABASES	Per site	Replicating	Read-only	<u>retired</u>	<u>retired</u>	W1 • DATA Bandwidth window for the first copy; align site clocks.
W1	SYNC SERVICE	<u>planned</u>	Live	Live	Live	<u>retired</u>	
W2	API LAYER	Per site	Per site	Dual-run	Cloud	Cloud	W2 • API Federate auth first; rollback is a routing switch.
W2	AUTH AND IDENTITY	Per site	Per site	Federated	Cloud	Cloud	
W3	REPORTING	Per site	Per site	Per site	Dual-run	Cloud	W3 • CLIENTS UI repoint needs a site maintenance window.
W3	UI CLIENTS ON SITE	Installed	Installed	Installed	Repointing	Repointed	

Legacy

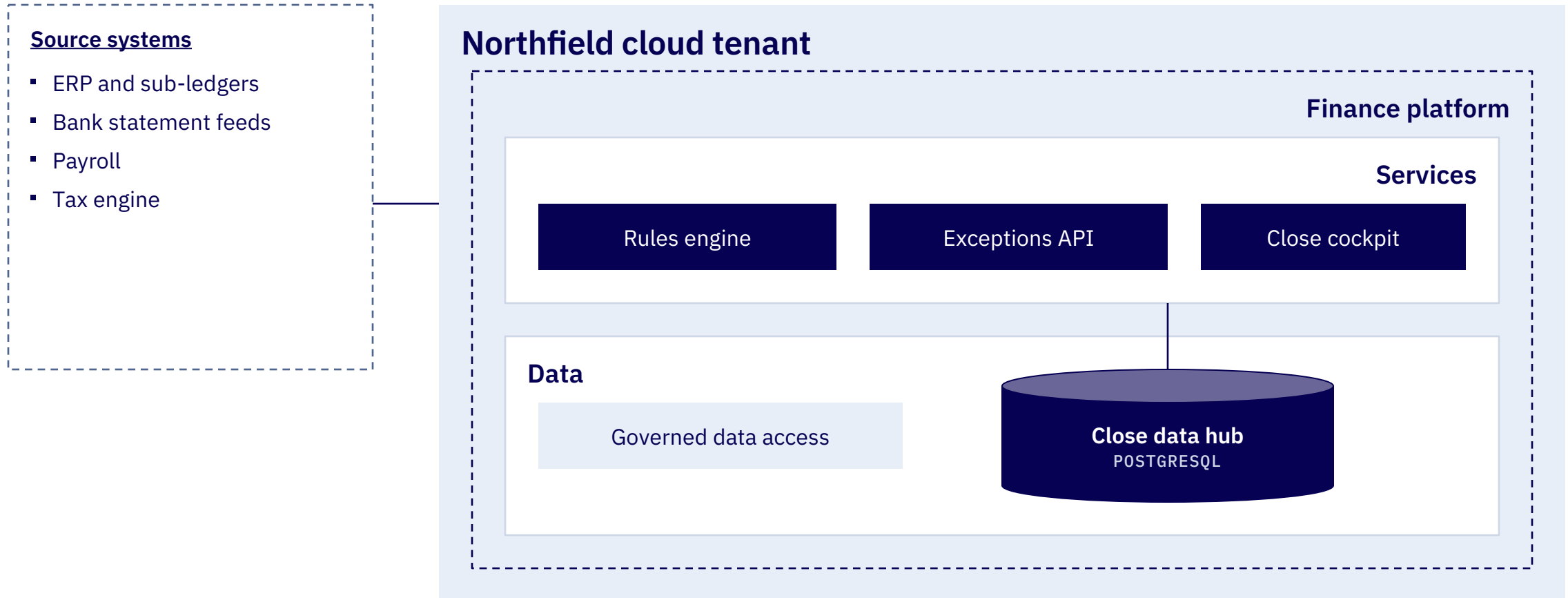
Coexisting

Migrated

Retired

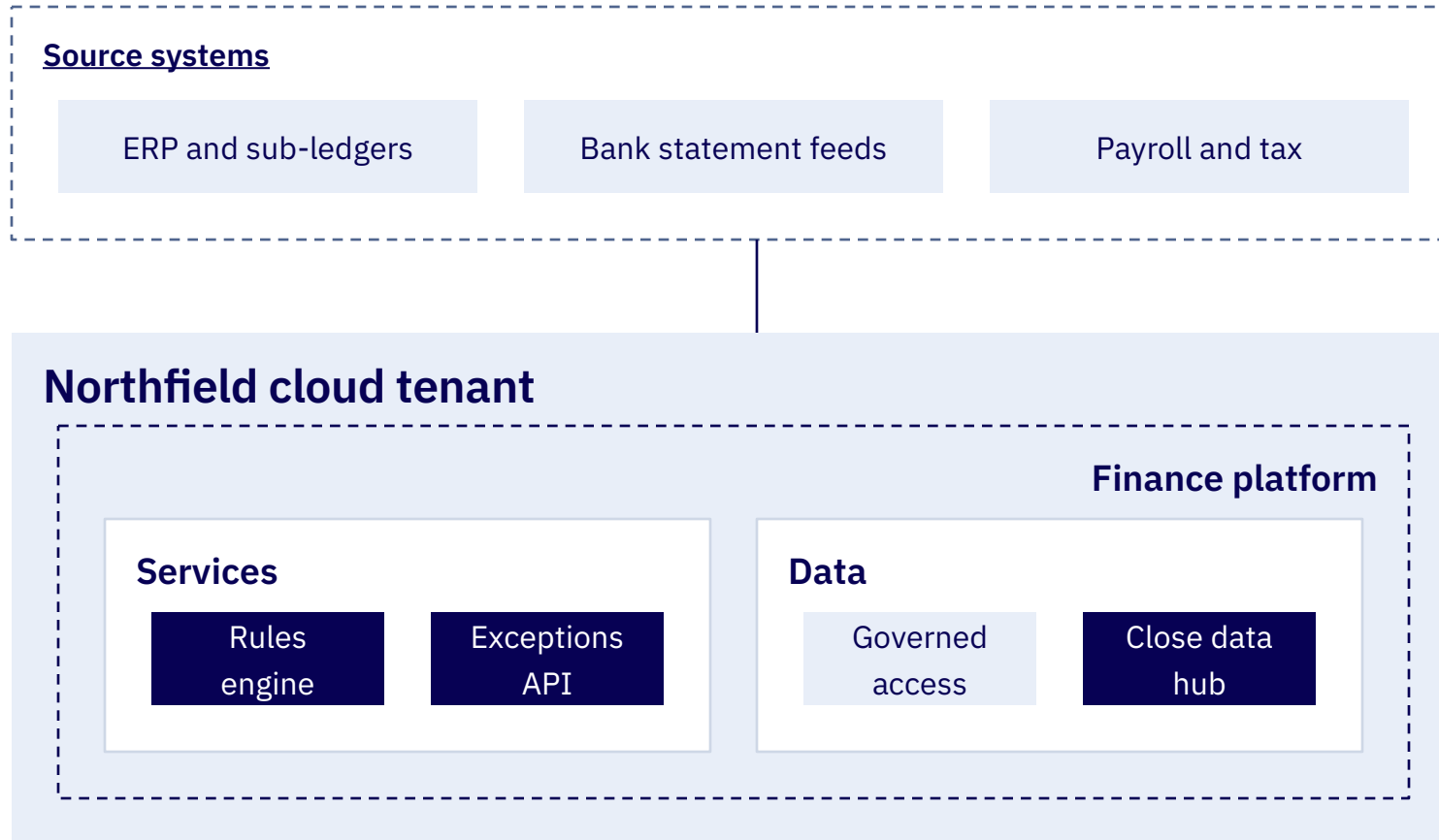
Source: Sample migration plan; waves and states are illustrative and confirmed with each site before cutover.

Everything runs in one governed Northfield tenant



Source: West Monroe reference architecture.

One governed tenant runs everything



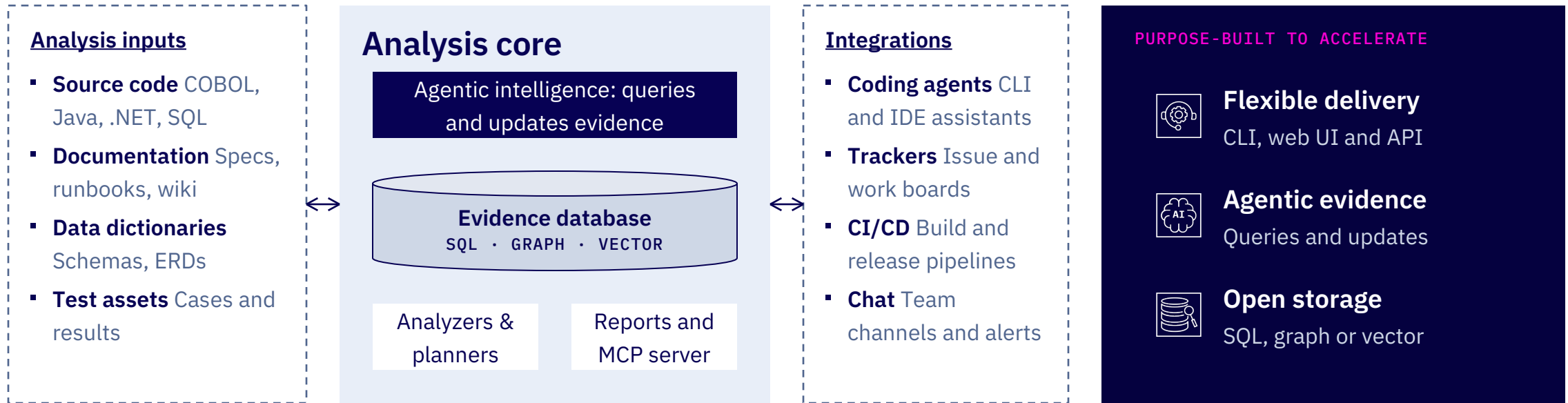
Source: West Monroe reference architecture.

WHAT CHANGES

One tenant, one data path

Every source lands once, and every service reads from the same governed hub.

AI and **code analysis** guide modernization that also improves the product itself



VALUE DELIVERED

12 months

to modernize, from 3–5 years

10+

pre-built AI-guided workflows

Up to 80%

savings in re-engineering effort

Source: West Monroe delivery experience on comparable modernization programs.

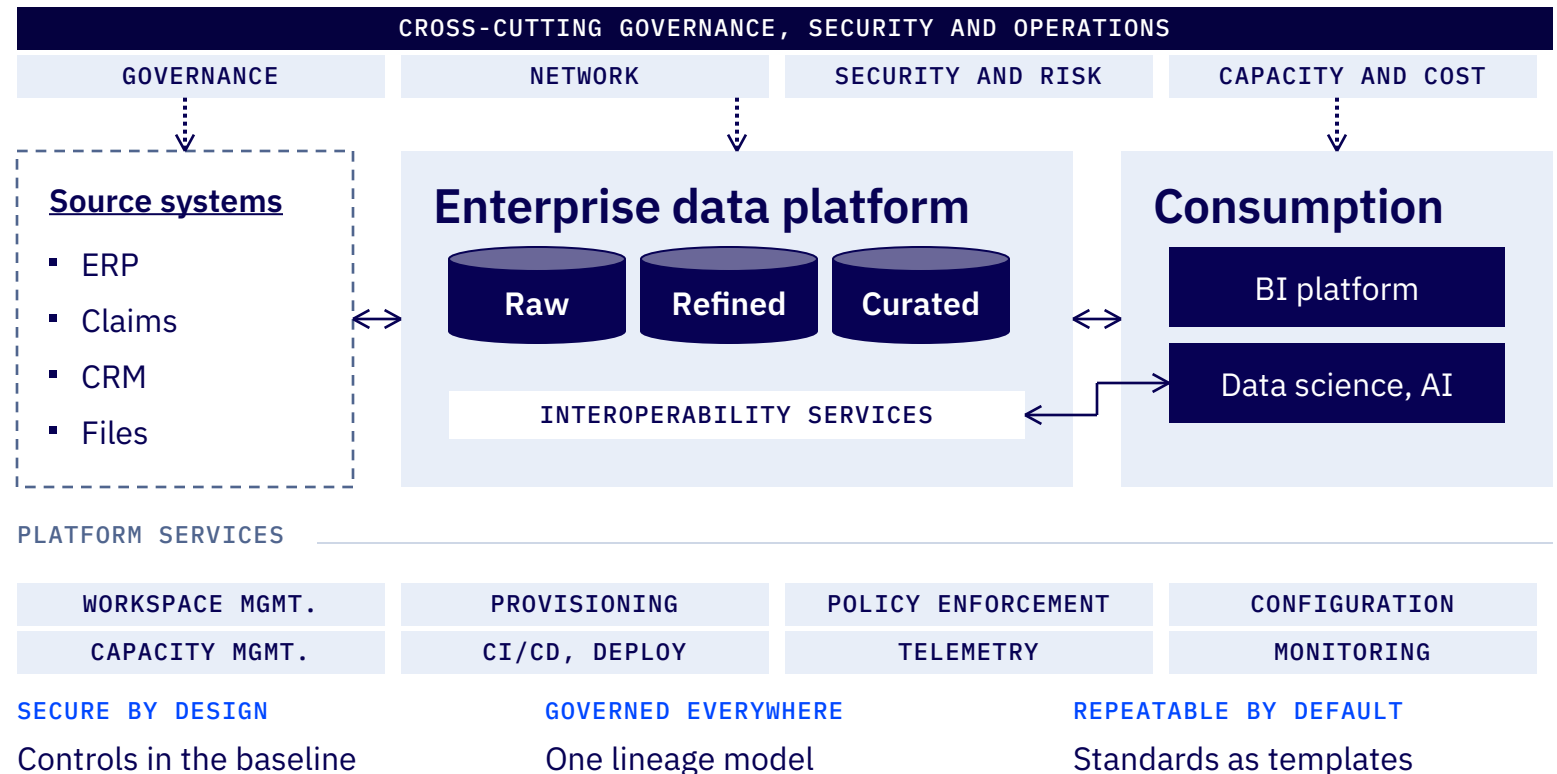
A secure, governed reference architecture lets each workload land on the right platform

Phase 1 proof points

- Workspace and environment pattern approved
- Code promotion demonstrated
- Lineage metadata visible for each store
- Telemetry externalized to shared monitoring
- Priority patterns demonstrated

Dependency disposition

- Gaps logged with owner and interim treatment
- Impact on general availability stated
- Reviewed at each steering meeting



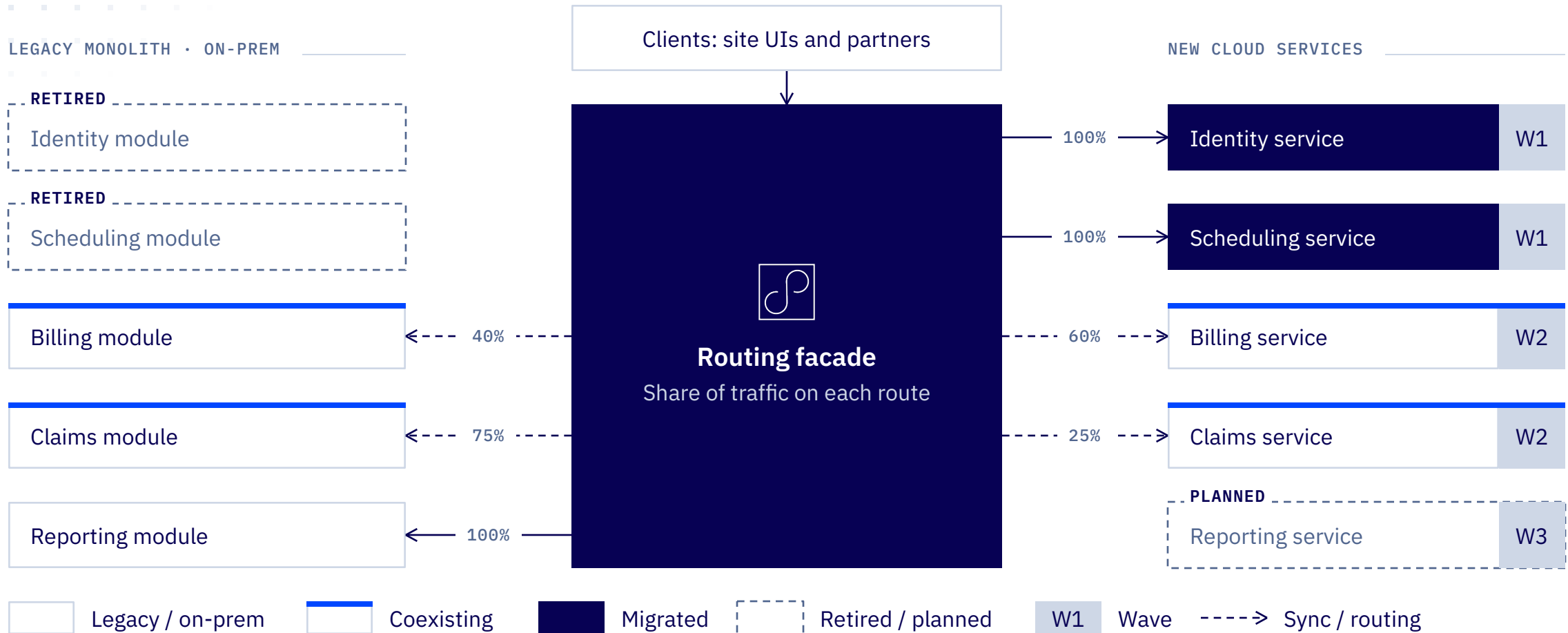
Four cohorts cut over, each behind a go/no-go gate



■ Coexisting (replicate and dual-run)
 □ On legacy
 ■ Migrated (cut over, site DB retired)
 — Gate

Go/no-go: data reconciles within 0.1 percent and no severity-1 defects for five days. Rollback: repoint the site UI to its local API; the site database stays read-only for 30 days.

A routing facade shifts traffic service by service



A routing facade shifts traffic service by service

Each percentage is the share of live traffic the facade sends down that route.

STATE KEY

 Legacy / on-prem

 Coexisting

 Migrated

 Retired / planned

 W1 Wave

----> Sync / routing

LEGACY • ON-PREM

RETIRED

Identity module

RETIRED

Scheduling module

Billing module

Claims module

Reporting module

Clients: site UIs, partners

Routing facade

Traffic share per route

NEW CLOUD SERVICES

Identity service

W1

Scheduling service

W1

Billing service

W2

Claims service

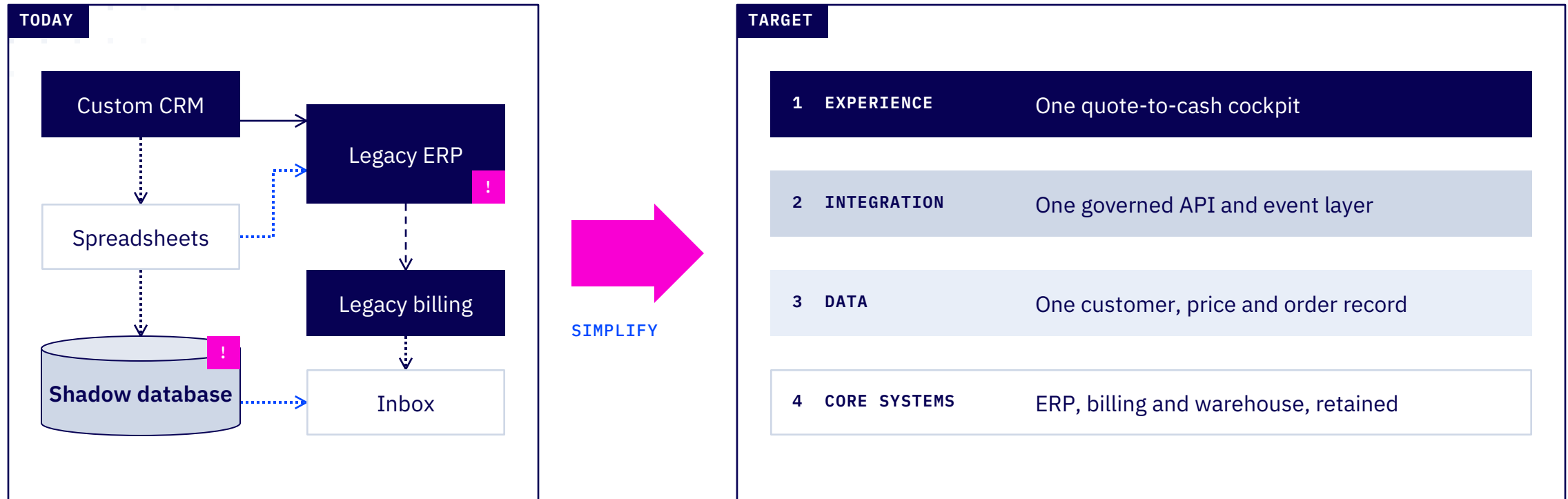
W2

PLANNED

Reporting service

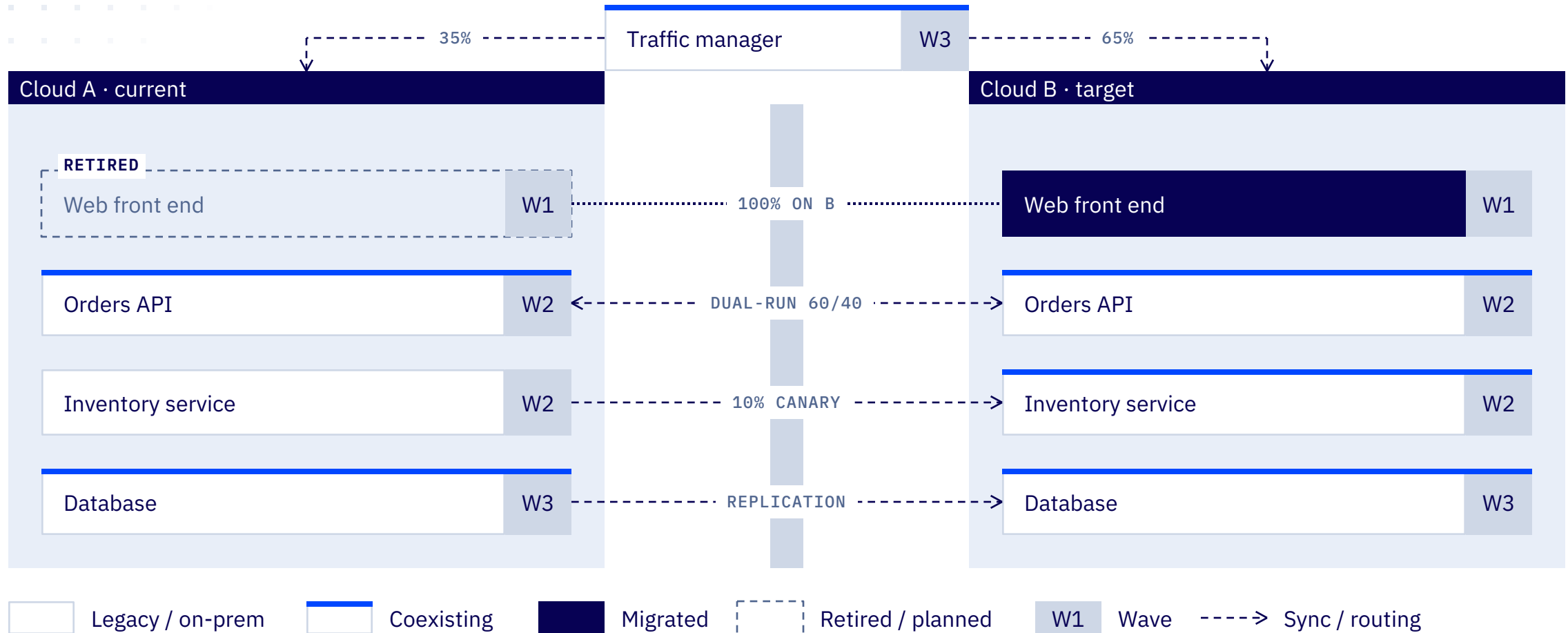
W3

From a tangled landscape to one connected platform

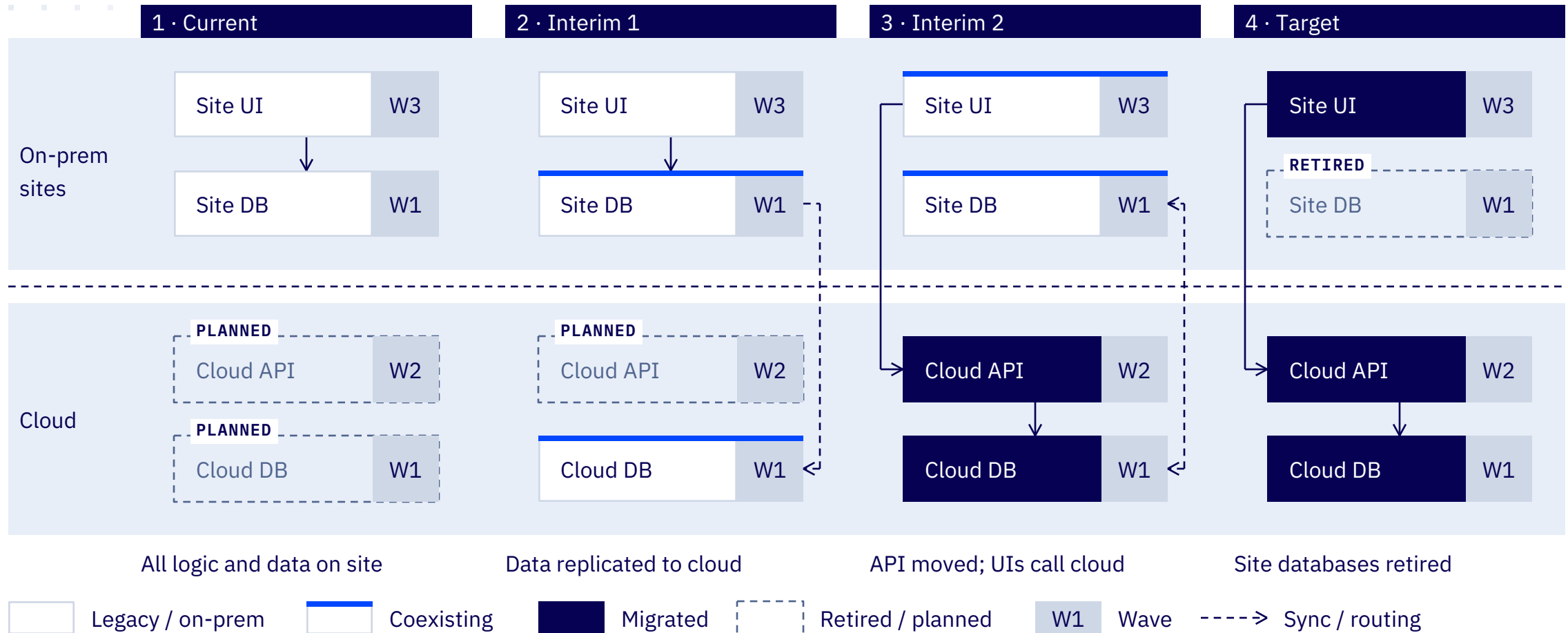


Replace point-to-point links and re-keying with one governed integration layer.

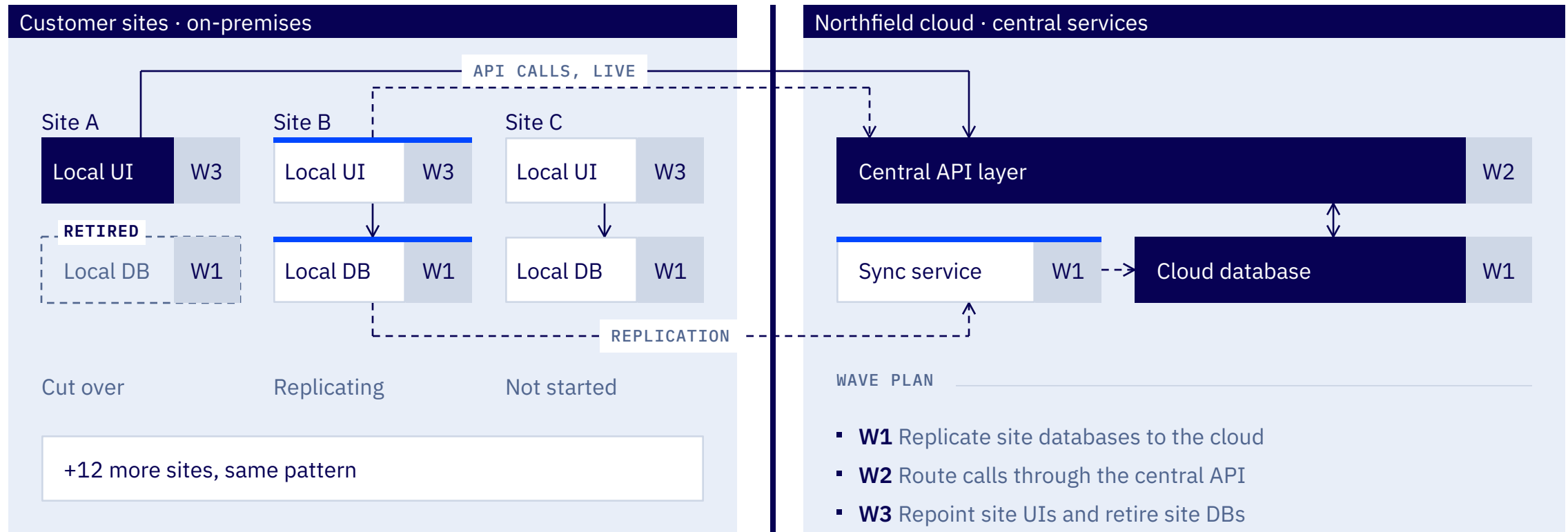
Services move from Cloud A to Cloud B in waves



Four steps take each site from local to cloud

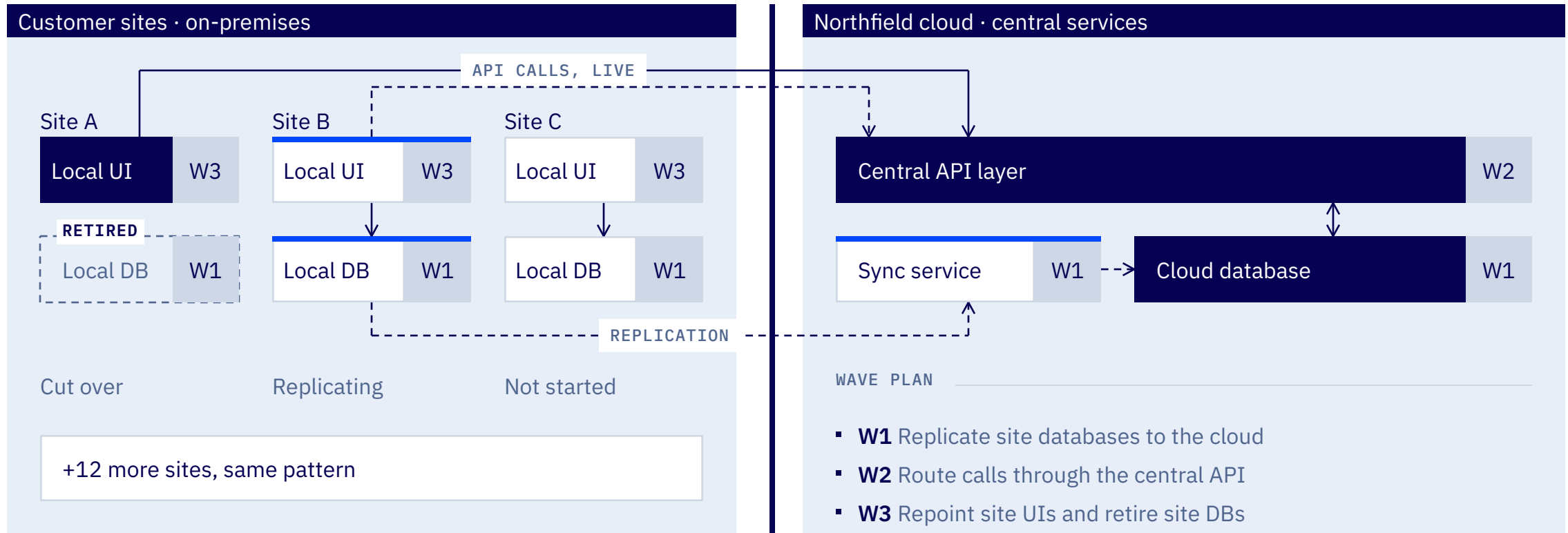


Data and API move to the cloud while UIs stay on site



Legacy / on-prem
Coexisting
Migrated
Retired / planned
W1 Wave
 -----> Sync / routing

Data and API move to the cloud while UIs stay on site



COMMERCIALS

Changes follow four steps, in writing

01 Request

Either party raises a change request in writing.

OWNER

Either party

02 Assess

We estimate the effect on scope, fee and dates.

OWNER

West Monroe

03 Approve

The steering committee approves or declines.

OWNER

Steering committee

04 Update

Plan, fee and dates are updated before work starts.

OWNER

Program lead

Impact assessed within three business days. No change work starts without written approval.

What we assume, and what we need from Northfield

WHAT WE ASSUME

- 01

Scope
14 business units on one ERP instance.
- 02

Fees
Fixed for the phases shown; changes follow change control.
- 03

Timing
Decisions are made within five business days.
- 04

Access
Read access to ERP and close calendars in week 1.

WHAT WE NEED FROM NORTHFIELD

DEPENDENCY	OWNER	BY
Named business-unit leads	Controller	Week 1
ERP and close-calendar access	Finance IT	Week 1
Steering committee cadence	CFO office	Week 2
Sign-off on design decisions	CFO	Week 8
Cutover window agreed	Finance IT	Week 12

Eighteen assumptions the plan and fee rest on

CLIENT RESPONSIBILITIES

ASSUMPTION	OWNER
Named lead for each business unit	Controller
Decisions within five business days	CFO
Subject experts 4 hrs a week	BU leads

TIMELINE

ASSUMPTION	OWNER
Kickoff by 12 January	Sponsor
No change freeze during weeks 6 to 8	CFO
Cutover window agreed by week 12	Finance IT

ENVIRONMENT AND ACCESS

ASSUMPTION	OWNER
Read access to the ERP granted in week 1	Finance IT
Secure workspace for the team	Finance IT
VPN and badges before start	Security

GOVERNANCE

ASSUMPTION	OWNER
Steering committee meets every two weeks	CFO
One accountable sponsor	CFO
Risks escalated within 48 hours of discovery	PMO

DATA

ASSUMPTION	OWNER
Close data for 24 months available	Controller
Master data owner named	Finance IT
Data quality issues logged, not fixed	Joint

COMMERCIAL

ASSUMPTION	OWNER
Fixed fee for phases shown	Joint
Expenses billed at cost, capped at 8%	Joint
Changes follow change control	PMO

If an assumption changes, here is what moves

ASSUMPTION	OWNER	IF IT CHANGES	WHAT HAPPENS TO SCOPE, FEE OR DATES
Business-unit leads named by week 1	Controller	Leads named after week 2	Diagnostic slips; about \$18K per week
Decisions made in five business days	CFO	Decisions take ten days or more	Dates move day for day
Read access to ERP in week 1	Finance IT	Access arrives after week 2	Data work starts late; plan reset
Close data for 24 months is complete	Controller	Months are missing or restated	Rework billed under change control
Fourteen business units in scope	Sponsor	A fifteenth unit is added	Fee rises about \$48K per unit
Existing interfaces stay unchanged	Finance IT	An interface is retired mid-build	Design and test effort reopened
Steering committee meets every two weeks	CFO	Meetings are skipped or deferred	Approvals queue; Build start moves
Fixed fee covers phases shown	Joint	A new phase is requested	New statement of work required

Ten numbered assumptions we will hold each other to

DELIVERY AND GOVERNANCE

NO.	ASSUMPTION
1	Client provides a named lead for each of the 14 business units Leads attend the weekly working session
2	Decisions are made within five business days Longer waits move dates day for day
3	Read access to the ERP and close calendars in week 1 Production data is read only
4	Close data for the last 24 months is complete Gaps are logged and carried forward
5	Steering committee meets every two weeks Quorum is the CFO plus two leads

DATA, ENVIRONMENT AND COMMERCIAL

NO.	ASSUMPTION
6	Cutover window agreed by week 12 No month-end inside the window
7	Existing interfaces stay unchanged during the program New interfaces are in scope
8	Fees are fixed for the phases shown Build is billed on the team run-rate
9	Expenses are billed at cost, capped at 8% of fees Travel is pre-approved
10	Changes follow the agreed change control Requests are sized within three days

In scope, and out of scope

IN SCOPE

Included in the fixed fee

- ✓ Close calendar mapping for 14 business units
- ✓ Delay ranking and diagnostic readout
- ✓ Target operating model and rules
- ✓ Business case and wave plan
- ✓ Design sign-off workshops
- ✓ Wave 1 readiness plan
- ✓ Handover to Northfield finance
- ✓ Weekly steering committee updates

OUT OF SCOPE

Software licenses and infrastructure

Build and configuration work

Data cleansing beyond the pilot

Northfield staff time

Priced separately through change control.

PROCESS LEAD

Sam Ortiz designs the new close

ROLE

Sam leads the diagnostic and designs the rules, routing and review calendar for wave 1.

BACKGROUND

A former assistant controller who now builds rules engines for finance teams.

SELECTED EXPERIENCE

- Rules engines for 3 clients
- Cut one close from 11 days to 6
- Lean certified



Sam Ortiz

Senior Manager · Lean, SAP



ENGAGEMENT LEAD

Jordan Ellis leads the engagement

Jordan Ellis • Managing Director

ROLE

Jordan sets direction, chairs the steering committee and is accountable for the five-day close.

BACKGROUND

Thirty years in health-system finance, most of them spent redesigning the close.

SELECTED EXPERIENCE

- 30+ close transformations
- 14-entity finance redesign
- Former health-system CFO

“

Our close became a calendar, not a fire drill.

VP FINANCE • REGIONAL HEALTH PARTNER



Jordan Ellis

Managing Director

CPA · Health care

ENGAGEMENT LEAD

Jordan Ellis leads the engagement

ROLE

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VP FINANCE · REGIONAL HEALTH PARTNER

OUR TEAM

A senior team that has done this before

ENGAGEMENT LEADERSHIP

PR

Priya Raman

Managing director

ENGAGEMENT LEAD

FINANCE

DATA

- 30+ finance transformations
- Led a 14-entity data redesign
- Speaks at CFO forums



Sam Ibarra

Senior manager

DELIVERY LEAD

AGILE

CLOUD

- Built 3 governed platforms
- Former controller
- Agile coach for 40 teams

TB

Tomas Berg

Director

SOLUTION ARCHITECT

DATA

SECURITY

- Designed 2 finance data hubs
- Leads architecture practice
- Patent holder, data lineage

AO

Aisha Okafor

Principal

CHANGE AND ADOPTION

TRAINING

HEALTH

- Ran 12 enterprise rollouts
- Certified change lead
- Trained 3,000 end users

SPECIALISTS ON CALL

HB

Helen Brandt

Governance, controls

RM

Ravi Menon

Finance data design

CD

Claire Dubois

Treasury and payments

JW

Jonas Weber

Cybersecurity

OUR TEAM

A senior team that has done this before

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The people who will run your close program



Jordan Ellis

Managing Director

ENGAGEMENT LEAD

CPA

HEALTH CARE

- 30+ close transformations
- Led a 14-entity finance redesign



Sam Ortiz

Senior Manager

PROCESS LEAD

LEAN

SAP

- Built rules engines for 3 clients
- Former assistant controller



Priya Kapoor

Manager

DATA ARCHITECT

AZURE

DATA MESH

- Designed 2 finance data hubs
- Leads data quality practice

TEAM

The people who will run your close program



Jordan Ellis
Managing Director

ENGAGEMENT LEAD

CPA HEALTH CARE

- 30+ close transformations
- Led a 14-entity finance redesign
- Chairs the steering committee



Sam Ortiz
Senior Manager

PROCESS LEAD

LEAN SAP

- Built rules engines for 3 clients
- Former assistant controller
- Cut one client's close by four days



Priya Kapoor
Manager

DATA ARCHITECT

AZURE DATA MESH

- Designed 2 finance data hubs
- Leads data quality practice
- Certified cloud data architect

All three work on the engagement full time, from kickoff to hand-off.

One row carries the largest gap to target

CAPABILITY	MAIN	LAKESIDE	RIDGEVIEW	HARBOR
Patient access	Small	Moderate	Moderate	Large
Scheduling	Moderate	Moderate	Large	Large
Revenue cycle	Small	Small	Moderate	Large
Documentation	Small	Moderate	Moderate	Moderate
<u>Care coordination</u>	Moderate	Moderate	Large	Critical
Supply chain	Small	Moderate	Large	Large
Data and analytics	Moderate	Moderate	Large	Large

- Nine other cells sit in the large band, all at Ridgeview or Harbor.
- Main campus has no gap above moderate.

BIGGEST GAP

Care coordination at Harbor

Referral status lives in faxes and phone notes, so no one sees a patient's whole path.

No gap Small Moderate Large Critical

Source: Northfield Health System assessment, Sep 2026.

Three gaps drive most of the distance to target

CAPABILITY	MAIN CAMPUS	LAKESIDE	RIDGEVIEW	HARBOR
Patient access				
Scheduling				2
Revenue cycle				
Documentation				
Care coordination				1
Supply chain				
Data and analytics			3	

No gap Small Moderate Large Critical

Source: Northfield Health System assessment, Sep 2026.

- 01 Care coordination at Harbor
No shared care plan or referral tracking; clinics hand off by phone and fax.
- 02 Scheduling at Harbor
Templates differ by clinic, so open slots never reach the central call center.
- 03 Data and analytics at Ridgeview
Reports are built by hand each month and cannot be compared with other sites.

-0.5
off the weighted gap score.

Maturity falls away from the main campus toward Harbor

CAPABILITY	MAIN CAMPUS	LAKESIDE	EASTGATE	RIDGEVIEW	ST. ANNE'S	HARBOR
Patient access	3	2	2	1	3	1
Scheduling	3	3	2	2	2	1
Revenue cycle	4	3	3	2	3	2
Clinical documentation	4	3	3	3	2	2
Care coordination	2	2	1	1	2	0
Supply chain	3	2	2	1	1	1
Workforce management	2	2	2	1	1	1
Data and analytics	3	2	1	1	2	0
Cybersecurity	3	3	2	2	2	1

Absent
Ad hoc
Defined
Managed
Optimized

Source: Northfield Health System assessment, Sep 2026.

Care coordination is the weakest row overall

Absent Ad hoc Defined Managed Optimized

CAPABILITY	MAIN CAMPUS	LAKESIDE	EASTGATE	RIDGEVIEW	ST. ANNE'S	HARBOR	AVERAGE
Patient access Front door and intake	3	2	2	1	3	1	2.0
Scheduling Slots and templates	3	3	2	2	2	1	2.2
Revenue cycle Claims to cash	4	3	3	2	3	2	2.8
Care coordination Referrals and plans	2	2	1	1	2	0	1.3
Cybersecurity Controls and response	3	3	2	2	2	1	2.2
Data and analytics Reports and models	3	2	1	1	2	0	1.5
Supply chain Orders and stock	3	2	2	1	1	1	1.7

Source: Northfield Health System assessment, Sep 2026.

CAPABILITY ASSESSMENT

Strong at the core, thin at the edges

- **Strong core.** Revenue cycle and documentation reach level 3 or better at the main campus.
- **Uneven spread.** Only the main campus holds a level 3 or 4 in more than half the rows.
- **Thin edge.** Harbor has no capability above level 2, and two sit at level 0.

LIFT FIRST

3

capabilities at Ridgeview and Harbor.

CAPABILITY	MAIN CAMPUS	LAKE SIDE	RIDGEVIEW	HARBOR
Patient access	3	2	1	1
Scheduling	3	3	2	1
Revenue cycle	4	3	2	2
Clinical documentation	4	3	3	2
Care coordination	2	2	1	0
Supply chain	3	2	1	1
Workforce management	2	2	1	1
Data and analytics	3	2	1	0
Cybersecurity	3	3	2	1
Patient billing	3	2	2	1
Quality reporting	3	3	2	1

Absent
Ad hoc
Defined
Managed
Optimized

Source: Northfield Health System assessment, Sep 2026.

Harbor trails on every capability

CAPABILITY	MAIN CAMPUS	LAKESIDE	RIDGEVIEW	HARBOR
Patient access	3	2	1	1
Scheduling	3	3	2	1
Revenue cycle	4	3	2	2
Documentation	4	3	3	2
Care coordination	2	2	1	0
Supply chain	3	2	1	1
Workforce	2	2	1	1
Data and analytics	3	2	1	0
Cybersecurity	3	3	2	1

Absent Ad hoc Defined Managed Optimized

Source: Northfield Health System assessment, Sep 2026.

TAKEAWAYS

9 of 9

capabilities sit at level 2 or below at Harbor.

1.3

is the six-site average for care coordination, the weakest row.

2.8

is the six-site average for revenue cycle, the pattern to replicate.

The full map: 48 capabilities across six domains

Access	Care	Billing	Engagement	Platform	Data
Find a provider	Care plans	Statements	Notifications	Identity, consent	Patient data store
Self-scheduling	Medication list	Pay online	Surveys	EHR integration	Analytics events
Waitlist	Lab results	Payment plans	Education library	Event stream	Consent registry
Virtual visit join	Visit summaries	Cost estimates	Programs	Observability	Data quality
Pre-visit forms	Secure messaging	Insurance capture	Reminders	Release pipeline	Reporting layer
Kiosk check-in	Remote vitals	Financial aid	Health goals	Feature flags	ML features
Wayfinding	Allergy list	Refund requests	Feedback intake	Audit trail	Patient index
Referral intake	Immunizations	Autopay	Language choice	Design system	Data catalog

■ Live
■ In build
■ Planned 2026
□ Not started

Access and billing are live; care builds next

Access	Care	Billing	Engagement
Find a provider	Care plans	Statements	Reminders
Self-scheduling	Medication list	Pay online	Proxy access
Waitlist and cancellations	Lab results	Payment plans	Feedback surveys
Virtual visit join	Secure messaging	Cost estimates	Health content
Pre-visit forms	Remote monitoring	Insurance card capture	Community programs

Live
 In build
 Planned 2026
 Not started

PATIENT APP CAPABILITIES

Access is live; care and billing carry the backlog

- **Live** booking, billing basics and the medication list.
- **In build** care plans, messaging and payment plans.
- **Open** kiosk, wayfinding and the community programs.

■ Live

■ In build

■ Planned 2026

□ Not started

Access

Find a provider

Self-scheduling

Waitlist and cancellations

Virtual visit join

Pre-visit forms

Kiosk check-in

Wayfinding

Care

Care plans

Medication list

Lab results

Secure messaging

Remote monitoring

Visit summaries

Allergy list

Billing

Statements

Pay online

Payment plans

Cost estimates

Insurance card capture

Financial aid

Autopay

Engagement

Reminders

Proxy access

Feedback surveys

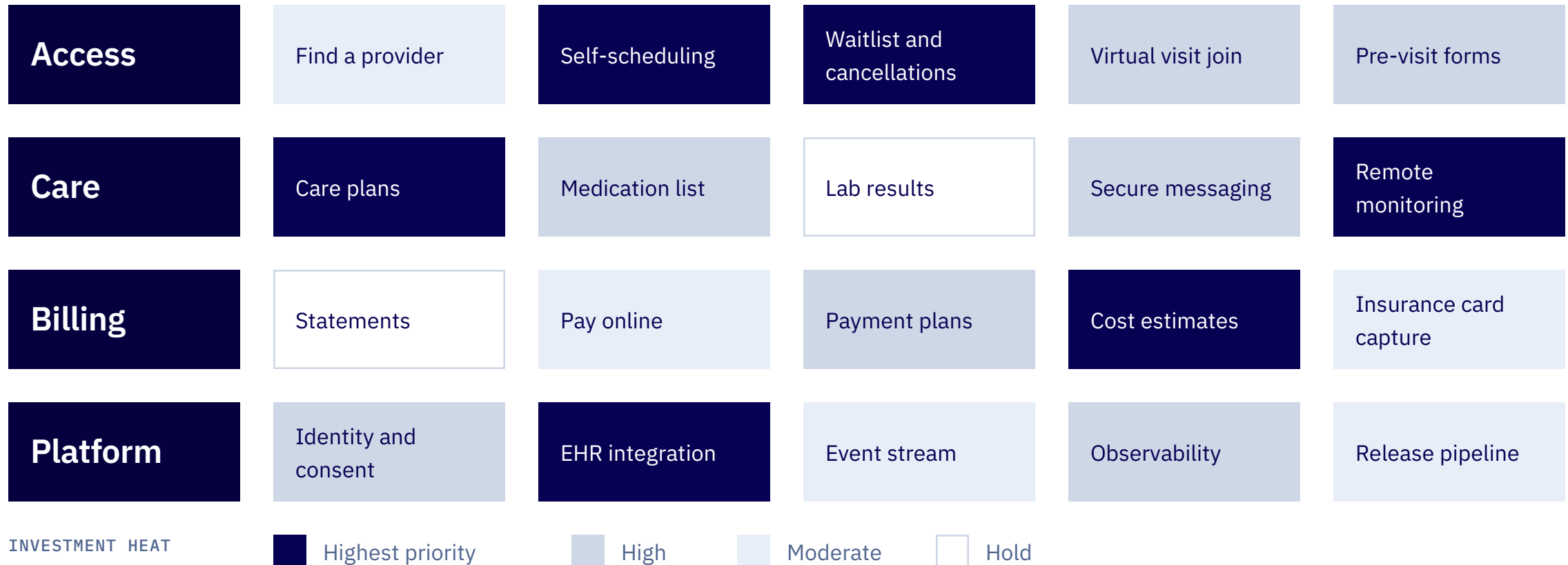
Health content

Community programs

Notifications

Language choice

Investment concentrates on scheduling and integration



Source: Northfield product council priorities, Sep 2026.

Eight of twenty capabilities are live today

Access

Find a provider
Self-scheduling
Waitlist and cancellations
Virtual visit join
Pre-visit forms

Care

Care plans
Medication list
Lab results
Secure messaging
Remote monitoring

Billing

Statements
Pay online
Payment plans
Cost estimates
Insurance card capture

Engagement

Reminders
Proxy access
Feedback surveys
Health content
Community programs

 Live  In build  Planned 2026  Not started

LIVE

8 capabilities are live, led by Access.

IN BUILD

6 are in build, spread across all four domains.

NOT YET BUILDING

6 are planned or not started, half of them in Engagement.

One model connects customers, capabilities and value

Customer	Patients			Payers		Employers		
	Self-pay · small balance			Commercial · large		Government · largest		
Channels	Portal	Phone	Text	Email	Mail	Kiosk	In person	
Enabling capabilities	Eligibility and intake			Charge capture		Claim status tracking		
	Registration and scheduling			Coding and edits		Denial workflow		
	Patient accounting platform							
Content and data	Enterprise data platform					Revenue analytics		
	Patient	Charge	Contract	Remit	Statement	Payroll	Ledger	
Operating model	Integrated channels			Revenue management		Organizational agility		
	Integrated technology			Message personalization		Process innovation		
Value drivers	CASH ACCELERATION - Days in A/R - Clean-claim rate			COST TO COLLECT - Touches per account - Rework volume		PATIENT EXPERIENCE - Billing clarity - Pay-by-text uptake		

READING THE MAP

Live where
patients start,
thin where care
continues.

8 of 20

capabilities are live; the build
backlog is spread across care and
billing.

PATIENT APP CAPABILITIES

Access leads and care trails

Access	Care	Billing	Engagement
Find a provider	Care plans	Statements	Reminders
Self-scheduling	Medication list	Pay online	Proxy access
Waitlist and cancellations	Lab results	Payment plans	Feedback surveys
Virtual visit join	Secure messaging	Cost estimates	Health content
Pre-visit forms	Remote monitoring	Insurance card capture	Community programs
Live	In build	Planned	Not started

Every layer has a clear build owner



Source: Sample names for layout only; replace with the client product inventory.

AI turns working evidence into durable capability

FOCUS	WHAT AI HELPS DO	HUMAN JUDGMENT THAT MATTERS
 Capture expertise	Structure interviews and surface knowledge gaps without asking experts to write every artifact.	Experts validate context, nuance and meaning.
 Observe the work	Use consented recordings and work artifacts to map steps, handoffs and workarounds.	Teams explain why the work happens that way.
 Analyze the systems	Examine source, architecture, data, integrations and risk to reveal constraints.	Technology leaders confirm trade-offs and boundaries.
 Co-design and validate	Create process alternatives and working prototypes, then test them with users.	People decide what should change and what will work.
 Transfer capability	Preserve operating knowledge, prepare people for new roles and retain context.	Leaders own adoption, readiness and accountability.

AI prepares evidence and first drafts; people decide what is true, what should change and when the business is ready.

Capabilities build across every domain, phase by phase

PHASE	APPLICATIONS	DATA	INFRASTRUCTURE	OPERATING MODEL
Rationalize Decide	Disposition model Per-application verdicts	Data inventory Sources, owners, quality	Hosting baseline Current cost and risk	Funding model Business case and gates
Build Deliver	Scheduling and billing Re-platformed and refactored	Migrated records Reconciled before cutover	Landing zone Network, identity, security	Cutover playbook Roles, rehearsals, rollback
Sustain Run	Release cadence Weekly, automated	Data stewardship Quality monitored	Cost optimization Rightsized each quarter	Modernization office Standards and backlog

Each phase leaves Northfield with new capabilities

Rationalize Decide	Application inventory Every system, owner, interface and cost	Value and risk scoring One scorecard across the portfolio	Disposition decisions Retire, retain, move or rebuild each	Funded roadmap Waves, budget and benefits agreed
Build Deliver	Cloud landing zone Secure, repeatable platform foundation	Modern scheduling Rebuilt on open, supported components	Reconciled data Patient and billing records verified	Controlled cutover Rehearsed waves with rollback
Sustain Run	Operational runbooks Support model and service levels in place	Cost and capacity Spend and capacity matched to demand	Release automation Changes ship weekly, not yearly	Modernization pipeline Next candidates identified each quarter

Across all phases: HIPAA compliance, clinical safety review and change management

Sprints 4 to 6 carry the heaviest commitments

TEAM	S1	S2	S3	S4	S5	S6	S7	S8	CAPACITY
Scheduling	24	28	32	34	36	36	30	22	32
Patient portal	20	24	28	30	34	38	34	26	30
Revenue cycle	16	22	26	30	32	34	30	24	28
Integration	28	32	36	40	42	40	34	24	36
Data platform	12	16	20	24	28	30	26	18	26

STORY POINTS Under 5 5–14 15–24 25–34 35 or more

01 Sprint 4 to 6

Every team commits above 24 points while go-live prep peaks.

02 Integration

Commits above its 36-point capacity in sprints 4 to 6.

03 Sprint 8

Load eases to 26 points or fewer everywhere before the freeze.

Source: Northfield delivery plan, Sep 2026.

Weeks 4 to 7 overload two roles

ROLE	W1	W2	W3	W4	W5	W6	W7	W8	W9	W10
Program manager	60%	60%	60%	70%	70%	70%	70%	60%	60%	50%
Integration architect	70%	90%	100%	120%	130%	130%	120%	100%	80%	60%
Data engineer	40%	60%	80%	100%	110%	120%	100%	80%	60%	40%
Clinical SME	20%	30%	50%	60%	60%	80%	100%	100%	70%	40%
Test lead	-	-	20%	40%	80%	100%	110%	110%	100%	80%
Change lead	20%	30%	40%	60%	60%	70%	90%	100%	100%	90%
Training developer	-	-	-	20%	50%	80%	100%	110%	100%	60%

Under 15%

15–44%

45–74%

75–104%

105% or more

OVER-ALLOCATION

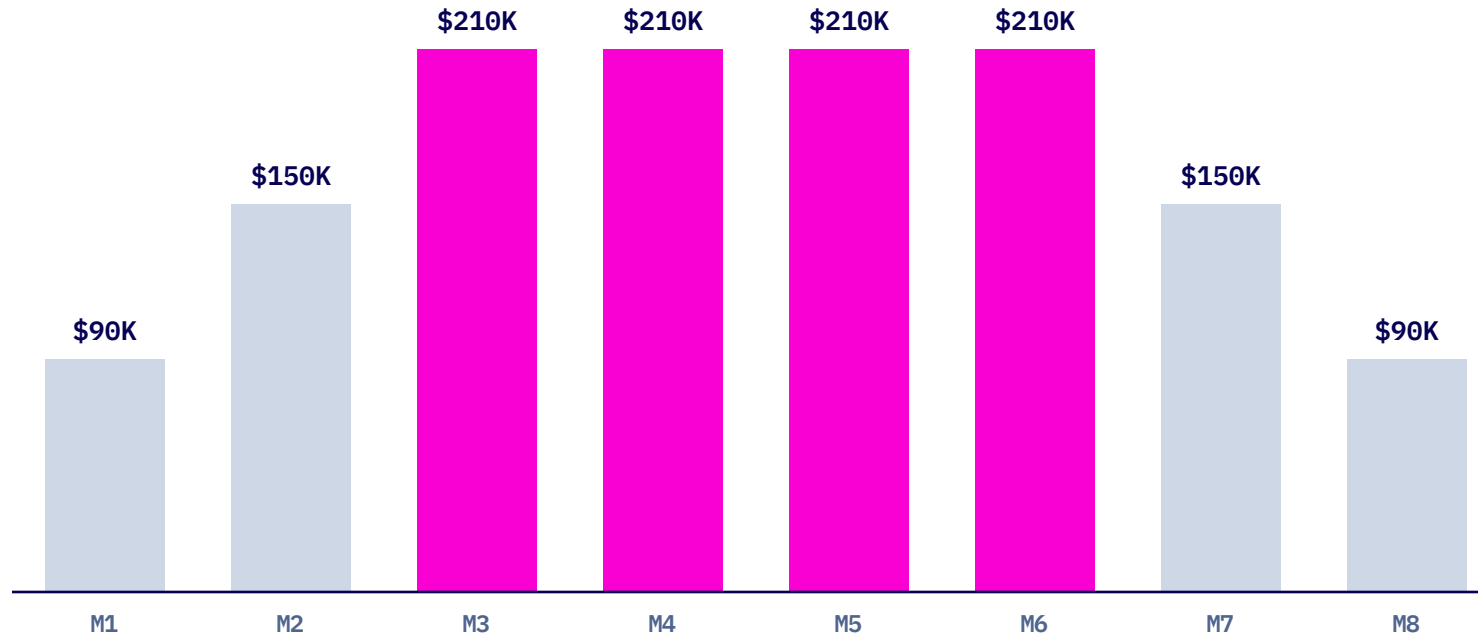
The integration architect runs over 100% from week 4 to week 7, and the data engineer joins in weeks 5 and 6.

Source: Northfield program resource plan, Sep 2026.

The run-rate ramps to \$210K a month, then holds

Team run-rate by month

TEAM MONTHLY RUN-RATE



- 01 Ramp-up, months 1–2**
Core team starts; specialists join as workstreams open.
- 02 Steady state, months 3–6**
Full team at \$210K a month.
- 03 Roll-off, months 7–8**
Capacity steps down as Northfield takes over.

\$1.32M

estimated total, eight months

COMMERCIALS

One team, one monthly run-rate

TEAM MONTHLY RUN-RATE

\$210K

for the whole team, invoiced monthly in arrears

4

roles on the team

30

days' notice to change the team

COVERS

The whole team

Process, data, accounting and change expertise, led by one engagement lead.

FLEXES

With demand

Add or release capacity with 30 days' notice; the run-rate moves with the team.

INVOICED

Monthly, in arrears

Expenses at cost, capped at 8% of fees.

COMMERCIALS

Three tiers of managed service, each one team run-rate

TIER	STANDARD	ENHANCED	PREMIUM
Coverage	Business hours	6 am to 10 pm	24 by 7
Critical issue response	4 hours	1 hour	15 minutes
Team	Shared pod	Dedicated pod	Dedicated pod and lead
Included	Run and monitor	Plus enhancements	Plus roadmap and optimization
Team monthly run-rate	\$45K	\$85K	\$140K

Three moves take the close from 12 days to 5

01 Diagnose

Map the close calendar for all 14 business units and rank each delay by days lost.

02 Design

Move review to the start of the cycle and set the rules that check entries as they post.

03 Deploy

Roll out in three waves, starting with the five entities that drive most of the delay.

Four steps, one owner for each

01 Diagnose

Rank every delay by the days it costs.

02 Design

Move review to the start of the cycle.

03 Deploy

Three waves, five entities first.

04 Sustain

Review close health every month.

Four design choices shape the new close

WHY IT MATTERS

A close that runs on rules, not reminders

Close takes 12 days because work waits for people, not because it is hard. We redesign the sequence so entries are checked as they post and exceptions reach an owner on day one.

01 Standard reconciliations

One template and one owner for every balance-sheet account, closed during the month.

02 Automated matching

Intercompany balances match overnight, so eleven entities stop trading spreadsheets.

03 Earlier review

Controllers review entries as they post, not in a single push on day nine.

04 Named ownership

Each close task has an owner, a due time and a clear escalation path.

Four design choices shape the new close

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Standard reconciliations

One template and owner for every balance-sheet account.



Automated matching

Intercompany balances match overnight across entities.



Earlier review

Controllers review entries as they post.



Named ownership

Every task has an owner and a due time.

Four design choices shape the new close

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Four design choices shape the new close

WHY IT MATTERS

A close that runs on rules, not reminders

Close takes 12 days because work waits for people, not because it is hard. We redesign the sequence so entries are checked as they post.

PROCESS

01 Standard reconciliations

One template and one owner for every balance-sheet account.

02 Automated matching

Intercompany balances match overnight across eleven entities.

PEOPLE AND GOVERNANCE

03 Earlier review

Controllers review entries as they post, not on day nine.

04 Named ownership

Every close task has an owner, a due time and an escalation path.

FUTURE STATE

Four design choices shape the new close

WHY IT MATTERS

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RECOMMENDED

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02 Automated matching

Intercompany balances match overnight, so eleven entities stop trading spreadsheets.

03 Earlier review

Controllers review entries as they post, not in a single push on day nine.

Close governance

TO BE CONFIRMED

Six workstreams deliver the five-day close

WHY IT MATTERS

A close that runs on rules

Close takes 12 days because work waits for people. We resequence it so entries are checked as they post.

01 Close calendar

One calendar for 14 business units, built backward from day five.

02 Reconciliations

Accounts reconciled during the month, with exceptions routed on day one.

03 Intercompany

Overnight matching replaces email and spreadsheets across eleven entities.

04 Review

Rules check entries as they post; controllers see only the exceptions.

05 Reporting

Flash results ready on day three, the full package on day five.

06 Controls

Evidence is captured as work happens, so audit asks stop reopening the close.

Six workstreams deliver the five-day close

WHY IT MATTERS

A close that runs on rules

Close takes 12 days because work waits for people. We resequence it so rules check entries as they post.

01 Close calendar

One calendar for 14 business units, built back from day five.

02 Reconciliation

Accounts reconciled in the month; exceptions routed on day one.

03 Intercompany

Overnight matching replaces email across eleven entities.

04 Review

Rules check entries as they post; controllers see exceptions.

05 Reporting

Flash results on day three, the full package on day five.

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Evidence captured as work happens, so audits stop reopening it.

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Close takes 12 days because work waits for people. We resequence it so entries are checked as they post.

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REVIEW AND CONTROL

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Together they cut close effort by a third, with no added headcount.

Six workstreams deliver the five-day close

WHY IT MATTERS

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03 Intercompany

Overnight matching replaces email and spreadsheets across eleven entities.

04 Review

Rules check entries as they post; controllers see only the exceptions.

05 Reporting

Flash results ready on day three, the full package on day five.

06 Controls

Evidence is captured as work happens, so audit asks stop reopening the close.

Source: West Monroe close calendar analysis, FY26, 14 business units.

Eight changes deliver the five-day close

WHY IT MATTERS

A close that runs on rules

Close takes 12 days because work waits for people. We resequence it so entries are checked as they post.

01 Reconcile

One template and owner for every balance-sheet account.

02 Matching

Intercompany balances match overnight across eleven entities.

03 Early review

Controllers review entries as they post, not in one push on day nine.

04 Ownership

Every close task has a named owner, a due time and an escalation path.

05 Flash report

Flash results are ready on day three and the full package on day five.

06 Controls

Evidence is captured as work happens, so audit requests stop reopening the close.

07 Calendar

One calendar for 14 business units, built backward from the day-five target.

08 Exceptions

Breaks reach the right owner the same day, with the supporting evidence attached.

Four groups of change deliver the five-day close

WHY IT MATTERS

A close that runs on rules

Close takes 12 days because work waits for people. We resequence it so entries are checked as they post.

CALENDAR

01 Calendar

One calendar, built back from day five.

02 Exceptions

Breaks reach an owner the same day.

RECONCILIATION

03 Reconcile

One template and owner per account.

04 Matching

Intercompany balances match overnight.

REVIEW

05 Early review

Entries are reviewed as they post.

06 Ownership

Every task has an owner and a due time.

REPORTING

07 Flash report

Flash on day three, full pack on day five.

08 Controls

Evidence is captured as work happens.

Eight changes deliver the five-day close

WHY IT MATTERS

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PEOPLE AND GOVERNANCE

03 Early review

Controllers review entries as they post, not in one push on day nine.

04 Ownership

Every close task has a named owner, a due time and an escalation path.

05 Flash report

Flash results are ready on day three and the full package on day five.

06 Controls

Evidence is captured as work happens, so audit requests stop reopening the close.

Four findings, four design responses

WHAT WE HEARD

Controllers find problems too late

- **Local calendars.** 14 business units close on 14 schedules.
- **Email matching.** Eleven entities reconcile by spreadsheet.
- **Late review.** Most entries are first seen on day nine.
- **Thin ownership.** Tasks have no named owner or due time.

01 Calendar

One close calendar for all 14 business units, built backward from day five.

02 Matching

Intercompany balances match overnight, so eleven entities stop trading spreadsheets.

03 Review

Rules check entries as they post, so controllers see exceptions instead of every line.

04 Ownership

Every task has a named owner, a due time and a clear path when it slips.

01 Calendar

One close calendar for all 14 business units, built backward from day five.

02 Matching

Intercompany balances match overnight, so eleven entities stop trading spreadsheets.

03 Review

Rules check entries as they post, so controllers see exceptions instead of every line.

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CURRENT STATE

Four findings, four design responses

WHAT WE HEARD

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- **Local calendars.** 14 business units close on 14 schedules.
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- **Thin ownership.** Tasks have no named owner or due time.

Clients say we drive value and build long-term partnerships

MULTI-STATE PAYER

Migrated 1,300 reports after a data model change, automating mapping and generation. Finished 8 months early.

\$2.2M

in cost savings

75%

less report build time

PLATFORM • CLOUD DATA WAREHOUSE

FAST-GROWING HEALTH PLAN

Built a scalable data platform that improved governance and enabled AI use cases, lifting RFP scores 25–40%.

\$2.5M

cost reduction

120K+

hours saved a year

PLATFORM • CLOUD LAKEHOUSE

VALUE DELIVERED

96%

beat their investment

TRUSTED ADVISOR

99%

call us a strategic advisor

“They earned our trust quickly and became experts in our processes.”

INTEGRATION DIRECTOR, REGIONAL PAYER

“The team was the critical differentiator in our platform selection.”

SYSTEMS DIRECTOR, HEALTH PLAN

“In days they understood our goals and put structure in place.”

STRATEGY DIRECTOR, NATIONAL INSURER

Source: Sample figures for layout only; replace with current firm data before use.

Five engagements cover what Northfield needs

ENGAGEMENT	CLOSE	DATA	ERP	RESULT
Regional health system · Close redesign	✓	—	✓	Close in 6 days, from 15
Multi-hospital network · Claims data platform	—	✓	✓	6 hospitals live, no billing gap
Physician services group · Reporting rebuild	✓	✓	—	65% faster payer reports
Academic medical center · ERP migration	✓	—	✓	Cutover on plan, zero audit findings
Regional payer · Shared finance services	✓	✓	✓	\$2.2M in annual savings

Source: Sample figures for layout only; replace with current firm data before use.

Finance teams like yours closed faster



Source: West Monroe engagement results, confirmed by each client.

Two engagements, one pattern: review moved earlier

REGIONAL HEALTH SYSTEM

A 15-day close with manual intercompany matching.

- Automated reconciliations
- Moved review to day 1
- Daily close status

\$4B

REVENUE

6 days

close

\$2.1M

savings

SPECIALTY MANUFACTURER

Consolidation took a week after close.

- Single data model
- Close cockpit
- One calendar for 22 entities

22

ENTITIES

-4 days

consolidation

92%

auto-matched

Source: West Monroe engagement results, confirmed by each client.

Two engagements, one pattern: review moved earlier

CONTEXT

APPROACH

PROOF

TEAM

FEES

REGIONAL HEALTH SYSTEM

A 15-day close with manual intercompany matching.

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- Moved review to day 1
- Daily close status

\$4B
REVENUE

6 days
close

\$2.1M
savings

SPECIALTY MANUFACTURER

Consolidation took a week after close.

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22
ENTITIES

-4 days
consolidation

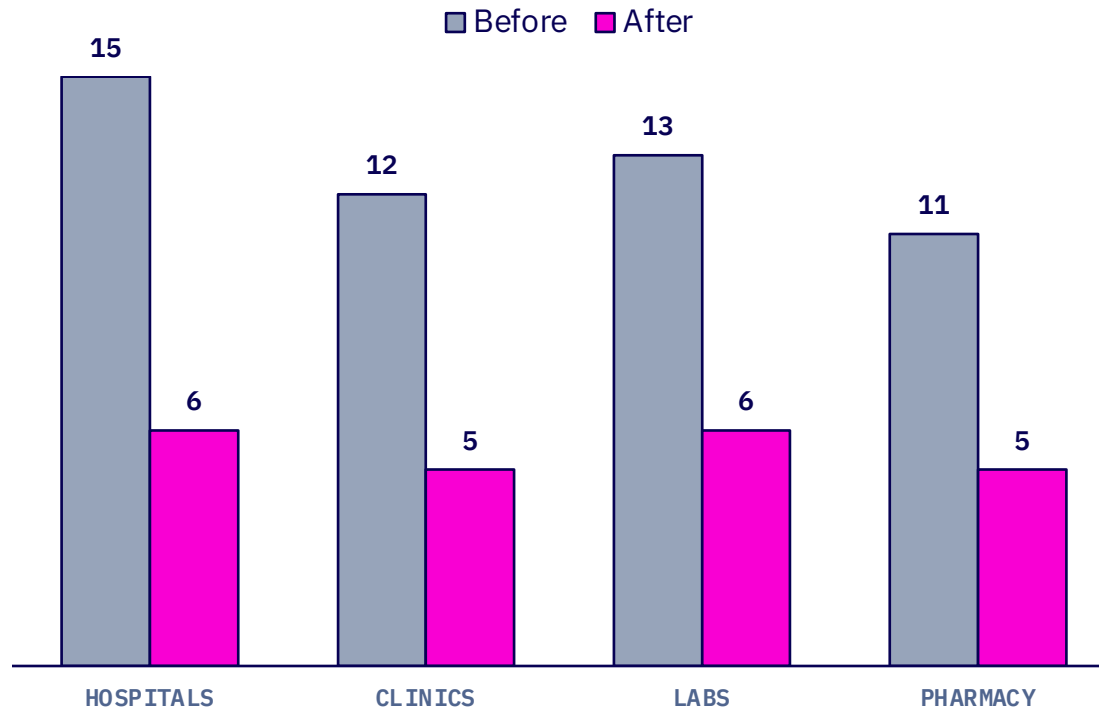
92%
auto-matched

Source: West Monroe engagement results, confirmed by each client.

Every entity now closes in under a week

Days to close, before and after

DAYS TO CLOSE



Source: Sample figures for layout only; replace with current firm data before use.

CHALLENGE

A 15-day close, with intercompany balances matched by email.

WHAT WE DID

- Automated reconciliations for the busiest accounts.
- Set rules at posting, so errors stop before review.
- Moved review to day one, entity by entity.

RESULTS

6 days

to close, from 15

\$2.1M

saved a year

CLIENT

Northfield Health System

Four-entity hospital group

CHALLENGE

A 15-day close, with intercompany balances matched by email.

RESULT

6 days

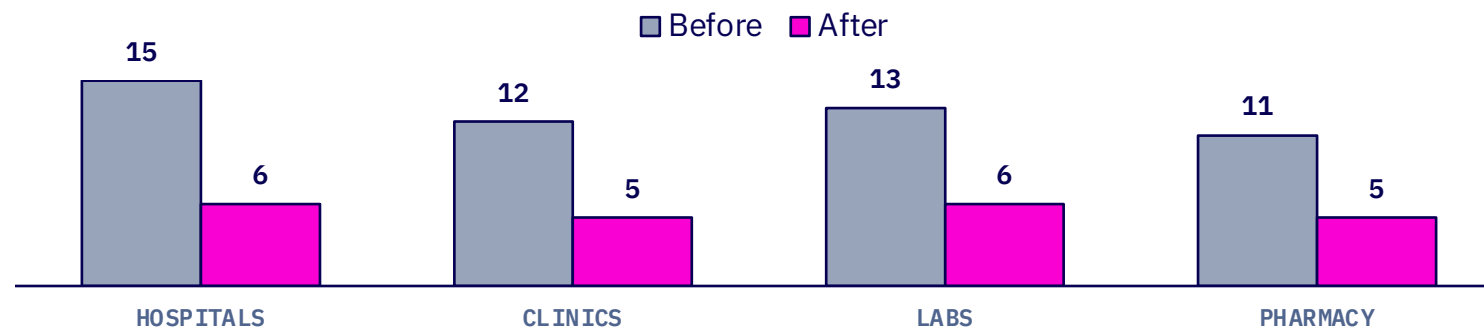
to close, from 15

CASE STUDY • HEALTHCARE

Every entity now closes in 6 days

Days to close, before and after

DAYS TO CLOSE



WHAT WE DID

- Automated reconciliations for the busiest accounts.
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RESULTS

92%

matches automated

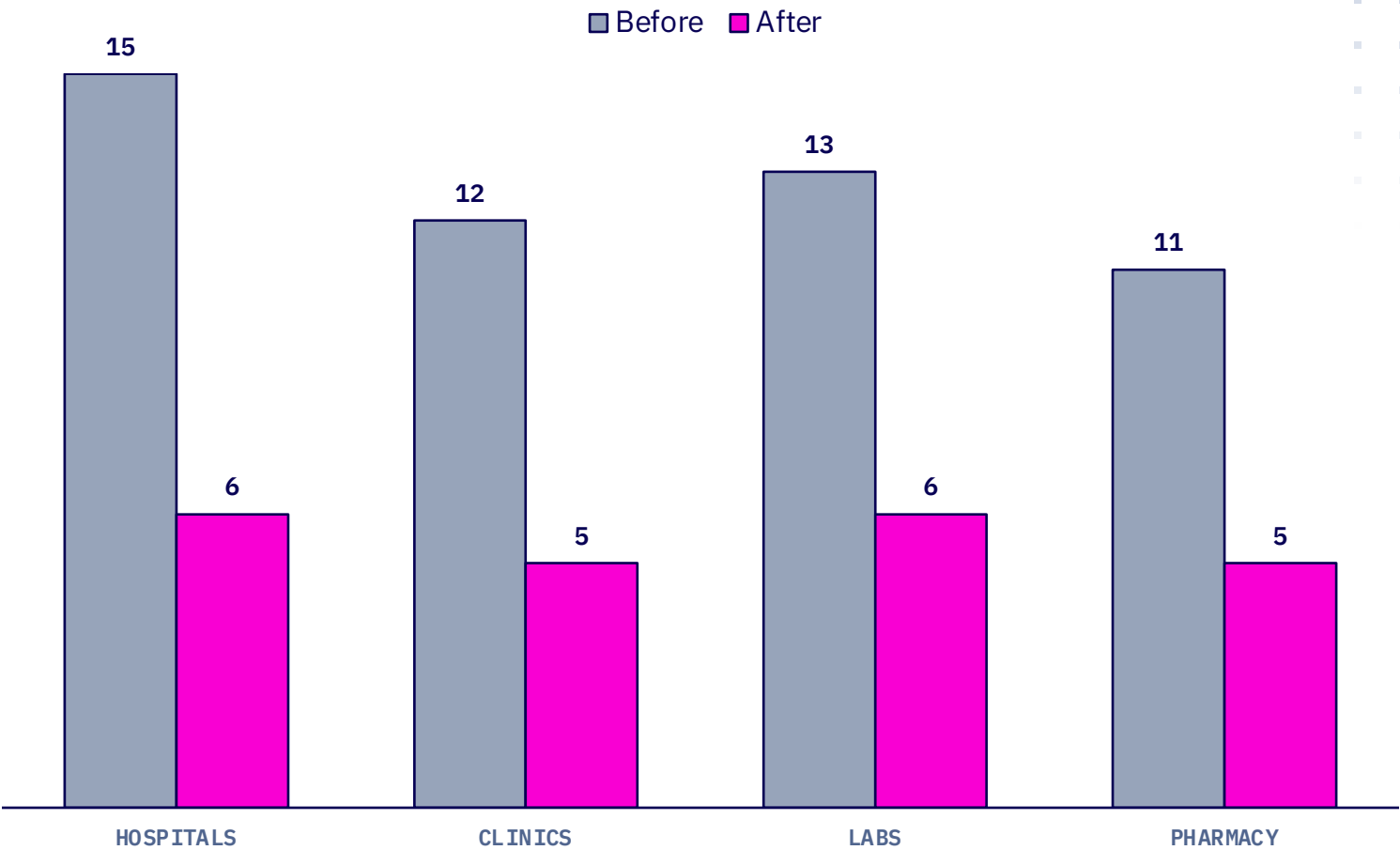
\$2.1M

saved a year

Source: Sample figures for layout only; replace with current firm data before use.

Days to close, before and after

DAYS TO CLOSE



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- Set rules at posting.
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RESULTS

6 days

to close, from 15

Source: Sample figures for layout only; replace with current firm data before use.

A shared data platform returned 3x its cost

SITUATION

A national payer wanted to grow by modernizing its data and cloud foundations, but legacy platforms and manual processes slowed every new use case.

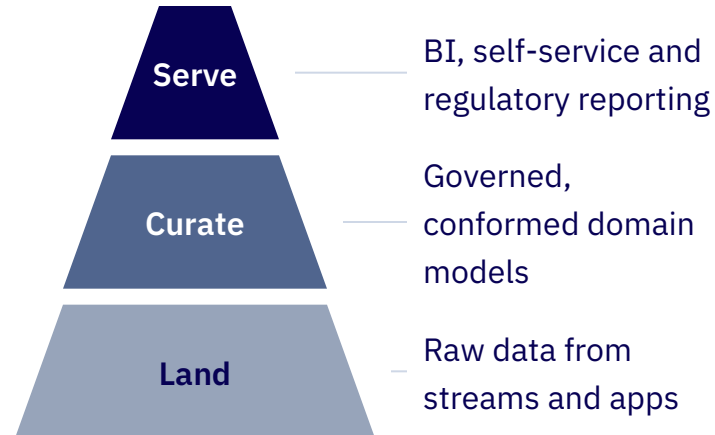
WHAT WE DID

Built the business case, then stood up a multi-tenant platform with governance, access patterns and a retirement plan for legacy systems.

THE RESULT

New data services, a governance framework and an operating model lifted trust and literacy.

PLATFORM ARCHITECTURE



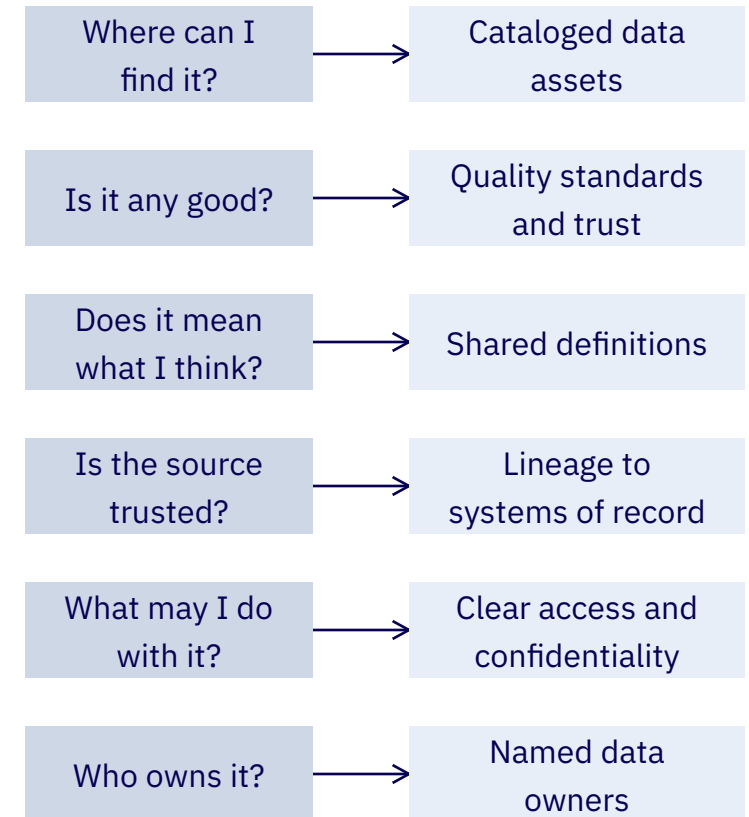
300%

ROI on the platform

\$10M

annual infrastructure savings

BUSINESS QUESTIONS TO VALUE DELIVERED



Source: West Monroe engagement results, confirmed by the client.

A 15-day close became a 6-day close



CHALLENGE

A 15-day close, with intercompany balances matched by email.

APPROACH

Automated reconciliations, rules at posting, review on day one.

6 days

to close, from 15

\$2.1M

annual savings

92%

of matches automated

We stopped finding problems on day nine.

CORPORATE CONTROLLER • REGIONAL HEALTH SYSTEM

Source: West Monroe engagement results, confirmed by the client.

A 15-day close became a 6-day close



“

CHALLENGE

A 15-day close, with intercompany balances matched by email.

APPROACH

Automated reconciliations, rules at posting, review on day one.

6 days

to close, from 15

\$2.1M

annual savings

92%

of matches automated

We stopped finding problems on day nine.

CORPORATE CONTROLLER • REGIONAL HEALTH SYSTEM

Source: West Monroe engagement results, confirmed by the client.

Close in 6 days: rules, reconciliations and a daily cockpit

6 days

to close, from 15

\$2.1M

annual savings

92%

of matches automated

WHAT WE DID

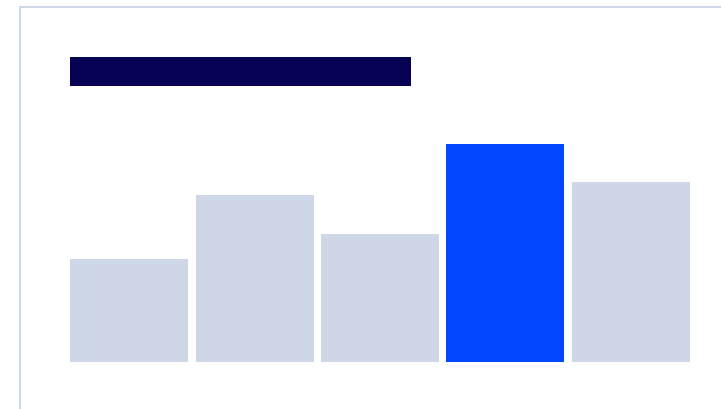
Approach

- **Automated** the account reconciliations.
- **Set rules** at posting.
- **Moved review** to day one.

“

We stopped finding problems on day nine.

CORPORATE CONTROLLER



Close cockpit, week 12

DELIVERABLE

A daily view of every entity's close status, built with the finance team.

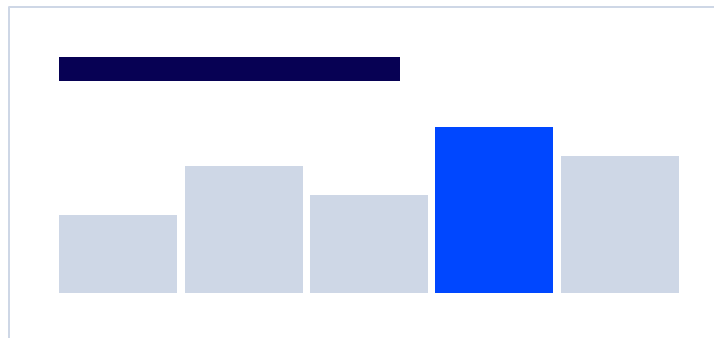
Source: Sample figures for layout only; replace with current firm data before use.

Close in 6 days: rules, reconciliations and a daily cockpit

WHAT WE DID

Approach

- **Automated** the account reconciliations.
- **Set rules** at posting.
- **Moved review** to day one.



Close cockpit, week 12

DELIVERABLE

A daily view of every entity's close status, built with the finance team.

“

We stopped finding problems on day nine.

CORPORATE CONTROLLER

Source: Sample figures for layout only; replace with current firm data before use.

RESULT

6 days

to close, from 15

\$2.1M

annual savings

92%

of matches automated

Regional health system cut its close from 15 days to 6



CHALLENGE

A 15-day close, with intercompany balances matched by email.

APPROACH

Automated reconciliations, rules at posting, review on day one.

RESULTS

6 days

to close, from 15

\$2.1M

annual savings

92%

of matches automated

Source: West Monroe engagement results, confirmed by the client.

A regional health system closed in 6 days, down from 15

WEEKS 1-2

Challenge

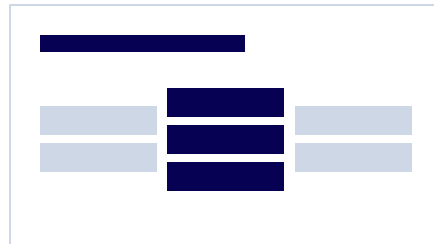
A 15-day close, with intercompany balances matched by email.



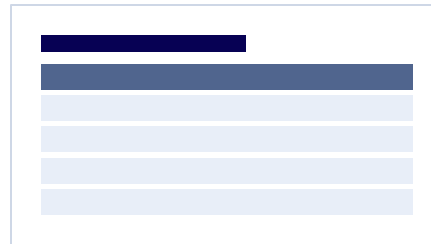
WEEKS 3-14

Approach

- **Mapped** every entity's close, task by task, to find the delays.
- **Automated** reconciliations for the 12 highest-volume accounts.
- **Set rules** at posting, so errors stop before review.
- **Moved review** to day one and rolled out entity by entity.



Close map, by entity



Reconciliation rules

WEEK 16 ON

Results

6 days

to close, from 15

8%

manual matches, from 70%

\$2.1M

saved a year

Source: Sample figures for layout only; replace with current firm data before use.

Three workstreams cut a regional close from 15 days to 6



Rules at posting

Errors are caught as entries post, not at review.

MANUAL MATCHES

fell from 70% to 8%.

Automated reconciliations

The 12 busiest accounts match without email.

CLOSE EFFORT

down 38,000 hours a year.

Day-one review

Controllers review exceptions from the first day.

REVIEW START

moved from day nine to day one.

RESULTS

6 days

close cycle, from 15

\$2.1M

annual savings

92%

of matches automated, from 30%

Source: Sample figures for layout only; replace with current firm data before use.

Three workstreams, one 6-day close

Rules at posting

Errors caught as entries post.

MANUAL MATCHES

fell from 70% to 8%.

Reconciliations

The busiest accounts match unaided.

CLOSE EFFORT

down 38,000 hours a year.

Day-one review

Exceptions reviewed from day one.

REVIEW START

moved up from day nine.

RESULTS

6 days

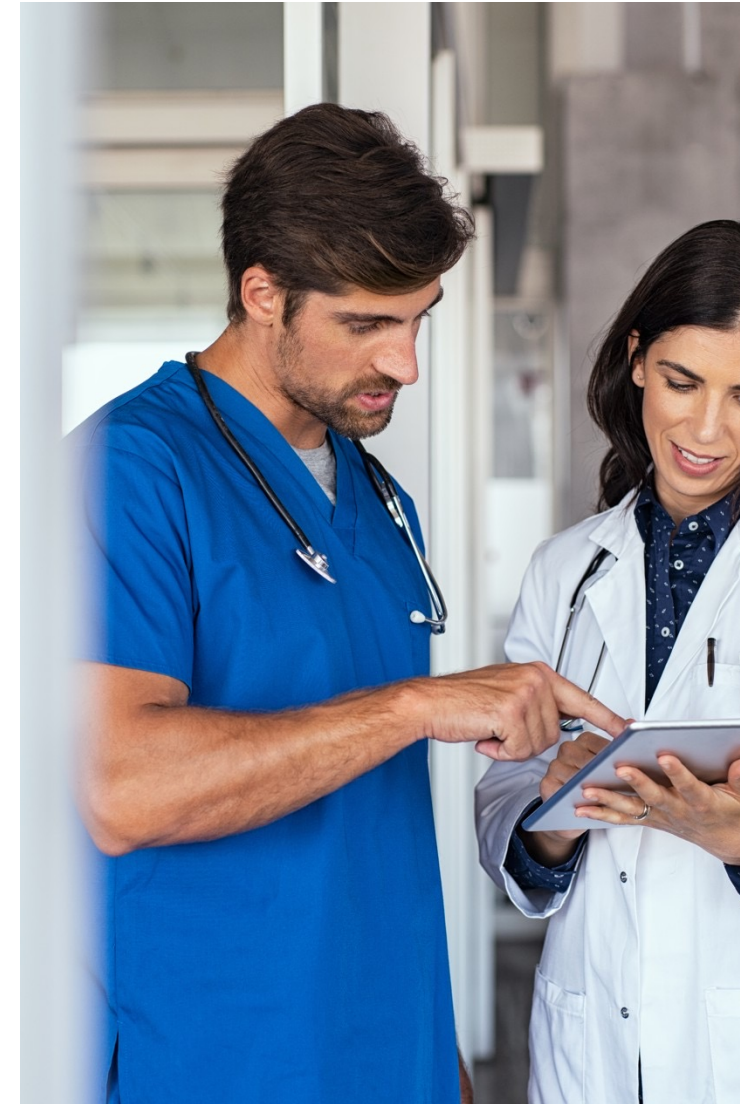
close, from 15

\$2.1M

saved a year

92%

matches automated

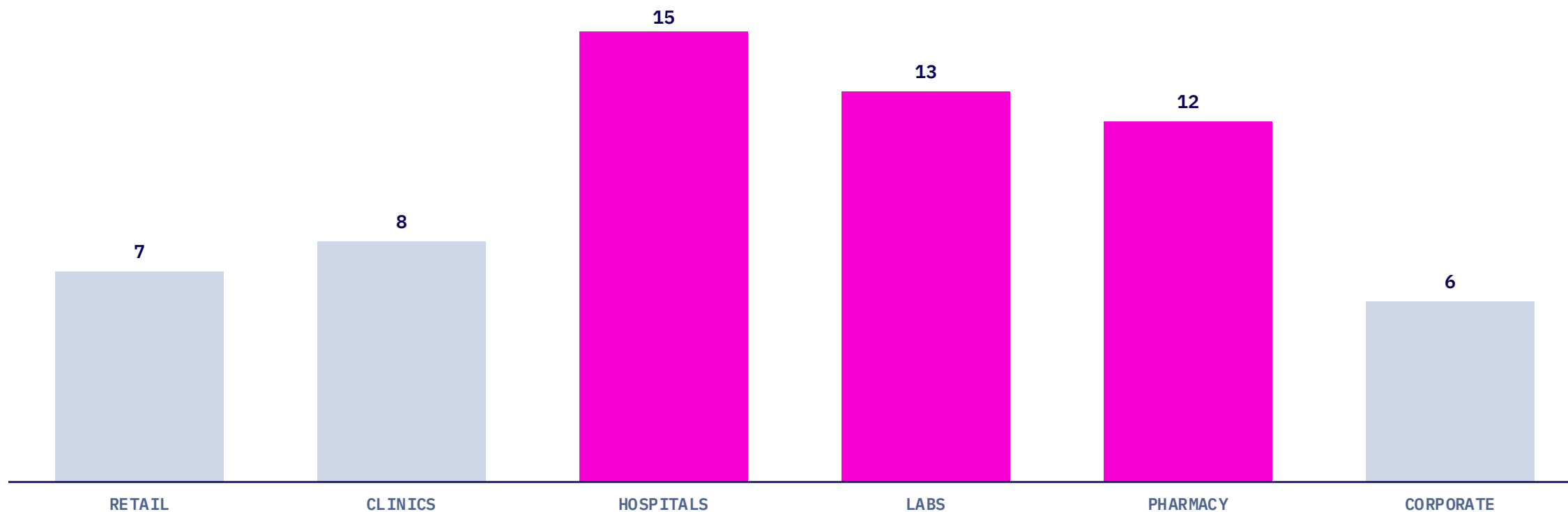


Source: Sample figures for layout only; replace with current firm data before use.

Three units take twice as long to close as the rest

Days to close by business unit group

DAYS TO CLOSE, AUG 2026



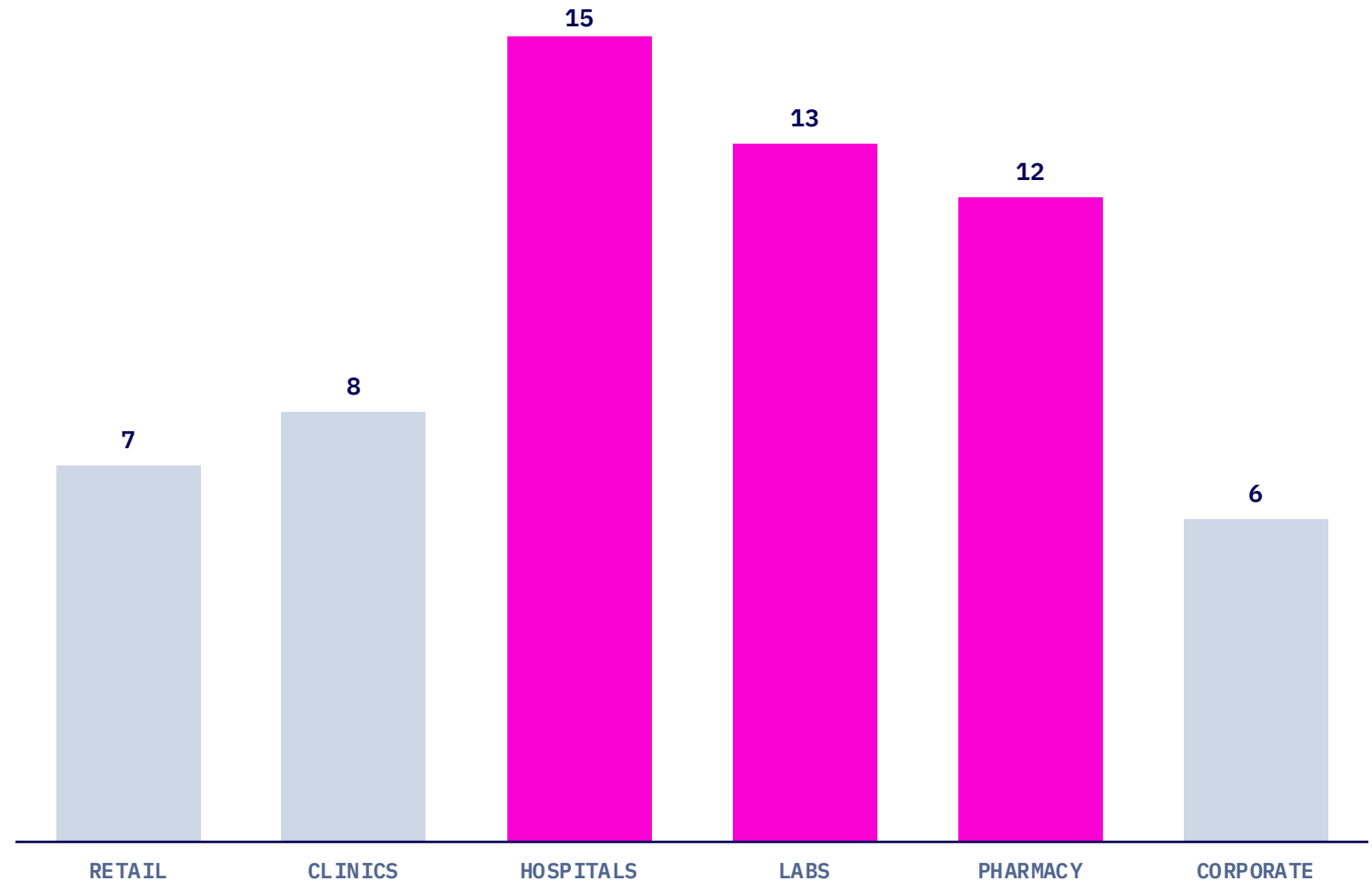
Source: ERP close calendar, Aug 2026; 14 business units grouped into six.

Three units take twice as long to close

Three units lag the rest

- **Slowest.** Hospitals take 15 days.
- **Fastest.** Corporate closes in 6.

Days to close by business unit group
DAYS TO CLOSE, AUG 2026



Source: ERP close calendar, Aug 2026.

WHAT THIS MEANS

Wave 1 sets the pace

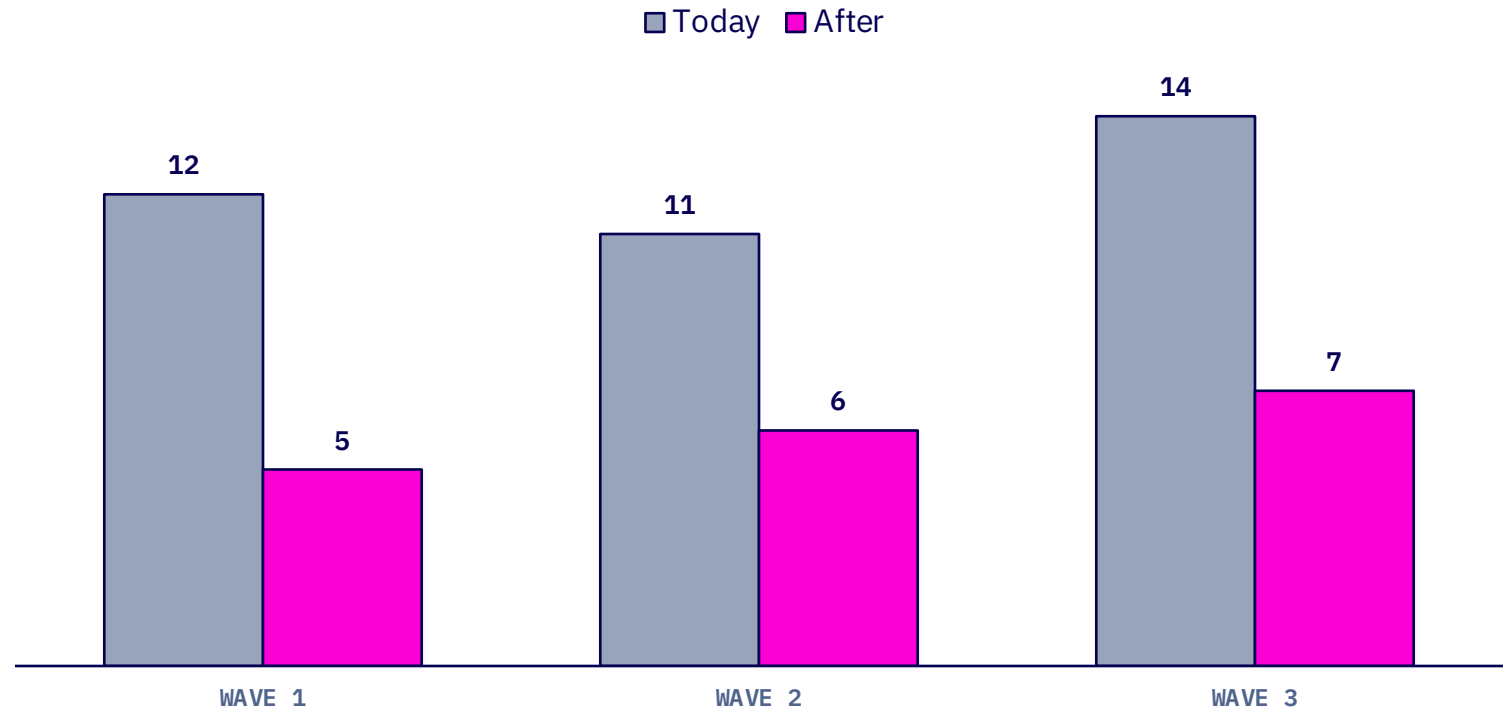
The five slowest entities move from 12 days to 5, which funds waves 2 and 3.

ROADMAP

Every wave cuts the close nearly in half

Days to close, before and after

DAYS TO CLOSE

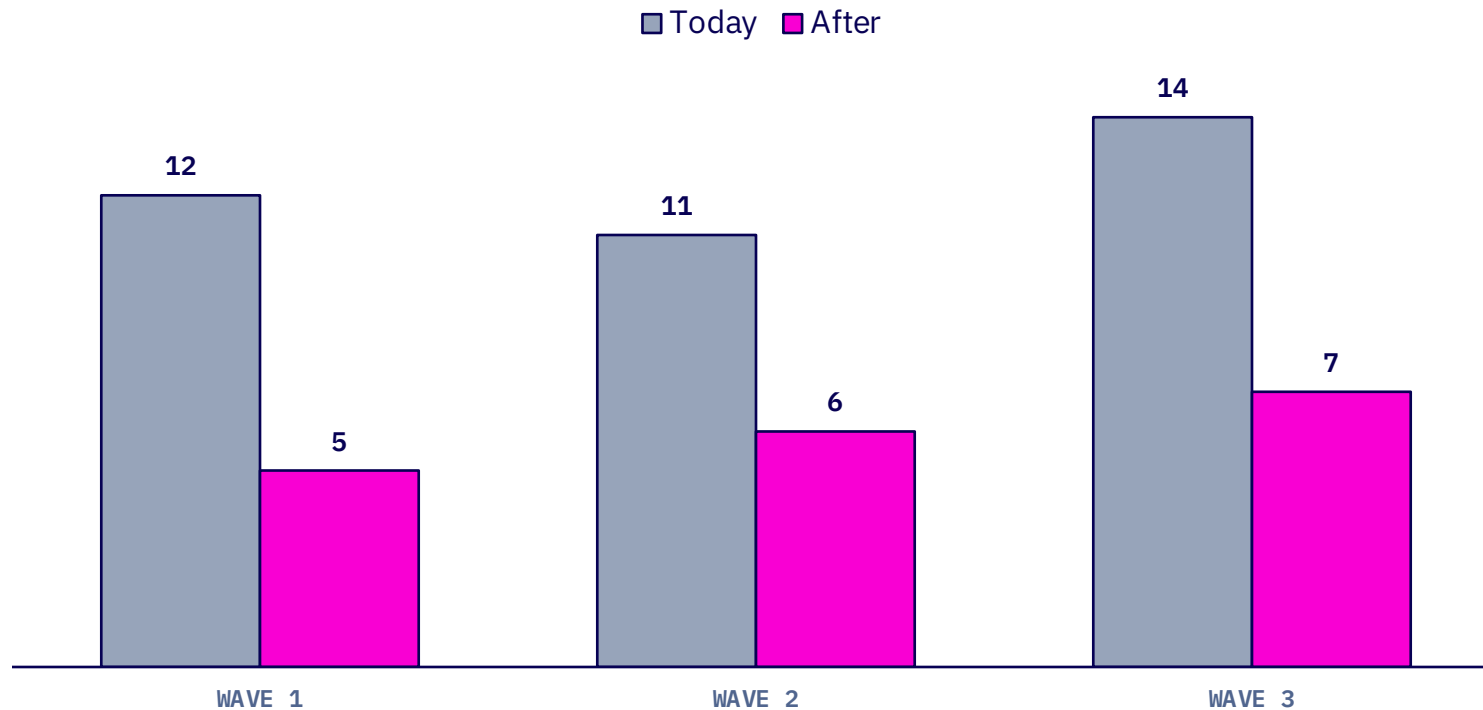


Source: Close calendar and wave plan, FY26 baseline.

Every wave cuts the close nearly in half

Days to close, before and after

DAYS TO CLOSE



Source: Close calendar and wave plan, FY26 baseline.

WHAT THIS MEANS

Wave 1 sets the pace

The five slowest entities move from 12 days to 5, which funds waves 2 and 3.

Clear Action. Lasting Impact.

NEXT STEP

Kick off wave 1 in January

Engagement lead · Finance Transformation
name@westmonroe.com

Each audience hears a message built for its day-to-day

01 Physicians

MESSAGE

What changes for my schedule

CHANNEL

Huddle and short video

CADENCE

Every 2 weeks

OWNER

Medical director

02 Frontline staff

MESSAGE

Steps, dates, who to call

CHANNEL

Floor-walkers, huddle cards

CADENCE

Weekly

OWNER

Practice managers

03 Leaders

MESSAGE

Progress, risks, decisions

CHANNEL

Steering briefing, dashboard

CADENCE

Monthly

OWNER

Program sponsor

04 Patients

MESSAGE

How to book, what to expect

CHANNEL

Email, text, lobby signs

CADENCE

From T-4 weeks

OWNER

Patient experience

Rule: staff hear from their own manager first.

Source: Northfield Health System communications plan, Sep 2026.

Six questions staff ask, with straight answers

01 Will I lose my job?

No roles are eliminated. Some tasks move; the role cards show which.

WHERE TO LOOK

Role cards on the intranet

02 When is my training?

Eight hours in the four weeks before your site goes live, scheduled by your manager.

WHERE TO LOOK

Your manager's schedule email

03 If the system is down?

A paper downtime kit and a hotline run for the first two weeks.

WHERE TO LOOK

Downtime kit at every desk

04 Who do I call?

Your floor-walker first, then the service desk hotline.

WHERE TO LOOK

Number on the huddle card

05 Keep my old templates?

Templates are rebuilt once by each clinic; your lead signs off.

WHERE TO LOOK

Template sign-off sheet

06 How will patients know?

Email, text and lobby signs begin four weeks before your site goes live.

WHERE TO LOOK

Lobby signs and website

Source: Northfield Health System change team, Sep 2026.

KEY MESSAGES

Three messages every leader repeats

01 Why now

Patients wait 11 days for a new appointment and hold times average eight minutes.

Three schedule systems cannot share open slots.
Staff lose time moving between screens.

02 What changes

One platform, one booking workflow, one set of rules for all nine sites.

Your manager shows you the steps before go-live.

Patient care itself does not change.

03 How we help

Role-based training, a champion on every floor and extra help for four weeks after launch.

Questions go to your floor-walker first.

Proof: 11-day wait today, 3-day goal.

Source: Northfield Health System communications plan, Sep 2026.

THE STORY IN ONE LINE

One schedule, easier access for every patient.

Use these words in every briefing, huddle and email. Local examples welcome; new claims are not.

Six audiences hear a tailored message on a fixed rhythm

AUDIENCE	KEY MESSAGE	CHANNEL	TIMING	OWNER
Executive team	Why the platform, funding and milestones	Steering briefing	Monthly	Program sponsor
Physicians	What changes for my schedule and referrals	Clinic huddle, short video	Every 2 weeks	Medical director
Schedulers and agents	New workflow, training dates, go-live support	Floor-walker, huddle card	Weekly	Practice managers
Nurses and clinic staff	How bookings and holds will look	Unit meeting, intranet	Every 2 weeks	Nurse leads
Union representatives	Role changes, training time, job security	Joint briefing	Per wave	HR director
Patients and families	New online booking and reminders	Email, text, signage	From T-4 weeks	Patient experience

Source: Northfield Health System communications plan, Sep 2026.

Every audience hears on a countdown to go-live

AUDIENCE	T-12 WKS	T-8 WKS	T-4 WKS	T-1 WK	GO-LIVE	T+4 WKS
Executives	Case and funding	Steering update	Readiness vote	Go/no-go brief	Launch note	Results review
Physicians	Vision video	Peer demo	Template sign-off	Cheat sheet	Hotline	Fix list
Schedulers	Town hall	Training dates	Sandbox practice	Huddle card	Floor-walkers	Tips and wins
Nurses	Unit meeting	Role cards	Unit demo	Downtime kit	Hotline	Pulse check
Patients	None yet	None yet	Website notice	Email and text	Lobby signage	Survey

- **Rule.** No audience hears a date from a channel it does not already trust.
- **Rule.** Frontline staff hear first from their own manager, never from a broadcast.
- **Rule.** Patients hear only after staff are trained at their site.

Source: Northfield Health System communications plan, Sep 2026.

TAKEAWAY

Four of five clinical systems have never tested a recovery.

Backups exist; the untested failover is the exposure.

RESILIENCE

Recovery is the weakest control

SYSTEM	BACKUP	FAILOVER	TESTED	DOCUMENTED
NorthChart EHR	1	2	4	1
Lab information	2	3	3	1
Pharmacy	1	2	2	1
Imaging archive	3	4	4	2
Scheduling	1	1	3	0

 None  Low  Some  High  Severe

WHAT WE RECOMMEND

- Test failover for the imaging archive and the EHR before go-live.
- Document recovery steps for the imaging archive, which scores worst on every control.

Inventory is the weakest data domain

DATA DOMAIN	COMPLETE	ACCURATE	TIMELY	CONSISTENT
Patient records	3	3	2	2
Orders	3	4	3	3
Charges	2	3	2	2
Inventory	2	2	1	2
Staff roster	4	3	3	3

Poor Weak Fair Good Strong

STRONGEST

Orders and roster

Both clear a good score on three of four checks.

WEAKEST

Inventory

Timely data scores one; counts lag the shelf by days.

HEADLINE

62%
of patient records
pass all four
checks.

Target for year end is 85%.

OUR DIFFERENCE

You will feel the difference in how we work

Our cross-functional teams combine hands-on finance, data and change experience with the technical depth to solve the whole problem.

We work alongside your team as player-coaches, moving from architecture and guardrails through engineering and operations to go-live.

HANDOVER TARGET

Day 90

Your team runs the platform.

OTHER FIRMS

- One-time benefit capture
- Technology-focused
- Generic blueprint
- Siloed workstreams
- Vendor-led delivery

HOW WE WORK

- Iterative path to go-live
- Platform-product focus
- Integrated architecture
- Player-coach partnership
- Cross-discipline teams

WHAT THAT MEANS FOR YOU



We build the capability to own, run and evolve the platform.



We apply data, cloud, security and change expertise.



We set measurable goals for speed, adoption and cost.



Cross-functional teams work together through delivery.



We target go-live outcomes, not only fixed scope.



Healthcare payer and platform experience shapes decisions.

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We set measurable goals for speed, adoption and cost.

This proposal is confidential

This document was prepared for Northfield Health System. It contains West Monroe's proprietary approach, pricing and staffing.

Use

Share it only with those evaluating this response.

Questions

Send questions to your West Monroe engagement lead.

Return

Return or destroy copies if we do not work together.

Shared context supports both **Build and Sustain**

SHARED FOUNDATION

- **Source-backed evidence** from code and data.
- **Validated process context** owned by the business.
- **Approved decisions** and current artifacts.
- **Review and test results** kept beside the evidence.

Governed retrieval connects the foundation to bounded work.

Build

HOW CONTEXT IS USED

Retrieve process, rules, requirements and open questions for a bounded design, coding, testing or cutover task.

VALUE OPPORTUNITY

Warmer handoffs, faster prototyping, stronger traceability and earlier visibility of gaps.

Sustain

HOW CONTEXT IS USED

Connect bug reports and change requests to documented intent, architecture and tests for impact analysis.

VALUE OPPORTUNITY

Faster investigation, better knowledge preservation and less maintenance effort over time.

The context layer compounds in value only while teams keep it current, governed and source-linked.

Persistent, agent-ready context can compress the path from prototype to production

CONTEXT CARRIED FORWARD

Evidence database

Source code, components, data and controls, with confidence.

Business process map

Steps, rules, exceptions and users linked to technology.

Decision trail

Approved outcomes, design choices and open questions.

Agent-ready context layer

■	PROCESS	business activity
■	RULE	what must hold true
■	COMPONENT	where behavior lives
■	REQUIREMENT	what must change
■	TEST	how it is proven

BUILD WORK ACCELERATED

- 1 Prototype** **FASTER FEEDBACK**
Options grounded in current behavior
- 2 Specify** **LESS REDISCOVERY**
Traceable stories and edge cases
- 3 Build** **FIRST-PASS FIT**
Relevant rules and design decisions
- 4 Test** **EARLIER DEFECTS**
Known states turned into tests



THE SHIFT

Retrieve the right context for the next task instead of rediscovering it, with review and testing still authoritative.

PROPOSAL • NORTHFIELD HEALTH SYSTEM

A five-day close that pays for itself

Record-to-report redesign for 14 business units

12 → 5
days to close



PROPOSAL FOR NORTHFIELD HEALTH SYSTEM

Close the books in **five days**

PREPARED BY

West Monroe · Finance Transformation

PROPOSAL CONTACTS

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Marcus Lee · Director
mlee@westmonroe.com



AI sequences each cutover wave and watches coexistence; people hold every go or no-go decision

1 Sequence

Find viable capability slices, expose hidden dependencies and simulate fallback paths.

2 Prepare

Profile and map data; draft test scenarios, runbooks, communications and support.

3 Monitor

Compare transactions and outcomes, flag exceptions and trace anomalies to dependencies.

ILLUSTRATIVE PHASED CUTOVER

	CURRENT STATE	WAVE 1	WAVE 2	WAVE 3
CLOSE AND CONSOLIDATION	Legacy	Coexistence	Modern	Modern
INTERCOMPANY AND PAYABLES	Legacy	Legacy	Coexistence	Modern
PLANNING AND REPORTING	Legacy	Legacy	Legacy	Coexistence

Source: Illustrative. The migration unit is a business-viable capability slice, not a single technical component.

Platform decisions at a glance, newest first

ID	DATE	DECISION	OWNER	STATUS	ADR LINK
D-021	29 Sep	Standardize on FHIR R4 for new APIs	Data architect	ACCEPTED	ADR-021
D-020	26 Sep	Retire the legacy scheduling database	Platform lead	PROPOSED	ADR-020
D-019	24 Sep	Adopt single sign-on for clinician apps	Security lead	ACCEPTED	ADR-019
D-018	22 Sep	Keep nightly batch for payer remittance	Finance partner	REJECTED	ADR-018
D-017	19 Sep	Host the analytics platform in a private cloud	Infrastructure lead	ACCEPTED	ADR-017
D-015	15 Sep	Move interface monitoring to one console	Platform lead	ACCEPTED	ADR-015
D-014	12 Sep	Route clinical events through an event bus	Chief architect	ACCEPTED	ADR-014
D-009	11 Mar 2024	Connect systems with point-to-point HL7 feeds	Chief architect	SUPERSEDED	ADR-009

ID · ADR	DATE	DECISION · OWNER	STATUS
D-021 ADR-021	29 Sep	Standardize on FHIR R4 for new APIs Data architect	ACCEPTED
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D-014 ADR-014	12 Sep	Route clinical events through an event bus Chief architect	ACCEPTED
D-009 ADR-009	Mar 2024	Connect systems with point-to-point HL7 feeds Chief architect	SUPERSEDED

DECISION LOG

Eight platform decisions,
newest first

STATUS OF THE EIGHT

5

accepted

1

rejected

1

proposed

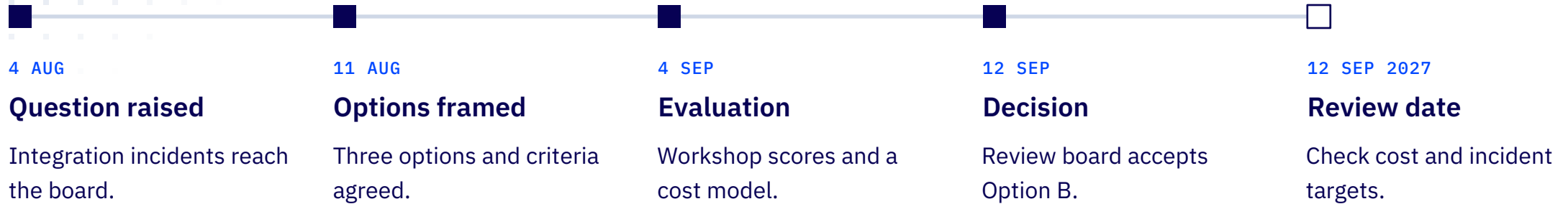
1

superseded

Every row links to its ADR in the architecture wiki.

HOW THE DECISION WAS MADE

Six weeks from question to a decision on the event bus



WHO TOOK PART

MO **Maya Okafor**
Chief architect

DH **Dev Hartley**
Platform lead

LS **Lena Soto**
Clinical informatics

RK **Rui Kato**
Finance partner

EVIDENCE REVIEWED

- Integration incident log, last 12 months
- Cost model for all three options
- Architecture workshop scores
- Reference check with two peer systems
- Vendor briefings for Option C

HOW THE DECISION WAS MADE

Six weeks from question to a decision

- **Question raised** 4 AUG
Integration incidents reach the board.
- **Options framed** 11 AUG
Three options and criteria agreed.
- **Evaluation** 4 SEP
Workshop scores and a cost model.
- **Decision** 12 SEP
Review board accepts Option B.
- **Review date** 12 SEP 2027
Check cost and incident targets.

WHO TOOK PART

MO **Maya Okafor**
Chief architect

LS **Lena Soto**
Clinical informatics

DH **Dev Hartley**
Platform lead

RK **Rui Kato**
Finance partner

EVIDENCE REVIEWED

- Integration incident log, last 12 months
- Cost model for all three options
- Architecture workshop scores

DECISION REQUEST

Approve the event bus by 17 October

OUR RECOMMENDATION

Fund Option B and retire the 61 point-to-point feeds in three waves. It scores highest against the criteria and is the only option that stops feed growth.

OPTION	3-YEAR COST	DURATION	STANCE
A · Keep point-to-point	\$6.2M	None	NOT ADVISED
B · Managed event bus	\$4.4M	9 months	RECOMMENDED
C · Integration suite	\$7.1M	14 months	ALTERNATIVE

IF WE WAIT

Each quarter adds about four feeds and delays the 2027 scheduling migration.

Source: Costs are sample figures for layout only; replace with current estimates before use.

THE ASK

Approve Option B and its \$4.4M budget

DECISION NEEDED BY

17 Oct

Next steering committee meeting.

DECISION OWNER

Steering committee chair

AI shifts buy versus build toward strategic fit

BUY

Accelerate standard capabilities

BEST WHEN

- Needs are common
- Speed beats customization
- Vendors carry the roadmap

TRADE-OFFS

- Fit gaps force compromise
- Roadmaps constrain change
- Switching costs build up

BUILD

Own differentiated capabilities

BEST WHEN

- Workflows give an edge
- Data and roadmap control
- Talent is already in place

TRADE-OFFS

- Needs durable capability
- You own maintenance
- Hiring takes time

MODERNIZATION ECONOMICS

AI lowers cost at every stage

PLAN AND UNDERSTAND

Faster discovery, mapping and requirements
Weeks of analysis become days.

IMPLEMENT AND BUILD

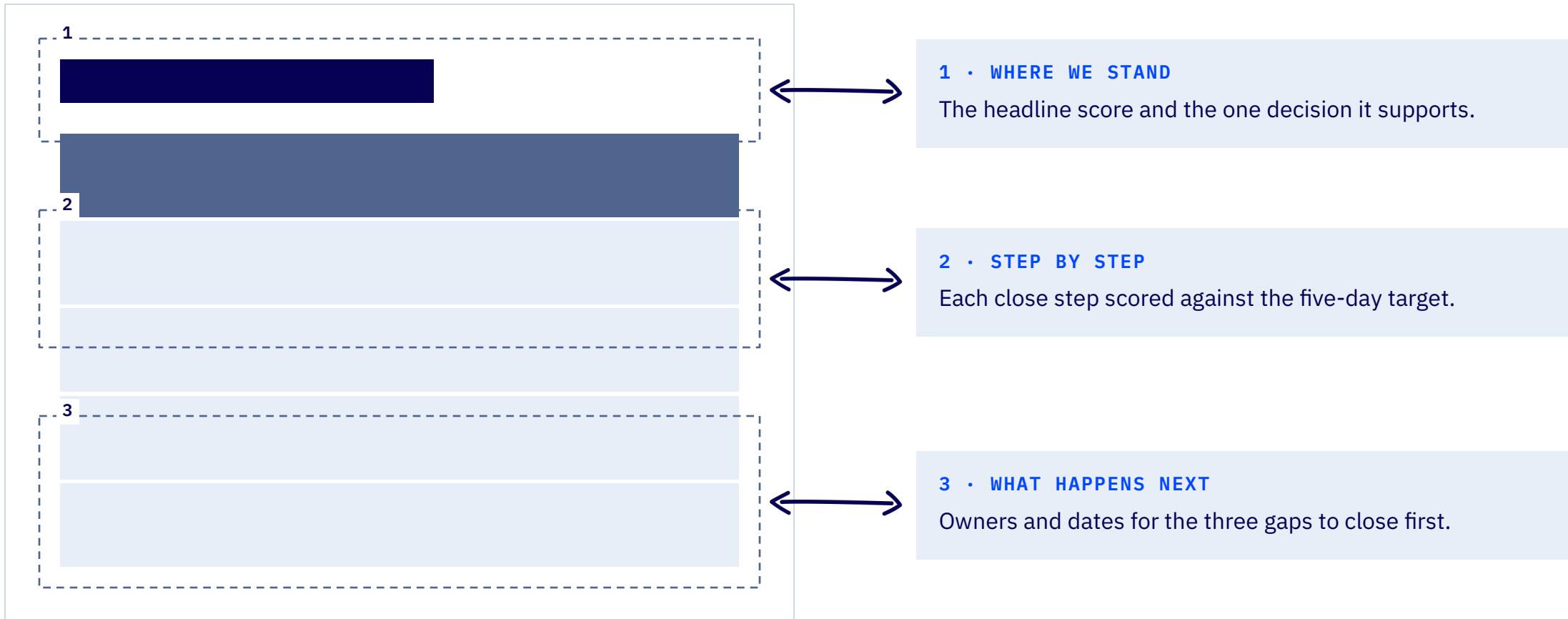
Accelerate coding, testing and migration
Agents draft, people review.

MAINTAIN LEGACY

Cut run cost and reliance on scarce skills
Fewer specialists on call.

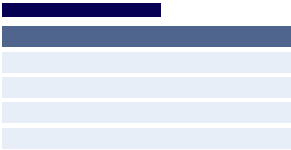
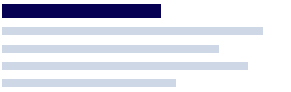
As AI lowers planning, build and run costs, choose by strategic fit, differentiation and ownership.

Each zone of the readiness scorecard has a job



Source: Illustrative page. Final outputs are tailored to Northfield's data and confirmed priorities.

Four deliverables, each built for a named audience

DELIVERABLE	PHASE	FORMAT	AUDIENCE	SAMPLE
Operating baseline	Weeks 1–4	Slide deck	Finance leadership	
Redesign portfolio	Weeks 5–8	Workbook	Steering committee	
Adoption plan	Weeks 9–10	Slide deck	Business unit leads	
KPI scorecard	Weeks 11–12	Dashboard	CFO and controllers	

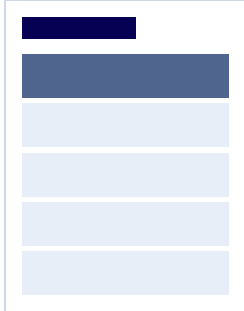
Source: Illustrative samples. Final outputs are tailored to Northfield's data and confirmed priorities.

Sample outputs show the rigor behind Phase 1

WEEK 4

Operating baseline and workflow findings

Workflow baselines, process maps, friction findings and capability gaps, owner-validated.



Readiness scorecard

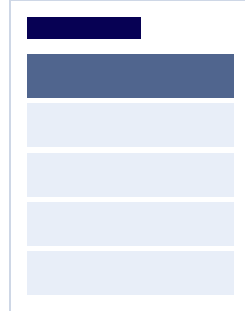


Process summary

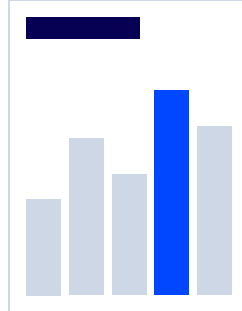
WEEK 8

Prioritized redesign portfolio

Ranked redesign opportunities, future-state workflows and value cases sized with Finance.



Opportunity portfolio

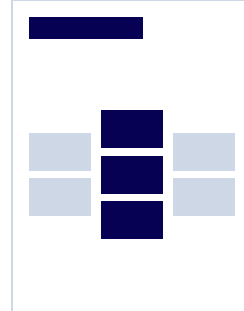


Feasibility map

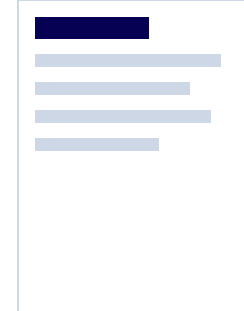
WEEK 10

Mobilization and adoption plan

Roadmap, integration readiness, controls, stage gates and adoption actions for each business unit.



Phased roadmap

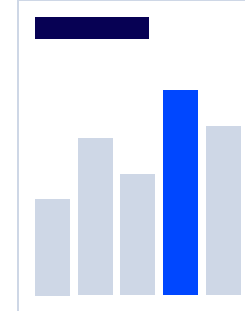


Adoption plan

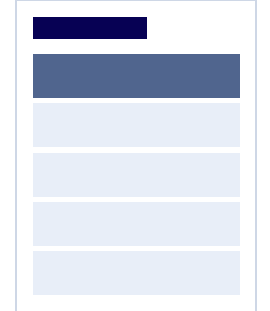
WEEK 12

Value realization and governance

Value tracking, KPI scorecards, leader routines and a signed handoff to the Northfield team.



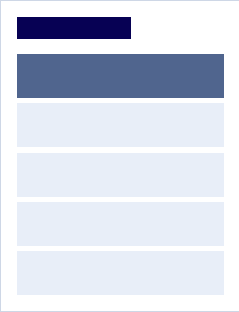
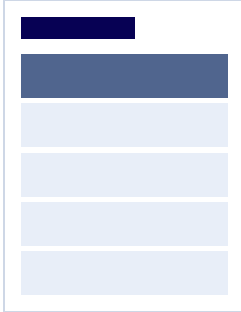
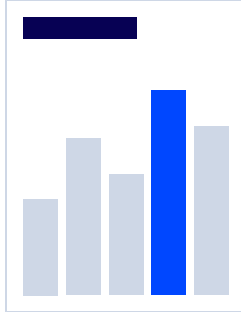
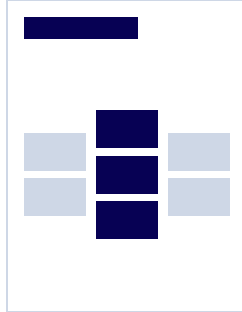
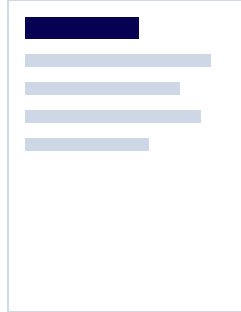
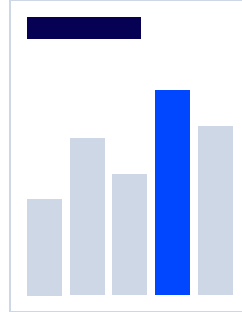
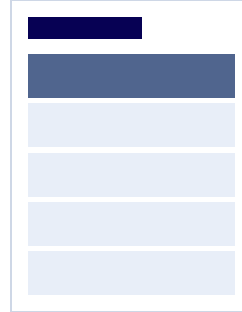
Value case



KPI scorecard

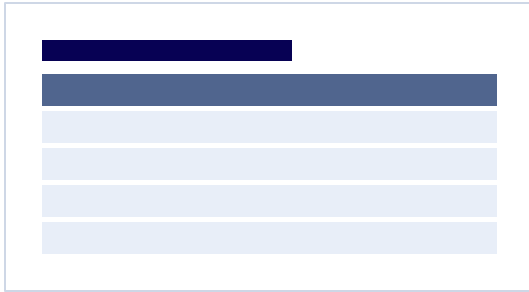
Source: Illustrative examples. Final outputs are tailored to Northfield's data, confirmed priorities and agreed level of detail.

Sample outputs show the rigor behind Phase 1

WEEK 4		WEEK 8		WEEK 10		WEEK 12	
Operating baseline and workflow findings		Prioritized redesign portfolio		Mobilization and adoption plan		Value realization and governance	
Workflow baselines, process maps, friction findings and capability gaps, owner-validated.		Ranked redesign opportunities, future-state workflows and value cases sized with Finance.		Roadmap, integration readiness, controls, stage gates and adoption actions for each business unit.		Value tracking, KPI scorecards, leader routines and a signed handoff to the Northfield team.	
							
Readiness scorecard	Process summary	Opportunity portfolio	Feasibility map	Phased roadmap	Adoption plan	Value case	KPI scorecard

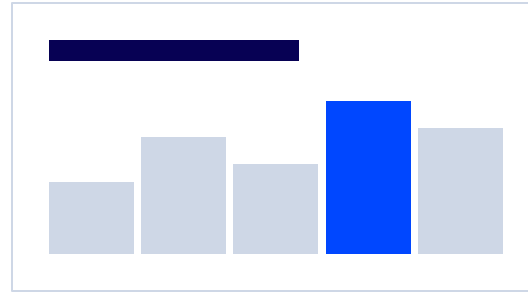
Source: Illustrative examples. Final outputs are tailored to Northfield's data, confirmed priorities and agreed level of detail.

The Phase 1 readout, walked through page by page



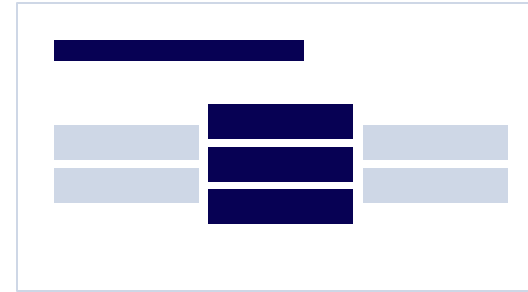
1 Baseline

Today's close, step by step, with the days each one takes.



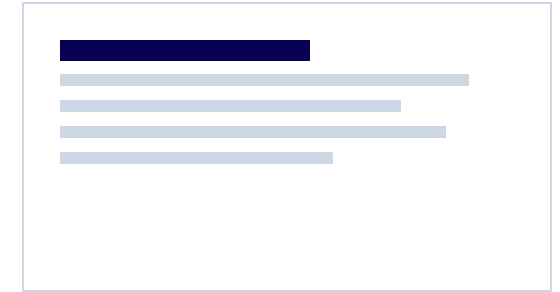
2 Findings

Where time is lost, ranked by days recovered.



3 Redesign

The future-state flow, with owners at each handoff.



4 Roadmap

Waves, stage gates and the first 90 days.

Every page also ships as an editable slide, so Northfield can reuse it in committee packs.

Source: Illustrative pages. Final outputs are tailored to Northfield's data and confirmed priorities.

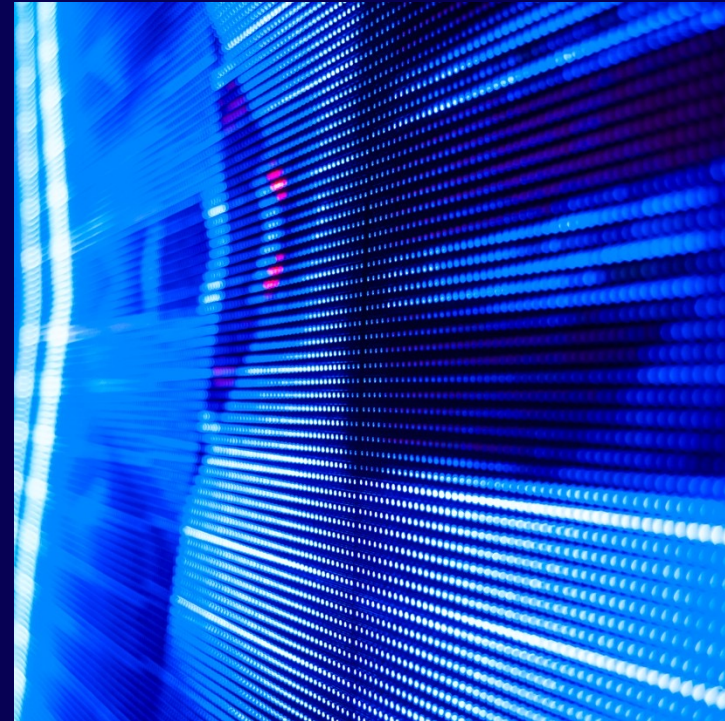


03

How we get there

SECTION 05 • TEAM

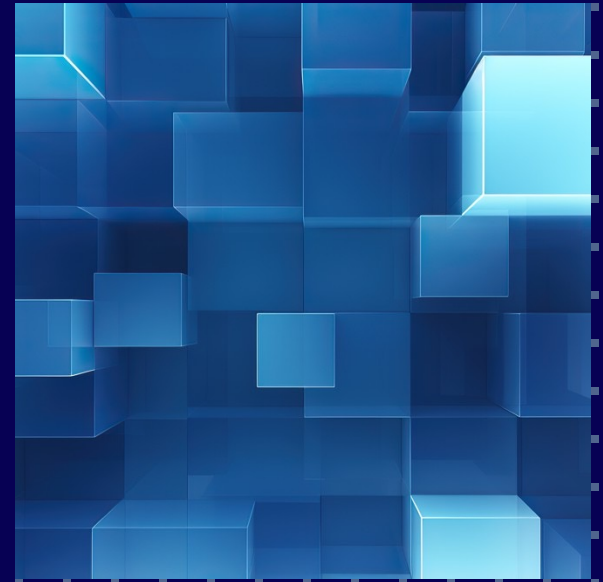
Who you will work with



04

SECTION 04 • BUSINESS CASE

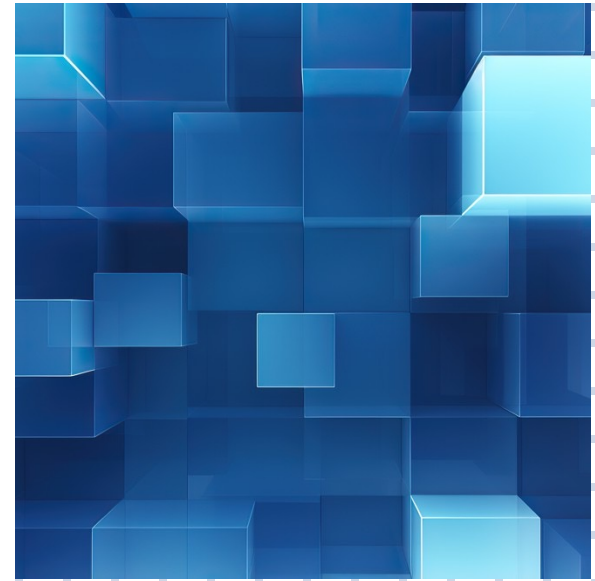
What it is worth



04

BUSINESS CASE

What it is **worth**



SECTION

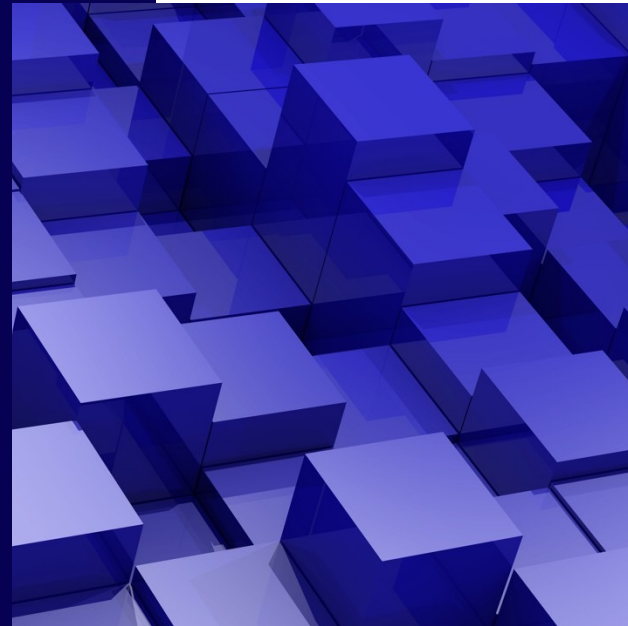
01

Introductions and objectives



02

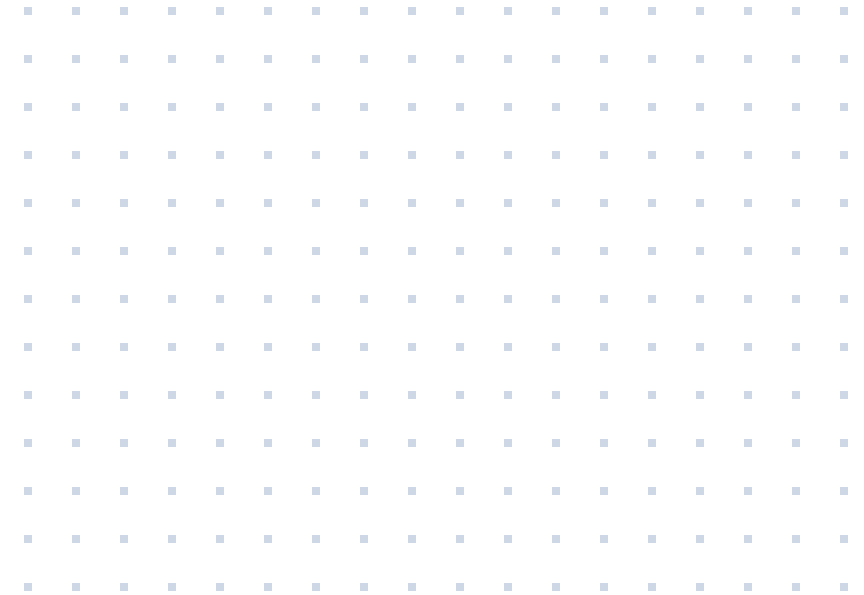
Our perspective



03

SECTION 03 OF 05

Our approach



01

02

03 APPROACH

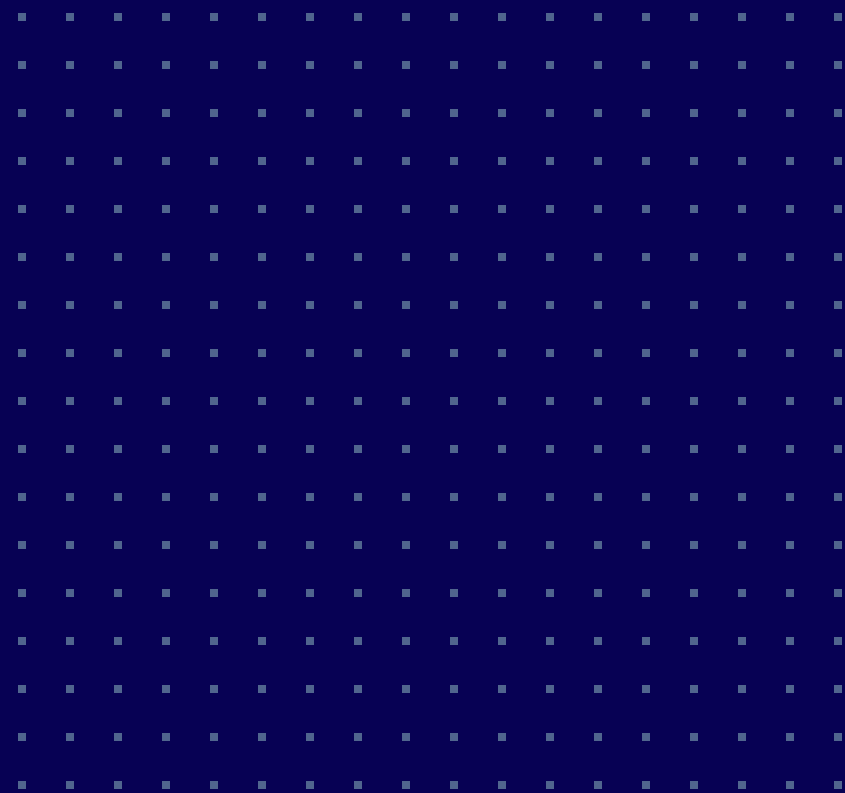
04

05

03

SECTION 03 OF 05

Our approach



01 CONTEXT

02 ECONOMICS

03 APPROACH

04 PROOF

05 NEXT STEPS

03

SECTION 03 OF 05

Our approach



01 CONTEXT

02 ECONOMICS

03 APPROACH

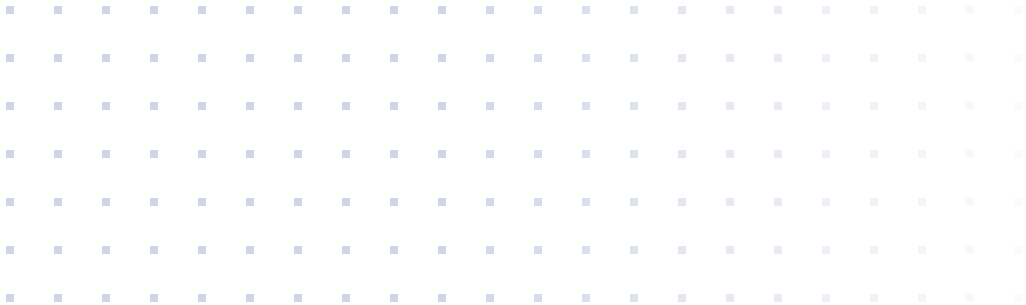
04 PROOF

05 NEXT STEPS

03

SECTION 03 OF 05

Our approach



01 Context

02 Economics

03 Approach

04 Proof

05 Next steps

03

SECTION 03 OF 05

Our approach



01 CONTEXT

02 ECONOMICS

03 APPROACH

04 PROOF

05 NEXT STEPS

03

SECTION 03 OF 05

Our approach

01 CONTEXT

02 ECONOMICS

03 APPROACH

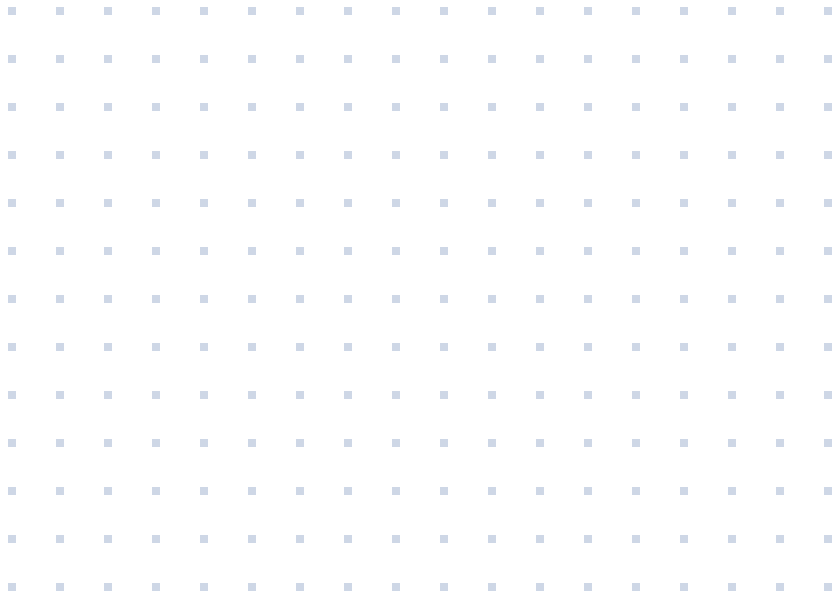
04 PROOF

05 NEXT STEPS

03

SECTION 03 OF 05

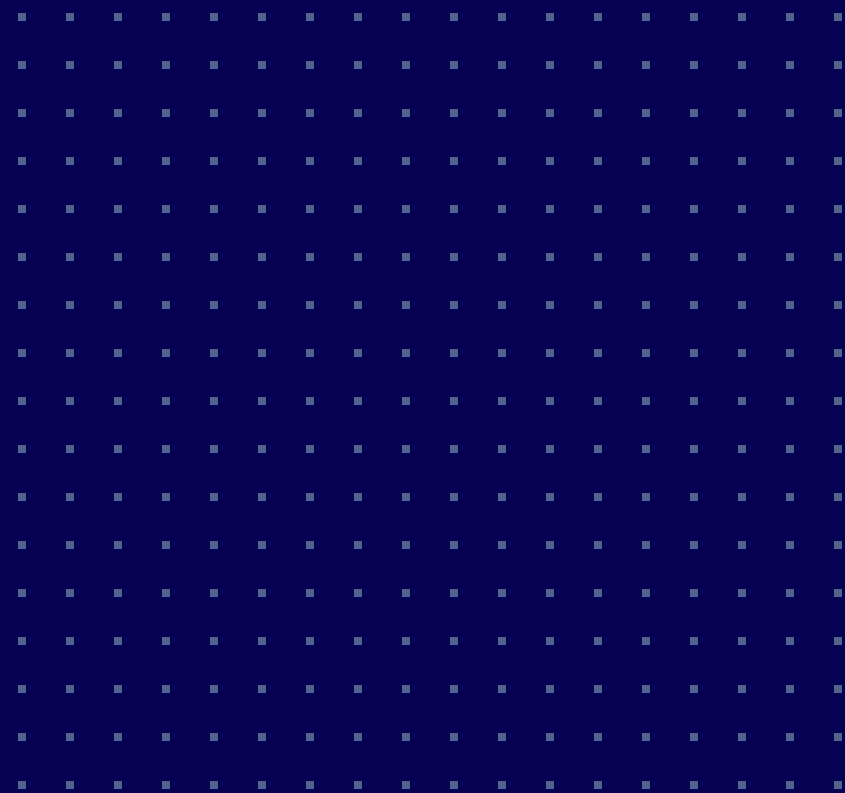
Our approach



03

SECTION 03 OF 05

Our approach



01 CONTEXT

02 ECONOMICS

03 APPROACH

04 PROOF

05 NEXT STEPS

A fixed base fee with four optional add-ons

BASE FEE

\$720K

fixed fee, 14 weeks

Diagnose and design

\$504K

Roadmap and handover

\$216K

TERMS

▪ Invoiced on phase acceptance

▪ Expenses capped at 8% of fees

OPTIONAL ADD-ONS, EACH A FIXED FEE

ADD-ON	WHAT IT ADDS	FEE
Wave 2 entity rollout	Five more entities	\$144K
Training academy	Role-based courses	\$96K
Hypercare, 90 days	Post-go-live support	\$72K
Benefits tracking	Quarterly value review	\$48K

Add-ons can be elected until design sign-off at the prices shown.

A fixed fee of \$720K, set phase by phase

PHASE 1

WEEKS 1-4

Diagnose

- Close calendar mapping
- Delay ranking by days lost
- Diagnostic readout

\$216K

30% of the fixed fee

PHASE 2

WEEKS 5-10

Design

- Rules and routing design
- Operating model and business case
- Design sign-off

\$288K

40% of the fixed fee

PHASE 3

WEEKS 11-14

Roadmap

- Wave plan and readiness
- Final readout
- Handover to Northfield

\$216K

30% of the fixed fee

Total fixed fee \$720K, invoiced as each phase is accepted

COMMERCIALS

\$720K rests on four assumptions

PHASE	TIMING	SHARE	FEE
Diagnose	Weeks 1–4	30%	\$216K
Design	Weeks 5–10	40%	\$288K
Roadmap	Weeks 11–14	30%	\$216K
Total fixed fee		100%	\$720K

HOW IT IS PAID

- Invoiced as each phase is accepted, net 30
- Expenses at cost, capped at 8% of fees

ASSUMPTIONS

14 business units in scope

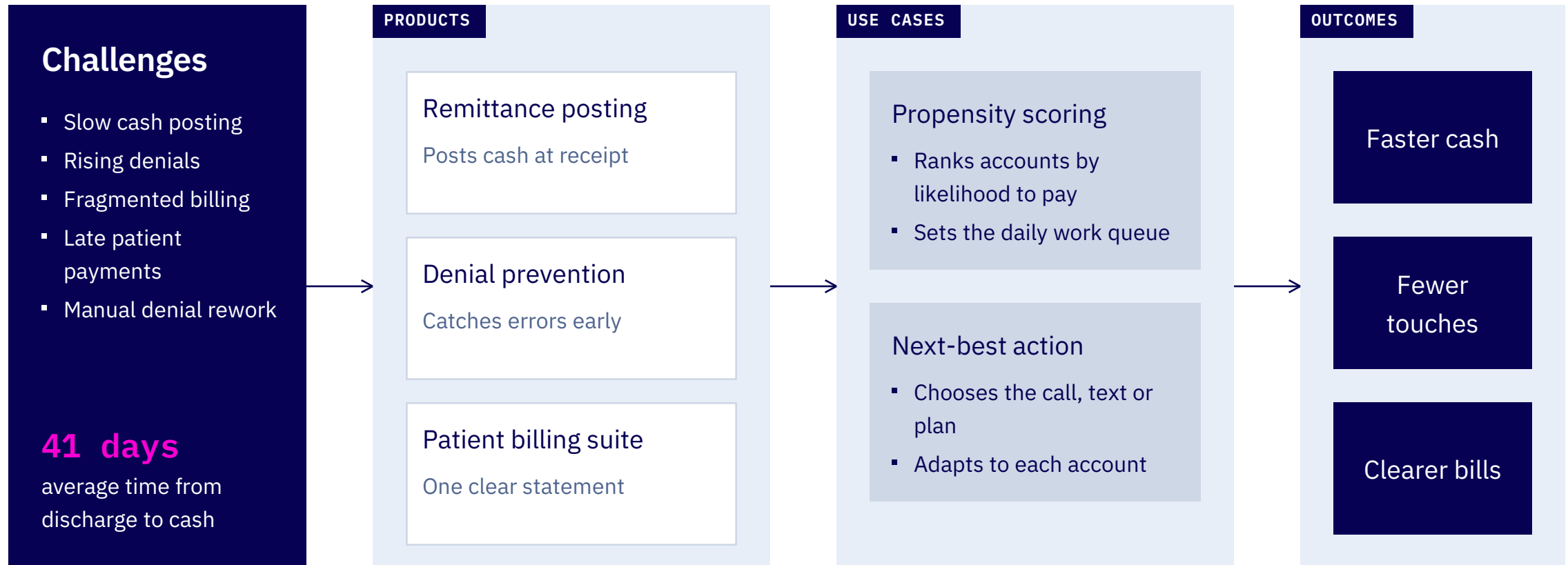
Data access in week 1

Decisions within 5 business days

One ERP instance, no new modules

Changes follow the change process.

From revenue-cycle challenges to measurable outcomes



Start from the challenge, not the tool: each product pays off when a use case makes it a repeatable action.

Frameworks are emerging for AI-powered delivery

FRAMEWORK LAYER	PUBLIC EXAMPLES	WHAT THEY EMPHASIZE	MATURITY
01 Product outcomes Why we build	Outcome-based operating model; continuous discovery	Frames outcomes with customer evidence.	Mainstream
02 AI product quality Whether to trust it	Human-AI interaction guidelines; trust patterns	Trust, feedback and failure recovery.	Growing
03 Product methods How we discover	Reusable discovery skills; context as code	Repeatable methods and shared product context.	Early
04 Specs and handoff How we hand off	Spec-driven development kits; change proposals	Reviewable specs, plans, tasks and changes.	Growing
05 Delivery flow How teams ship	Cross-role planning and QA playbooks	Cross-role planning, review and QA.	Early
Workshop application	Use today's learnings to upgrade your delivery operating system		

Move review to the start of the cycle

TODAY

Review happens after the work

- Entries reviewed on day 9
- Errors found late
- Controllers act as the last line of defense



WITH THE NEW MODEL

Review is built into the work

- Rules check entries as they post
- Exceptions routed the same day
- Controllers review trends

OPERATING MODEL

Four shifts take the close from 12 days to 5

TODAY

Entries reviewed on day 9

Intercompany matched by email

Controllers review every entry

Close takes 12 days



WITH THE NEW MODEL

Rules check entries as they post

92% AUTO-CHECKED

Matched automatically, daily

11 ENTITIES

Controllers review exceptions

-70% REVIEW TIME

Close takes 5 days

-7 DAYS

Source: West Monroe close assessment, 14 business units, Aug 2026.

Custom software scales what makes you unique

WHY BUILD

Make software conform to the business

- 01 Unique processes**
Encode the workflows that set you apart.
- 02 Familiar language**
Keep the vocabulary your people use.
- 03 Selective change**
Modernize only what needs to change.

CHANGE ON YOUR TERMS

Less translation, less retraining.



ADVANTAGE 01

Scale what makes you unique

Turn distinctive processes into repeatable operations.

ADVANTAGE 02

Modernize without upheaval

Keep familiar roles and vocabulary.

Build when differentiation matters and you want to choose where the business changes.

Redesigned registration doubled online completions

Before: Jan 2026



12% of invitees completed a visit.

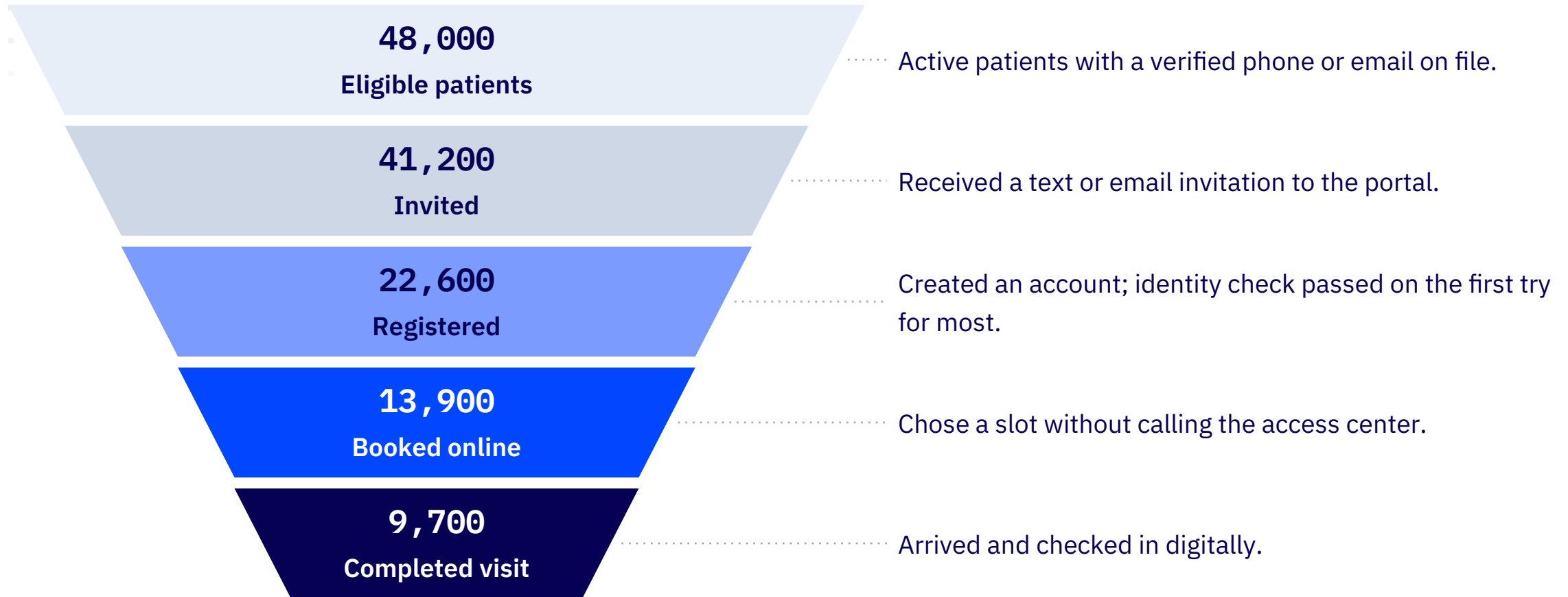
After: Sep 2026



24% of invitees completed a visit.

Source: Sample figures for layout only; replace with current firm data before use.

Only one in five eligible patients completes a visit online



Source: Northfield Health System digital front door program, Sep 2026.

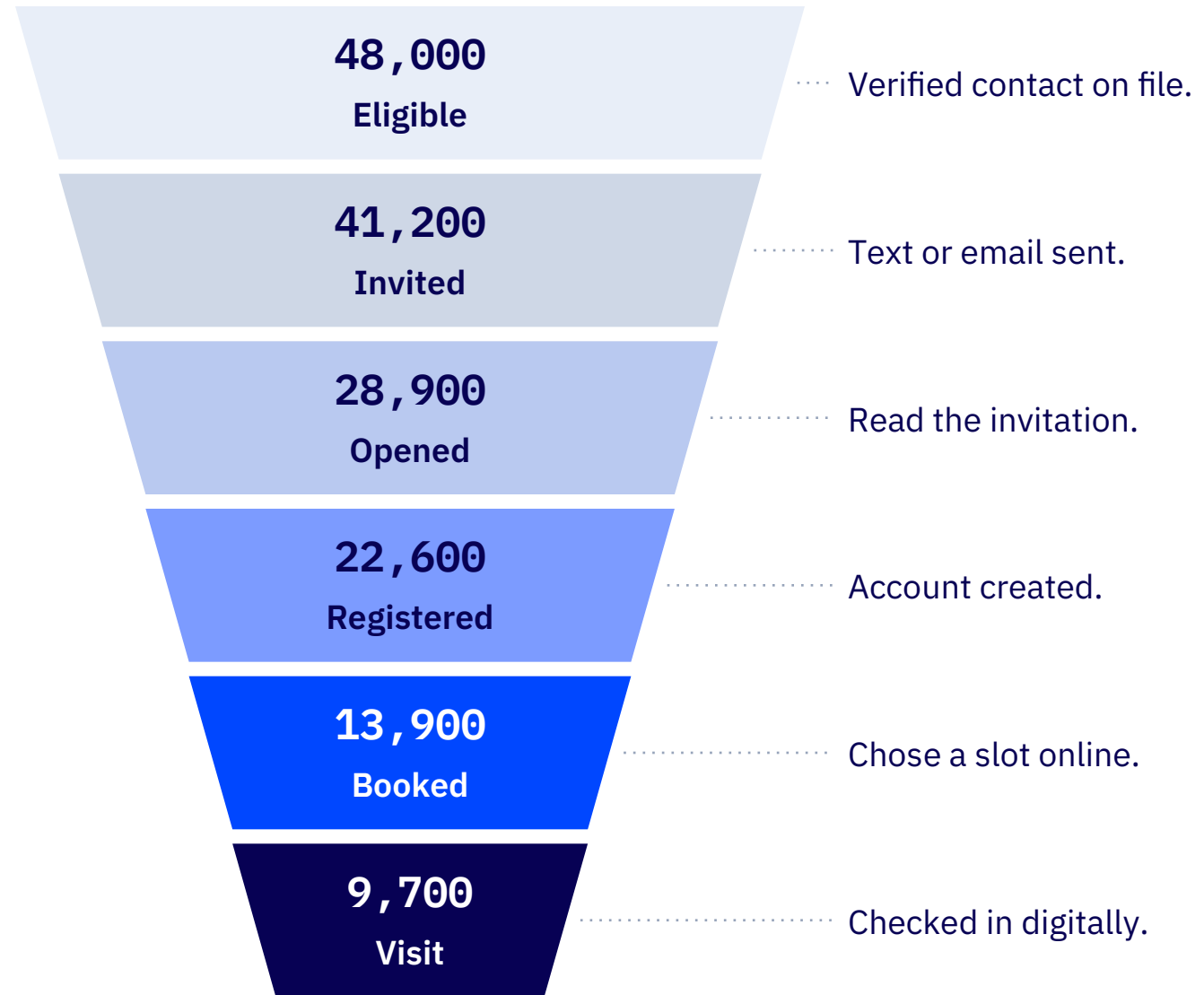
DIGITAL FRONT DOOR

One in five eligible patients completes a visit

Each stage removes patients for a different reason, so each needs its own fix.

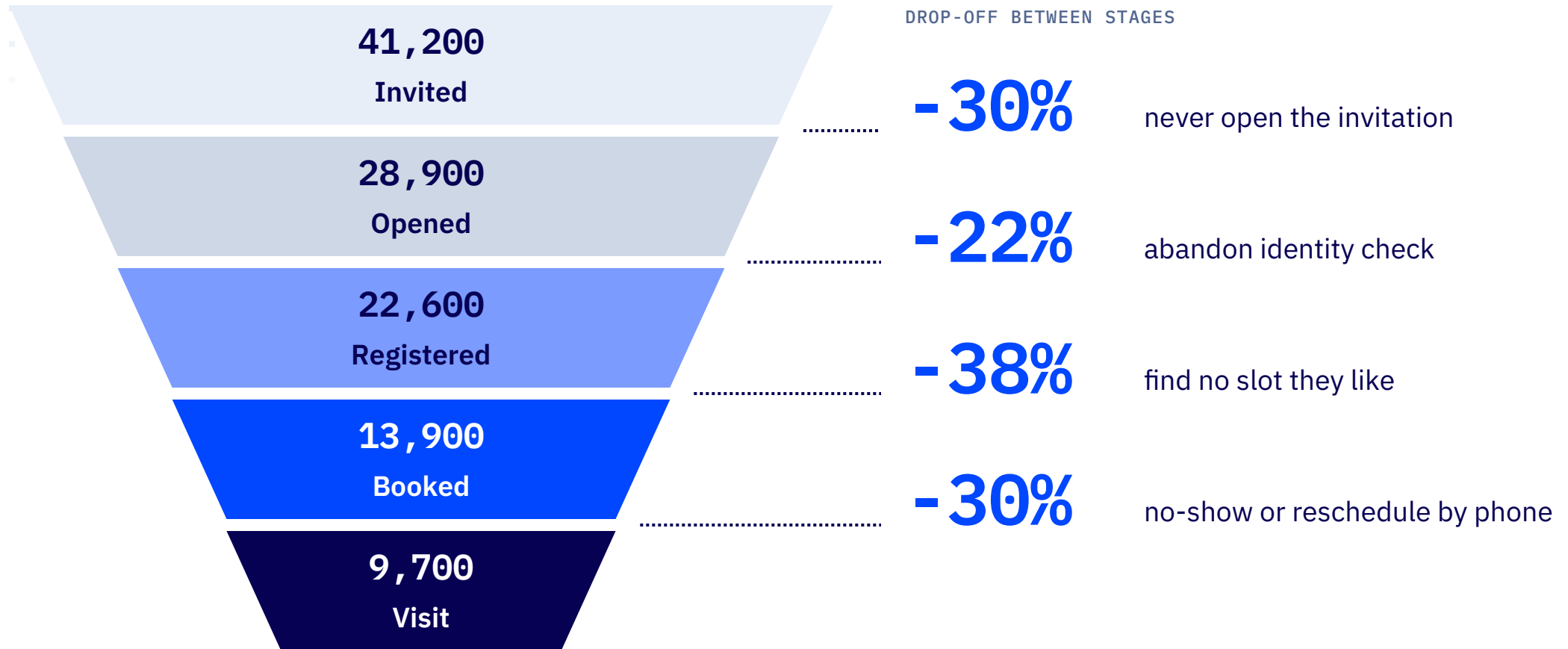
20%

of eligible patients complete a visit online, against a 35% target for FY27.



Source: Sample figures for layout only.

The biggest leak is at slot selection, not sign-up



Source: Sample figures for layout only; replace with current firm data before use.

Three insights explain where patients drop away



Insight 1: Registration loses 45% of invitees

Identity checks fail on the first try for one in five.

Insight 2: Booking is a phone habit

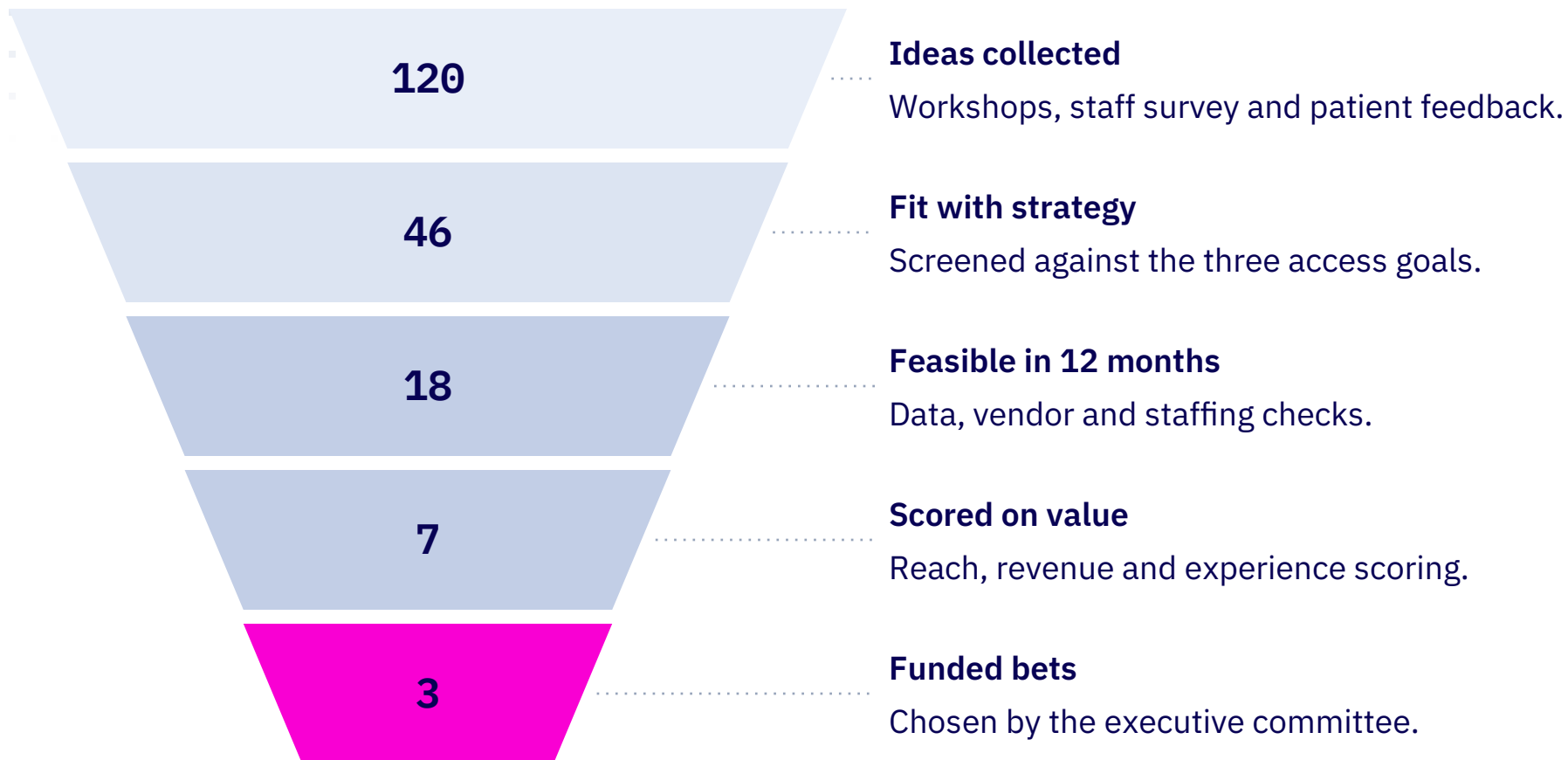
Patients over 65 book 3x more by phone than online.

Insight 3: Visits convert once booked

70% of online bookings are completed, above phone.

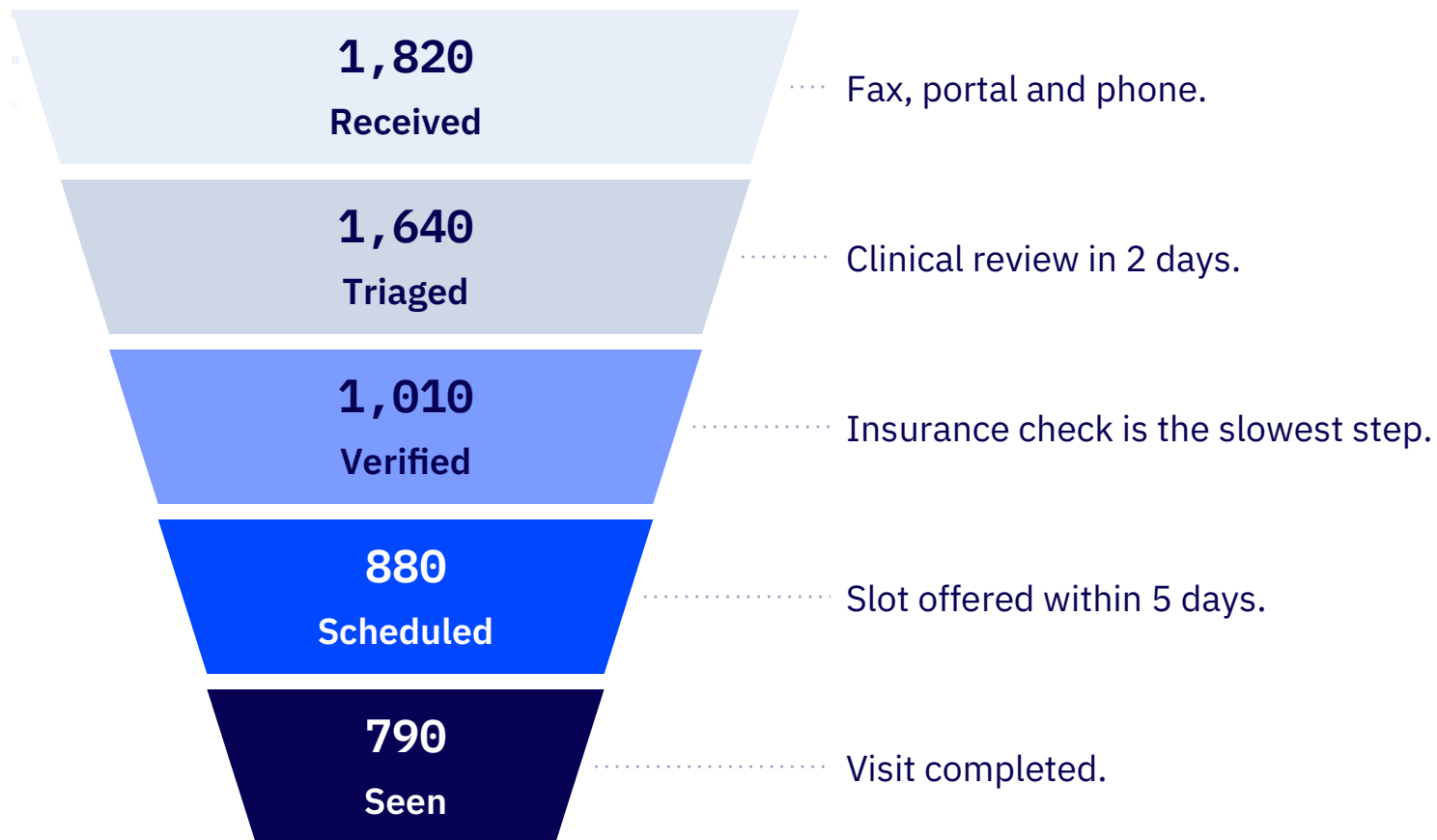
Source: Sample figures for layout only; replace with current firm data before use.

A long list of ideas narrows to three funded bets



Source: Northfield Health System digital front door program, Sep 2026.

Referrals stall at insurance verification



Source: Sample figures for layout only; replace with current firm data before use.

INTAKE THIS QUARTER

43%

of referrals reach a visit.

9 days

median from receipt to scheduled slot.

38%

of verification losses are missing member IDs.

Select one opportunity to anchor the day

GOAL

Choose one participant-owned, full-stack opportunity that is valuable enough for Product and feasible enough for Engineering to advance today.

1 opportunity by 10:00

STEPS TO GET THERE

15 MIN

01 Frame the candidates

Product shares up to three candidates with evidence.

Output: a shortlist with evidence.

Facilitator: clarify the decision criteria and keep the group out of solution mode.

25 MIN

02 Stress-test together

Engineering tests technical meaning, touchpoints, constraints and a path.

Output: constraints and a path for each.

Facilitator: compare value, feasibility and full-stack depth.

20 MIN

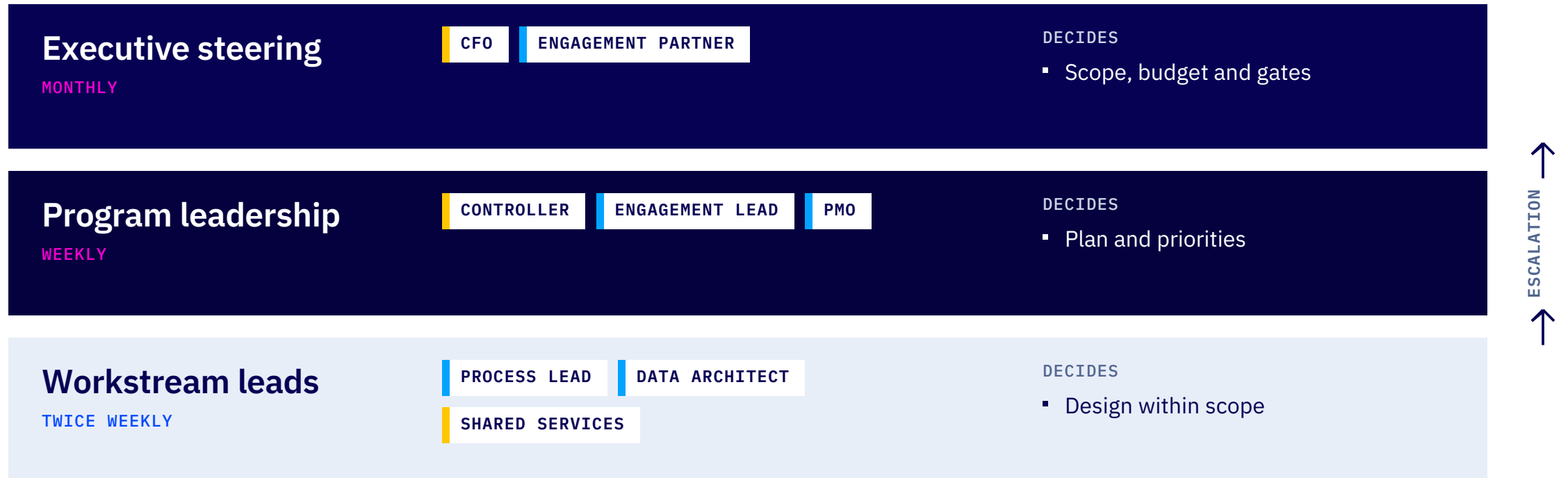
03 Decide and document

The group picks one opportunity and records the rationale.

Output: one opportunity and its rationale.

Facilitator: resolve or assign open questions, then confirm the gate.

Decisions stay close to the work, with a clear path up



■ West Monroe ■ Northfield

How to use this deck

NARRATIVE

This deck is a library, not a story told start to finish. Each section answers one buyer question for Northfield Health System, from why change now to what happens next. Pull only the slides that fit the buyer and the moment.

HOW IT IS ORGANIZED

01 Context

Why the buyer acts now

02 Economics

What modernization costs and returns

03 Approach

How the work gets delivered

04 Proof

Results from comparable programs

05 Next steps

Decisions and first steps

HOW TO USE IT

1. Start with the buyer and the reason to act.
2. Pick the sections that match the conversation.
3. Choose depth: one slide, or the full set.
4. Close with proof and one clear next step.

INTERNAL USE ONLY: REMOVE THIS GUIDE AND EVERY SECTION GUIDE BEFORE SHARING THE DECK EXTERNALLY.

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Approach: how the work gets delivered

NARRATIVE OF THIS SECTION

Approved outcomes become working capability in four subphases. Design fixes the target, build and validation prove it with Northfield teams, and cutover moves entities in controlled waves behind leader sign-off.

SLIDE	TITLE	WHAT IT COVERS	WHEN TO USE
31	Approach at a glance	Four subphases take approved outcomes to a live capability	Opening any delivery talk
32	Subphase detail	Activities, outputs and owners for each subphase	Buyer asks for specifics
33	Working model	What each Northfield team does, week by week	Resourcing questions
34	Phased cutover	Entities move in controlled waves, not one event	Risk-averse operations leaders
35	Decision gates	Where leaders sign off before work moves on	Steering committee review
36	Delivery risks	Top risks and how we contain each one	Late-stage diligence
37	Governance cadence	Who meets, how often and what they decide	Sponsor wants oversight

Approach: how to use this section

HOW TO USE THIS DECK

- 1 Start with the buyer and the reason to act.
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HOW TO USE THIS SECTION

SLIDE	TITLE	WHAT IT COVERS	WHEN TO USE
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Four domains set the investment order



Denials, master data and self-scheduling top the list



Eight years of work that this program builds on



2020

Mapped the close across 14 entities and found the day-nine review.

2022

Stood up intercompany matching for 11 entities.

2024

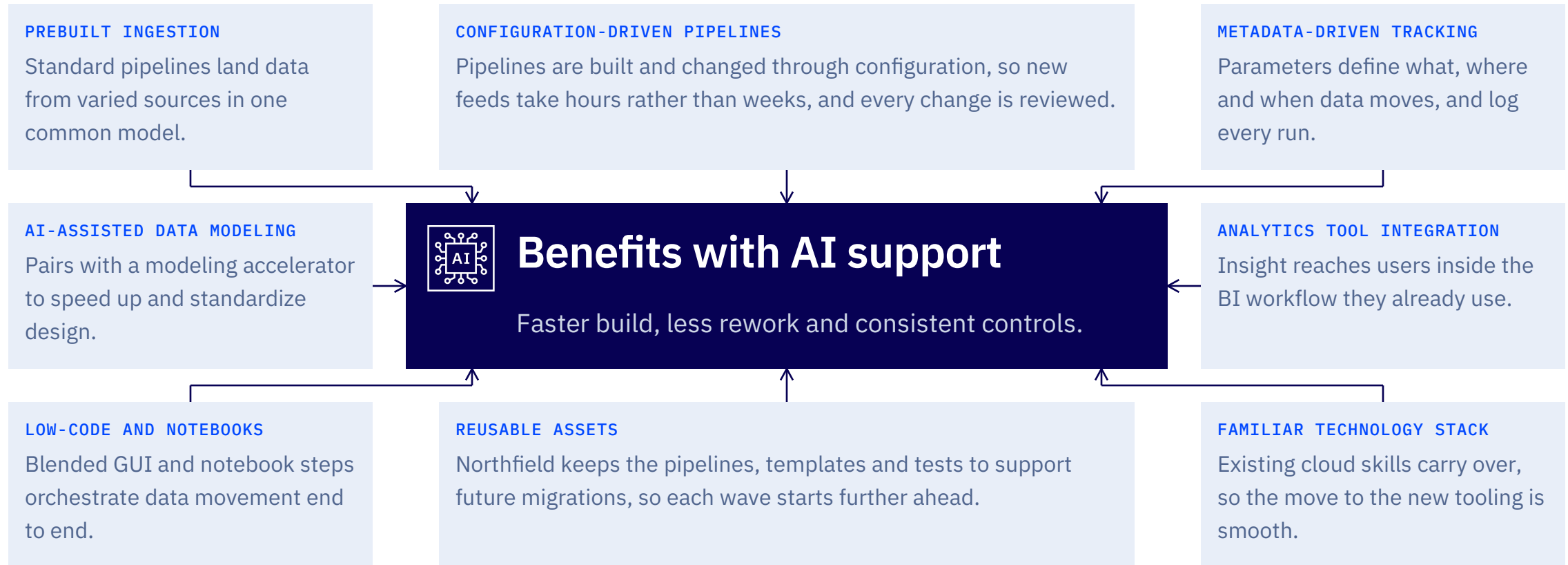
Supported the ledger migration without a missed close.

2026

\$4.2M

run-rate savings in the program we propose

Reusable patterns speed up the platform build



Source: West Monroe engineering patterns, Northfield delivery plan.

Shared services close from one queue

One team, one list of exceptions

Work moves to whoever has capacity, so no entity waits for its own team to catch up. Controllers review the queue each morning.



Shared services hub, wave 1 pilot.



OPERATING MODEL

Shared services close from one queue

One team, one list of exceptions

Work moves to whoever has capacity, so no entity waits for its own team to catch up. Controllers review the queue each morning.

- **Less waiting.** Capacity follows the queue.
- **Faster fixes.** Exceptions are reviewed daily.

Shared services hub, wave 1 pilot.

Give every entity one view of its close



The close cockpit starts every day from the same status

Controllers see open reconciliations, unmatched intercompany balances and review queues for all 14 business units.

- **Fewer chases.** Status is visible without a meeting.
- **Earlier fixes.** Exceptions surface on day one.

Front-desk schedulers are hit hardest on process, technology and role

HIGH IMPACT

01 Process

Walk-in and phone booking merge into one scheduling workflow with new double-booking rules.

Support: Redesign sessions with five clinics.

HIGH IMPACT

02 Technology

Three legacy schedule screens are replaced by a single web platform with new login.

Support: Sandbox practice from week 6.

HIGH IMPACT

03 Role

Schedulers own referral holds and waitlist outreach that used to sit with nurses.

Support: Updated role cards and job aids.

MEDIUM IMPACT

04 Skills

Template building and rules-based overbooking are new skills for the team.

Support: Eight hours of role-based training.

Scale: 142 schedulers, nine sites

Limit: 61 work part time

Source: Northfield Health System change impact assessment, Sep 2026.

STAKEHOLDER GROUP	PROCESS	TECHNOLOGY	ROLE	SKILLS
Front-desk schedulers	High	High	High	High
Call-center agents	High	Medium	High	Medium
Clinic nurses	Medium	Medium	Medium	Medium
Physicians	Medium	High	Low	Low
Imaging technologists	High	Medium	Medium	Low
Practice managers	Medium	High	High	Medium
Revenue cycle staff	Medium	Medium	Medium	Low
IT service desk	Low	High	Medium	High
Patients and families	Medium	Medium	None	None
Lab and pharmacy staff	Low	Medium	Low	Low

None
Low
Medium
High

CHANGE IMPACT

Four groups need targeted support before go-live

- **Heaviest.** Front-desk schedulers change on all four dimensions at once.
- **Technology-led.** Physicians and the service desk feel the tool, not the process.
- **Light.** Patients meet the change only through the new online booking page.

4 of 10

high on three or more dimensions

Source: Northfield impact assessment, Sep 2026.

Schedulers and agents carry the heaviest change load

STAKEHOLDER GROUP	PROCESS	TECHNOLOGY	ROLE	SKILLS
Front-desk schedulers	High	High	High	High
Call-center agents	High	Medium	High	Medium
Clinic nurses	Medium	Medium	Medium	Medium
Physicians	Medium	High	Low	Low
Imaging technologists	High	Medium	Medium	Low
Practice managers	Medium	High	High	Medium
Revenue cycle staff	Medium	Medium	Medium	Low
IT service desk	Low	High	Medium	High
Patients and families	Medium	Medium	None	None

None Low Medium High

Source: Northfield Health System change impact assessment, Sep 2026.

Each month of delay gives up \$217K of value

VALUE FORGONE EACH MONTH

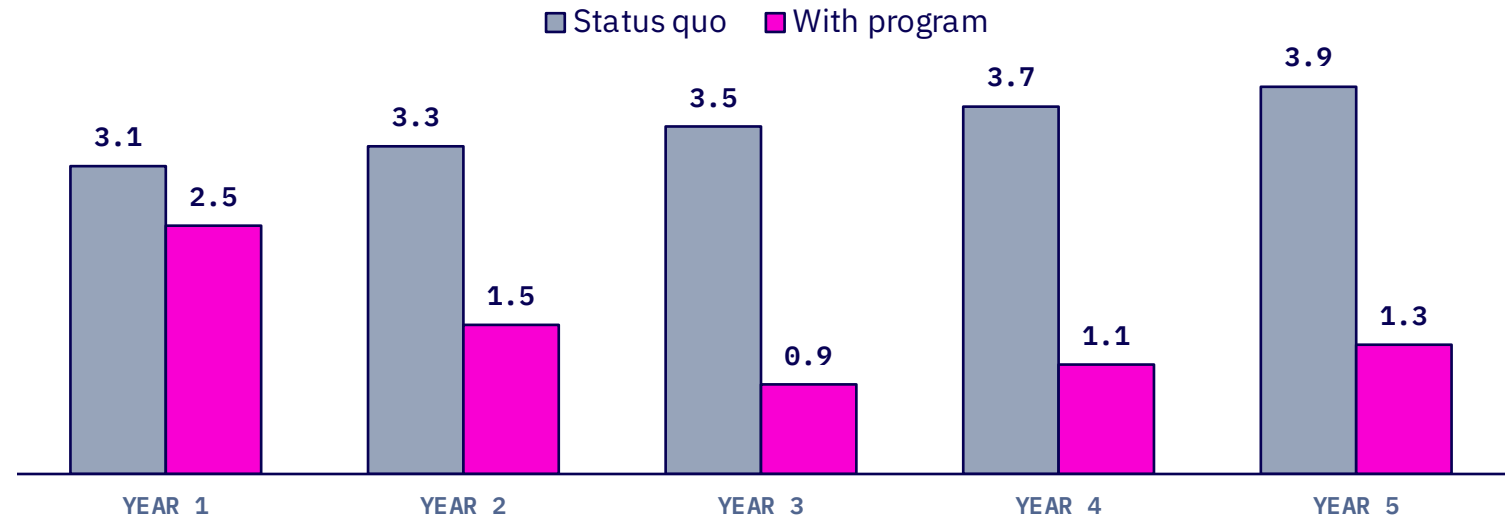
\$217K

\$2.6M a year, divided by 12

Six-month delay costs **\$1.3M**Payback moves to **month 37**A year of delay costs **\$2.6M**

Avoidable cost if nothing changes

\$M A YEAR



- Manual close hours grow 6% a year with volume
- Denied claims keep writing off 9.1% of billed revenue

Source: Sample figures for layout only; replace with current program data before use.

INVESTMENT AND VALUE

Doing nothing costs \$2.6M a year

DELAY OF ONE YEAR

\$2.6M

of value not captured, and payback pushed out a year

\$217K

forgone each month

43

months to payback

The loss is permanent: the ramp starts when the program does.

WHAT THE STATUS QUO COSTS EACH YEAR

Close and reporting rework  **\$1.0M**

Denied claims written off  **\$0.8M**

Off-contract spend  **\$0.4M**

Manual corrections  **\$0.3M**

Open audit findings  **\$0.1M**

Source: Sample figures for layout only; replace with current program data before use.

Intellio accelerators cover every phase and PDLC segment

MODERNIZATION PHASE	IDENTIFY	DEFINE	BUILD	OPTIMIZE
Rationalize Decide what to modernize	Portfolio Analyzer Scans and scores the application estate	Disposition Modeler Models retire, retain, move and rebuild	Target Blueprints Pre-built scheduling and billing patterns	Value Tracker Tracks the business case against benefits
Build Deliver the move	Code Scanner Finds dependencies and hot spots in code	Migration Planner Wave plans and cutover runbooks	Replatform Kit Landing zones, pipelines, test harnesses	Data Mover Mapping, loads and reconciliation
Sustain Run and improve	Estate Monitor Continuous inventory and drift alerts	Service Playbooks Runbooks, service levels, support models	Release Factory Automated build, test and release	FinOps Optimizer Cost, performance and capacity tuning

Source: Sample names for layout only; replace with the current catalog before use.

Twenty-four accelerators by segment and phase

Read down a column to follow one phase through the PDLC, or across a row to see everything available in a segment.

HOW TO USE

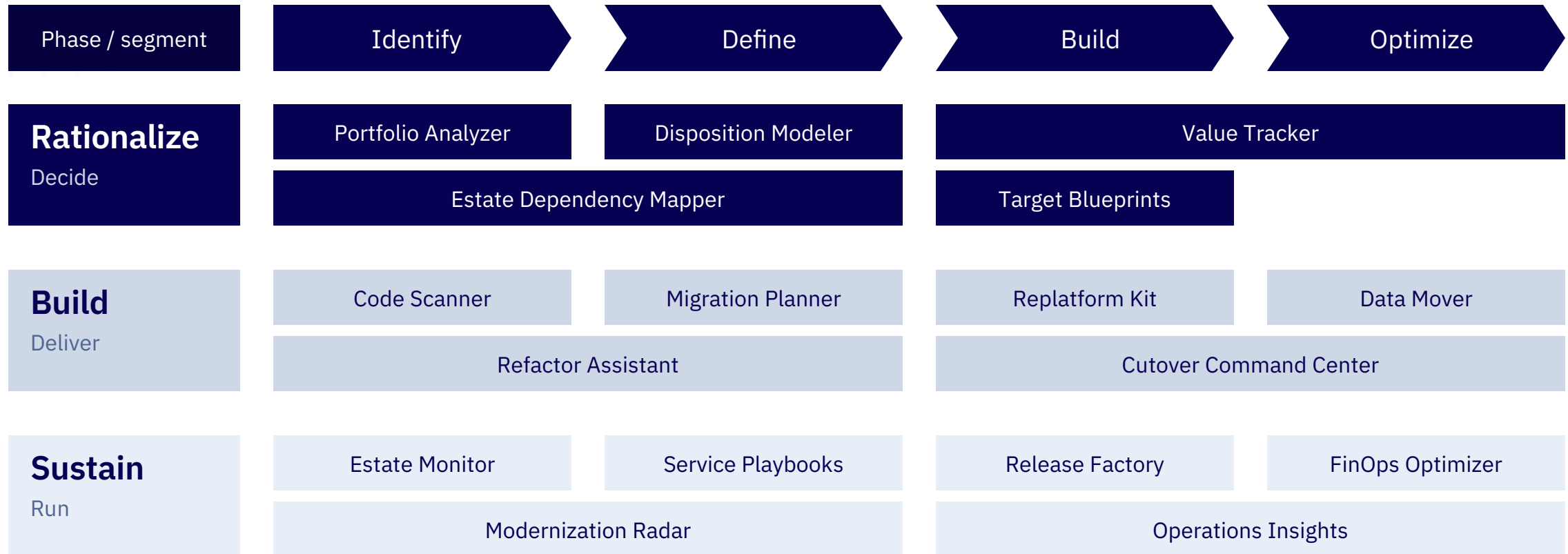
Two accelerators each

- Start with the first in every cell
- Add the second as scope grows
- Both ship with playbooks

Source: Sample names for layout only; replace with the current catalog before use.

	Rationalize	Build	Sustain
Identify	Portfolio Analyzer Dependency Mapper	Code Scanner Interface Profiler	Estate Monitor Drift Alerts
Define	Disposition Modeler Risk Scorer	Migration Planner Cutover Playbooks	Service Playbooks SLO Designer
Build	Target Blueprints Architecture Linter	Replatform Kit Test Harness	Release Factory Runbook Automator
Optimize	Value Tracker Benefit Dashboard	Data Mover Reconciler	FinOps Optimizer Capacity Forecaster

Intellio assets span the PDLC within each phase



Source: Sample names for layout only; replace with the current catalog before use.

Five key stakeholders are still out of reach

STAKEHOLDER	ACCESS STATUS	DAYS OUT	ESCALATION ASK	OPEN QUESTIONS FOR THEM
Chief Medical Officer Clinical ops	Blocked	34	. Sponsor to ask CEO's office to hold 45 minutes	. Who owns documentation standards? What is the physician time ask?
Patient Access Supervisors Patient access	No response	21	. Ask COO to release team for a 60-minute session	. Which tools do registrars trust? Where are errors found?
VP Supply Chain Finance	Rescheduled	12	. Move to a shared Thursday slot, no escalation yet	. What spend sits outside contracts?
Chief Digital Officer IT	Rescheduled	9	. Rescheduled twice; sponsor note pending	. How does this fit the platform roadmap?
Board finance chair Finance	Awaiting date	7	. Route request through the CFO	. What return does the board expect?

Days out counts from the first request. Escalation asks go to the engagement sponsor.

Source: Sample names for layout only; replace with engagement stakeholders before use.

Four stakeholders need a sponsor push

19 days

average wait across the four

BLOCKED • 34 DAYS

Chief Medical Officer · Clinical ops

- Ask: CEO office holds 45 minutes
- Open: who owns documentation standards

NO RESPONSE • 21 DAYS

Patient Access Supervisors

- Ask: COO releases team for 60 minutes
- Open: where registration errors are found

RISK IF UNRESOLVED

2

findings stay unvalidated at the readout on Nov 3.

RESCHEDULED • 12 DAYS

VP Supply Chain · Finance

- Ask: shared Thursday slot
- Open: spend outside contracts

RESCHEDULED • 9 DAYS

Chief Digital Officer · IT

- Ask: sponsor note on priority
- Open: platform roadmap fit

ASKS THIS WEEK

- CEO office: 45 minutes for the CMO
- COO: release registrars for 60 minutes
- CFO: route the board chair request

Source: Sample names for layout only; replace with engagement stakeholders before use.

Three stakeholders have waited more than ten days

STAKEHOLDER	STATUS	DAYS	REASON
Chief Medical Officer Clinical ops	Blocked	34	Calendar blocked by assistant
Patient Access Supervisors Patient access	No response	21	No reply to three emails
VP Supply Chain Finance	Rescheduled	12	Rescheduled twice
Chief Digital Officer IT	Rescheduled	9	Rescheduled; sponsor note due
Board finance chair Finance	Awaiting date	7	Awaiting CFO routing
Dir., Revenue Integrity Revenue cycle	Awaiting date	5	Rescheduled once

Source: Sample names for layout only; replace with engagement stakeholders before use.

3

sponsor actions needed this week

- CEO office to hold 45 minutes for the CMO
- COO to release registrars for 60 minutes
- CFO to route the board chair request

Until then: clinical and front-end findings rest on documents only.

Compliance and IT are the thinnest in coverage

DEPARTMENT	INTERVIEWS	DENIALS	REGISTRATION	CLOSE	INTEGRATION	CONTROLS	STAFFING
Revenue cycle	9	In depth	Covered	Touched	Discussed	Discussed	Covered
Patient access	7	Covered	In depth	Not covered	Discussed	Touched	Covered
IT	6	Touched	Touched	Not covered	In depth	Covered	Discussed
Finance	6	Covered	Touched	In depth	Discussed	Discussed	Discussed
Clinical ops	7	Discussed	Discussed	Not covered	Discussed	Discussed	In depth
Compliance	3	Discussed	Touched	Touched	Touched	In depth	Touched
HIM	4	Covered	Touched	Not covered	Discussed	Covered	Covered

Not covered Touched Discussed Covered In depth

Source: Sample names for layout only; replace with engagement stakeholders before use.

Interview log: weeks 1 to 3, interviews 1 to 11

PERSON	ROLE / TITLE	DEPARTMENT	MET	FORMAT	REQUEST STATUS	INTERVIEW
J. Alvarez	VP Revenue Cycle	Revenue cycle	Aug 4	ONSITE	Received	#1 of 42
M. Chen	Dir., Patient Access	Patient access	Aug 5	ONSITE	Received	#2 of 42
E. Haynes	Mgr, Denials	Revenue cycle	Aug 6	PHONE	Received	#3 of 42
R. Okafor	CIO	IT	Aug 7	VIDEO	Received	#4 of 42
S. Patel	Dir., Enterprise Apps	IT	Aug 7	VIDEO	Pending	#5 of 42
L. Brandt	CFO	Finance	Aug 11	ONSITE	Received	#6 of 42
D. Moreau	Controller	Finance	Aug 12	ONSITE	Received	#7 of 42
A. Nguyen	CMO	Clinical ops	Aug 13	VIDEO	Blocked	#8 of 42
T. Wallace	VP Clinical Operations	Clinical ops	Aug 14	ONSITE	Pending	#9 of 42
K. Ito	Chief Compliance Officer	Compliance	Aug 18	VIDEO	Received	#10 of 42
P. Silva	Dir., HIM	HIM	Aug 19	ONSITE	Pending	#11 of 42

Page 1 of 2 · continued on the next page

Source: Sample names for layout only; replace with engagement stakeholders before use.

Interview log: weeks 3 to 6, interviews 12 to 22

INTERVIEW LOG • CONTINUED

PERSON	ROLE / TITLE	DEPARTMENT	MET	FORMAT	REQUEST STATUS	INTERVIEW
G. Larsen	Coding Manager	HIM	Aug 20	ONSITE	Received	#12 of 42
C. Dubois	Privacy Officer	Compliance	Aug 21	VIDEO	Received	#13 of 42
N. Rahman	Dir., Financial Planning	Finance	Aug 25	ONSITE	Received	#14 of 42
B. Kowalski	Patient Access Supervisor	Patient access	Aug 26	ONSITE	Overdue	#15 of 42
H. Mensah	Chief Nursing Officer	Clinical ops	Aug 27	ONSITE	Pending	#16 of 42
F. Osei	Dir., Supply Chain	Finance	Sep 2	VIDEO	Received	#17 of 42
V. Ruiz	Mgr, Cash Posting	Revenue cycle	Sep 3	PHONE	Received	#18 of 42
I. Novak	Dir., Quality	Clinical ops	Sep 4	VIDEO	Pending	#19 of 42
Q. Adeyemi	Service Desk Lead	IT	Sep 8	VIDEO	Received	#20 of 42
U. Lindgren	Dir., Scheduling	Patient access	Sep 9	ONSITE	Received	#21 of 42
Z. Barrett	Mgr, Credentialing	Compliance	Sep 10	PHONE	Not yet asked	#22 of 42

Source: Sample names for layout only; replace with engagement stakeholders before use.

42 interviews over ten weeks cover seven departments

42

interviews held

10

weeks of fieldwork

88%

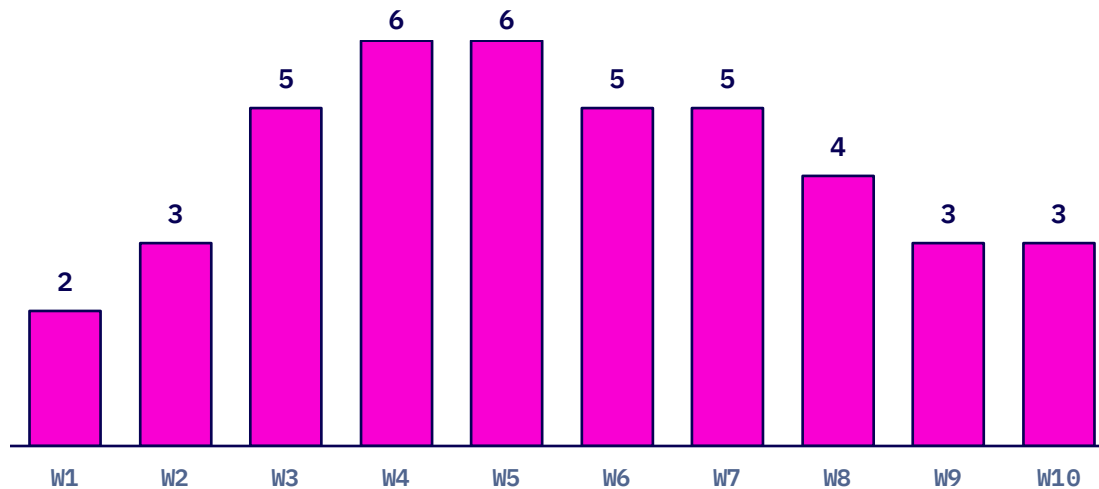
of planned stakeholders met

6

still to schedule

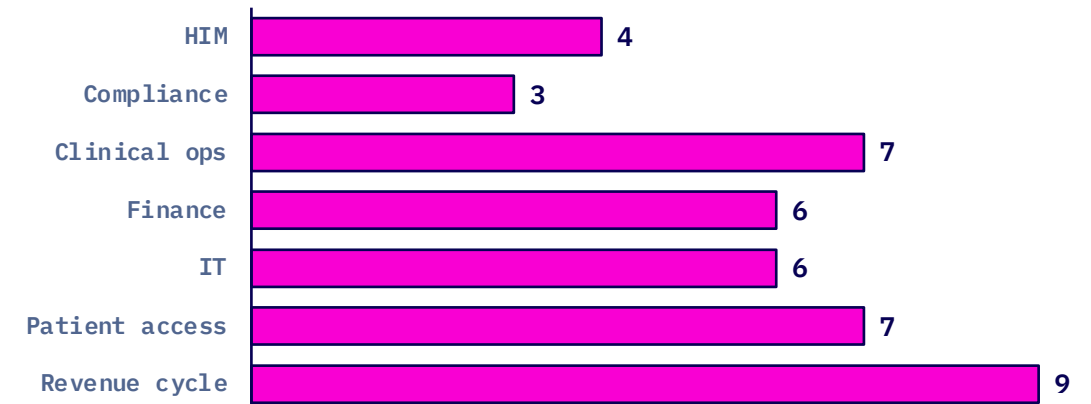
Interviews by week

COUNT



Interviews by department

COUNT



Source: Sample names for layout only; replace with engagement stakeholders before use.

Revenue cycle sees denials start at authorization

STAKEHOLDER	DEPARTMENT AND DATE	FORMAT	INTERVIEWERS
J. Alvarez	Revenue cycle	Onsite, Main campus	K. Ortiz, W. Lee
VP Revenue Cycle	Sep 8, 60 minutes		

TOPIC 1

Denials

- Authorization gaps cause 40% of denials
- Appeals take 45 days

“

We find it at billing, three weeks late.

VP REVENUE CYCLE

Source: Sample names for layout only; replace with engagement stakeholders before use.

TOPIC 2

Registration

- Front-desk errors found at billing
- No eligibility check at check-in

PAIN AND OPPORTUNITY

- Pain: manual rework
- Pain: no shared view
- Idea: one work queue

TOPIC 3

Tooling

- Three work queues, no single view
- Reports rebuilt by hand each month

FOLLOW-UPS AND QUESTIONS

- Denial extract: received
- Appeal staffing: pending Oct 6
- Open: who owns payer contracts?

Five findings from the revenue cycle interview



J. Alvarez
VP Revenue Cycle

DEPARTMENT AND DATE
Revenue cycle · Sep 8

INTERVIEWERS
K. Ortiz, W. Lee

“

We find it at billing,
three weeks late.

J. ALVAREZ

- 01** **Denials start at authorization**
40% of denials trace to missing authorizations. So what: fix intake first.
- 02** **Errors surface at billing**
Front-desk errors are found three weeks late. So what: add check-in checks.
- 03** **Three queues, no shared view**
Teams work denials in separate queues. So what: build one queue by payer.
- 04** **Appeals average 45 days**
Two analysts hold the appeal know-how. So what: cross-train and staff the tail.
- 05** **Reports are hand-built**
Monthly reports restart in spreadsheets. So what: automate the denial extract.

FOLLOW-UP	STATUS
Denial extract	■ Received
Appeal staffing	■ Pending
Contract owner	□ Open

Source: Sample names for layout only; replace with engagement stakeholders before use.

J. Alvarez interview: six findings and next steps



J. Alvarez
VP Revenue Cycle

DEPARTMENT

Revenue cycle

DATE AND FORMAT

Sep 8 · Onsite · 60 min

INTERVIEWERS

K. Ortiz, W. Lee

Source: Sample names for layout only; replace with engagement stakeholders before use.

FINDING	EVIDENCE	IMPACT	RECOMMENDED NEXT STEP
Authorization gaps	40% of denials start at intake	High	Add an authorization check at scheduling
Late error discovery	Found at billing, 3 weeks on	High	Pilot check-in eligibility at two clinics
Three work queues	No shared view of backlog	Med	Design one queue by payer
Slow appeals	45 days on average	High	Triage appeals by dollar value
Concentrated know-how	Two analysts hold practice	Med	Cross-train and document playbook
Manual reporting	Rebuilt by hand monthly	Low	Automate the denial extract

Patient access leads agree on registration as the first fix

Group session · Sep 25 · Onsite, Main campus · 90 min · K. Ortiz, A. Cole

DIRECTOR, PATIENT ACCESS

M. Chen

- Errors begin at the front desk
- Three scheduling tools in use
- Wants real-time eligibility

PATIENT ACCESS SUPERVISOR

B. Kowalski

- Staff re-key data twice
- Turnover is highest at intake
- Prior-auth data is slow to arrive

SCHEDULING LEAD

U. Lindgren

- Templates differ by clinic
- Cancellations are not tracked
- Open to a shared scheduling tool

3 of 3

name registration quality as the first fix

OPPORTUNITIES

- Check eligibility at check-in
- One scheduling template set
- Shared error dashboard

FOLLOW-UPS AND OPEN QUESTIONS

- Registration error log: received
- Prior-auth data: overdue 3 days
- Open: who owns template changes?

Source: Sample names for layout only; replace with engagement stakeholders before use.

Revenue cycle sees denials start at authorization



J. Alvarez

VP Revenue Cycle

DEPARTMENT

Revenue cycle

DATE AND FORMAT

Sep 8 · Onsite · 60 min

INTERVIEWERS

K. Ortiz, W. Lee

01 Denials start at authorization

40% of denials trace to missing authorizations. So what: fix intake before appeals.

02 Errors surface at billing

Front-desk errors are found about three weeks after the visit. So what: add a check-in step.

03 Three work queues, no single view

Teams hide backlog in separate queues and hand-built reports. So what: one queue by payer.

04 Appeals depend on two analysts

Appeals average 45 days and the know-how sits with two people. So what: cross-train now.

Source: Sample names for layout only; replace with engagement stakeholders before use.



J. Alvarez

VP Revenue Cycle

DEPARTMENT

Revenue cycle

DATE AND FORMAT

Sep 8 · Onsite · 60 min

INTERVIEWERS

K. Ortiz, W. Lee

STAKEHOLDER INTERVIEWS

Denials start at authorization

- 01 Denials start at authorization**
Evidence: 40% of denials trace to authorization gaps. Implication: fix intake, not appeals.
- 02 Errors surface late**
Evidence: front-desk errors found at billing, three weeks on. Implication: add a check-in eligibility step.
- 03 No single work view**
Evidence: three queues, reports rebuilt by hand monthly. Implication: one queue by payer.
- 04 Appeal know-how is concentrated**
Evidence: two analysts hold the practice behind 45-day appeals. Implication: cross-train first.
- 05 Payer contracts have no owner**
Evidence: nobody could name one owner. Implication: assign an owner before redesign.

FOLLOW-UPS AND QUESTIONS

- **Received:** denial extract; appeal staffing due Oct 6
- **Open:** who owns payer contracts?

Source: Sample names for layout only; replace with engagement stakeholders before use.

J. Alvarez, VP Revenue Cycle: Sep 8 interview

DEPARTMENT	DATE AND FORMAT	INTERVIEWERS
Revenue cycle	Sep 8 · Onsite · 60 min	K. Ortiz, W. Lee
TOPIC	KEY POINT	PAIN POINT OR OPPORTUNITY
Denials	40% of denials trace to authorization gaps	Rework after day 30 is manual
Registration	Front-desk errors surface at billing	No eligibility check at check-in
Tooling	Three work queues, no single view	Reports rebuilt by hand monthly
Appeals	Appeals average 45 days to resolve	Two analysts hold the know-how

Source: Sample names for layout only; replace with engagement stakeholders before use.

“

We find it at billing, three weeks late.

J. ALVAREZ

OPPORTUNITIES

- One work queue by payer
- Eligibility check at check-in

FOLLOW-UPS AND QUESTIONS

- **Denial extract:** received Sep 22
- **Appeal staffing:** pending, due Oct 6
- **Open:** who owns payer contracts?

Four topics from the VP Revenue Cycle interview

J. Alvarez, VP Revenue Cycle · Revenue cycle · Sep 8 · In person · K. Ortiz, W. Lee

TOPIC	KEY POINTS	IN THEIR WORDS	OPPORTUNITY	FOLLOW-UP
Denials Authorization, appeals	40% trace to authorization - Appeals average 45 days	We find it at billing, three weeks late.	One queue by payer	☒ Received
Registration Front desk, eligibility	- Errors found at billing - No check-in eligibility	Registrars are blamed for tool gaps.	Real-time eligibility	☐ Pending
Tooling Queues, reporting	- Three work queues - Hand-built monthly reports	Every report starts in a spreadsheet.	Shared denial view	☐ Pending
Staffing Analysts, coders	- Two analysts hold appeal know-how - Coder backlog 9 days	If either leaves, appeals stall.	Cross-train analysts	☐ Not asked

Source: Sample names for layout only; replace with engagement stakeholders before use.

Revenue cycle and patient access agree: fix intake first



J. Alvarez

VP Revenue Cycle

DEPARTMENT AND DATE

Revenue cycle · Sep 8



M. Chen

Director Patient Access

DEPARTMENT AND DATE

Patient access · Sep 10

INTERVIEWERS: K. ORTIZ, W. LEE (BOTH SESSIONS)

THEME	J. ALVAREZ, REVENUE CYCLE	M. CHEN, PATIENT ACCESS
Root cause	Denials start at authorization; 40% trace there	Registration is rushed; staff lack payer rules
Tooling	Three work queues, no single view	No eligibility check at check-in
Staffing	Two analysts hold appeal know-how	Front-desk turnover near 30%
Where they agree	Fix intake before appeals	Fix intake before appeals

Source: Sample names for layout only; replace with engagement stakeholders before use.

STAKEHOLDER INTERVIEWS

Wave 1 is booked; waves 2 and 3 are not yet scheduled

STAKEHOLDER	DEPARTMENT	TARGET	PURPOSE	SCHEDULING
WAVE 1 • EXECUTIVES • BY OCT 16				
Chief Nursing Officer	Clinical ops	Oct 7	Clinical priorities and nurse time lost to rework	☒ Scheduled
Chief Operating Officer	Operations	Oct 8	Operating model and decision rights	☒ Scheduled
Chief Digital Officer	IT	Oct 13	Roadmap fit and platform timelines	☐ Not scheduled
WAVE 2 • OPERATIONAL LEADERS • BY OCT 23				
Director, Billing	Revenue cycle	Oct 20	Statement and collections process	☐ Not scheduled
Manager, Credentialing	Compliance	Oct 21	Payer enrollment delays	☐ Not scheduled
WAVE 3 • FRONT-LINE AND SMES • BY OCT 30				
Registration leads (3)	Patient access	Oct 24	Workflow walk at the front desk	☐ Not scheduled
Coding specialists (4)	HIM	Oct 27	Query volume and documentation gaps	☐ Not scheduled

Source: Sample names for layout only; replace with engagement stakeholders before use.

Front-line sessions close out the plan in late October

STAKEHOLDER	TARGET	PURPOSE
Registration leads (3) Patient access	Oct 24	Front-desk workflow walk
Coding specialists (4) HIM	Oct 27	Query volume and gaps
Denials analysts (3) Revenue cycle	Oct 28	Appeal tooling and rework
Service desk lead IT	Oct 29	Incident trends by system
Unit clerks (3) Clinical ops	Oct 30	Charge capture steps
Billing leads (2) Finance	Oct 30	Statement exceptions

- ☒ **Wave 1 • Executives** OCT 7 TO 16
Five sessions on priorities and funding.
- ☐ **Wave 2 • Leaders** OCT 17 TO 23
Five sessions on process and tooling.
- ☐ **Wave 3 • Front line** OCT 24 TO 30
Fifteen people in six group sessions.
- ☐ **Readout** NOV 3
Findings to the steering committee.

STILL TO SCHEDULE

10

of 15 interviews have no date yet.

Source: Sample names for layout only; replace with engagement stakeholders before use.

Fifteen more interviews run in three waves through October

BY OCT 16

Wave 1 · Executives

- Chief Nursing Officer
 - Oct 7 · Clinical priorities
- Chief Operating Officer
 - Oct 8 · Operating model
- VP Supply Chain
 - Oct 9 · Spend scope
- Chief Digital Officer
 - Oct 13 · Roadmap fit
- Board finance chair
 - Oct 15 · Funding view

BY OCT 23

Wave 2 · Operational leaders

- Director, Scheduling
 - Oct 17 · Template rules
- Director, Billing
 - Oct 20 · Statement process
- Mgr, Credentialing
 - Oct 21 · Payer enrollment
- Dir., Quality
 - Oct 22 · Reporting load
- Mgr, Cash Posting
 - Oct 23 · Posting lag

BY OCT 30

Wave 3 · Front-line and SMEs

- Registration leads (3)
 - Oct 24 · Workflow walk
- Coding specialists (4)
 - Oct 27 · Query volume
- Denials analysts (3)
 - Oct 28 · Appeal tooling
- Service desk lead
 - Oct 29 · Incident trends
- Unit clerks (3)
 - Oct 30 · Charge capture

Source: Sample names for layout only; replace with engagement stakeholders before use.

Twelve interviews done; every one has a follow-up

PERSON	ROLE / TITLE	DEPARTMENT	DATE MET	FORMAT	ASKED	REQUEST STATUS	NOTE
J. Alvarez	VP Revenue Cycle	Revenue cycle	Sep 8	ONSITE	✓	Received	Due Sep 30
M. Chen	Director, Patient Access	Patient access	Sep 9	ONSITE	✓	Received	Log received
R. Okafor	Chief Information Officer	IT	Sep 10	VIDEO	✓	Pending	2 systems out
S. Patel	Dir., Enterprise Apps	IT	Sep 11	VIDEO	✓	Pending	Map in review
L. Brandt	Chief Financial Officer	Finance	Sep 15	ONSITE	✓	Received	Received Sep 22
D. Moreau	Controller	Finance	Sep 16	ONSITE	✓	Received	Received Sep 29
A. Nguyen	Chief Medical Officer	Clinical ops	Sep 17	VIDEO	✓	Blocked	Needs sponsor
T. Wallace	VP Clinical Operations	Clinical ops	Sep 18	ONSITE	✓	Pending	Draft in review
K. Ito	Chief Compliance Officer	Compliance	Sep 22	VIDEO	✓	Received	Received Oct 1
P. Silva	Director, HIM	HIM	Sep 23	ONSITE	✓	Pending	Weekly refresh
E. Haynes	Manager, Denials	Revenue cycle	Sep 24	PHONE	✓	Received	Report in
B. Kowalski	Patient Access Supervisor	Patient access	Sep 25	ONSITE	✓	Overdue	3 sent

Source: Sample names for layout only; replace with engagement stakeholders before use.

We met 12 stakeholders; 5 follow-up requests are met

PERSON	ROLE / TITLE	DEPARTMENT	MET	FORMAT	FOLLOW-UP REQUEST	REQUEST STATUS
J. Alvarez	VP Revenue Cycle	Revenue cycle	Sep 8	ONSITE	Denial reason-code extract	Received
M. Chen	Director, Patient Access	Patient access	Sep 9	ONSITE	Registration error log	Received
R. Okafor	Chief Information Officer	IT	Sep 10	VIDEO	Application inventory	Pending
S. Patel	Dir., Enterprise Apps	IT	Sep 11	VIDEO	Interface and integration map	Pending
L. Brandt	Chief Financial Officer	Finance	Sep 15	ONSITE	FY26 budget by department	Received
D. Moreau	Controller	Finance	Sep 16	ONSITE	Month-end close calendar	Received
A. Nguyen	Chief Medical Officer	Clinical ops	Sep 17	VIDEO	Clinical documentation audit	Blocked
T. Wallace	VP Clinical Operations	Clinical ops	Sep 18	ONSITE	Staffing model by unit	Pending
K. Ito	Chief Compliance Officer	Compliance	Sep 22	VIDEO	Open audit findings	Received

Source: Sample names for layout only; replace with engagement stakeholders before use.

- **Finance and compliance.** Every request met inside a week of the interview.
- **Clinical operations.** One request blocked pending CMO sponsor action.
- **Patient access.** Prior-authorization data is slow to arrive.
- **IT.** Two inventories are in progress, due next week.

REQUESTS MET

5 of 12

with four pending and three open.

STAKEHOLDER INTERVIEWS

Finance and compliance answered fastest

PERSON	DEPARTMENT	MET	FOLLOW-UP REQUEST	STATUS
J. Alvarez VP Revenue Cycle	Revenue cycle	Sep 8	Denial reason-code extract	Received
M. Chen Director, Patient Access	Patient access	Sep 9	Registration error log	Received
R. Okafor Chief Information Officer	IT	Sep 10	Application inventory	Pending
S. Patel Dir., Enterprise Apps	IT	Sep 11	Interface and integration map	Pending
L. Brandt Chief Financial Officer	Finance	Sep 15	FY26 budget by department	Received
D. Moreau Controller	Finance	Sep 16	Month-end close calendar	Received

Source: Sample names for layout only; replace with engagement stakeholders before use.

Each leader gave us one data request; two are still open

PERSON	DEPARTMENT	MET	FOLLOW-UP REQUEST	REQUEST STATUS	OWNER
J. Alvarez VP Revenue Cycle	Revenue cycle	Sep 8 Onsite	Denial reason-code extract Asked Sep 8 · due Oct 6	 Received	K. Ortiz
M. Chen Director, Patient Access	Patient access	Sep 9 Onsite	Registration error log Asked Sep 9 · due Oct 8	 Received	W. Lee
R. Okafor Chief Information Officer	IT	Sep 10 Video	Application inventory Asked Sep 10 · due Oct 10	 Pending	K. Ortiz
S. Patel Dir., Enterprise Apps	IT	Sep 11 Video	Interface and integration map Asked Sep 11 · due Oct 12	 Pending	A. Cole
L. Brandt Chief Financial Officer	Finance	Sep 15 Onsite	FY26 budget by department Asked Sep 15 · due Oct 14	 Received	W. Lee
D. Moreau Controller	Finance	Sep 16 Onsite	Month-end close calendar Asked Sep 16 · due Oct 16	 Received	A. Cole
A. Nguyen Chief Medical Officer	Clinical ops	Sep 17 Video	Clinical documentation audit Asked Sep 17 · due Oct 18	 Blocked	K. Ortiz

 Received  Pending, inside due date  Overdue or blocked

Source: Sample names for layout only; replace with engagement stakeholders before use.

STAKEHOLDER INTERVIEWS

Five of twelve follow-up requests are already met

12

interviews held

12

follow-ups requested

5

requests met

2

requests blocked or overdue

PERSON	DEPARTMENT	MET	FORMAT	FOLLOW-UP REQUEST	STATUS	DUE
J. Alvarez VP Revenue Cycle	Revenue cycle	Sep 8	ONSITE	Denial reason-code extract	Received	Oct 2
M. Chen Director, Patient Access	Patient access	Sep 9	ONSITE	Registration error log	Received	Oct 4
R. Okafor Chief Information Officer	IT	Sep 10	VIDEO	Application inventory	Pending	Oct 6
S. Patel Dir., Enterprise Apps	IT	Sep 11	VIDEO	Interface and integration map	Pending	Oct 8
L. Brandt Chief Financial Officer	Finance	Sep 15	ONSITE	FY26 budget by department	Received	Oct 10
D. Moreau Controller	Finance	Sep 16	ONSITE	Month-end close calendar	Received	Oct 12

Source: Sample names for layout only; replace with engagement stakeholders before use.

REQUESTS MADE

10

3

received

5

pending, due soon

2

overdue

STAKEHOLDER INTERVIEWS

Seven requests are still open, two of them overdue

REQUEST	DUE	DAYS	STATUS
Application inventory R. Okafor, IT	Sep 24	-6	 Pending
Interface and integration map S. Patel, IT	Sep 25	-8	 Pending
Clinical documentation audit A. Nguyen, Clinical ops	Oct 1	+2	 Overdue
Staffing model by unit T. Wallace, Clinical ops	Oct 2	-3	 Pending
Coding backlog by week P. Silva, HIM	Oct 7	-4	 Pending
Prior-auth turnaround data B. Kowalski, Patient access	Sep 30	+3	 Overdue

Source: Sample names for layout only; replace with engagement stakeholders before use.

Three of ten requests are met; two are overdue

REQUEST	ASKED OF	DEPARTMENT	WM OWNER	REQUESTED	DUE	DAYS	STATUS
Denial reason-code extract	J. Alvarez	Revenue cycle	K. Ortiz	Sep 8	Sep 22	met	Received
Registration error log	M. Chen	Patient access	W. Lee	Sep 9	Sep 23	met	Received
Application inventory	R. Okafor	IT	A. Cole	Sep 10	Sep 24	-6	Pending
Interface and integration map	S. Patel	IT	A. Cole	Sep 11	Sep 25	-8	Pending
FY26 budget by department	L. Brandt	Finance	W. Lee	Sep 15	Sep 29	met	Received
Clinical documentation audit	A. Nguyen	Clinical ops	K. Ortiz	Sep 17	Oct 1	+2	Overdue
Staffing model by unit	T. Wallace	Clinical ops	W. Lee	Sep 18	Oct 2	-3	Pending
Open audit findings	K. Ito	Compliance	A. Cole	Sep 22	Oct 6	met	Received
Coding backlog by week	P. Silva	HIM	K. Ortiz	Sep 23	Oct 7	-4	Pending
Prior-auth turnaround data	B. Kowalski	Patient access	W. Lee	Sep 25	Sep 30	+3	Overdue

Days: negative is days to the due date, positive is days overdue. Pending requests are inside their due date.

Source: Sample names for layout only; replace with engagement stakeholders before use.

Leaders agree on the problem and differ on the order

REVENUE CYCLE

J. Alvarez, VP Revenue Cycle

- Denials rise on missing authorization
- Rework is manual after day 30
- Wants one work queue

PATIENT ACCESS

M. Chen, Director

- Registration errors start at the front desk
- Three scheduling tools in use
- Asks for real-time eligibility

IT

R. Okafor, CIO

- Two platforms near end of support
- Integration is the main risk
- Prefers phased cutover

FINANCE

L. Brandt, CFO

- Close takes 12 days
- Sees denials as a cash lever
- Needs a case by January

COMPLIANCE

K. Ito, Chief Compliance Officer

- Audit findings repeat each year
- Wants controls built in
- Open to automation

HIM

P. Silva, Director

- Coding backlog averages 9 days
- Documentation gaps drive denials
- Short two coders

Source: Sample names for layout only; replace with engagement stakeholders before use.

Every group points to the same front-end causes

GROUP	KEY TAKEAWAYS	OPEN ITEMS	FOLLOW-UPS
Revenue cycle 4 interviews · Sep 8 to 24	- Denials start with missing authorization - Manual rework after day 30	- Denial code extract - Appeal staffing	Received
Patient access 3 interviews · Sep 9 to 25	- Registration errors drive front-end denials - Three scheduling tools	- Prior-auth data - Eligibility vendor	Overdue
IT and applications 3 interviews · Sep 10 to 29	- Two platforms near end of support - Integration is the main risk	- Interface map - Cutover window	Pending
Finance 3 interviews · Sep 15 to 30	- Close takes 12 days - Needs a case by January	Budget baseline	Received
Clinical ops and HIM 4 interviews · Sep 17 to 30	- Documentation gaps drive denials - Coding backlog averages 9 days	- Documentation audit - Coder capacity	Blocked

Source: Sample names for layout only; replace with engagement stakeholders before use.

What eight leaders told us

- **J. Alvarez, VP Revenue Cycle.** Denials start at authorization; rework after day 30 is manual.
- **M. Chen, Patient Access.** Registration errors begin at the front desk; three scheduling tools.
- **R. Okafor, CIO.** Two platforms near end of support; integration is the main risk.
- **L. Brandt, CFO.** Close takes 12 days; wants a funded case by January.
- **K. Ito, Compliance.** Audit findings repeat; wants controls built into workflow.
- **P. Silva, HIM.** Coding backlog averages 9 days; short two coders.
- **T. Wallace, Clinical Ops.** Staffing by unit varies; nurses re-key charge data.
- **D. Moreau, Controller.** Manual journals at close; reconciliation lags a week.

4 of 8

name authorization and registration quality as the first fix

3 of 8

want controls and automation built into the workflow

2 of 8

need a funded case before January

HOW TO READ THIS

Four of eight put authorization and registration first

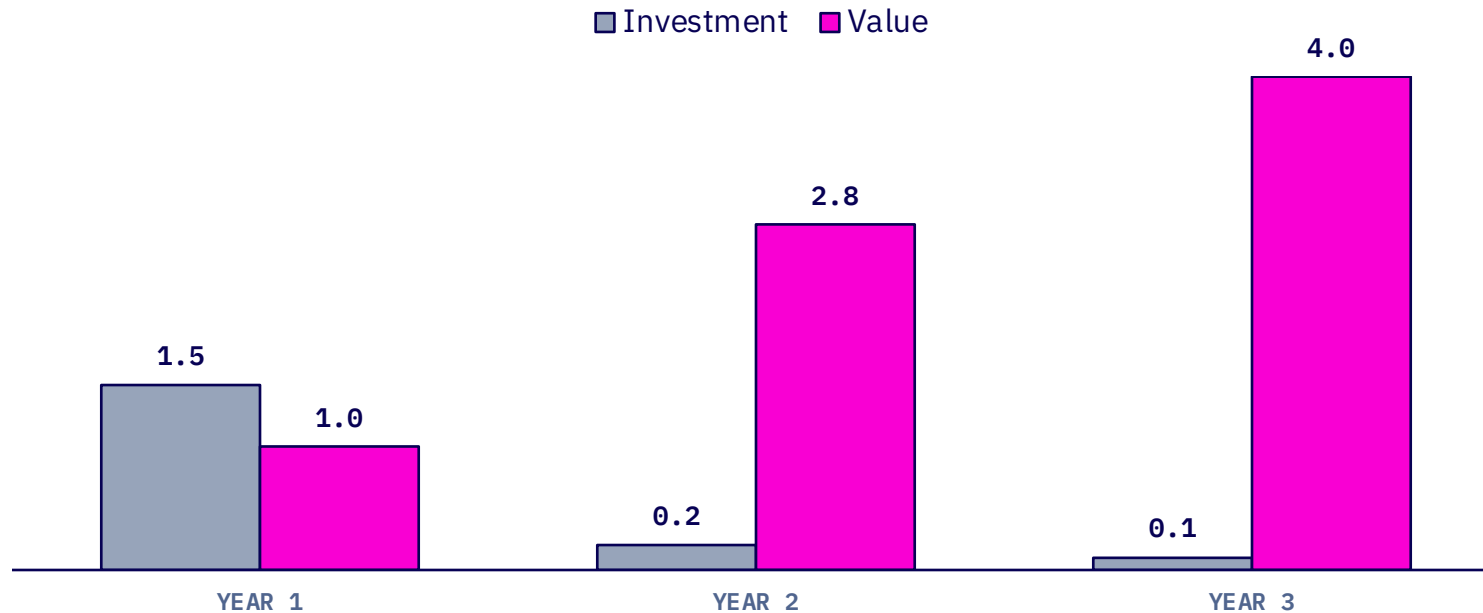
Finance, IT and compliance want a funded case, a phased cutover and built-in controls before any tool change.

Source: Sample names for layout only; replace with engagement stakeholders before use.

Value outruns cost from year two

Investment and value by year

\$M



Source: Illustrative Northfield business case; replace with validated figures before use.

THREE-YEAR NET VALUE

\$6.0M

\$7.8M of value on \$1.8M of investment

Value exceeds cost in year two.

Year three carries almost no new spend.

Four phases, \$1.68M in fees

PHASE	TIMING	WHAT IT DELIVERS	FEE MODEL	FEE
Diagnose	Weeks 1–4	Diagnostic readout	Fixed fee	\$288K
Design	Weeks 5–10	Target design and business case	Fixed fee	\$432K
Build, Wave 1	Months 3–6	Wave 1 live in five entities	Team run-rate	\$840K
Hypercare	Months 7–8	Stabilized and handed over	Fixed fee	\$120K
Total fees	8 months			\$1.68M

COMMERCIALS

A \$1.8M investment pays back in 14 months

\$1.8M

total investment

\$2.6M

annual run-rate benefit

14

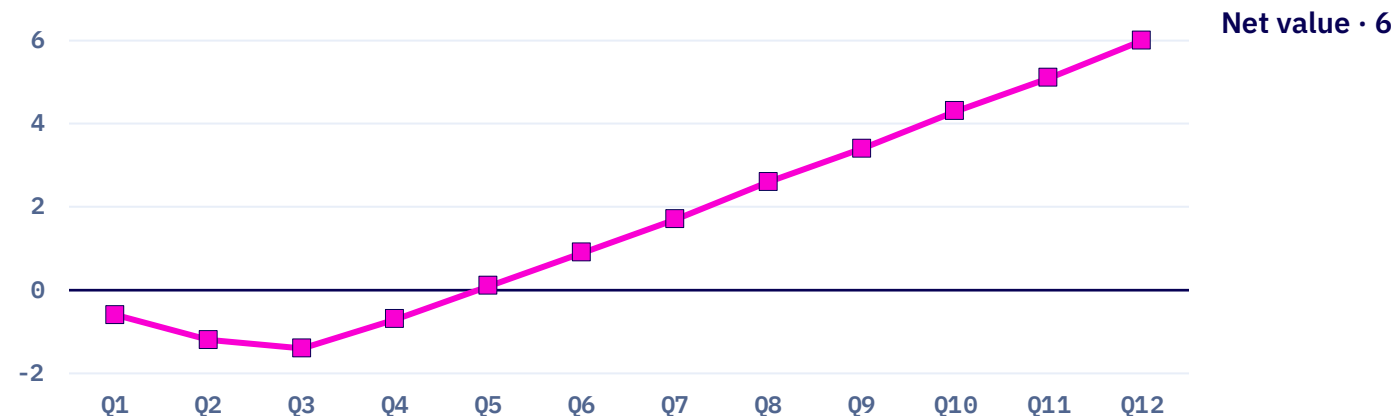
months to payback

330%

three-year return

Cumulative net value

\$M, BY QUARTER



WHAT IT MEANS

- Investment peaks in quarter 3
- Net value turns positive in quarter 5
- \$6.0M net value by year three

Source: Illustrative Northfield business case; replace with validated figures before use.

COMMERCIALS

Total investment: \$1.8M over eight months

TOTAL INVESTMENT

\$1.8M

fees of \$1.68M plus expenses capped at 8%

14

months to payback

330%

three-year return

BY PHASE

Diagnose **\$288K**

Design **\$432K**

Build, Wave 1 **\$840K**

Hypercare **\$120K**

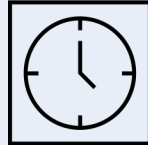
Expenses, capped **\$134K**

FINANCE TRANSFORMATION

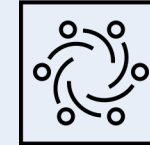
The longest, riskiest step is understanding what to change, how, and in what order.

Teams often spend months analyzing the current close and then take safe, small steps. The result is a slow program with little business impact.

MOST COMMON COMPLICATING FACTORS



Manual steps nobody has documented



Knowledge held by a few individuals



Hidden dependencies between entities



Finance and IT working to different priorities

The key to success is a partner who compresses the time to understand, so decisions come sooner.

The script has **flipped** on how companies approach application modernization

AI is cutting the time and cost of complex modernization by changing how teams analyze, design, build and test.

How are legacy systems holding your business back?

If modernizing legacy software cost one-tenth of what you expect, what would that unlock?

AI-NATIVE READINESS

How can you get your business ready to run on AI?

GROWTH VELOCITY

What products could you launch without release-cycle delays?

COST EFFICIENCY

Where would fewer handoffs unlock efficiency?

CUSTOMER MOAT

How could a connected experience deepen loyalty?

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MODERNIZATION ECONOMICS

The script has **flipped** on legacy systems

AI is cutting the time and cost of complex modernization by changing how teams analyze, design, build and test.

How are legacy systems holding your business back?

If it cost one-tenth of what you expect, what would that unlock?

AI-NATIVE READINESS

How can you get your business ready to run on AI?

Map data, interfaces and controls to see which systems block AI-enabled work.

COST EFFICIENCY

Where would fewer handoffs unlock efficiency?

Fewer handoffs and less rework free capacity that today goes to keeping old systems running.

GROWTH VELOCITY

What products could you launch without release-cycle delays?

Modular platforms let teams ship weekly instead of waiting on quarterly releases.

CUSTOMER MOAT

How could a connected experience deepen loyalty?

One connected experience across channels gives customers fewer reasons to leave.

The script has **flipped** on how companies approach modernization

AI is cutting the time and cost of complex modernization by changing how teams analyze, design, build and test.

How are legacy systems holding your business back?

AI-NATIVE READINESS

How can you get your business ready to run on AI?

Most legacy estates were built before AI. Mapping data, interfaces and controls first shows which systems block AI-enabled workflows and which are ready now.

GROWTH VELOCITY

What products could you launch without release-cycle delays?

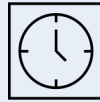
Quarterly release trains slow every new idea. Modern, modular platforms let teams ship weekly, so a good idea reaches customers while it still matters.



FINANCE TRANSFORMATION

You do not need another tool rollout.
You need a partner who treats the close as a **business change**, not a systems project.

The right partner for this program will be able to:



Understand the current close in days, not months



Prioritize by business value, not by effort



Move in steps, balancing speed with control



Build the new process while the old one runs

And the approach adapts to your teams, not the other way round.

Close the books in 5 days instead of 12

**We standardize reconciliations,
automate intercompany matching and
move review earlier.**

The controller signs off on day five, and the team stops finding problems on day nine.



\$4.2M

run-rate savings



12 → 5 days

Close, before and after

Close the books in 5 days instead of 12

We standardize reconciliations, automate intercompany matching and move review earlier.

The controller signs off on day five, and the team stops finding problems on day nine.



\$4.2M

run-rate savings

12 → 5 days

FINANCE TRANSFORMATION

Close the books in **5 days** instead of 12

We standardize reconciliations, automate intercompany matching and move review earlier.

The controller signs off on day five, and the team stops finding problems on day nine.



\$4.2M

run-rate savings



12 → 5 days

Close, before and after

Close the books in **5 days** instead of 12

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\$4.2M

run-rate savings



WHAT CHANGES

Review moves to the start of the close, not the end

Exceptions route to owners on day one, so day nine is for sign-off, not discovery.

WHAT CHANGES

Review moves to the start of the close, not the end

Exceptions route to owners on day one, so day nine is for sign-off, not discovery.

Build is strong; the loops back to Define are weak

PHASE	CADENCE	QUALITY	FEEDBACK	OWNERSHIP	TOOLING
Identify	3	2	2	3	2
Define	2	3	2	2	3
Build	4	3	2	3	3
Optimize	1	2	1	2	1
Loop-backs	1	1	0	2	1

 Absent  Ad hoc  Defined  Managed  Optimized

STRONGEST

Build cadence

Two-week sprints and managed quality since the 2025 reset.

OPTIMIZE

Feedback is ad hoc

Usage data rarely reaches the backlog.

WEAKEST

Loop-backs have no owner

Nobody owns the return from Optimize to Define; cadence is missing.

Source: Northfield Health System lifecycle review, Sep 2026.

Roles shift from product to engineering and back

ROLE	IDENTIFY	DEFINE	BUILD	OPTIMIZE
Product owner	Leads	Leads	Core	Core
UX researcher	Leads	Core	Informed	Supports
Solution architect	Informed	Leads	Core	Informed
Engineering lead		Supports	Leads	Supports
QA lead		Informed	Leads	Supports
Release manager			Core	Core
Product analyst	Supports	Informed	Informed	Leads
Clinical lead	Core	Core	Supports	Core

None Informed Supports Core Leads

Source: Northfield Health System lifecycle review, Sep 2026.

EARLY PHASES

Product and research lead

Identify and Define depend on the owner, researcher and clinical lead.

BUILD

Engineering and QA lead

The product owner stays core to keep scope steady.

OPTIMIZE

Analysts take over

Only the analyst leads; the owner stays core to act on the data.

Each phase ends in a concrete outcome

01 Rationalize

Decide what to modernize, in what order, and why it pays back.

- Portfolio assessment
- Application rationalization
- Target architecture
- Business case

OUTCOME

A funded roadmap: every application has a disposition and a wave.

02 Build

Move the chosen applications and data onto the new platform, safely.

- Re-platform
- Refactor
- Data migration
- Cutover

OUTCOME

Applications live on the new platform with their data reconciled.

03 Sustain

Run the new estate well and keep it from aging into the next legacy.

- Run and operate
- Optimize
- Modernization governance
- Continuous modernization

OUTCOME

A run model and a pipeline that keep the estate current.

Every phase opens on **criteria** and closes at a gate

1 Rationalize

ENTRY CRITERIA

- Mandate and sponsor named
- Application data accessible
- Budget for discovery released

EXIT CRITERIA

- Every application dispositioned
- Business case agreed by finance

GATES

G1 Portfolio decision

G2 Architecture and funding

2 Build

ENTRY CRITERIA

- Roadmap and funding approved
- Target architecture signed
- Wave 1 team in place

EXIT CRITERIA

- Rollback rehearsed and passed
- Data reconciled to the penny

GATES

G3 Cutover readiness

G4 Steady-state handover

3 Sustain

ENTRY CRITERIA

- Cutover complete, hypercare done
- Runbooks accepted by support
- Run budget transferred

EXIT CRITERIA

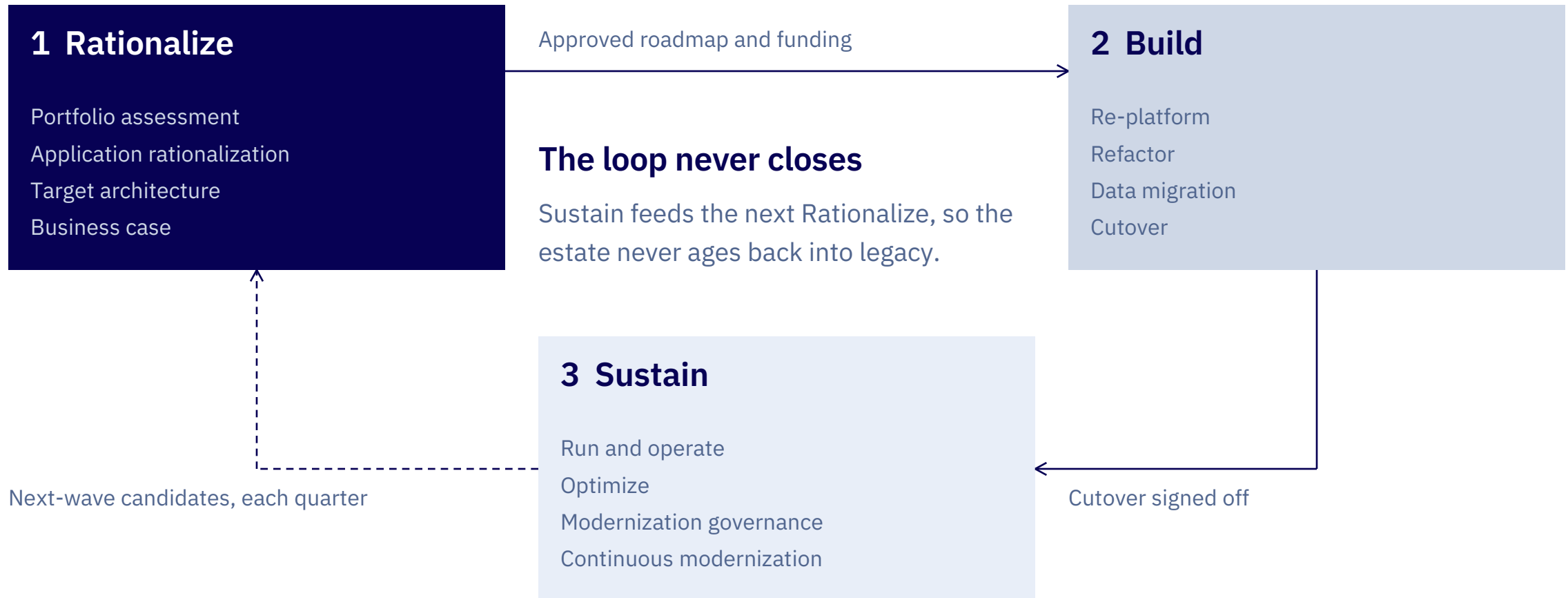
- Service levels met two quarters
- Next-wave candidates ranked

GATES

Quarterly health review

Annual re-rationalization

Modernization is a **loop**, not a line



THE IDEA

Decide, deliver, then keep it current

Most estates modernize once and then age. The lifecycle makes the third phase as deliberate as the first two.

12

subtracks across three phases

MODERNIZATION LIFECYCLE

Three phases, one lifecycle

Rationalize

Decide

What to modernize, in what order

- Portfolio assessment
- Application rationalization
- Target architecture
- Business case

Build

Deliver

Move applications and data safely

- Re-platform
- Refactor
- Data migration
- Cutover

Sustain

Run

Run it well, keep it current

- Run and operate
- Optimize
- Modernization governance
- Continuous modernization

Each phase answers **one question** for the client

01**Rationalize**

THE QUESTION

Which applications should we modernize, and in what order?

KEY DELIVERABLES

Application scorecard
Target architecture
Funded roadmap

TYPICAL DURATION

8–12
weeks

02**Build**

THE QUESTION

Can we move them safely, without disrupting care?

KEY DELIVERABLES

Working platform and services
Reconciled data
Cutover and rollback plan

TYPICAL DURATION

6–12
months per wave

03**Sustain**

THE QUESTION

Will the new estate stay current and affordable?

KEY DELIVERABLES

Service levels and runbooks
Cost and capacity plan
Modernization pipeline

TYPICAL DURATION

Ongoing
quarterly reviews

Source: Sample figures for layout only; replace with current firm data before use.

Three phases, twelve subtracks, four gates

Northfield's scheduling and billing estate moves through the same three phases, each closed by a gate the steering group signs.

FOUR GATES

Exit gates

- G1 · Portfolio decision
- G2 · Architecture and funding
- G3 · Cutover readiness
- G4 · Steady-state handover

01 Rationalize

Portfolio assessment
Score 140 applications

Application rationalization
Retire, keep, move, rebuild

Target architecture
Scheduling, billing design

Business case
Cost, risk, benefit by wave

02 Build

Re-platform
Move to the landing zone

Refactor
Split the billing monolith

Data migration
Reconciled patient data

Cutover
Waves, rehearsals, go-live

03 Sustain

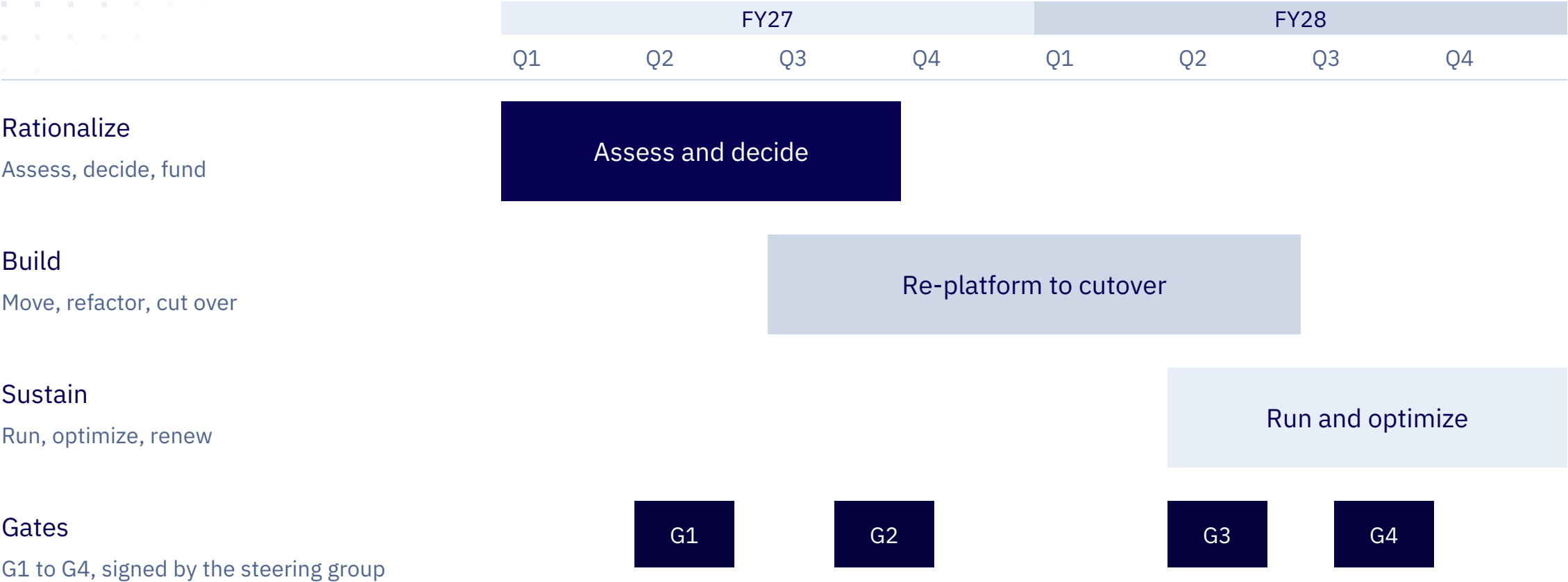
Run and operate
Monitoring and support

Optimize
Tune cost and capacity

Modernization governance
Standards and funding

Continuous modernization
Keep the estate current

Phases overlap across eight quarters, with four gates



Source: Sample dates for layout only; replace with the engagement plan.

Each phase has its own leads and Intellio accelerators

Northfield's team and the Intellio accelerators change with each phase, but the three leads stay constant.

HOW TO READ

Reading the rows

- Roles lead through the handoff
- Accelerators are tuned to the estate
- Each phase closes at a gate

Source: Sample names for layout only; replace with the current catalog before use.

01 Rationalize

LEAD ROLES

Engagement lead
Enterprise architect
Portfolio analyst
Finance partner

INTELLIO ACCELERATORS

Portfolio Scan
Disposition Scorecard
Reference Architecture Kit
Business Case Model

02 Build

LEAD ROLES

Delivery lead
Cloud engineer
Data engineer
Quality lead

INTELLIO ACCELERATORS

Landing Zone Blueprint
Refactor Toolkit
Data Reconciler
Cutover Runbook

03 Sustain

LEAD ROLES

Service owner
Site reliability lead
FinOps analyst
Change lead

INTELLIO ACCELERATORS

Operations Playbook
Cost Optimizer
Governance Cadence Kit
Modernization Radar

Modernization runs in three phases

1 Rationalize

Decide what to modernize, in what order, and why it pays back.

Portfolio assessment

Inventory and score 140 applications

Application rationalization

Retire, retain, move or rebuild

Target architecture

Scheduling and billing reference design

Business case

Cost, risk and benefit by wave

2 Build

Move the chosen applications and data onto the new platform, safely.

Re-platform

Move workloads to the new landing zone

Refactor

Break the billing monolith into services

Data migration

Patient and account data, reconciled

Cutover

Waves, rehearsals, go-live, rollback

3 Sustain

Run the new estate well and keep it from aging into the next legacy.

Run and operate

Service levels, monitoring, support

Optimize

Tune cost, performance and capacity

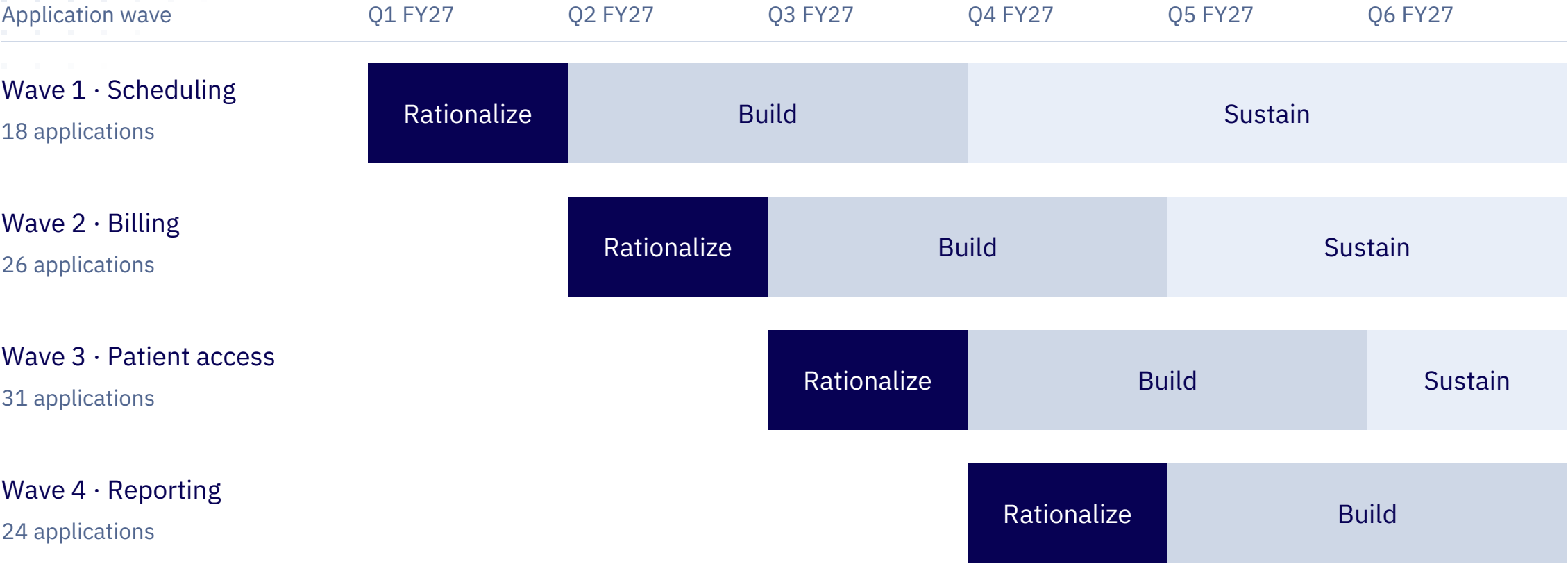
Modernization governance

Standards, funding, decision rights

Continuous modernization

Keep the estate current each release

Applications move through the phases in four waves



Source: Sample figures for layout only; replace with current firm data before use.

Seven capabilities, each two stages from target

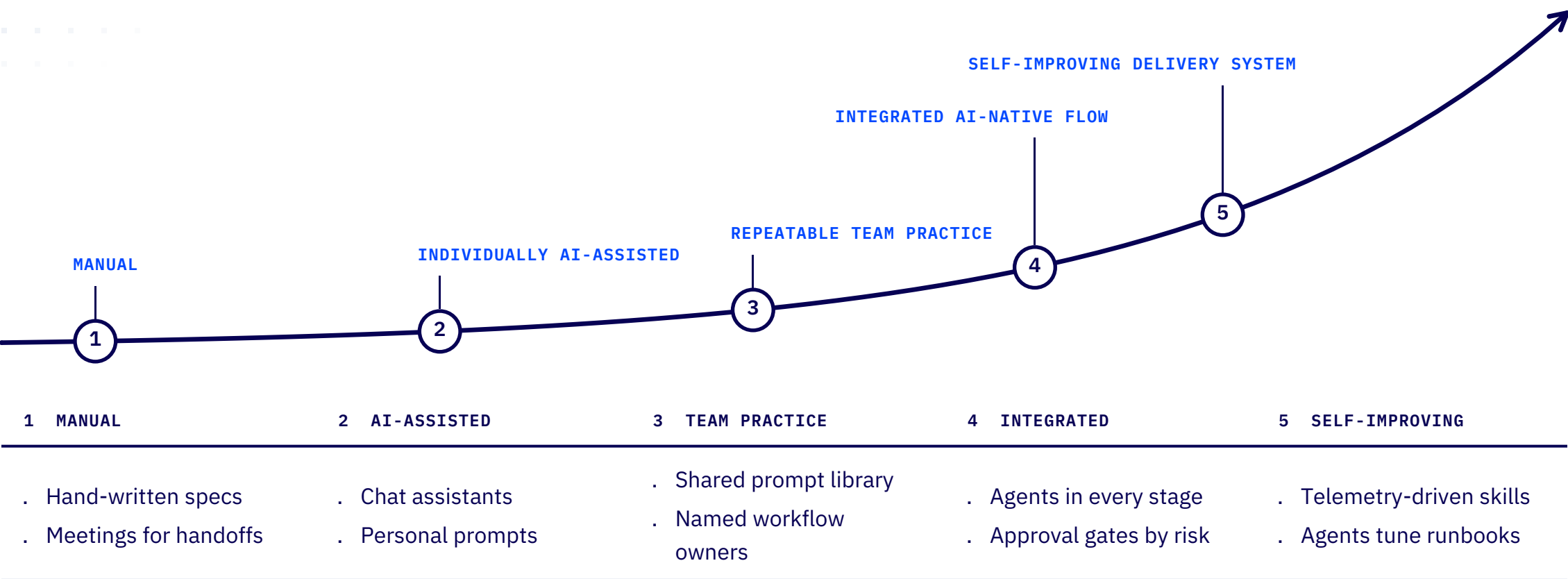
CAPABILITY	STAGE • NOW AND TARGET	NEXT MOVE
Adoption, ownership and learning	2 → 3	Name workflow owners in each team
Work design and flow	2 → 4	Map handoffs and decision rights
Context and knowledge	1 → 3	Curate sources and set freshness rules
AI workflow architecture	2 → 4	Standardize agent steps and handoffs
Platform integration	1 → 3	Connect PM tool, repo and CI by API
Trust, risk and control	2 → 4	Grade risk and set approval gates
Measurement and improvement	1 → 3	Baseline outcomes, then track weekly

Source: Northfield assessment workshops, Sep 2026.

The curve bends at stage four, then reaches further



Each stage adds capabilities the last one lacked



Four steps move Northfield from stage one to five

1 Assess

STAGE 1 → 2

- Baseline each team on seven capabilities
- Pick two pilot teams
- Set the target stage

DONE WHEN

Every team has a scored baseline

2 Pilot

STAGE 2 → 3

- Pair coaches with pilot teams
- Build shared workflows and prompts
- Name a workflow owner

DONE WHEN

Pilot teams reuse one shared workflow

3 Embed

STAGE 3 → 4

- Connect agents to PM tool, repo and CI
- Add risk-based approval gates
- Train the next teams

DONE WHEN

Handoffs run through agents with review

4 Extend

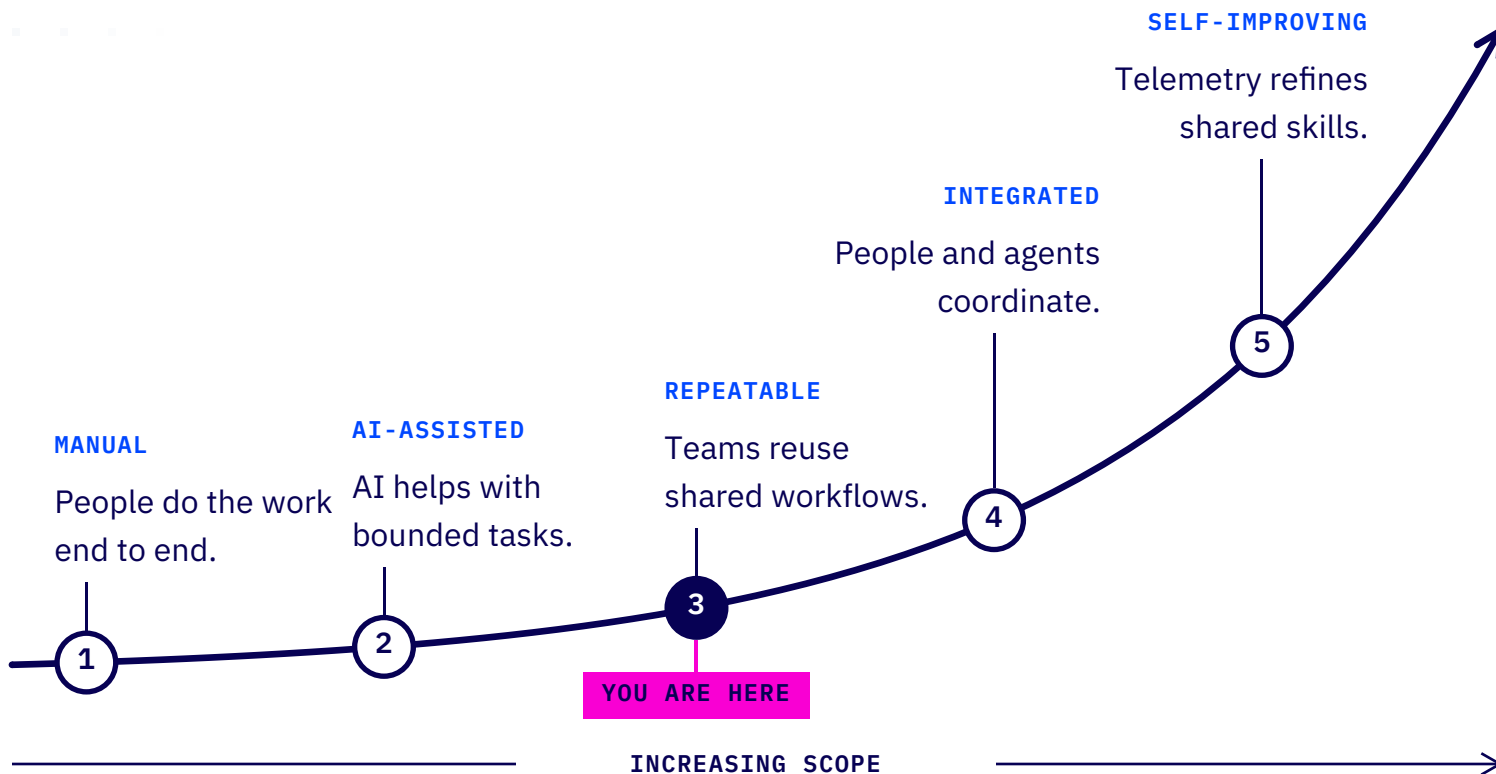
STAGE 4 → 5

- Add telemetry and monitoring agents
- Extend to engineering, SRE and support
- Hand the practice to Northfield

DONE WHEN

Skills improve from delivery data

Northfield is at stage three; stage four needs three moves



TO REACH STAGE FOUR

NAME ONE OWNER PER WORKFLOW

Each shared workflow needs an accountable owner and a review step.

TO REACH STAGE FOUR

DEFINE THE HANDOFFS

Write down where agents hand work to people and back.

TO REACH STAGE FOUR

MEASURE CYCLE TIME

Track one flow metric so gains show up in delivery.

Three lessons shape the climb to AI-native delivery



INSIGHT 1

WORKFLOWS BEFORE TOOLS

Gains stall until teams share workflows, not just licenses.

INSIGHT 2

REVIEW KEEPS PACE

Defined review lets agents take on more of the work safely.

INSIGHT 3

TELEMETRY IS THE UNLOCK

Self-improvement needs delivery data agents can learn from.

Delivery matures in five stages



What changes in each stage

	1 MANUAL	2 AI-ASSISTED	3 TEAM PRACTICE	4 INTEGRATED	5 SELF-IMPROVING
People	Individuals own all work and context	Individuals pick their own AI tools	Named workflow owners and shared standards	Agentic process architects lead cross-team flow	Teams curate skills; agents handle routine work
Process	Manual handoffs and meetings	Workflows unchanged; tasks faster	Shared workflows with defined review	Humans and agents share one flow	Telemetry reshapes the flow itself
Tooling	Documents and trackers	Chat assistants and code helpers	Shared prompts, templates and skills	Agents connected to PM, repo and CI	Monitoring agents tune skills and runbooks
Governance	Review by the manager	Personal judgment on AI use	Review rules by risk	Controls built into the flow	Evidence loop guides policy changes

Maturity has two axes: how wide and how deep

Go deep, then go wide

Depth is how far AI is built into the workflow. Breadth is how many functions share it.

- Product teams lead at stage three
- Engineering and SRE follow one stage behind
- Support and go-to-market start at stage one

DEPTH OF INTEGRATION

	PRODUCT TEAMS	ENGINEERING AND SRE	WIDER ORGANIZATION
5 SELF-IMPROVING			
4 INTEGRATED FLOW	Target		
3 TEAM PRACTICE	Today	Target	
2 AI-ASSISTED		Today	Target
1 MANUAL			Today

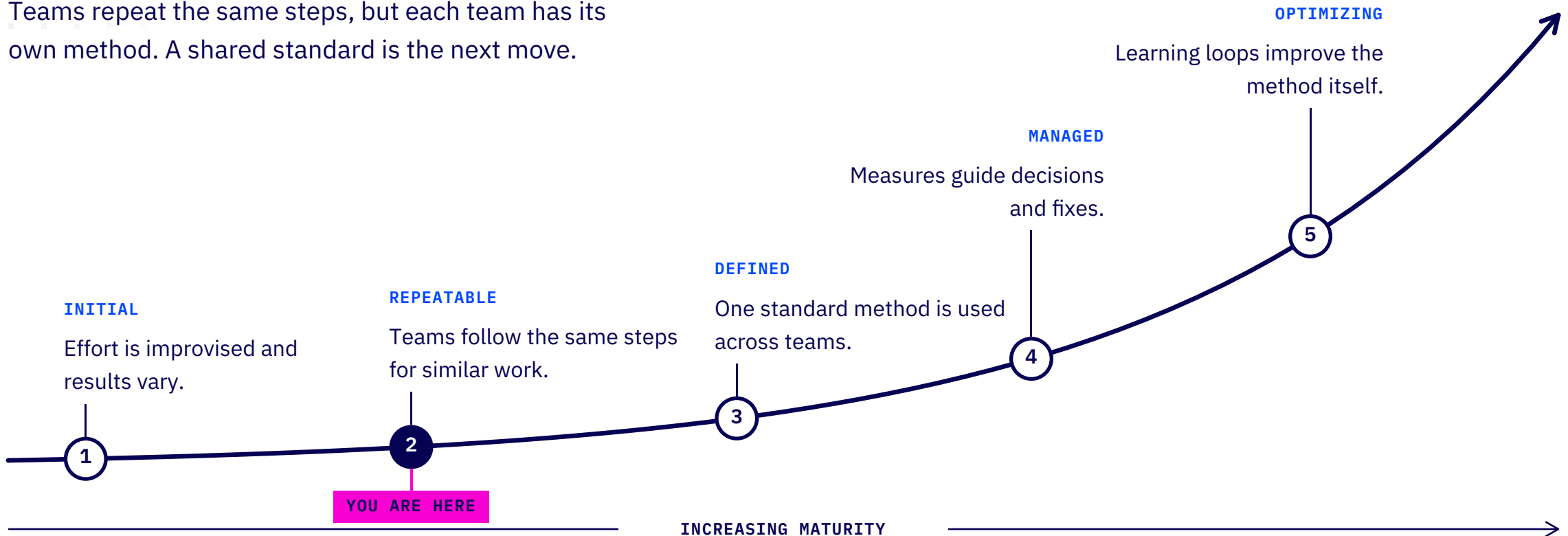
BREADTH ACROSS THE ORGANIZATION

Shaded cells are stages already reached.

CURRENT STATE

Northfield sits at stage two of five

Teams repeat the same steps, but each team has its own method. A shared standard is the next move.



The filled marker is where Northfield is today.

Four stages take the practice from ad hoc to optimized

Each stage builds on the last. Early stages take effort and show little; the climb steepens once practice is shared.



MATURITY MODEL

Most of the gain arrives after the inflection



INSIGHT 1

HABITS COME FIRST

Early stages cost effort and show little; hold the line until stage three.

INSIGHT 2

STANDARDS UNLOCK SCALE

A shared method is the inflection that makes later gains compound.

INSIGHT 3

MEASURES CLOSE THE LOOP

Optimizing starts when results feed back into the method.

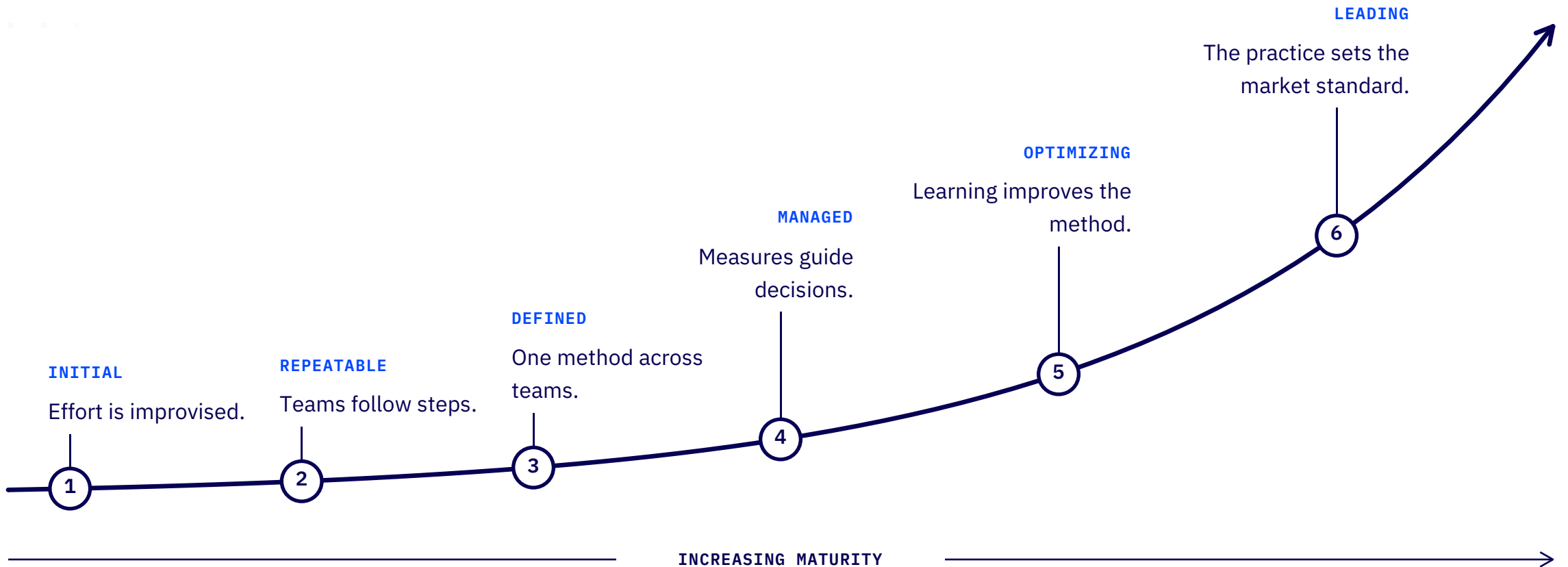
MATURITY MODEL

Four stages take the practice from ad hoc to optimized

Each stage builds on the last. Early stages take effort and show little; the climb steepens once practice is shared.



Six stages describe the full climb



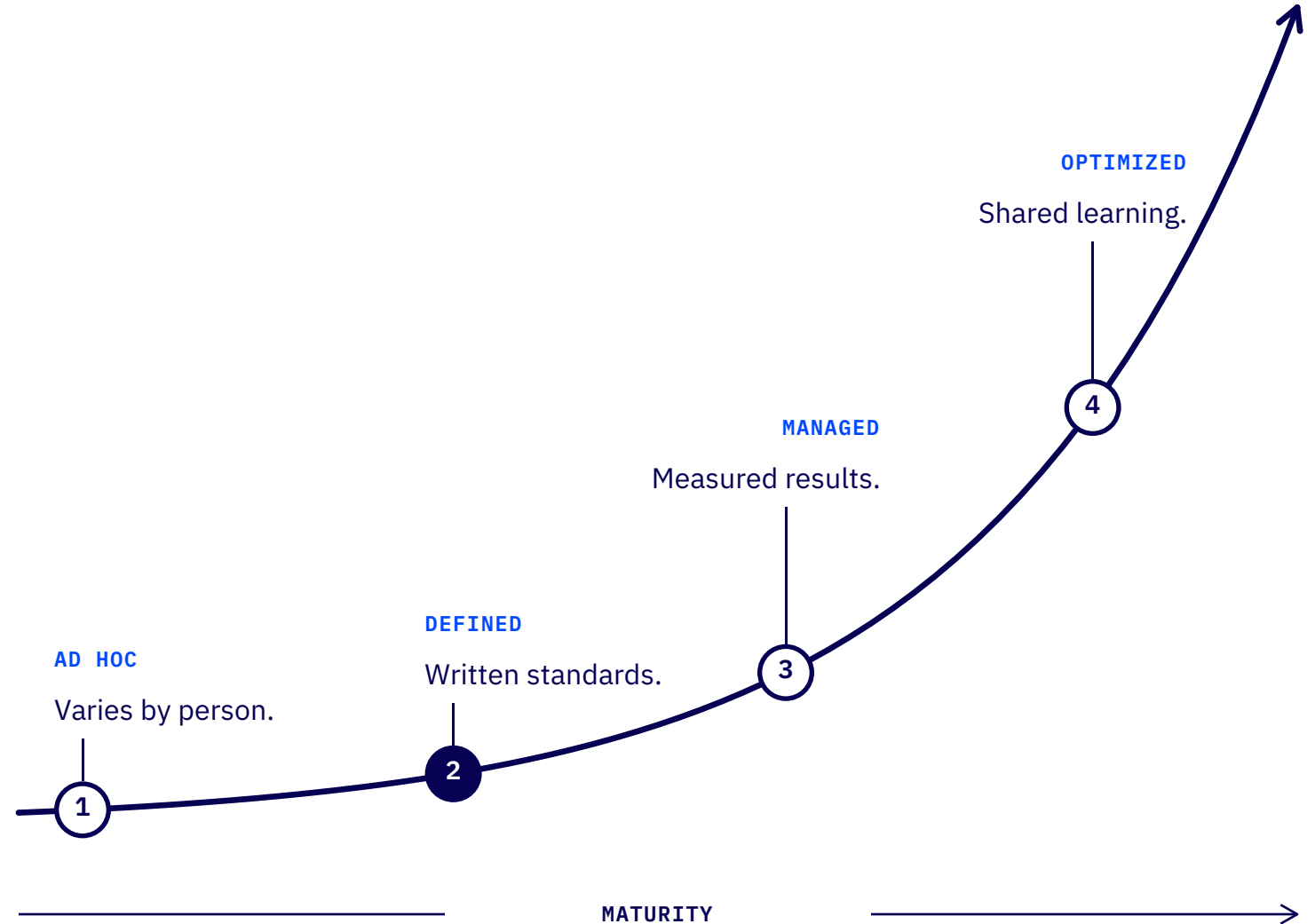
MATURITY MODEL

Most of the gain comes late in the climb

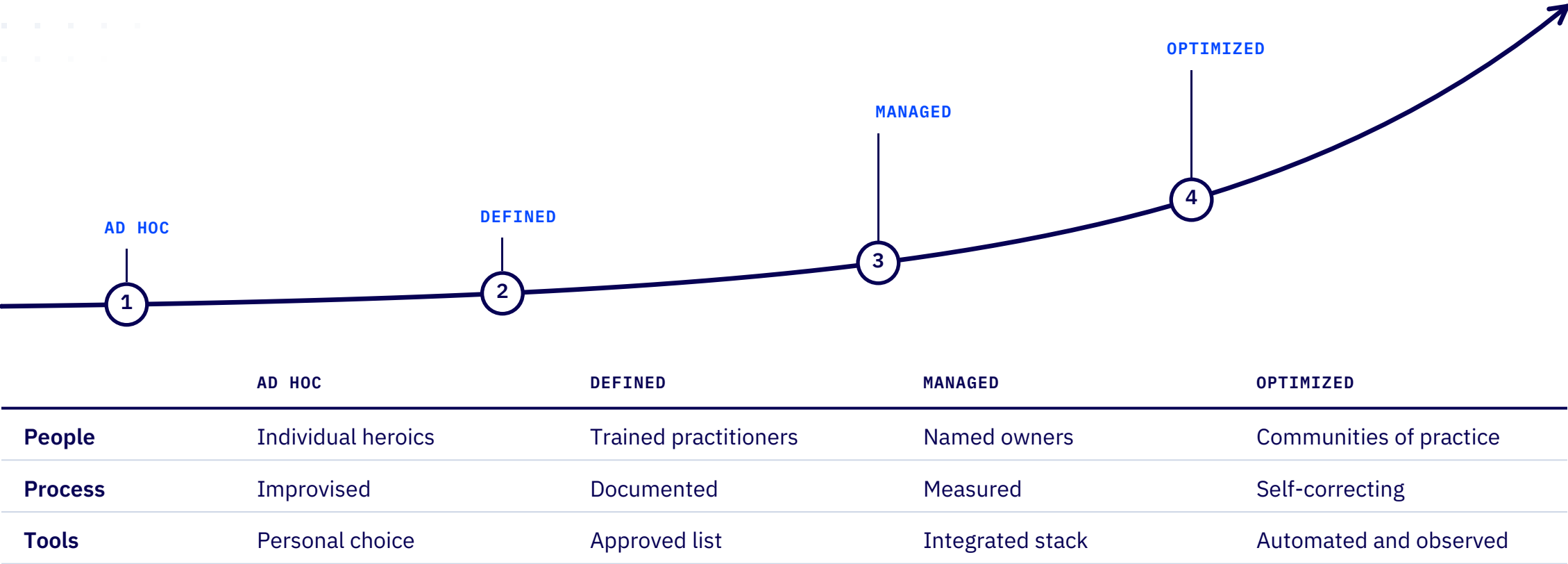
How to read it

The curve stays flat while teams build habits, then rises once the practice is shared.

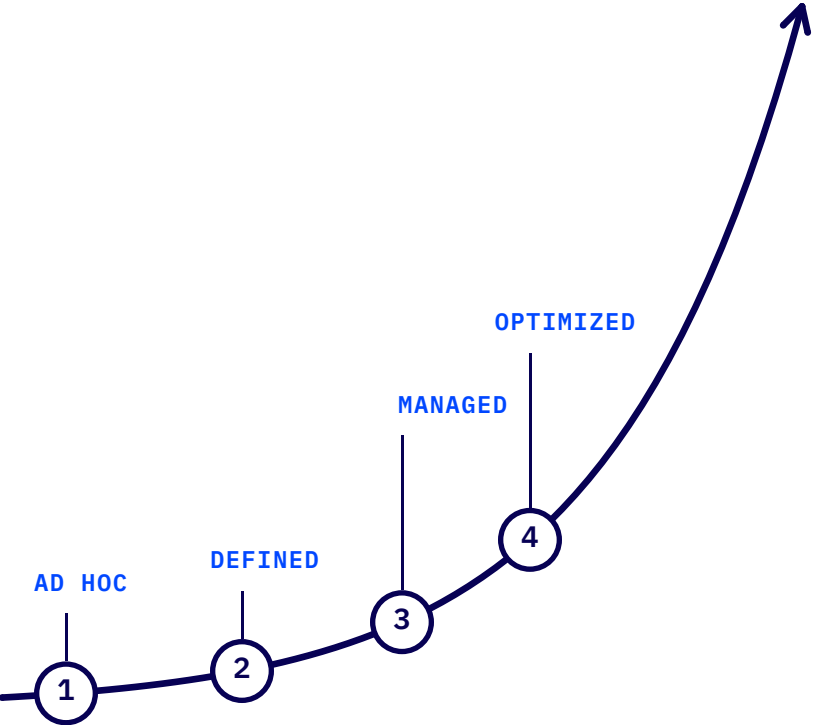
- Early stages cost effort and show little
- Stage three is the inflection
- Northfield is at stage two today



What changes at each stage

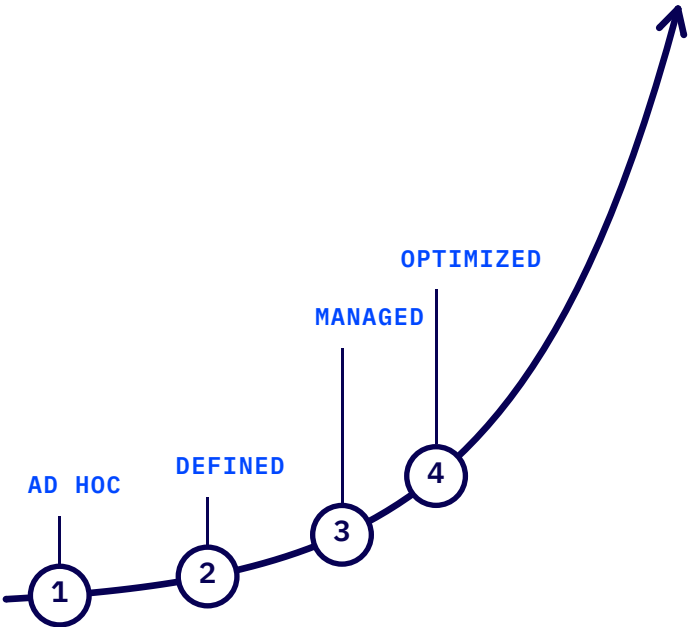


What people, process and tools look like at each stage



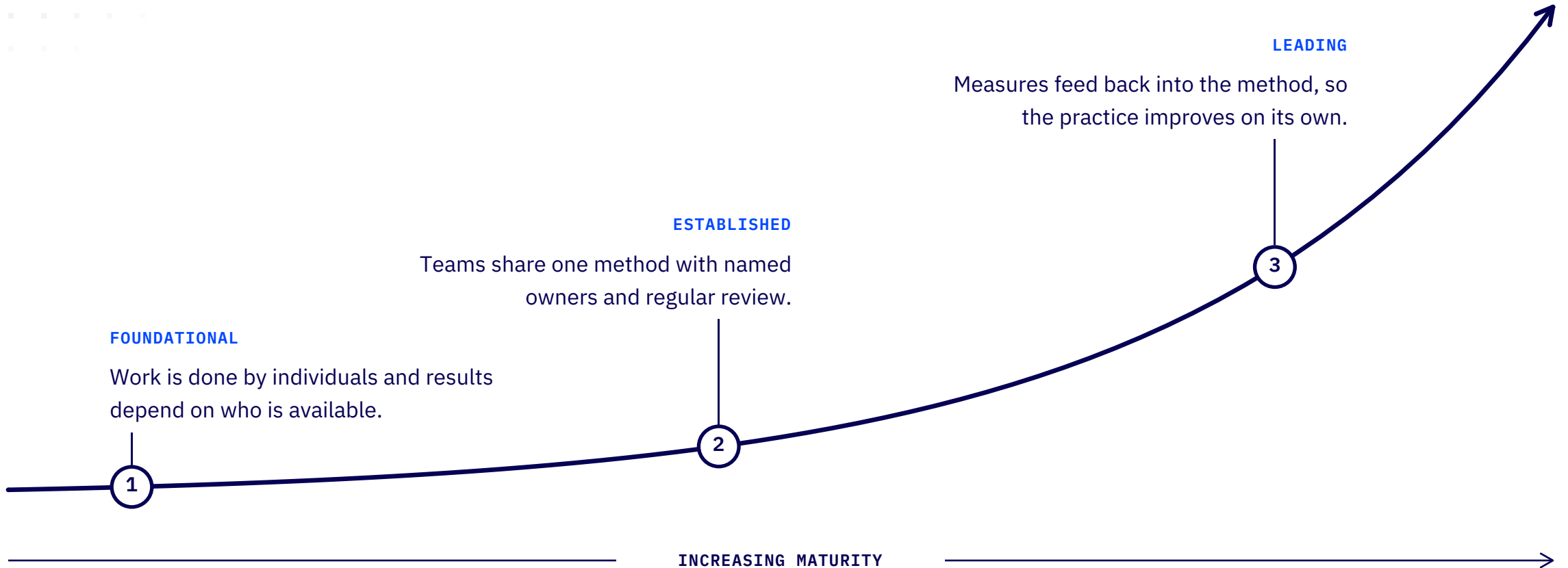
	PEOPLE	PROCESS	TOOLS
Ad hoc	Individual heroics	Improvised	Personal choice
Defined	Trained practitioners	Documented steps	Approved list
Managed	Named owners	Measured results	Integrated stack
Optimized	Communities of practice	Self-correcting	Automated and observed

What people, process and tools look like at each stage



	PEOPLE	PROCESS	TOOLS
Ad hoc	Individual heroics	Improvised, varies by person	Personal choice of tools
Defined	Trained practitioners	Documented, repeatable steps	Approved list of tools
Managed	Named owners per practice	Measured against targets	Integrated, shared stack
Optimized	Communities of practice	Self-correcting through review	Automated and observed

Three stages take a practice from habit to advantage



One method finds and ranks how work should change

PROCESS ANALYSIS ACCELERATOR

What it is, what you get, why it works

WHAT IT IS

A repeatable method that combines process artifacts, operating data, interviews and observation to decide how work should change.

WHAT YOU GET

- Evidence-based view of workflows, handoffs and exceptions
- Prioritized moves to simplify or automate work
- Quantified capacity and value, with assumptions
- Future-state workflows with roles and measures

WHY IT WORKS

- Every recommendation grounded in standards
- Faster discovery through a repeatable method
- Initiatives linked to adoption and Finance
- Outputs trace back to source evidence

#	STEP	WHAT HAPPENS
01	Intake	Scans and registers every available document.
02	Process discovery	Maps the operation into areas and processes.
03	Current-state analysis	Builds process maps and characteristics.
04	Pain points and waste	Finds what optimization and AI can address.
05	Redesign options	Options to eliminate, simplify or automate.
06	Quantify and prioritize	Scores value, capacity, feasibility and risk.
07	Transformation waves	Packages priorities into sequenced initiatives.

Three causes explain most of the delay

Close takes 12 days today. About 7 of them are spent waiting, not working.

- **Late reconciliations.** Balance-sheet accounts are reconciled after close, not during the month.
- **Manual intercompany.** Eleven entities match balances by email and spreadsheet.
- **Review at the end.** Controllers see most entries for the first time on day nine.

Source: West Monroe close calendar analysis, FY26, 14 business units.

CURRENT STATE

Three causes explain most of the delay

Close takes 12 days today. About 7 of them are spent waiting, not working.

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THE SHORT VERSION

Close takes 12 days today. About 7 of them are spent waiting, not working.

Source: West Monroe close calendar analysis, FY26, 14 business units.

Four roles, each with a clear job and a time commitment

01 Sponsor

TIME

2 hours a month

DOES

- Clears blockers
- Speaks at town halls
- Owns go/no-go
- Reviews adoption monthly

02 Change lead

TIME

Full time

DOES

- Runs the network
- Tracks adoption
- Reports weekly
- Escalates blockers

03 Site champion

TIME

2 hours a week

DOES

- Coaches peers
- Relays feedback
- Flags local risks
- Hosts a weekly huddle

04 Super user

TIME

1 hour a week

DOES

- First-line help
- Shares daily tips
- Logs issues
- Joins daily floor huddle

Ratio: one champion for every 35 staff.

Source: Northfield Health System change plan; sample figures for layout only.

A sponsor, a lead and 54 champions reach every floor



- 01 Sponsor**
Removes blockers, speaks at every town hall, owns the go/no-go vote.
- 02 Change lead**
Runs the network, tracks adoption and reports weekly to the steering committee.
- 03 Champions**
Two hours a week; coach peers, gather feedback, relay local risks.
- 04 Super users**
One hour a week; first-line help and a daily tip for their floor.

Each site champion leads a cell of four to five super users who sit on the floor and answer questions in real time.

Source: Northfield Health System change plan; sample figures for layout only.

AI changes how product and engineering operate

WHY AI CHANGES THE OPERATING MODEL

01

Speed changes the unit of work

Drafting, coding, testing and prototyping move faster, so teams can work in smaller, testable slices.

02

Ambiguity becomes the bottleneck

Unclear evidence, intent and acceptance criteria spread through delivery just as quickly.

03

The product interface is the advantage

Shared evidence, review and governance turn isolated gains into repeatable team performance.

HOW WE WILL RUN IT

Guide the process without taking over the work

- Short prompts, then participant work
- Named checkpoints and timeboxes
- Active coaching and unblocking
- Human approval at every gate
- Shared notes captured live
- Open risks logged for follow-up

WHY THIS MATTERS

AI changes how product and engineering operate

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WHAT PARTICIPANTS WILL UNDERSTAND

Six outcomes define the workshop

Frame the right opportunity

Pick a feasible problem

Turn evidence into intent

Stories and criteria

Agents across the SDLC

Plan, code, test, release

Product-to-R&D interface

Agree a demonstrable slice

Compress feedback loops

Prototype in parallel

Scale AI-enabled delivery

Adoption and governance

QUALIFICATIONS

We are deeply embedded across healthcare, with proven work in payer, provider and pharmacy

We deliver modernization through targeted offerings across the healthcare value chain.

TECHNOLOGY ALLIANCES

CLOUD PLATFORM	LAKEHOUSE
CRM	CORE ADMIN
INTEGRATION	BI TOOLS
CONTACT CENTER	IDENTITY
ERP	EHR
DATA QUALITY	GENERATIVE AI
SUPPLY CHAIN	PAYMENTS
REVENUE CYCLE	TELEHEALTH
CYBERSECURITY	INTEROPERABILITY

Industry offerings

PAYER

- Care management modernization
- Claims platform modernization
- Member digital strategy

PROVIDER

- Population health modernization
- Care experience design
- Clinical audit readiness

PHARMACY AND LIFE SCIENCES

- Clinical program management
- Manufacturing excellence
- Commercial enablement

Cross-functional capabilities

ADVANCED ANALYTICS AND AI

Generative AI accelerators · ML delivery · Data architecture · Cloud data strategy

EXPERIENCE STRATEGY

Service design · Persona development · CRM implementation · Market research

ENTERPRISE STRATEGY AND TECHNOLOGY

Application rationalization · IT cost optimization · ERP selection · Cybersecurity

PRODUCT AND WORKFORCE

Product strategy · Software engineering · DevOps · Org change management

Source: West Monroe engagement history.

OFFERINGS

Revenue cycle diagnostic

1 • WHAT WE WILL DO

Find where claims stall between registration and cash, size every leak and leave a fix list the revenue cycle team can start on. We work from 24 months of your own claims, not from benchmarks, and agree every number with Finance before it goes in the plan.

2 • HOW WE WILL DO IT • 6 WEEKS

01 • WEEKS 1 TO 2

Baseline

- Pull 24 months of claims by payer
- Map registration to cash steps
- Set KPI baselines with Finance

02 • WEEKS 3 TO 4

Diagnose

- Root-cause denials and underpayments
- Trace A/R aging by payer
- Validate leaks with billing leads

03 • WEEKS 5 TO 6

Size and plan

- Size each fix: low, base, high
- Sequence by effort and value
- Name owners and start weeks

\$145K fixed fee
6 weeks

Deliverables: KPI baseline, leak heat map, opportunity sizing, 90-day plan. Assumes a claims extract in week 1; no new tools.

Source: Sample figures for layout only; replace with current firm data before use.

PROOF • SAMPLE CLIENT

Sample case: 14 hospitals, 220 clinics.

18% fewer denials

\$11M annual cash opportunity

9d faster to cash

“We stopped debating the numbers and started fixing the top five leaks.”
VP Revenue Cycle, sample client

01 • FRAME • W1 TO 2

- Triage six sources; agree naming
- Set security and access standards
- Design the target model

02 • BUILD • W3 TO 7

- Pipelines for six sources, with tests
- Model the core with data owners
- Weekly review of lineage and quality

03 • PROVE • W8 TO 9

- Build two priority reports
- Validate numbers with Finance and clinical
- Fix gaps before go-live

04 • HAND OVER • W10

- Run book and monitoring
- Train 30 report builders
- Backlog for the next quarter

OFFERINGS

Data platform jumpstart, proven at a health system

1 • WHAT WE WILL DO

A governed starter platform on real Northfield data in ten weeks: six sources ingested, a tested core model and two live reports that finance and clinical leaders both use, built beside your team so you can extend it.

FIXED FEE

\$240K

fixed fee, 10 weeks
3 invoices, net 30

DELIVERABLES

- Platform blueprint and standards
- Tested core model, six sources
- Two live reports, run book

Team: lead, architect, two engineers, analyst. Assumes cloud tenant by week 2; up to six sources.

14

reports on one
platform

- 32%

less manual
rework

“One source we all trust.”
CFO, sample client

Source: Sample figures for layout only; replace with current firm data before use.

OFFERINGS

Revenue cycle diagnostic: three assets you keep

1 • WHAT WE WILL DO

Every diagnostic ends with three assets a CFO can act on: a heat map of where claims leak, a sized opportunity and a 90-day plan with owners.

2 • HOW WE WILL DO IT • 6 WEEKS

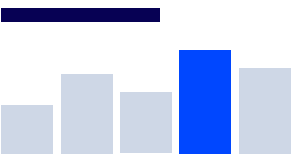
PHASE	WHAT WE DO	WHAT YOU GET
Baseline Weeks 1 to 2	• Pull 24 months of claims by payer • Map steps from registration to cash	KPI baseline
Diagnose Weeks 3 to 4	• Root-cause denials, underpayments, A/R • Validate each leak with billing leads	Leak heat map
Size and plan Weeks 5 to 6	• Size each fix: low, base and high case • Sequence fixes; name owners, start weeks	Sizing and 90-day plan

Source: Sample figures for layout only; replace with current firm data before use.

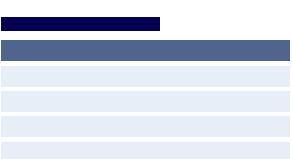
FIXED FEE

\$145K

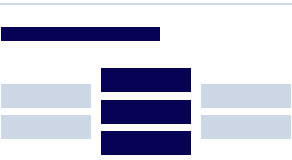
fixed fee, 6 weeks
2 invoices, net 30



Leak heat map: denials by payer and cause



Opportunity sizing: low, base, high per fix



90-day plan: owner, effort, start week

Team: lead, SME, two analysts. Assumes a claims extract in week 1.

OFFERINGS

AI readiness assessment: see what you get

1 • WHAT WE WILL DO

In eight weeks we assess data, people, risk and use cases across Northfield, and rank what to do first. You keep a scorecard, a ranked portfolio and a governance model you can act on.

2 • HOW WE WILL DO IT • 8 WEEKS

PHASE	WHAT WE DO	WHAT YOU GET
Align Weeks 1 to 2	- Sponsor goals, risk appetite; 20 interviews - Agree the scoring criteria and domains	Scoring model
Assess Weeks 3 to 5	- Scan data, platform and talent in five domains - Score each domain 1 to 5 against peers	Readiness scorecard
Prioritize, plan Weeks 6 to 8	- Rank 30 use cases; governance and risk model 90-day roadmap, business case, readout	Portfolio and roadmap

3 • FIXED FEE

\$185K

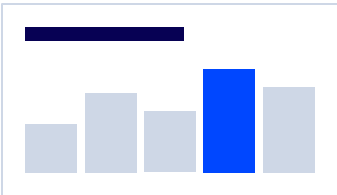
8 weeks, 2 invoices

VALUE

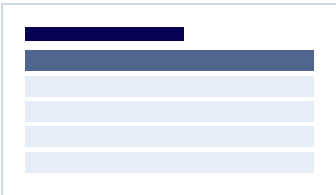
A funded first wave in 90 days, with risk owners named.

TEAM AND ASSUMPTIONS

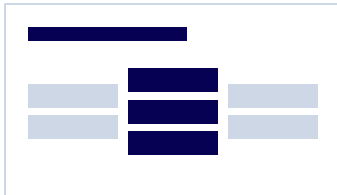
Lead, AI architect, two analysts.
Data access in week 1.



Sample scorecard: five domains scored 1 to 5, with a peer view



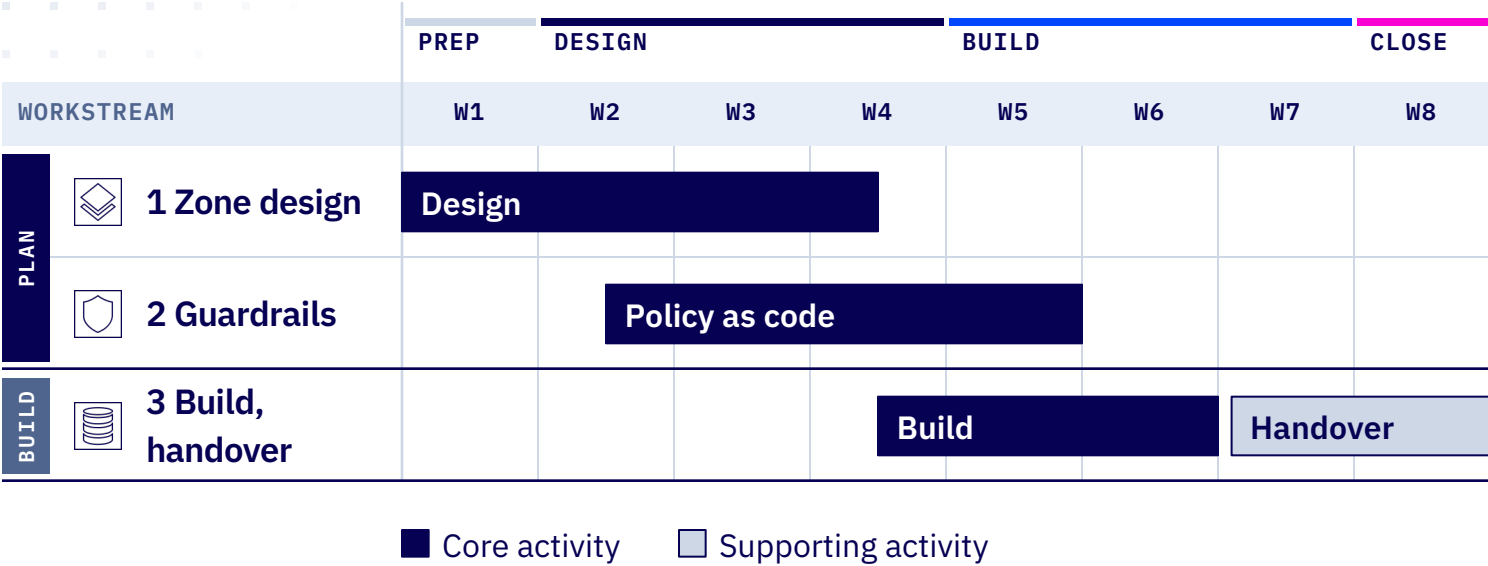
Sample portfolio: use cases ranked by value, risk and readiness



Sample governance model: roles, review gates and risk tiers

Source: Sample figures for layout only; replace with current firm data before use.

Cloud landing zone sprint: eight weeks to production



1 • ZONE DESIGN • W1 TO 4

- Network and account design
- Identity and tagging standards
- Three environments agreed

2 • GUARDRAILS • W2 TO 5

- Security baselines as code
- Drift and compliance checks
- Security lead sign-off

3 • BUILD, HANDOVER • W4 TO 8

- Pipelines tested in all three
- Operations team trained
- Readout and cost baseline

WHAT WE WILL DO

A secure, governed cloud foundation Northfield can build on: designed, built and tested in eight weeks, then handed over with runbooks.

FIXED FEE

\$210K fixed fee, 8 weeks
Invoiced at weeks 4, 8

Team: lead 25%, architect, two engineers

DELIVERABLES

- Landing zone in production
 - Policy-as-code library
 - Runbooks and cost baseline
- Assumes tenant and SSO ready in week 1

Source: Sample figures for layout only; replace with current firm data before use.

OFFERINGS

Six weeks to a clear scale or stop call

WHAT WE WILL DO

Test one AI use case on your own data before you commit to scale. We build and measure a working prototype, then give you a clear scale-or-stop call.

FIXED FEE

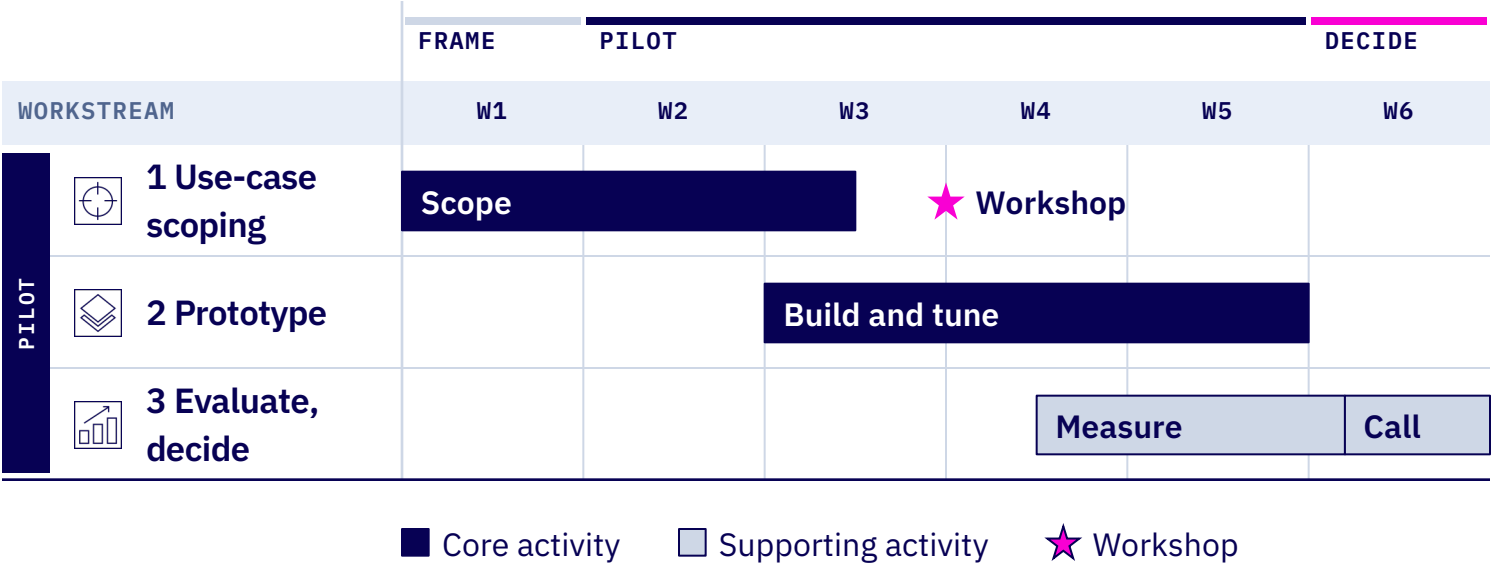
\$150K

fixed fee
6 weeks

DELIVERABLES

- Working prototype on Northfield data
- Evaluation scorecard
- Scale-or-stop recommendation

Source: Sample figures for layout only; replace with current firm data before use.



1 • SCOPING • W1 TO 3

- Pick one use case and success measure
- Map data, risks and clinical owners
- Workshop to agree the scope

3 • EVALUATE, DECIDE • W4 TO 6

- Score accuracy, safety and usability
- Test with 10 clinical users
- Scale-or-stop case with costed options

2 • PROTOTYPE • W3 TO 5

- Build on a slice of Northfield data
- Tune prompts, models and guardrails
- Weekly demo to the clinical lead

TEAM AND ASSUMPTIONS

- Lead 25%, engineer, analyst, advisor
- One use case chosen in week 1
- Clinical lead 4 hours a week

OFFERINGS

ERP readiness review: a costed roadmap and a clear go or stop call

1 • WHAT WE WILL DO

Know before you commit. In seven weeks we test how well your ERP fits Northfield's finance and supply processes, and give the board a costed roadmap and a clear go, wait or stop call.

3 • FIXED FEE

\$130K 7 weeks, 2 invoices

Team: lead, ERP architect, two analysts. Needs a named business owner in week 1.

Source: Sample figures for layout only; replace with current firm data before use.

PHASE	WHAT WE DO	WHAT YOU GET
Scope Week 1	- Confirm goals, scope and sponsors - Inventory 14 interfaces and modules - Agree success criteria	Scope and criteria
Assess Weeks 2 to 3	- Review finance and supply processes - Rate fit gaps by module - Interview 15 process owners	Fit-gap register
Design Weeks 4 to 5	- Draft target process and data model - Map integrations and cutover risks - Size the data clean-up	Target-state blueprint
Plan Week 6	- Sequence waves and release plan - Build the budget and staffing case - Set governance and decision rights	Roadmap, budget, risks
Decide Week 7	- Present options to the steering group - Resolve open decisions - Agree the go, wait or stop call	Decision readout

AI readiness assessment: one page, one fixed fee

1 • WHAT WE WILL DO

Health systems are piloting AI faster than they can govern it. In eight weeks we assess data, people, risk and use cases across Northfield, rank what to do first and leave a funded, sequenced first wave.

FIXED FEE

\$185K

fixed fee, 8 weeks
2 invoices, net 30

2 • HOW WE WILL DO IT • 8 WEEKS

01 • WEEKS 1 TO 2

Align

- Confirm sponsor goals and risk appetite
- 20 stakeholder interviews
- Agree scoring criteria

DELIVERABLES

Readiness scorecard, ranked use-case portfolio, governance model, 90-day roadmap

02 • WEEKS 3 TO 5

Assess

- Scan data, platform and talent against five domains
- Score each domain 1 to 5
- Compare with peer systems

03 • WEEKS 6 TO 7

Prioritize

- Long-list 30 use cases with clinical and finance leads
- Rank on value, risk and readiness
- Agree the top ten

ASSUMPTIONS

Data access and a data owner in week 1; sponsors decide in five days; no model build

04 • WEEK 8

Plan

- Governance and risk model with review gates
- 90-day roadmap with owners
- Business case and readout

Source: Sample figures for layout only; replace with current firm data before use.

3 • COST

\$240K

fixed fee, 10 weeks

Three milestone invoices

TEAM OF FIVE

Lead 25% • architect 100% • two engineers 100% • governance analyst 50%

ASSUMPTIONS

Cloud tenant ready by week 2; up to six source systems; Northfield owns licenses

OFFERINGS

Data platform jumpstart: a governed starter platform in ten weeks

1 • WHAT WE WILL DO

A governed starter platform on real Northfield data: six sources ingested, a modeled core and two priority reports that finance and clinical leaders both use, built beside your team.

2 • HOW WE WILL DO IT • 10 WEEKS

STAGE	WHAT WE DO	OUTPUT
Frame Weeks 1 to 2	• Triage six sources; agree naming and security • Design the target model	Platform blueprint
Build Weeks 3 to 7	• Build pipelines for six sources; write data tests • Model the core; review weekly with data owners	Tested core model
Prove Weeks 8 to 10	• Build two priority reports; validate with users • Train users and hand over the run book	Two live reports, run book

Source: Sample figures for layout only; replace with current firm data before use.

OFFERINGS

Cloud cost review: four weeks, \$75K fixed

1 • WHAT WE WILL DO

A cloud cost review finds the 20 to 30 percent of Northfield's cloud spend that nobody owns. In four weeks we map every dollar to an owner, find the waste and commitment gaps, and leave a ranked savings backlog.

2 • HOW WE WILL DO IT • 4 WEEKS

01 • WEEK 1

Collect

- Read-only billing and usage access
- Pull 12 months of spend data
- Agree the account hierarchy

02 • WEEK 2

Map

- Tag and map spend to owners
- Flag untagged and shared cost
- Validate with account owners

03 • WEEK 3

Analyze

- Find idle and oversized assets
- Check commitment coverage
- Estimate savings per lever

04 • WEEK 4

Decide

- Rank savings by effort and risk
- Draft tagging and budget model
- Readout for the CFO and CIO

3 • FIXED FEE

\$75K 4 weeks

DELIVERABLES	TEAM	ASSUMPTIONS
Spend map by service and owner; ranked savings backlog	Lead, FinOps analyst, cloud engineer	Billing access in week 1; up to 40 accounts; savings are estimates

Source: Sample figures for layout only; replace with current firm data before use.

Revenue cycle diagnostic in six weeks

1 • WHAT WE WILL DO

Revenue leaks in small places. We trace claims from registration to cash, size every leak and give the revenue cycle team a ranked fix list with owners.

2 • HOW WE WILL DO IT • 6 WEEKS

PHASE	WHAT WE DO	WHAT YOU GET
Baseline Weeks 1 to 2	• Pull 24 months of claims by payer • Map front, mid and back office steps • Set KPI baselines with Finance	KPI baseline pack
Diagnose Weeks 3 to 4	• Root-cause denials and underpayments • Trace A/R aging by payer and cause • Validate leaks with billing leads	Leak heat map
Size and plan Weeks 5 to 6	• Size each fix: low, base, high • Sequence by effort and value • Name owners and start weeks	Opportunity sizing and 90-day plan

Source: Sample figures for layout only; replace with current firm data before use.

3 • COST

\$145K

fixed fee, 6 weeks
Invoiced at weeks 3 and 6

VALUE

1.5% net revenue opportunity, typical range; quick-win denial fixes in 60 days

TEAM

Lead 25% • SME 100% • two analysts 100%

Assumes: claims extract by week 1; one payer-mix view

Finance close accelerator: what, how and what it costs

1 • WHAT

The offer and what you keep

Close slips because reconciliations, approvals and journal entries run in sequence. We map the close, remove the longest delays and put a five-day target calendar in place, so Finance spends days on analysis, not on chasing.

WHAT YOU KEEP

- Close map with a delay heat map
- Redesigned close calendar and RACI
- Approval rules, dashboard, playbook

2 • HOW

Three phases, ten weeks

01 • WEEKS 1 TO 3

Diagnose

- Time every close task end to end
- Interview 12 preparers, approvers
- Rank the 10 longest delays

02 • WEEKS 4 TO 7

Redesign

- Parallelize reconciliations
- Set approval thresholds and SLAs
- Pilot the calendar on one close

03 • WEEKS 8 TO 10

Embed

- Run a full close on the new design
- Train 40 preparers and approvers
- Hand over dashboard and playbook

3 • COST

\$120K

fixed fee

Ten weeks, three milestone invoices at weeks 3, 7 and 10

Fees exclude expenses; net 30

TEAM AND ASSUMPTIONS

Lead 25%, close SME and analyst full time. Needs ledger access in week 1.

Source: Sample figures for layout only; replace with current firm data before use.

OFFER	HOW WE DO IT	FIXED FEE
AI readiness scan Strategy	. Interview 8 leaders . Scan data, risk, talent . Rank top five use cases	\$65K 3 weeks
AI readiness assessment Strategy	. Score five domains vs. peers . Rank 30 use cases . Governance and 90-day plan	\$185K 8 weeks
Revenue cycle diagnostic Finance	. Baseline 24 months of claims . Root-cause denials and A/R . Size fixes; 90-day plan	\$145K 6 weeks
Data platform jumpstart Data	. Frame sources and standards . Build core model, six sources . Prove with two live reports	\$240K 10 weeks
Cloud landing zone sprint Cloud	. Design zone and guardrails . Build and test pipelines . Train and hand over	\$210K 8 weeks
Cloud cost review Cloud	. Pull billing and usage data . Map spend to owners . Rank savings by effort	\$75K 4 weeks

OFFERINGS

Six packaged offers, one fixed fee each

WHAT WE WILL DO

Pick a package, not a proposal. Each offer is a fixed fee for a fixed time, with named deliverables.

EVERY OFFER INCLUDES

- A named lead and team
- Weekly sponsor check-in
- Readout and handover pack

HOW OFFERS CHAIN

- Scan, then assessment, then pilot
- Diagnostic, then jumpstart

Assumes data access in week 1. Fees exclude expenses.

Source: Sample figures for layout only; replace with current firm data before use.

Cloud landing zone: pick the package that fits

1 • WHAT WE WILL DO

Three packages for a secure, governed cloud foundation: review it, build it, or build it and move the first workloads.

2 • HOW WE DO IT

- Review the estate against the design
- Design and build the landing zone, guardrails as code
- Prove it on workloads; hand over

ASSUMPTIONS

- Tenant and SSO ready in week 1
- Up to three environments
- Northfield holds the licenses

WHAT IS INCLUDED	A • REVIEW	B • SPRINT	C • PROGRAM
Current-state review	✓	✓	✓
Target landing zone design	✓	✓	✓
Security guardrails as code	—	✓	✓
Production landing zone	—	✓	✓
Runbooks and cost baseline	—	✓	✓
Migration wave plan	—	—	✓
First two waves migrated	—	—	✓
Hypercare, 60 days	—	—	✓
FIXED FEE Team: lead plus architects and engineers	\$75K 3 weeks, fixed	\$210K 8 weeks, fixed	\$480K 16 weeks, fixed

Source: Sample figures for layout only; replace with current firm data before use.

Three ways to start with AI, each a fixed fee

SKU 1 • SCAN

AI readiness scan

Where do we stand? A rapid read of data, risk and talent.

HOW WE DO IT

- Interview 8 leaders
- Scan data, risk and talent
- Rank the top five use cases

YOU KEEP

Scorecard, top five use cases, two-page summary

\$65K

fixed fee, 3 weeks · lead, analyst

SKU 2 • ASSESSMENT

MOST CHOSEN

AI readiness assessment

What should we do first? Full portfolio, governance and roadmap.

HOW WE DO IT

- Align on goals; 20 interviews
- Score five domains vs. peers
- Rank 30 use cases; plan 90 days

YOU KEEP

Scorecard, ranked portfolio, governance model, roadmap

\$185K

fixed fee, 8 weeks · lead, architect, 2 analysts

SKU 3 • PILOT

AI use-case pilot

Can it work here? Scope, build and test one use case.

HOW WE DO IT

- Scope one use case; set measures
- Build and tune on your data
- Evaluate; scale-or-stop call

YOU KEEP

Working prototype, scorecard, scale-or-stop case, handover pack

\$150K

fixed fee, 6 weeks · lead, engineer, analyst

Source: Sample figures and names for layout only; replace with current firm data and catalog before use.

OFFERINGS

Managed cloud: one team, one monthly run-rate

1 • WHAT WE WILL DO

Run the Northfield cloud estate for you: patching, security posture, cost and a monthly roadmap session, delivered by one named team on one monthly run-rate.

Up to 40 cloud accounts; three-month minimum, then 30 days' notice.

TEAM MONTHLY RUN-RATE

\$88K per month, whole team
In arrears

2 • HOW WE WILL DO IT • EVERY MONTH

01 • WEEK 1

Operate

- Patch and upgrade cycle
- Incident and ticket review
- Backup and recovery checks

02 • WEEK 2

Secure

- Posture scan and findings
- Access and key reviews
- Fix plan with owners

03 • WEEK 3

Optimize

- Cost and tagging review
- Right-size and commit decisions
- Savings tracked to target

04 • WEEK 4

Plan

- Health report for the CIO
- Roadmap and backlog session
- Next-month priorities

TEAM	Cloud lead · 25%	Platform engineer · 100%	Security engineer · 50%	FinOps analyst · 50%
	Service owner	Operate, patch	Posture, access	Cost, tagging

Source: Sample figures for layout only; replace with current firm data before use.

A four-month data team, one \$96K monthly run-rate

1 • WHAT WE WILL DO

A standing team that ships a governed data platform in four months and leaves Northfield able to run it. We build, you learn by pairing.

Assumes the cloud tenant is ready in month 1; four-month minimum term.

TEAM MONTHLY RUN-RATE

\$96K

per month, whole team
Billed monthly

2 • HOW WE WILL DO IT • FOUR MONTHS

01 • MONTH 1	02 • MONTH 2	03 • MONTH 3	04 • MONTH 4
Frame and mobilize - Stand up the team and the working rhythm - Triage sources; agree standards - Design the target platform	Build the core - Build ingestion and the modeled core - Write data quality tests - Pair with Northfield engineers	Prove with reports - Deliver two priority reports - Validate numbers with Finance - Stand up catalog and ownership	Hand over - Run book and on-call guide - Train owners and report builders - Backlog for the next six months
TEAM Engagement lead · 25% Steering, quality	Data architect · 100% Platform design	Data engineers · 100% Pipelines, tests	Governance analyst · 50% Standards, catalog

Source: Sample figures for layout only; replace with current firm data before use.

Three ways to connect 14 systems; the bus wins

OPTION A

Keep point-to-point

PROS

- Known to every interface team
- No new platform to run

CONS

- Feeds grow with every new system
- Failures hard to trace

2.6 / 5

Fit 3 · Cost 2 · Risk 2

OPTION B

Managed event bus

PROS

- One place to monitor and replay
- New systems connect in weeks

CONS

- A new service to operate
- Event design skills needed

4.1 / 5

Fit 5 · Cost 3 · Risk 4

CHOSEN

OPTION C

Integration platform suite

PROS

- Rich tooling and connectors
- Vendor-supported runtime

CONS

- Highest licence cost
- Locks flows into one product

3.3 / 5

Fit 4 · Cost 2 · Risk 3

Why we chose it: Option B fits 14 systems at a manageable cost and removes the feed growth behind most incidents.

The event bus scores highest on speed and resilience

CRITERION	A · POINT-TO-POINT	B · EVENT BUS	C · INTEGRATION SUITE
Speed to connect a new system	Each feed takes a quarter	Subscribe in weeks	Connectors speed set-up
Three-year run cost	Low build, rising upkeep	New service to operate	Highest licence cost
Resilience and replay	Failures hard to trace	One place to replay	Vendor-supported
Skills Northfield has today	Known to every team	Event design to learn	Product training needed

Recommendation: Option B scores 4.1 of 5 against 2.6 and 3.3. Retire the 61 feeds in three waves.

Source: Scores from the 4 September architecture workshop, 1 (weak) to 5 (strong).

Weighting the criteria puts the event bus clearly ahead

CRITERION	WEIGHT	A • P2P	B • BUS	C • SUITE
Speed to connect a system	30%	2	5	4
Three-year run cost	25%	3	3	1
Resilience and replay	20%	2	5	4
Skills Northfield has today	15%	5	3	3
Audit and compliance	10%	2	4	4
Weighted score	100%	2.6	4.1	3.3

WHY WE CHOSE IT

Option B, the event bus

It leads on speed and resilience, which carry half the weight, and ties Option A on cost.

- Cost is level with Option A over three years
- Skills gap is closed by a two-quarter ramp-up
- Replay supports audit requests
- Fits the existing cloud contract

Scores run 1 (weak) to 5 (strong); weights were set before scoring.

Three programs cut delivery time and operating cost

REGIONAL HEALTH SYSTEM

01 Month-end close rewrite

A critical close application was assessed and rebuilt in under half the usual time.

12 → 5

months to deliver

58%

faster delivery

BUSINESS VALUE

Protected the close calendar and cut deadline risk.

MULTI-HOSPITAL NETWORK

02 Claims data platform

A claims platform was modernized across six hospitals without interrupting billing.

6

hospitals live

700K

lines modernized

BUSINESS VALUE

Cutover risk down, with a scalable base for payer growth.

PHYSICIAN SERVICES GROUP

03 Billing and reporting

A legacy reporting stack was redesigned to deliver payer reports faster and cheaper.

65%

faster reports

\$30K

saved per client

BUSINESS VALUE

Less rework, faster payer reports, a platform that scales.

Source: Illustrative client outcomes; baselines are directional and should be validated before external use.

General availability

Make governance executable

Scale depends on clear decision rights, auditable controls and operational handoffs, not one-time enablement.

- Named approvers per decision
- SLA tracking on every request
- Access controls by pattern
- Adoption measured by team
- Exceptions reviewed quarterly

Governance runs through rollout and optimization

TWO CRITICAL PATHWAYS TO OPERATIONALIZE PLATFORM GOVERNANCE

01 Request intake: team to approved pattern access



02 Use-case intake: idea to governed build



Result: controlled scale, approval-only tool access and a repeatable model.

1 Designed to align with Northfield's governance structure, data enablement team and operational support functions.

Three patterns define the lakehouse role

Pattern 1: Lakehouse as the analytics layer over the existing warehouse

Curated warehouse → Semantic services → BI layer

ROLE: Native analytics and consumption over data governed in the warehouse

Pattern 2: Lakehouse as the consumption layer for the engineering platform

Engineering platform → Curated layer → BI layer

ROLE: Business consumption while engineering and curation stay put

Pattern 3: End-to-end lakehouse platform

Ingestion and engineering → Lakehouse → Semantic and reporting

ROLE: One platform for qualified net-new workloads

Key decisions to validate

- Data access versus movement
- Semantic model ownership
- Security and lineage

Key decisions to validate

- Open formats and shortcuts
- Metadata and lineage sync
- Curated layer ownership

Key decisions to validate

- Fit with placement criteria
- Capacity and isolation
- Engineering standards

Source: West Monroe analysis of the Northfield data estate.

What is invoiced each month, and on what terms

CHARGE	MONTH 1	MONTH 2	MONTH 3	MONTH 4	MONTH 5	MONTH 6
Milestone fees	\$216K	\$288K	\$216K	-	-	-
Team run-rate	-	-	-	\$210K	\$210K	\$210K
Total invoiced	\$216K	\$288K	\$216K	\$210K	\$210K	\$210K

01 On acceptance

Milestone fees are invoiced when Northfield accepts.

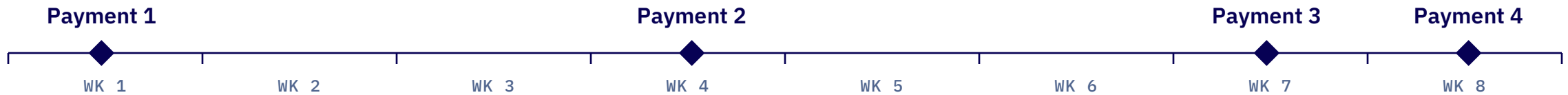
02 Monthly in arrears

The team run-rate is invoiced after each month.

03 Net 30

Expenses at cost, capped at 8% of fees.

Four payments, each tied to an accepted milestone



PAYMENT 1 • WEEK 1

**Mobilization
complete**

\$72K

10% of fixed fee

PAYMENT 2 • WEEK 4

Diagnostic readout

\$216K

30% of fixed fee

PAYMENT 3 • WEEK 7

Design sign-off

\$288K

40% of fixed fee

PAYMENT 4 • WEEK 8

**Final readout
accepted**

\$144K

20% of fixed fee

MILESTONE AND ACCEPTANCE CRITERION	TARGET DATE	% OF FEE	AMOUNT
Mobilization complete Team onboarded; plan accepted	9 Jan	10%	\$72K
Diagnostic readout Findings accepted by CFO	6 Feb	10%	\$72K
Target design approved Design signed off by steering	6 Mar	10%	\$72K
Business case accepted Case validated by Finance	27 Mar	10%	\$72K
Wave 1 build complete Passes unit and integration test	1 May	10%	\$72K
User acceptance passed No open severity 1 or 2 defects	29 May	10%	\$72K
Wave 1 go-live Close completes in new system	26 Jun	15%	\$108K
Hypercare exit Stable 30 days; runbooks accepted	24 Jul	10%	\$72K
Final readout accepted Outcomes accepted by sponsor	28 Aug	15%	\$108K
Total fixed fee		100%	\$720K

COMMERCIALS

Nine payments, one per accepted milestone

PAYMENT TERMS

Net 30 from the invoice date, paid by electronic transfer.

INVOICING

Raised only after written acceptance of the milestone.

HOLDBACK

10% of each payment is held until the final readout.

ACCEPTANCE

Five business days to review; silence counts as acceptance.

COMMERCIALS

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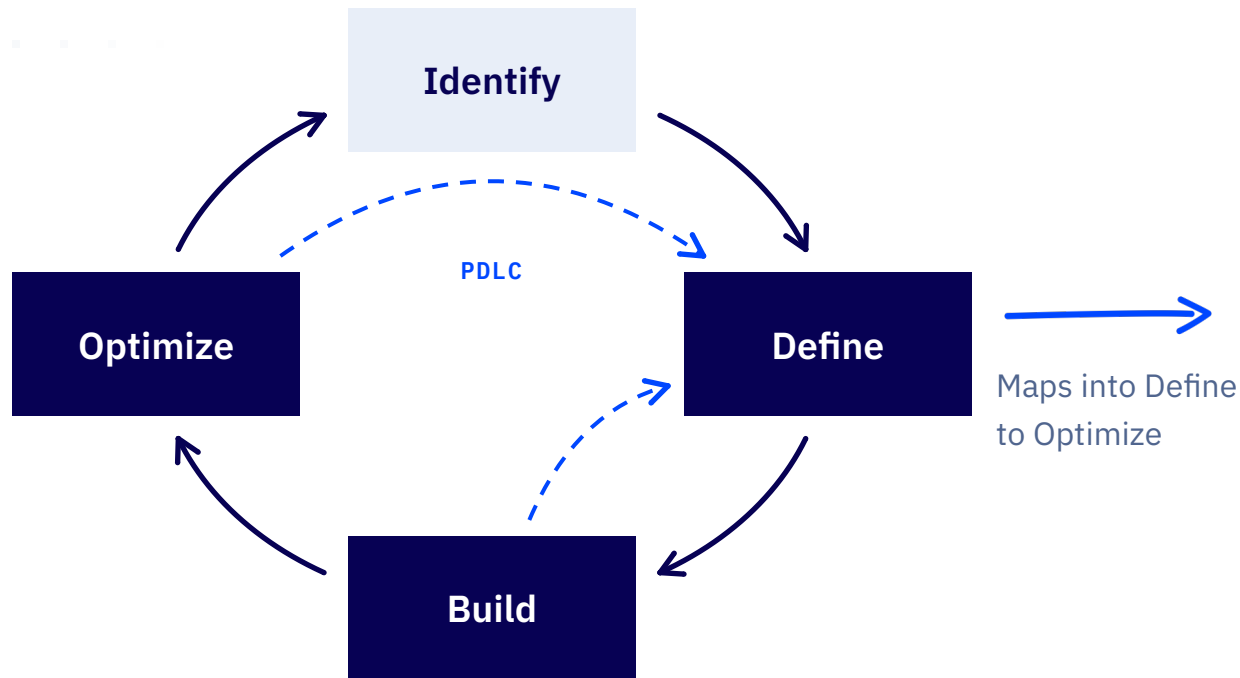
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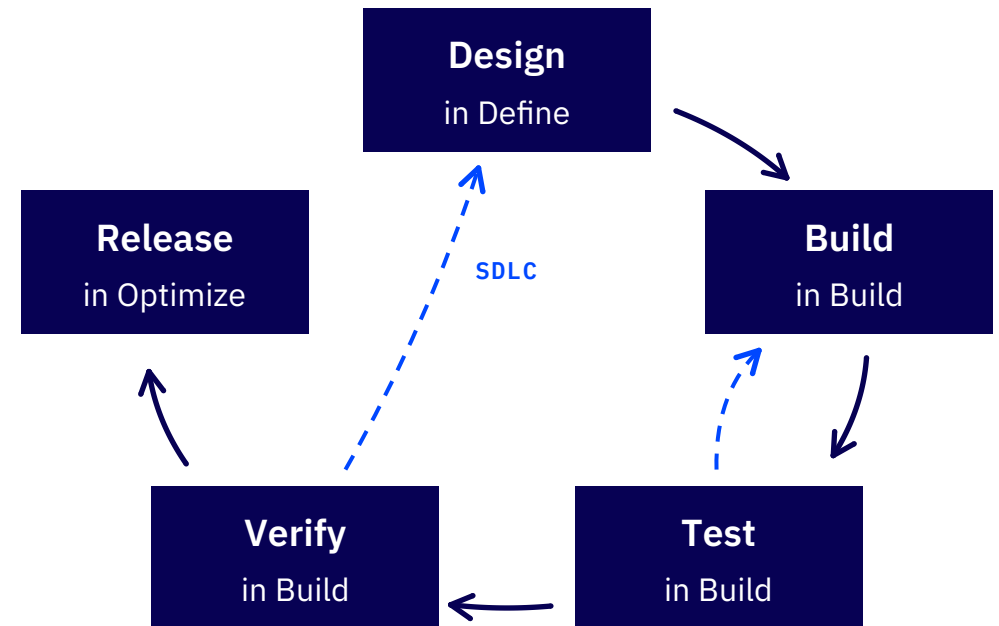
The SDLC is the slice of the PDLC from Define to Optimize

PDLC: THE SUPERSET



Four phases that loop back. Dark phases are where the SDLC runs.

SDLC: A SUBSET



Five engineering steps that loop back on defects and gaps. Tags show the PDLC phase.

HOW TO READ IT

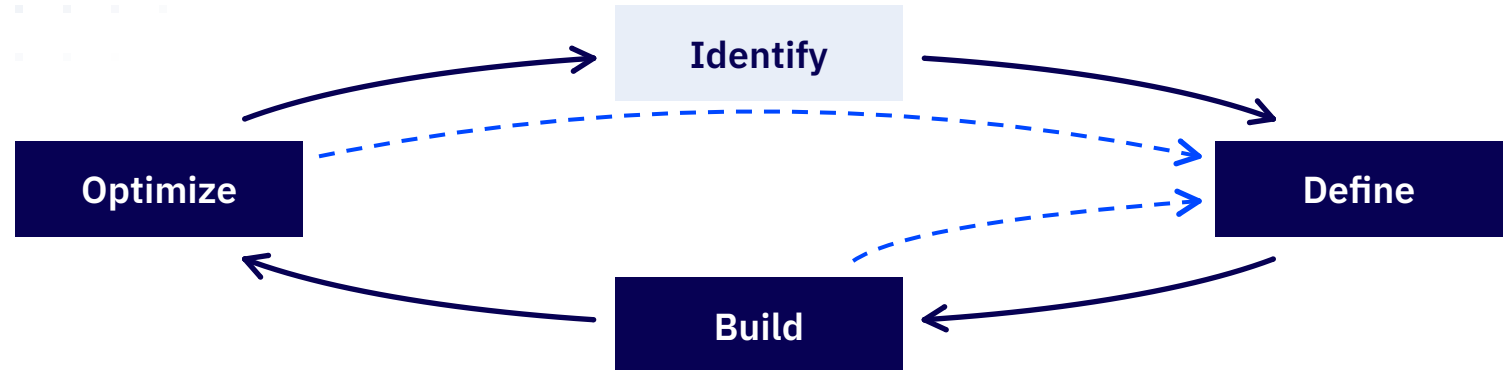
PDLC is the superset; the SDLC is a subset.

The SDLC runs from Define to Optimize. Dark phases are where it applies; both loop back.

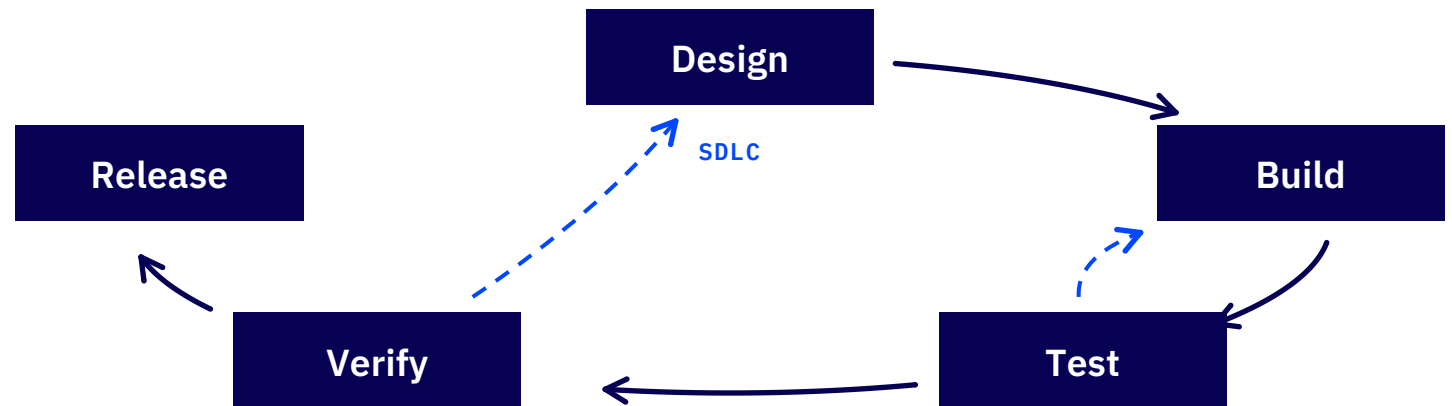
PRODUCT LIFECYCLE

Where the SDLC sits inside the PDLC

PDLC: THE SUPERSET, FOUR PHASES THAT LOOP BACK

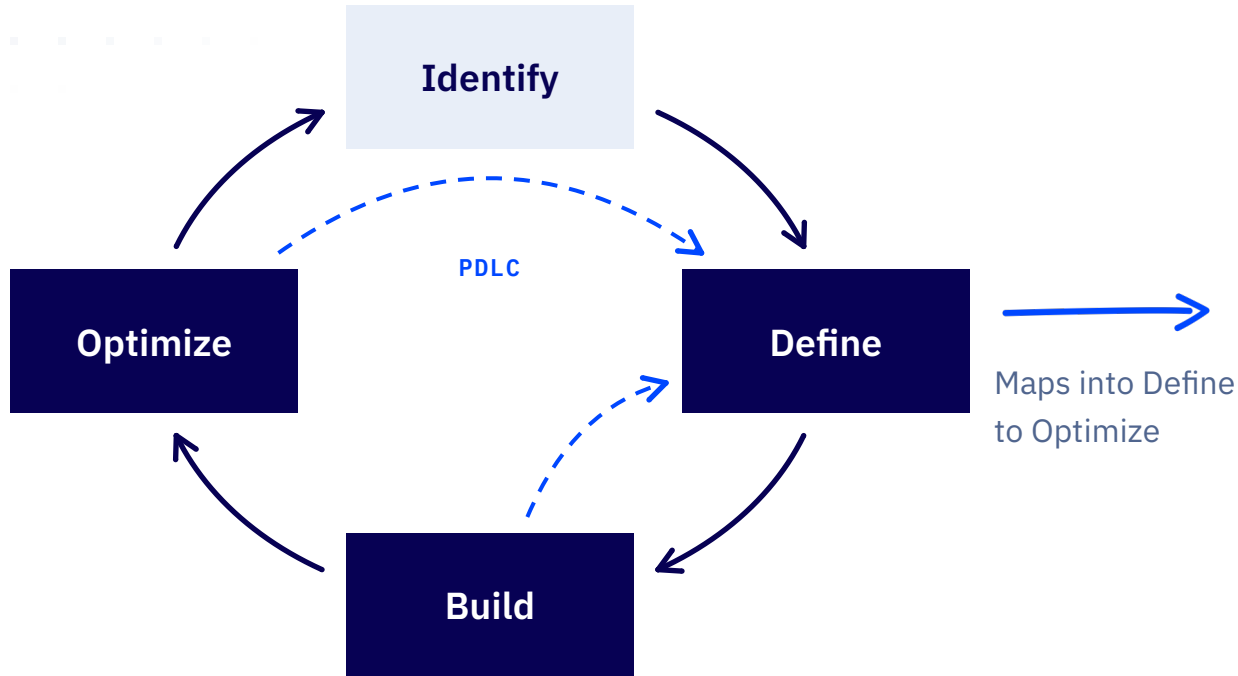


SDLC: A SUBSET, DEFINE TO OPTIMIZE



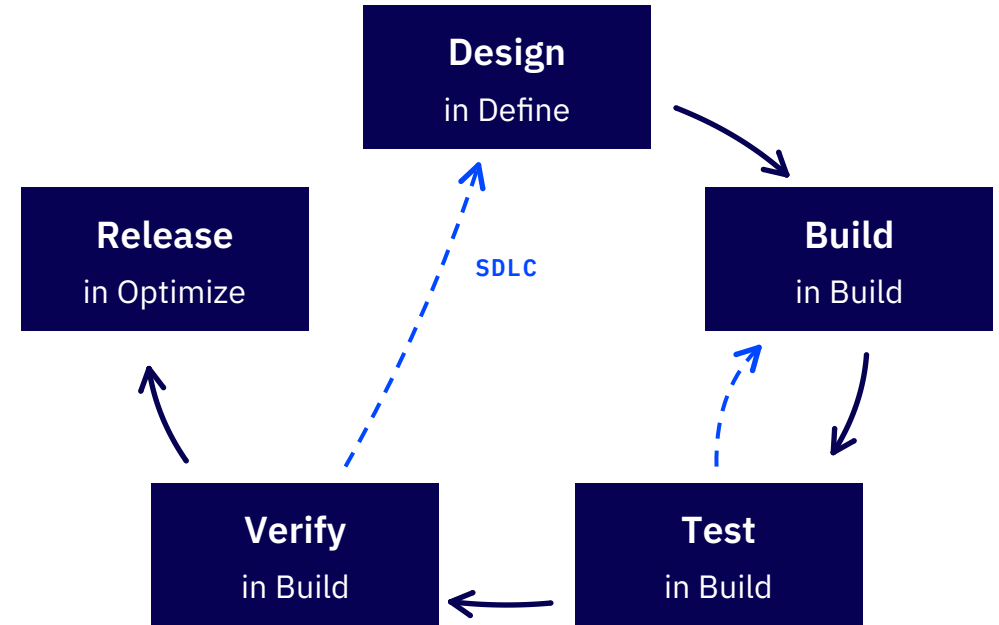
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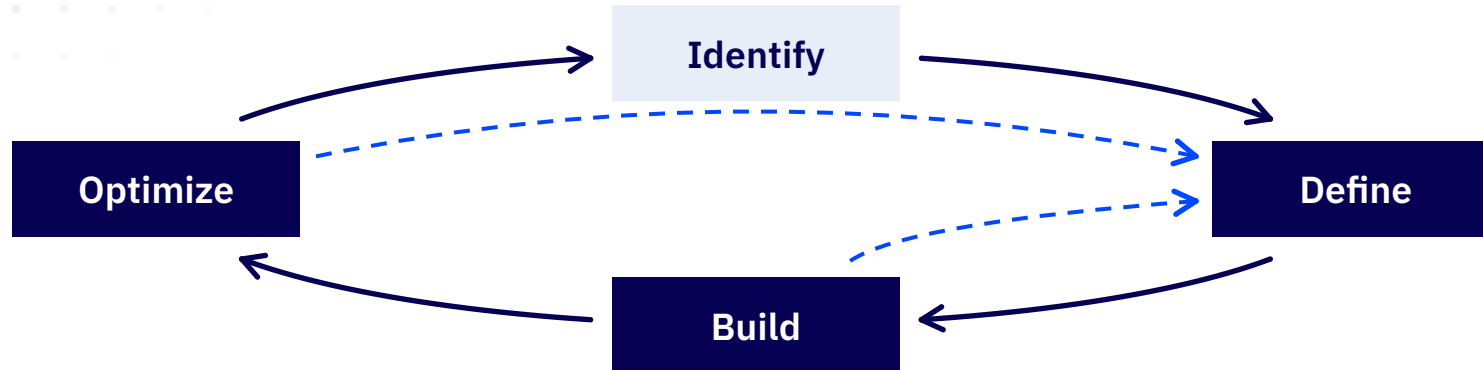
SDLC: A SUBSET



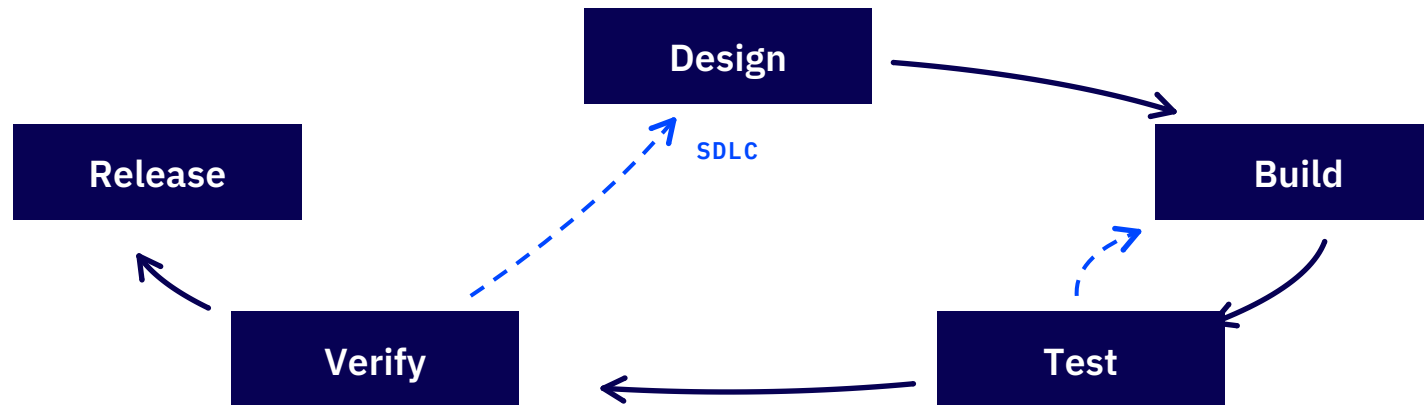
Five engineering steps that loop back on defects and gaps. Tags show the PDLC phase.

Where the SDLC sits inside the PDLC

PDLC: THE SUPERSET, FOUR PHASES THAT LOOP BACK



SDLC: A SUBSET, DEFINE TO OPTIMIZE



HOW TO READ IT

PDLC is the superset; the SDLC is a subset.

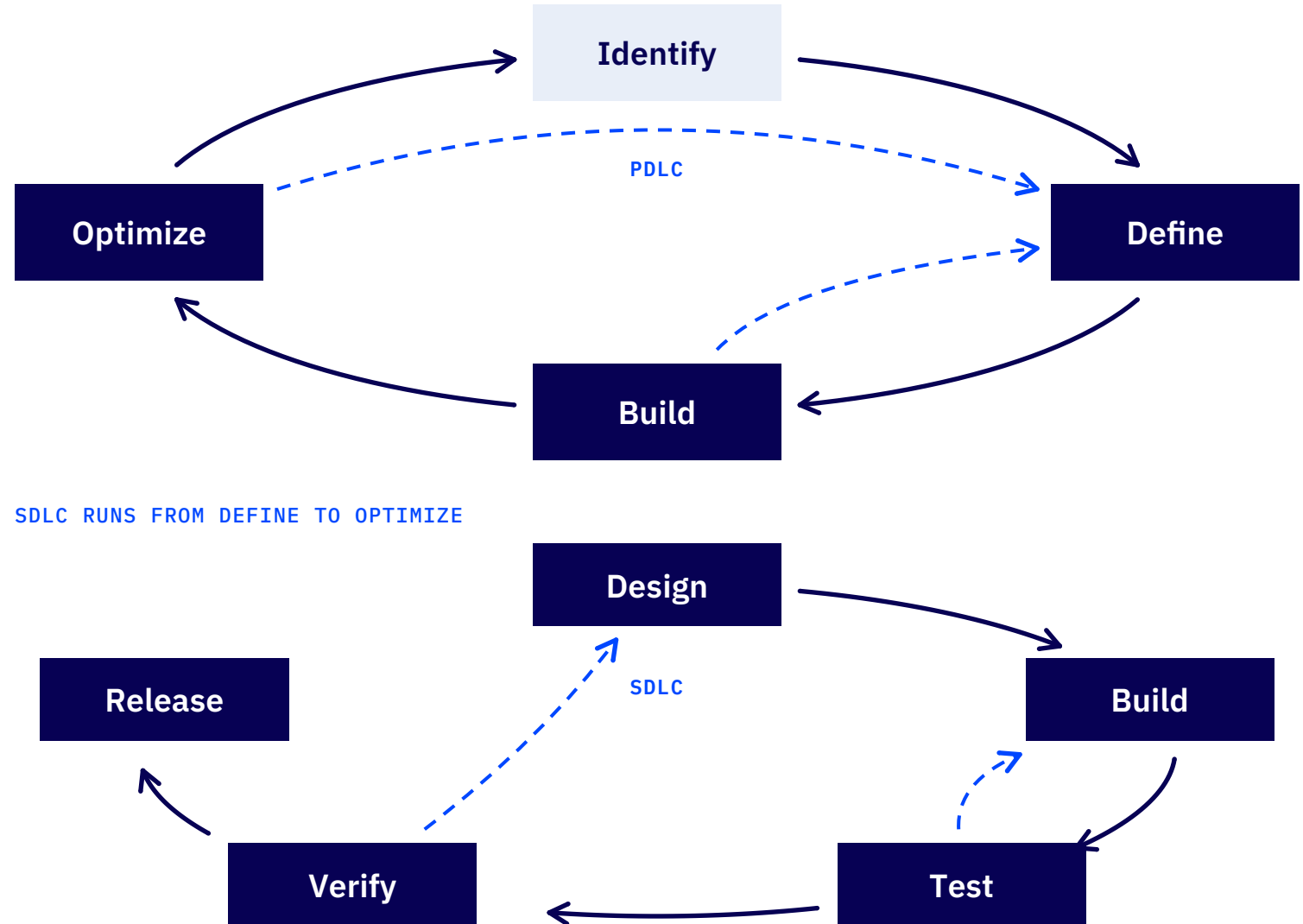
The SDLC runs from Define to Optimize. Dark phases are where it applies; both loop back.

The SDLC is a slice of the PDL

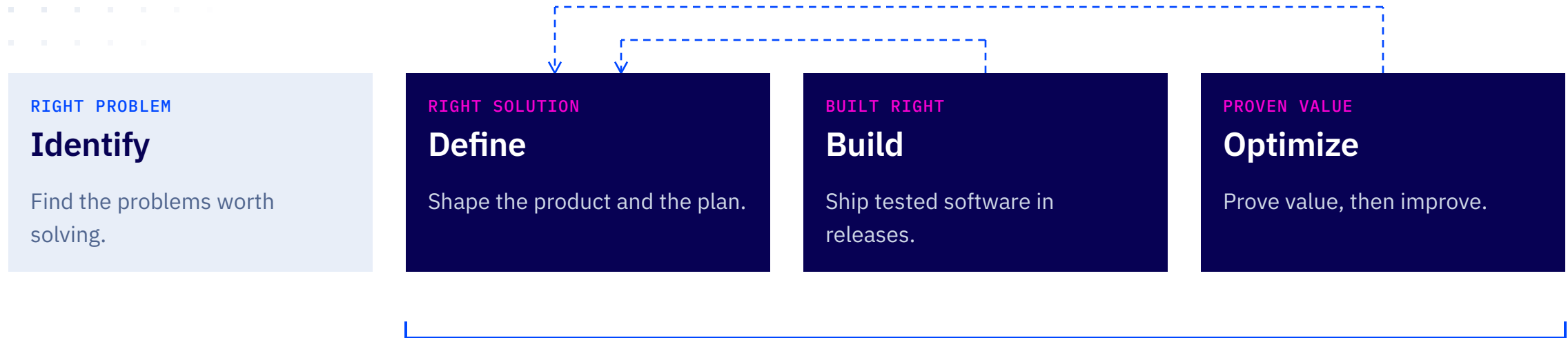
- **PDL.** The superset, from problem to proven value.
- **SDLC.** The subset that engineers and tests the product.

WHERE THE SDLC SITS

- Design in Define
- Build, Test, Verify in Build
- Release in Optimize



The SDLC is the slice of the PDLC from Define to Optimize

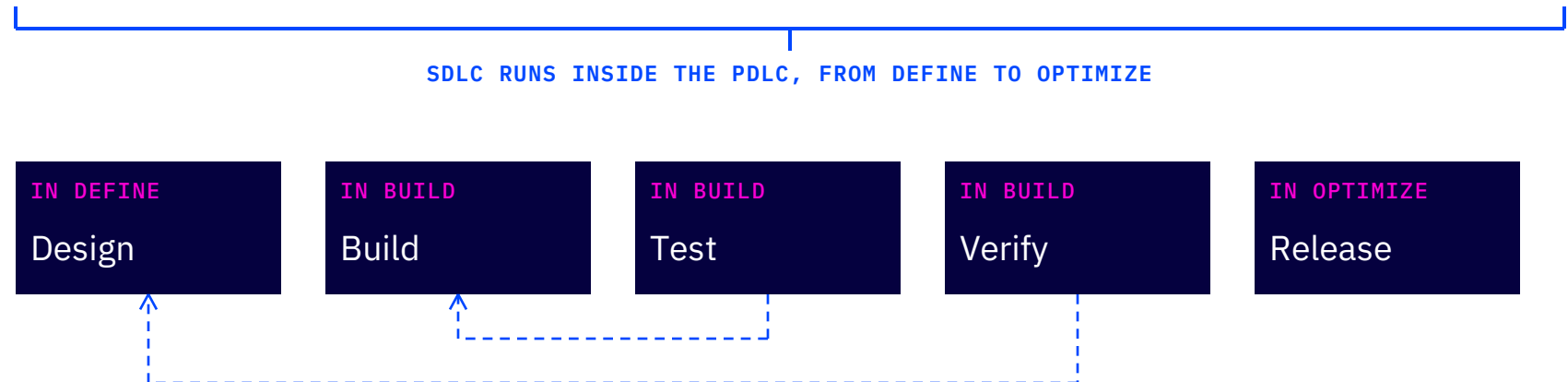


PDLC: THE SUPERSET

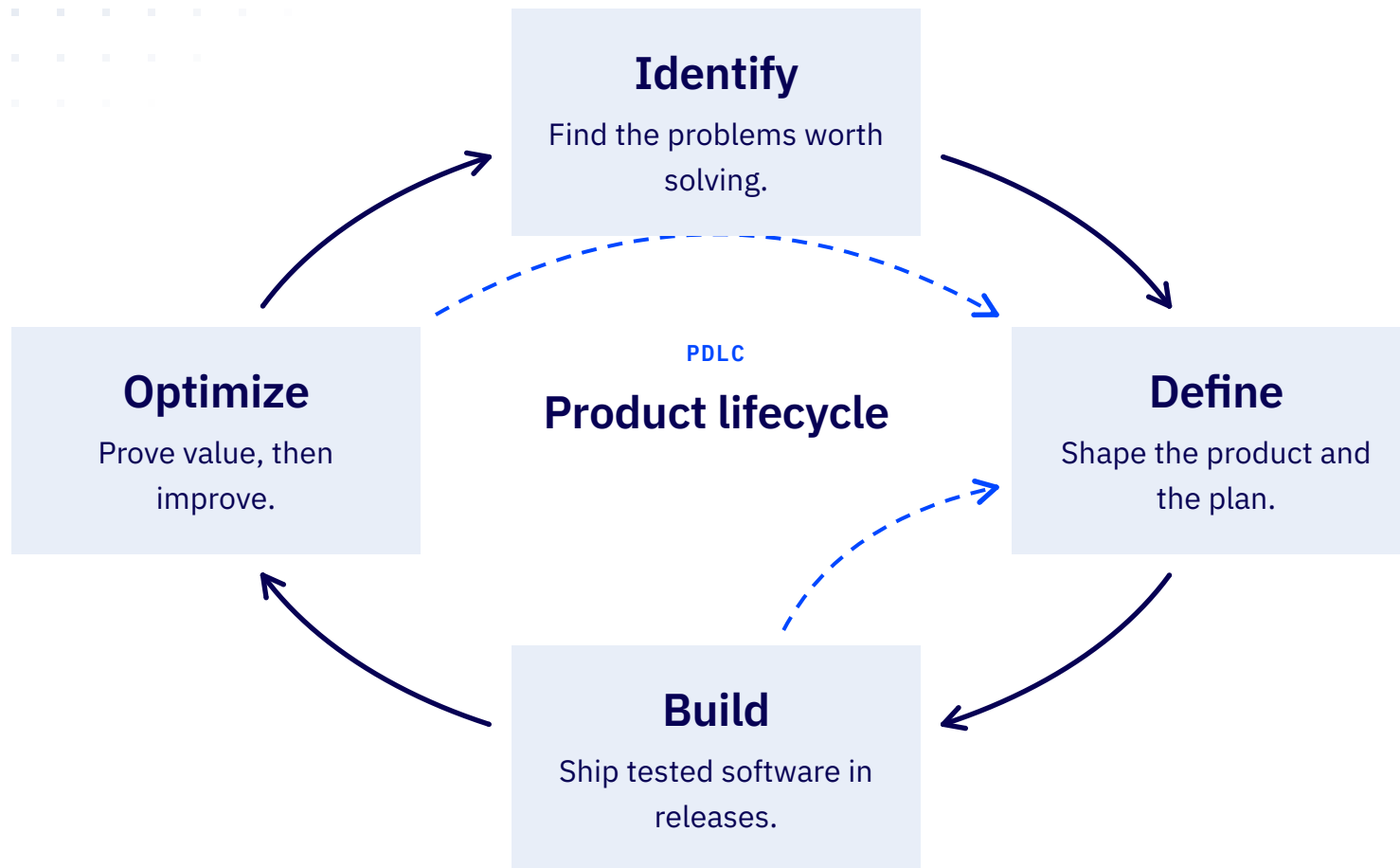
Four phases that loop back. Identify happens before engineering starts.

SDLC: A SUBSET

Five steps that loop back on defects.



The lifecycle is a loop: every phase can send work back



BUILD TO DEFINE

Findings reshape scope

Testing and clinician feedback change what we build next.

OPTIMIZE TO DEFINE

Live data reopens design

Adoption and usage evidence change the solution.

ANY PHASE

New problems enter anywhere

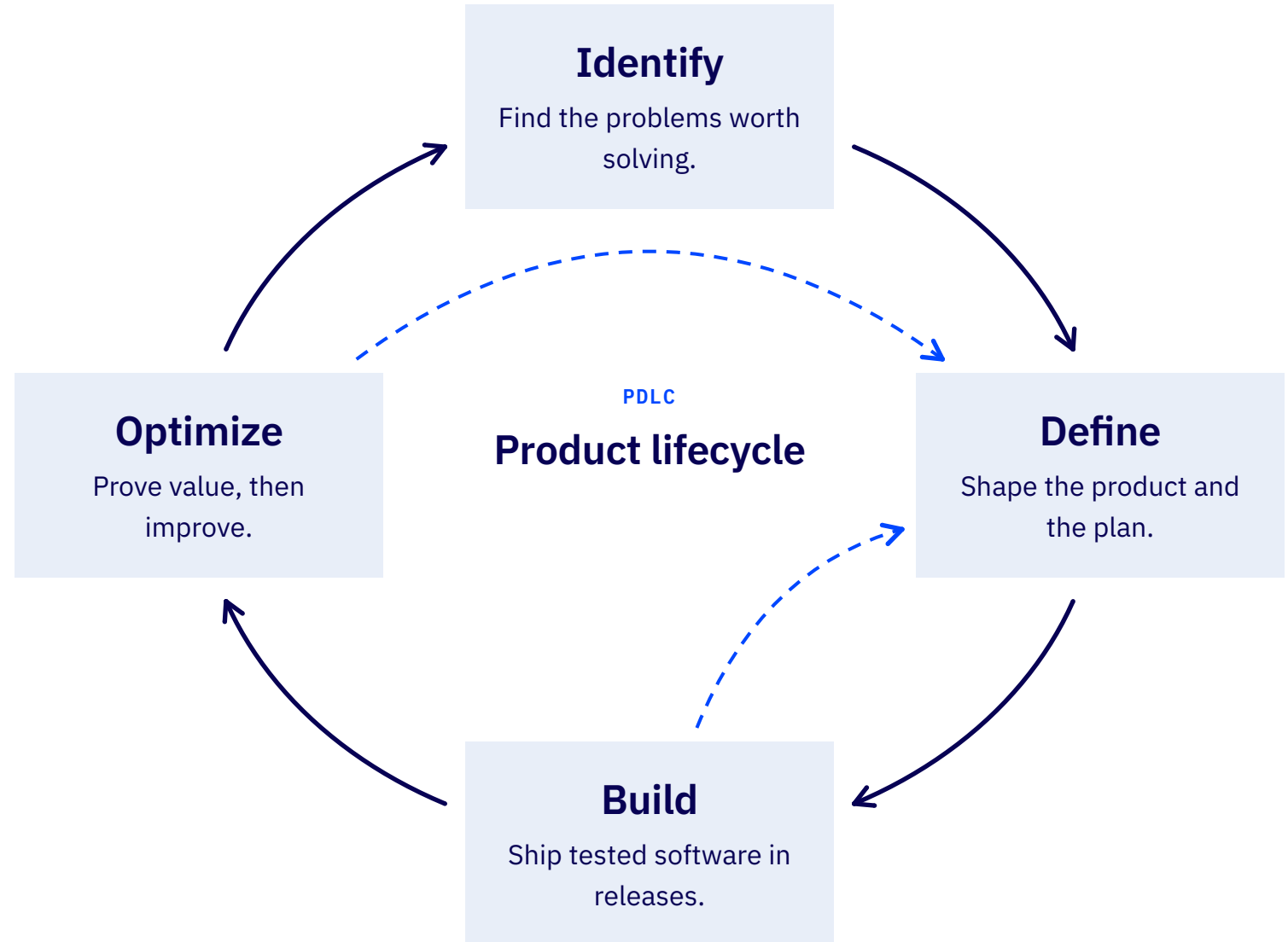
The patient app backlog is never closed.

Phases loop back as evidence arrives

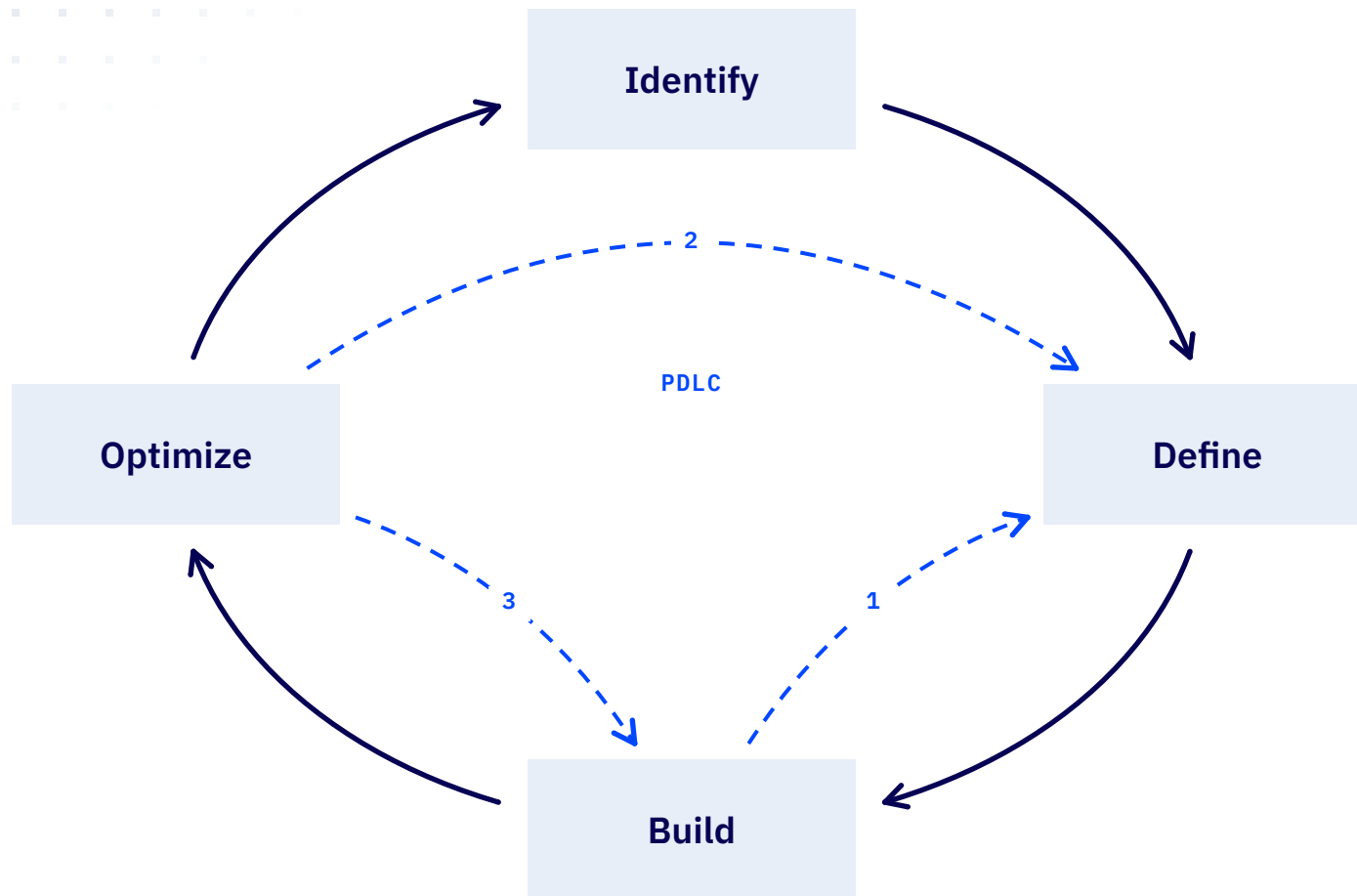
- **Build to Define.** Test findings reshape scope.
- **Optimize to Define.** Live data reopens the design.
- **Any phase.** New problems enter at any time.

READING THE CYCLE

Solid arrows are the plan; dashed arrows are rework.



Three loop-backs keep the product honest



01 Build back to Define

Test findings change scope before release.

02 Optimize back to Define

Adoption data reopens the booking design.

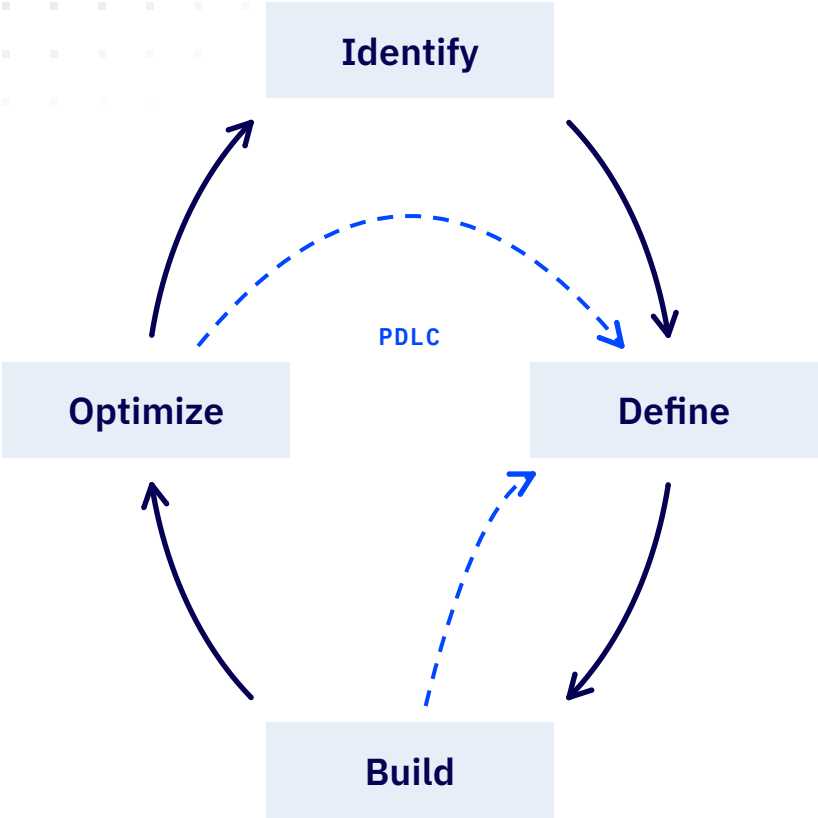
03 Optimize back to Build

Live defects return to the next sprint.

RULE OF THUMB

Give every loop-back an owner and a review cadence, or it will not happen.

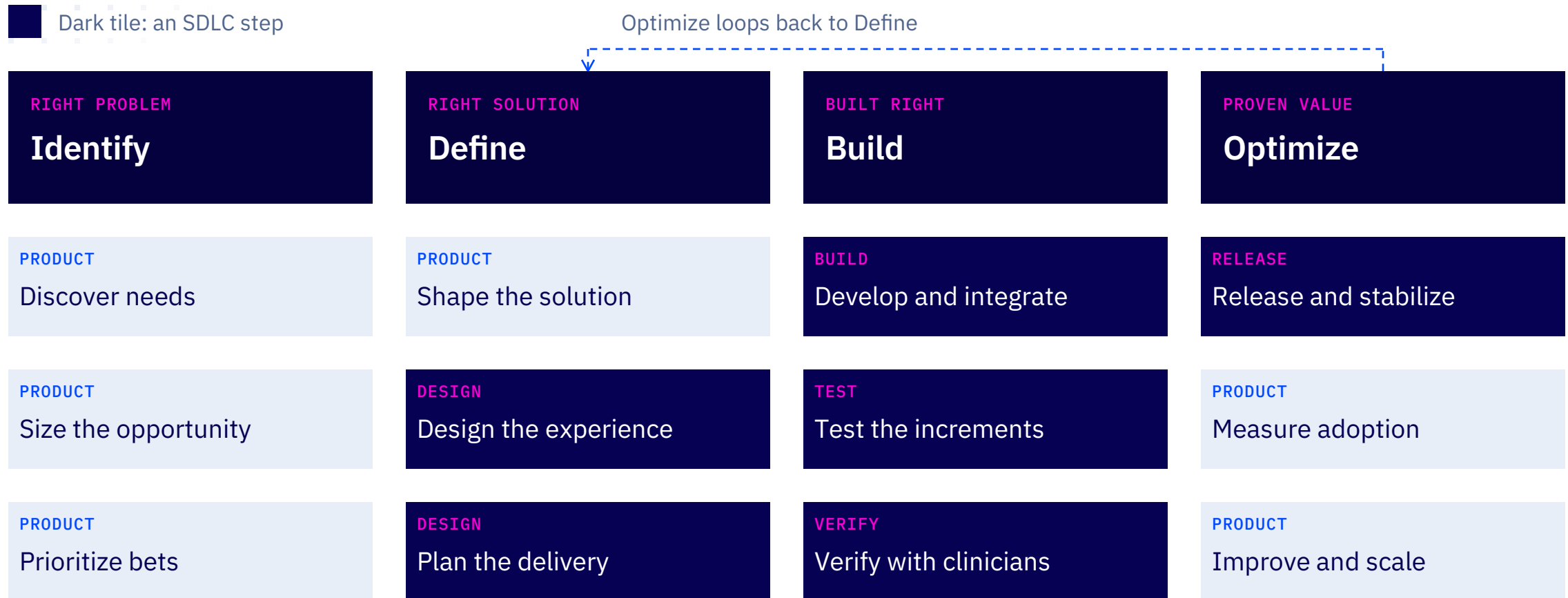
What we see and do in each phase



PHASE	WHAT WE OBSERVE	WHAT WE DO
Identify	38% of patients abandon booking at step 3	Interview patients and staff; rank the bets
Define	Three booking journeys overlap	Shape one journey; plan releases and risks
Build	Eight-week cycle; manual regression tests	Two-week sprints; automate tests
Optimize	Adoption plateaus with no loop to the backlog	Track adoption; run experiments

Source: Northfield Health System lifecycle review, Sep 2026.

Each phase has three sub-phases; the SDLC covers six



OUR APPROACH

Phase 01

Data Platform Foundation

Weeks 0–16



Early-adopter scorecard

STRATEGIC VALUE

Outcomes and sponsorship

PATTERN COVERAGE

Warehouse, lakehouse or both

TECHNICAL READINESS

Data, security, dependencies

TEAM READINESS

Owner, engineers, timeline

LEARNING VALUE

Reusable patterns and controls

Three workloads prove the platform patterns

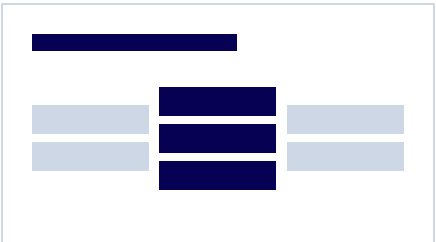
Selected to maximize enterprise learning and to test each architecture pattern once.

WAREHOUSE A TO PLATFORM

Analytics

Build analytics models on curated warehouse data.

Validates data movement and lineage.



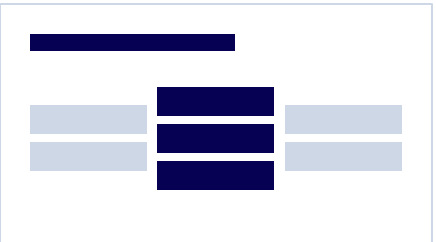
Warehouse A architecture

LAKEHOUSE B TO PLATFORM

Reporting

Rebuild reports and dashboards on curated data.

Validates ownership of curated layers.



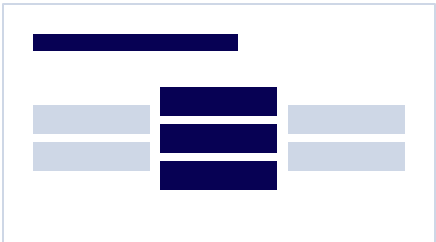
Lakehouse B architecture

END TO END ON PLATFORM

Engineering

Net-new ingestion, curation and BI for one use case.

Validates engineering standards.



End-to-end architecture

Phase 02

Scale and Early Adoption

Weeks 17–34



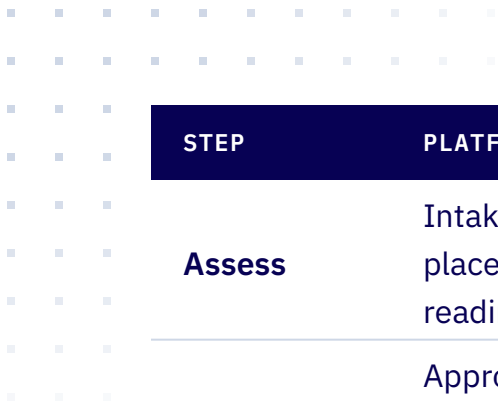
Objective

Prove that Northfield can assess, onboard, enable, govern and support a workload through one repeatable enterprise process.

Fixed or variable

Shared services are fixed and priced once.

Workload delivery varies with the number of early adopters.



STEP	PLATFORM SERVICE	WORKLOAD ACTIVITY	EVIDENCE
Assess	Intake, workload placement and readiness assessment	Confirm outcomes, data and team readiness; list dependencies	Approved placement decision and scope
Design	Approved patterns, security model and governance	Complete solution design and confirm component ownership	Signed design, exceptions and delivery backlog
Provision	Workspace automation, access and policy assignment	Confirm users, roles, connections and monitoring needs	Compliant environment with controls active
Validate	Production-readiness criteria and governance checks	Test the workload, resolve defects, secure acceptance	Readiness decision and acceptance record
Operate	Monitoring, support, escalation and service management	Assume ownership; respond to issues; track value and usage	Support model validated; lessons defined

Fixed shared services: intake · placement · provisioning · governance · monitoring · support
Variable delivery: solution design · build · data integration · testing · launch · hypercare

OUR APPROACH

Phase 01

Data Platform Foundation

Weeks 0–16



Objective

Stand up and validate the enterprise data platform: target architecture, security, governance and onboarding patterns for the first finance workloads.

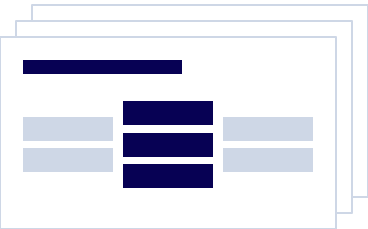
Phase 01 exit

- Architecture patterns approved
- Foundation environments live
- Integrations demonstrated
- Three pilot workloads chosen

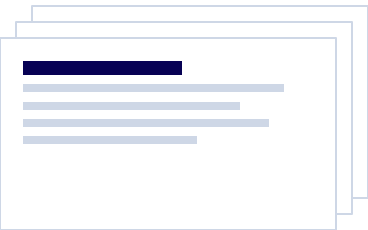


STREAM	ACTIVITIES	WORK PRODUCTS
Design	. Define target architecture and capacity . Create the placement framework	. Reference architecture . Placement framework
Build	. Configure environments and pipelines . Set up health and cost telemetry	. Configured environments . Telemetry baseline
Govern	. Define security and catalog patterns . Set lineage and classification rules	. Access control framework . Lineage standards
Launch	. Define the service catalog and onboarding . Set adopter criteria; plan Phase 02	. Service catalog and kit . Early-adopter cohort

Deliverables



Onboarding framework



Reference design

Assumptions

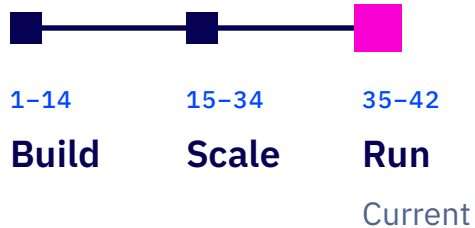
- Access is provided
- Pilots test patterns only

APPROACH

Phase 3 hands platform ownership to the Northfield team

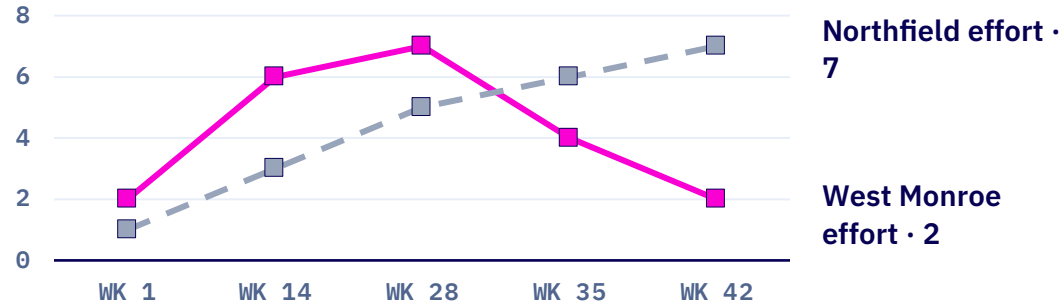
PHASE 03 · WEEKS 35-42

General availability and optimization



EXIT CRITERIA

- Runbooks accepted
- Northfield on call
- Benefits tracked
- Risks transferred



West Monroe tapers its driving role so the Northfield core team owns execution. Effort crosses at about week 30, when Northfield staff lead more work than West Monroe.

Phase 2 · Scalability and early adoption

Phase 3 · General availability

West Monroe team

Deploying provisioning and monitoring, and onboarding the first business units.

Optimizing performance, cost and adoption while transferring ownership and operations.

Northfield team

Refining standards and overseeing the early adopter program.

Dedicated staff are learning to support the platform and beginning to lead.

Evidence from three early adopters decides whether the platform is ready for general availability

01 Platform

EVIDENCE REQUIRED

- Patterns validated
- Integrations reliable
- Exceptions understood
- Standards adopted

ILLUSTRATIVE EXIT

Priority patterns approved for GA

DECISION OWNER

Chief architect

02 Operations

EVIDENCE REQUIRED

- Provisioning automated
- Telemetry active
- Runbooks exercised
- Alerts routed

ILLUSTRATIVE EXIT

Core services supportable

DECISION OWNER

Platform owner

03 Governance

EVIDENCE REQUIRED

- Controls applied
- Lineage and evidence
- Access reviews done
- Gaps closed

ILLUSTRATIVE EXIT

No unresolved critical gaps

DECISION OWNER

Governance lead

04 Service

EVIDENCE REQUIRED

- Ownership clear
- Support tiers tested
- Onboarding working
- Handover agreed

ILLUSTRATIVE EXIT

Support accepted by Northfield

DECISION OWNER

Service owner

05 Financial

EVIDENCE REQUIRED

- Cost visible
- Demand forecastable
- Value demonstrated
- Savings tracked

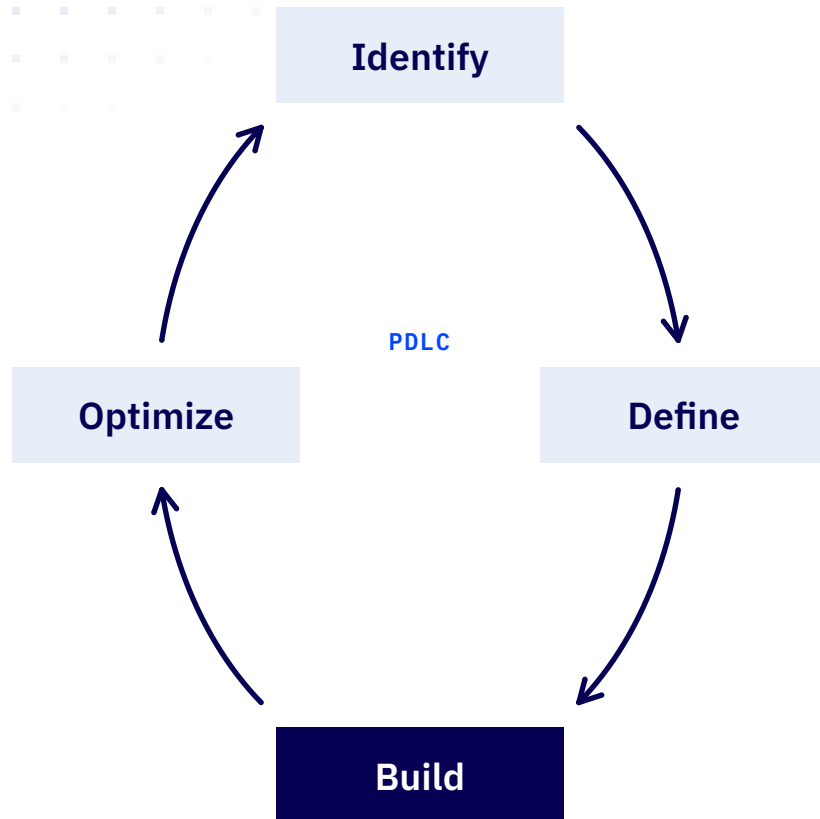
ILLUSTRATIVE EXIT

Sustainable operating basis

DECISION OWNER

Finance sponsor

Build ships tested software in small releases



Dark phase is the one shown on the right.

PHASE 3 OF 4 • BUILT RIGHT

Build

Deliver the patient app and scheduling platform in two-week sprints with automated checks.

ACTIVITIES

- Build in two-week sprints
- Automate tests and releases
- Verify with clinicians

OUTPUTS

- Working releases
- Test evidence
- Release notes

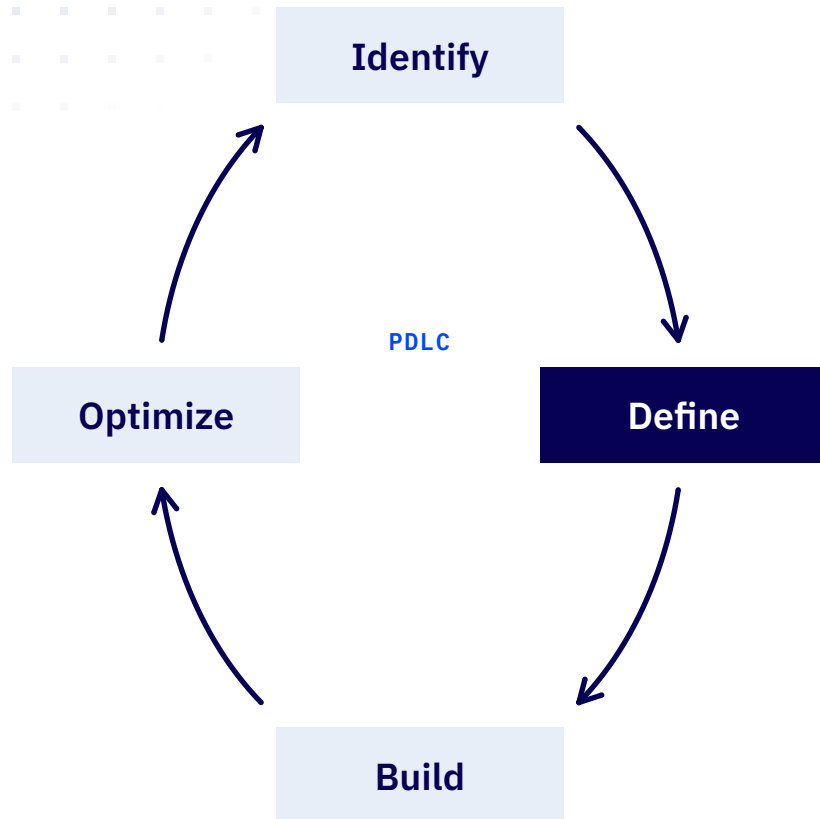
ROLES

- Engineering lead
- QA lead
- Release manager

EXIT GATE

Release approved for patients

Define turns a ranked problem into a plan



Dark phase is the one shown on the right.

PHASE 2 OF 4 • RIGHT SOLUTION

Define

Shape the product, the patient experience and the delivery plan with clinicians.

ACTIVITIES

- Shape the vision and prototypes
- Design the booking journeys
- Plan releases and risks

OUTPUTS

- Product vision
- Release plan
- Architecture

ROLES

- Product owner
- Solution architect
- UX designer

EXIT GATE

Approved scope and plan

Our **three-phase** approach turns a defined architecture into a Northfield-owned enterprise service



Source: Fixed platform scope is priced once; workload scope scales with the number of early adopters.

APPROACH

Four phases take the close from 12 days to 5

PHASE 1

1 WK

Mobilize

Team, data access and plan in place.

ACTIVITIES

- Kickoff and interviews
- Data access

DELIVERABLES

- Project plan

PHASE 2

3 WKS

Diagnose

Know where the 12 days go.

ACTIVITIES

- Close calendar mapping
- Delay ranking

DELIVERABLES

- Diagnostic readout

PHASE 3

3 WKS

Design

A five-day close, designed.

ACTIVITIES

- Rules and routing
- Operating model

DELIVERABLES

- Target design

PHASE 4

1 WK

Roadmap

Waves 1–3 planned and funded.

ACTIVITIES

- Wave sequencing
- Business case

DELIVERABLES

- \$4.2M business case

G1 • MOBILIZED

G2 • READOUT

G3 • SIGN-OFF

G4 • FINAL

APPROACH

Four phases to a 5-day close

PHASE 1

1 WK

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Team, data access and plan in place.

ACTIVITIES

- Kickoff and interviews
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DELIVERABLES

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DELIVERABLES

- \$4.2M business case

G1 • MOBILIZED

G2 • READOUT

G3 • SIGN-OFF

G4 • FINAL

OUR COMMITMENT

8 weeks

from kickoff to a funded roadmap

WHAT YOU CAN HOLD US TO

A five-day close designed with your team, and a business case your board can approve at Gate 4.

Three phases from baseline to a funded roadmap

Phase 1 · Baseline

6–8 weeks · establish the fact base

ACTIVITIES

- Gather data and benchmarks
- Interview finance leaders
- Size the opportunity by function
- Map workflows and handoffs
- Test findings with sponsors

DELIVERABLES

- Baseline and benchmark report
- Opportunity sizing model
- Fact-base readout

Phase 2 · Design

2–3 weeks · shape the options

ACTIVITIES

- Develop recommendations per area
- Set savings and value targets
- Define the target mix of roles
- Rank options by impact and effort
- Align leaders on the options

DELIVERABLES

- Prioritized value opportunities
- Target role-mix view
- Value-target summary

Phase 3 · Roadmap

1–2 weeks · agree the plan

ACTIVITIES

- Facilitate a trade-off workshop
- Build the integrated roadmap
- Sequence initiatives by value
- Name owners and dependencies
- Read out the priorities

DELIVERABLES

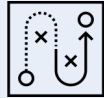
- Value-creation roadmap
- Initiative owner register
- Executive readout deck

AI-powered modernization compresses time and risk



Value first

- We rank work by business value, not technical elegance.
- Every decision weighs complexity.
- Return comes first.



Purpose built

- We choose retain, replatform, rewrite or retire per system.
- Stable components stay in place.
- Nothing is rebuilt for show.



AI accelerated

- Agents scan and map codebases at scale.
- Evidence, planning and tooling stay connected.
- Months of analysis take days.



Co-build and scale

- Co-sourced teams sit with your stakeholders.
- Onshore and nearshore flex with demand.
- Knowledge transfer is built in.

WHY THIS MATTERS

Effort goes where return is highest, so promised value actually lands.

WHY THIS MATTERS

We design around your portfolio, keeping what works and replacing only what drives value.

WHY THIS MATTERS

Analysis that takes months is done in days, with less reliance on tribal knowledge.

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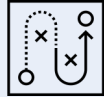
You get capacity without disruption, and a team that can run the result on its own.

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Why modernization has been slow and costly

Coding is only one part. The harder work is rediscovering, deciding and cutting over safely.

DISCOVER AND DEFINE

01 Reconstruct business behavior

Document current processes, rules and workarounds.

02 Decide what to retain

Separate essential controls from obsolete features.

03 Align business and technology

Turn operator intent into shared designs and tests.

DELIVER AND TRANSITION

04 Untangle dependencies

Map integrations, then clean duplicate and incomplete data.

05 Validate during cutover

Run acceptance tests while old and new systems coexist.

06 Execute a safe cutover

Sequence migration, rollback and controls to protect operations.

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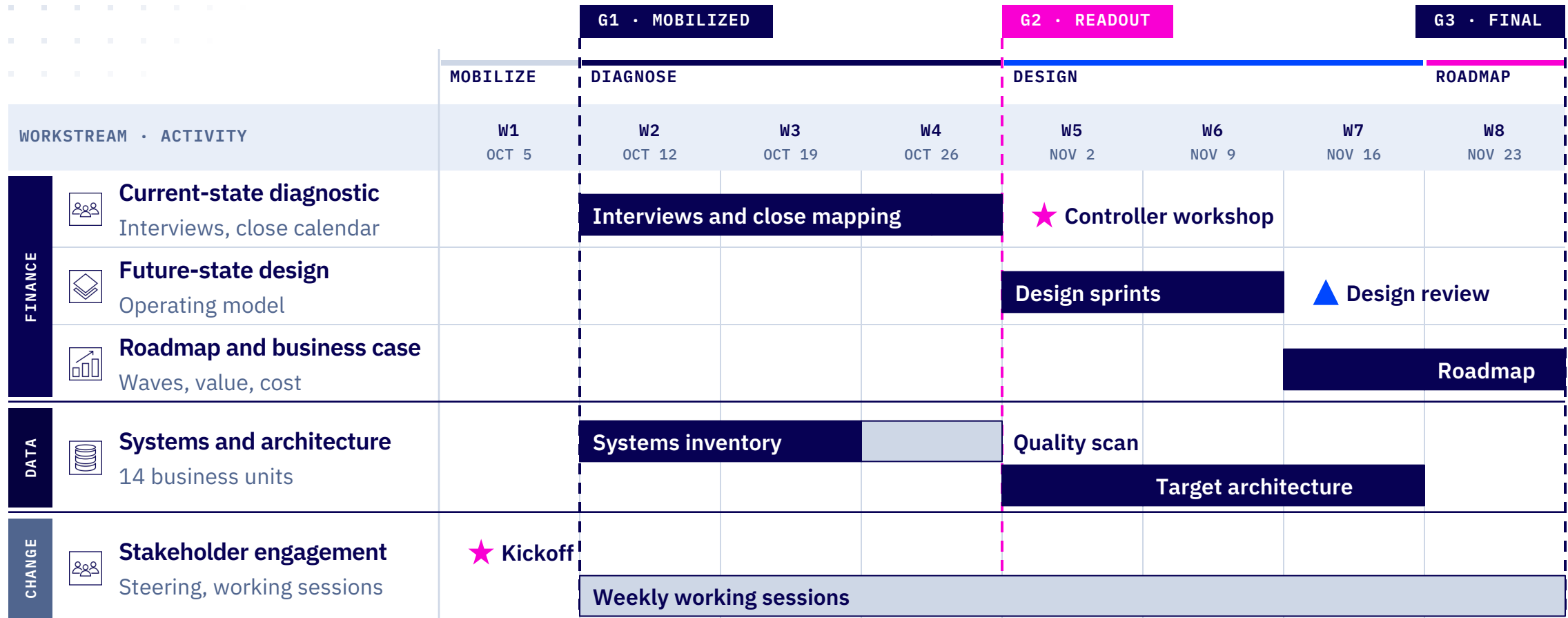
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Sequence migration, rollback and controls to protect operations.

Eight weeks from kickoff to a signed-off roadmap



■ Core activity □ Supporting activity ▨ Hatched: in progress, soft start or tentative ★ Workshop ▲ Review — Gate

COMMERCIALS

Option B is the sweet spot of scope and fee

OUR RECOMMENDATION

Accelerate

\$720K

fixed fee, 24 weeks

Covers everything to Wave 1 live. Training and hypercare can be added later.

WHAT IS INCLUDED	A · FOUNDATION	B · ACCELERATE	C · TRANSFORM
Close diagnostic	✓	✓	✓
Target design	✓	✓	✓
Roadmap and business case	—	✓	✓
Wave 1 build	—	✓	✓
Change and training	—	—	✓
Hypercare, 90 days	—	—	✓
FIXED FEE	\$288K	\$720K	\$1.14M

Three tiers, each building on the one before

GOOD • FIXED FEE

Foundation

- ☒ Close diagnostic
- ☐ Target design
- ☐ Roadmap and business case
- ☐ Wave 1 build
- ☐ Training and hypercare

\$288K

fixed fee, 4 weeks

RECOMMENDED

BETTER • FIXED FEE

Accelerate

- ☒ Close diagnostic
- ☒ Target design
- ☒ Roadmap and business case
- ☒ Wave 1 build
- ☐ Training and hypercare

\$720K

fixed fee, 24 weeks

BEST • FIXED FEE

Transform

- ☒ Close diagnostic
- ☒ Target design
- ☒ Roadmap and business case
- ☒ Wave 1 build
- ☒ Training and hypercare

\$1.14M

fixed fee, 36 weeks

Two options, and the one we recommend

OPTION A • DESIGN ONLY

Diagnose and design

Prove the case first, then decide on build.

- ☒ Close diagnostic
- ☒ Target design
- ☐ Wave 1 build

\$720K

fixed fee, 8 weeks

RECOMMENDED

OPTION B • DESIGN AND BUILD

Diagnose, design, build

Carry one team through to Wave 1.

- ☒ Close diagnostic
- ☒ Target design
- ☒ Wave 1 build

\$720K + \$210K/mo

fixed, then run-rate

WHY OPTION B

- 01 No hand-off gap**
The design team stays to build what it designed.
- 02 Fixed where known**
Design is a fixed fee; build flexes with demand.
- 03 Earlier benefit**
Wave 1 goes live 12 weeks sooner.

Decision by 16 October keeps the Wave 1 start date.

IN BOTH OPTIONS

Program management and weekly steering

Knowledge transfer to Northfield finance

A fixed-fee diagnostic and design

NOT IN EITHER

Licenses and infrastructure

Northfield staff time and expenses

COMMERCIALS

Two options for Northfield, one recommended

OPTION A

Design only

Prove the case, then decide on build.

DURATION

8 weeks

\$720K

fixed fee

OPTION B

Design and build

Carry one team through to Wave 1.

DURATION

8 weeks fixed, then 16 weeks capacity

\$720K + \$210K/mo

fixed, then run-rate

RECOMMENDED

One team, one monthly run-rate of \$210K

CAPACITY

\$840K

estimated, four months

Team run-rate \$210K / mo
Duration 4 months

TERMS

- Invoiced monthly in arrears
- 30 days' notice to change the team
- Expenses capped at 8% of fees

ROLE	LEVEL	ALLOCATION
Engagement lead	Managing Director	25%
Process lead	Senior Manager	100%
Data architect	Manager	100%
Accountants	Consultant × 2	100%
Team monthly run-rate		\$210K

COMMERCIALS

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Duration 4 months

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ROLE	LEVEL	ALLOCATION
Engagement lead	Managing Director	25%
Process lead	Senior Manager	100%
Data architect	Manager	100%
Integration lead	Manager	50%
Accountants	Consultant × 2	100%
Analysts	Analyst × 2	100%
Change lead	Senior Consultant	50%
Team monthly run-rate		\$210K

A fixed fee of \$720K, paid as milestones are accepted

MILESTONE	TIMING	% OF FEE	AMOUNT	CUMULATIVE
Mobilization complete	Week 1	10%	\$72K	\$72K
Diagnostic readout	Week 4	30%	\$216K	\$288K
Design sign-off	Week 7	40%	\$288K	\$576K
Final readout accepted	Week 8	20%	\$144K	\$720K
Total fixed fee		100%	\$720K	

FIXED FEE

\$720K

total fixed fee

Diagnose · weeks 1–4	\$288K
Design and roadmap · weeks 5–8	\$432K

TERMS

- Invoiced on milestone acceptance
- Expenses capped at 8% of fees

Source: West Monroe estimate. Fees exclude expenses.

TOTAL FIXED FEE

\$720K

eight weeks, two phases

Diagnose, weeks 1 to 4: \$288K

Design and roadmap, weeks 5 to 8:
\$432K

Invoiced on milestone acceptance.
Expenses capped at 8% of fees.

COMMERCIALS

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Source: West Monroe estimate. Fees exclude expenses.

Three ways to engage, one we recommend

OPTION A • FIXED FEE

Diagnose only

- ☒ Close diagnostic
- ☐ Target design and roadmap
- ☐ Wave 1 build

DURATION

4 weeks

\$288K

fixed fee

RECOMMENDED

OPTION B • HYBRID

Diagnose and build

- ☒ Close diagnostic
- ☒ Target design and roadmap
- ☒ Wave 1 build

DURATION

8 weeks fixed, then 16 weeks capacity

\$720K + \$210K/mo

fixed, then run-rate

OPTION C • CAPACITY

Embedded pod

- ☒ Close diagnostic
- ☒ Target design and roadmap
- ☒ Wave 1 build

DURATION

Rolling, 3-month minimum

\$240K/mo

pod run-rate

COMMERCIALS

Three ways to engage, one we recommend

OPTION A • FIXED FEE

Diagnose only

- ☒ Close diagnostic
- ☐ Target design and roadmap
- ☐ Wave 1 build

DURATION

4 weeks

\$288K

fixed fee

OPTION B • HYBRID

Diagnose and build

- ☒ Close diagnostic
- ☒ Target design and roadmap
- ☒ Wave 1 build

DURATION

8 weeks fixed, then 16 weeks capacity

\$720K + \$210K/mo

fixed, then run-rate

RECOMMENDED

OPTION C • CAPACITY

Embedded pod

- ☒ Close diagnostic
- ☒ Target design and roadmap
- ☒ Wave 1 build

DURATION

Rolling, 3-month minimum

\$240K/mo

pod run-rate

Three options, one recommended

OPTION	SCOPE	DURATION	FEE
A • Diagnose only	Close diagnostic	4 weeks	\$288K fixed
B • Diagnose and build	Diagnostic, design and Wave 1 build	24 weeks	\$720K + \$210K/mo
C • Embedded pod	One team, all work	Rolling	\$240K/mo

OUR RECOMMENDATION

Option B • Diagnose and build

Fixed fee through design, then a monthly run-rate through build

\$720K

fixed, then \$210K a month

Fixed where scope is known, flexible where it is not.

With constant AI innovation, modernization is faster, cheaper and more necessary than ever

But most organizations do not yet know how to capture AI's value across the modernization lifecycle.



Legacy systems hold the business back

Legacy architecture is expensive to maintain and increasingly vulnerable, and it slows innovation and developer productivity.



Teams cannot optimize what they own

Most efforts become lift-and-shift exercises that copy the past instead of rearchitecting for AI.

WHAT CHANGES THE ODDS

Start with value, pick the right path per system, use AI throughout.

MOST MODERNIZATIONS FALL SHORT

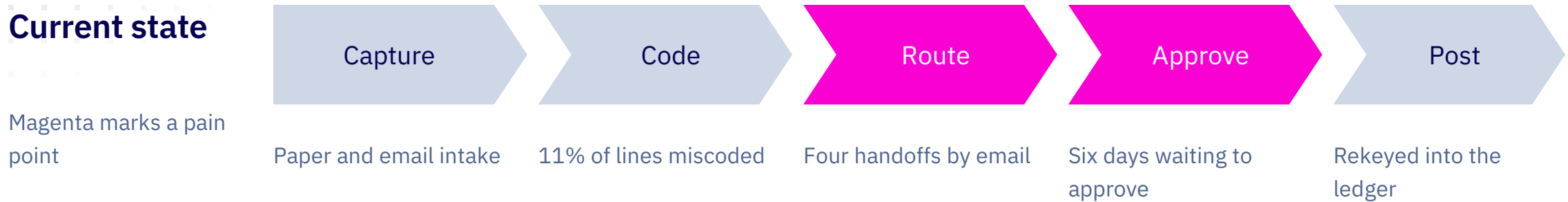
60–75%

of programs run over budget or timeline, are scaled back, or deliver less value than promised.

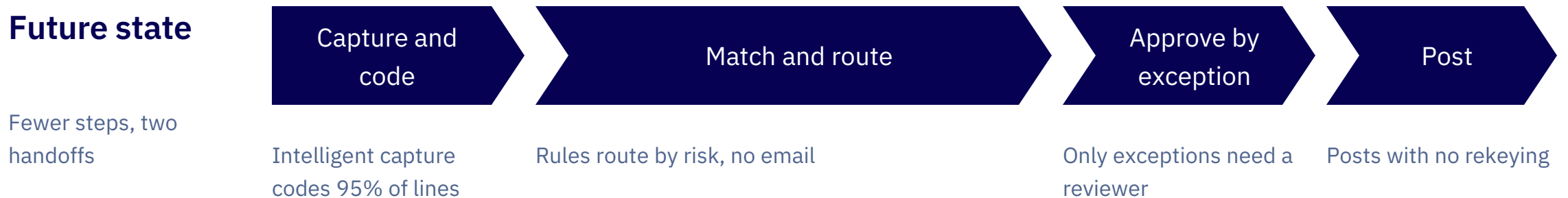
1 in 3 scope cut after year one

Redesign cuts invoice handling from six handoffs to two

Current state



Future state



Cycle time 14 days to 5 days · Handoffs 6 to 2 · Rekeying removed

Source: Northfield Health System invoice sample, Q3 FY26.

Redesign cuts handoffs from six to two

Current state: magenta marks a pain point



Future state: fewer steps, two handoffs



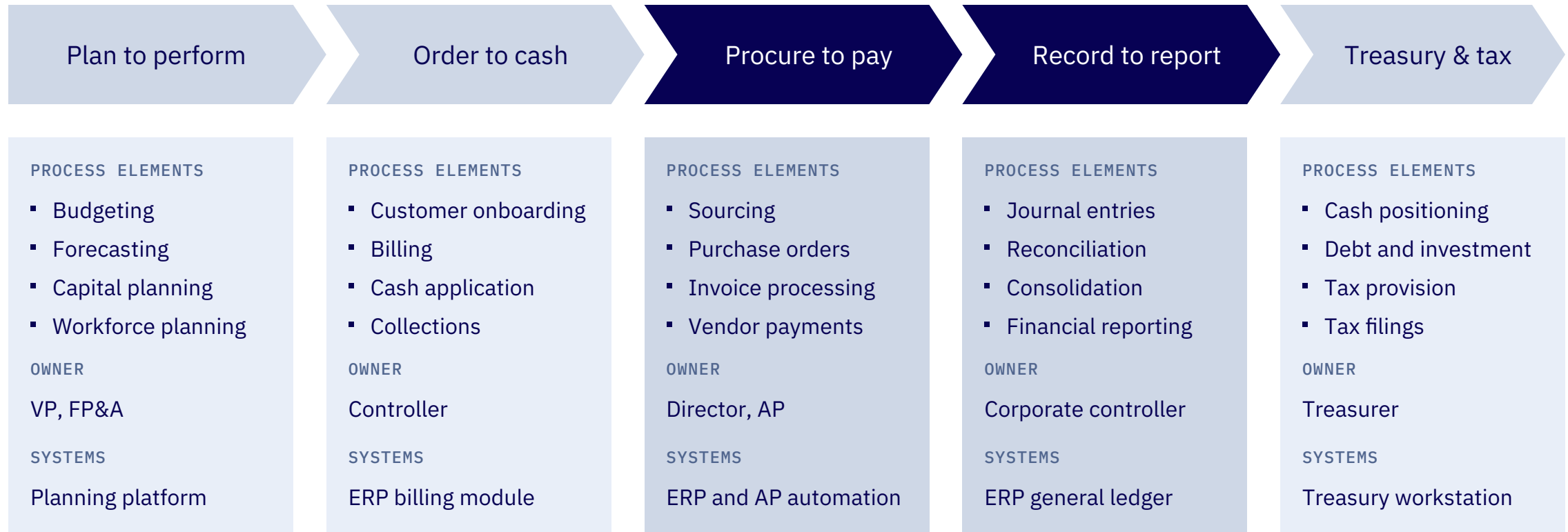
Source: Northfield Health System invoice sample, Q3 FY26.

RESULT

14 days down to 5

Handoffs fall from six to two, and no invoice is rekeyed.

Wave one covers two of five end-to-end finance processes



Dark chevrons are in scope for wave one. Enablers: shared services, finance IT and master data.

Source: Northfield Health System process inventory, Sep 2026.

Approval holds most of the 14-day invoice cycle

	Receive	Match	Approve	Pay	Reconcile
Cycle time	1.5 days	2.0 days	8.0 days	1.5 days	1.0 day
Handoffs	1	2	4	1	1
Invoices a month	12,400	12,400	12,400	11,900	11,900
Rework rate	4%	11%	9%	2%	1%

Approval holds 57% of the 14-day cycle and four of nine handoffs: redesign it first.

Source: Northfield Health System AP system data, Q3 FY26.

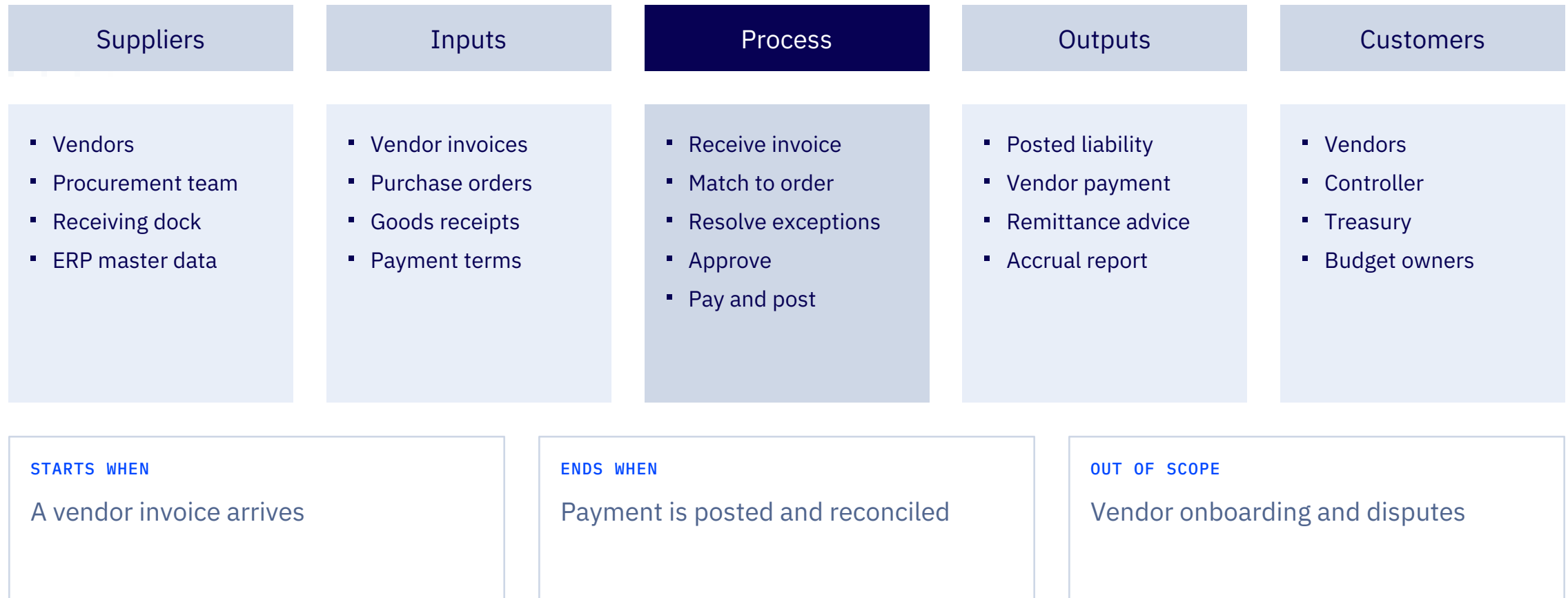
Approval holds most of the 14-day invoice cycle

	Receive	Match	Approve	Pay	Reconcile
Cycle time	1.5 days	2.0 days	8.0 days	1.5 days	1.0 day
Handoffs	1	2	4	1	1
Invoices a month	12,400	12,400	12,400	11,900	11,900
Rework rate	4%	11%	9%	2%	1%

Approval holds 57% of the 14-day cycle and four of nine handoffs: redesign it first.

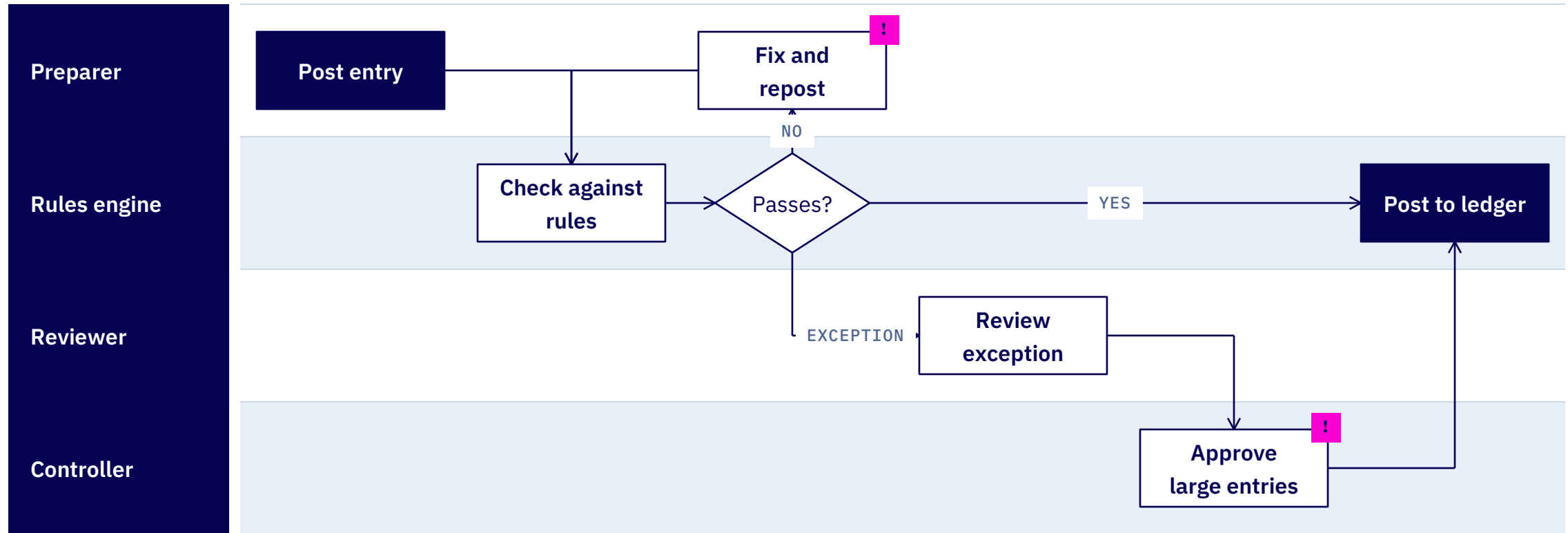
Source: Northfield Health System AP system data, Q3 FY26.

A SIPOC fixes the scope of invoice to pay before we map it



Source: Northfield Health System scoping workshop, Sep 2026.

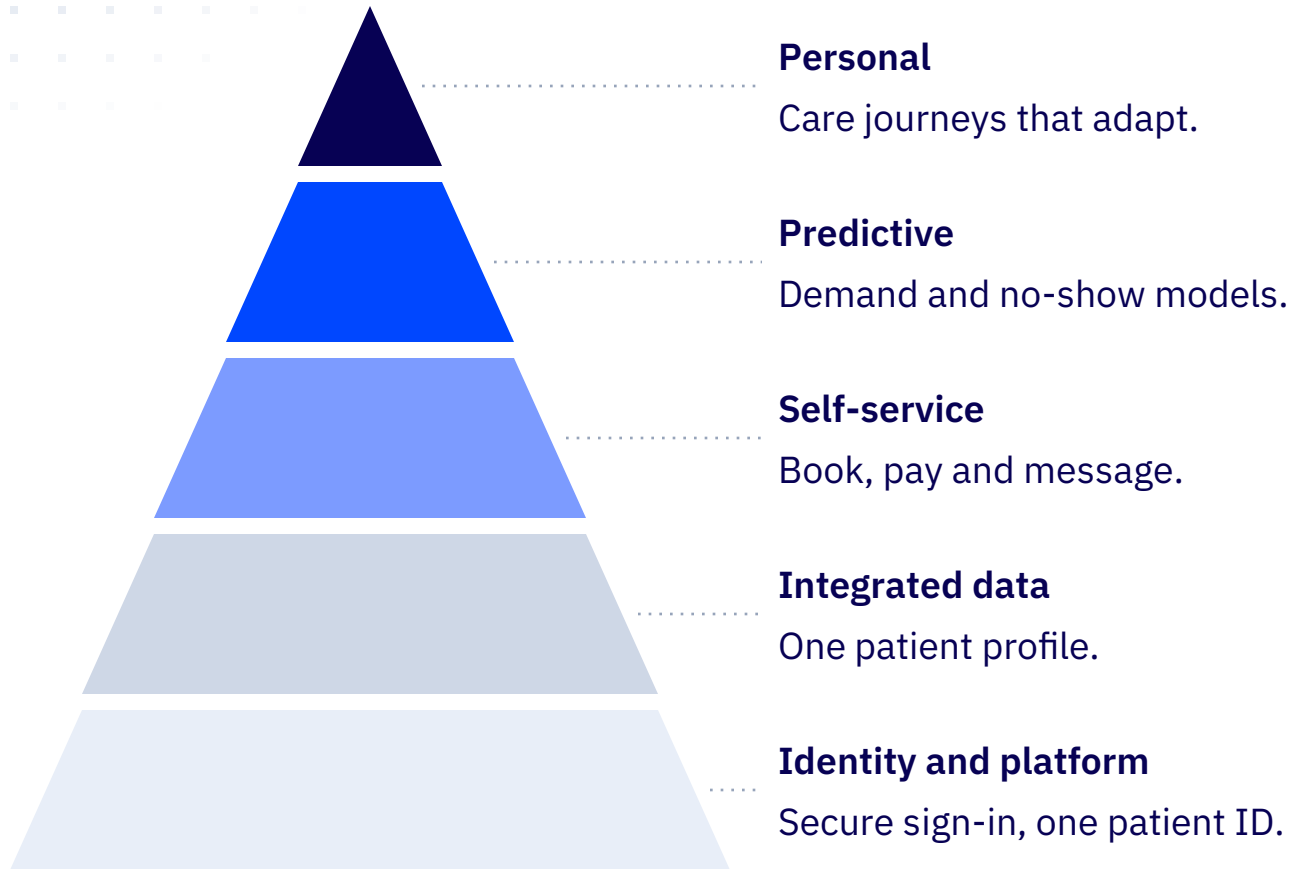
Journal review hands off four times before posting



! PAIN POINT RAISED IN INTERVIEWS

Source: Northfield close interviews, Sep 2026.

Each layer of the digital front door rests on the one below

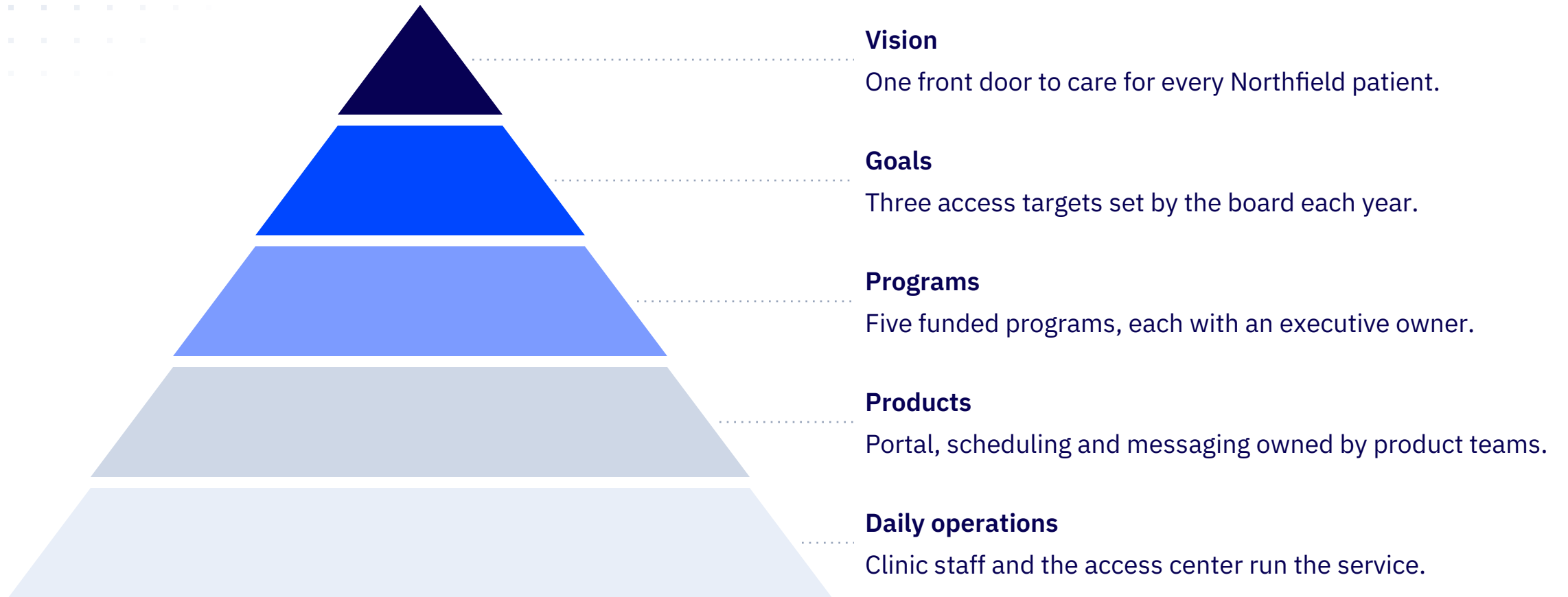


BUILD ORDER

- 01 Lay the base first**
Identity and a secure platform come before any new service.
- 02 Connect the data**
Scheduling, billing and clinical records feed one profile.
- 03 Then add intelligence**
Prediction needs clean, connected data.

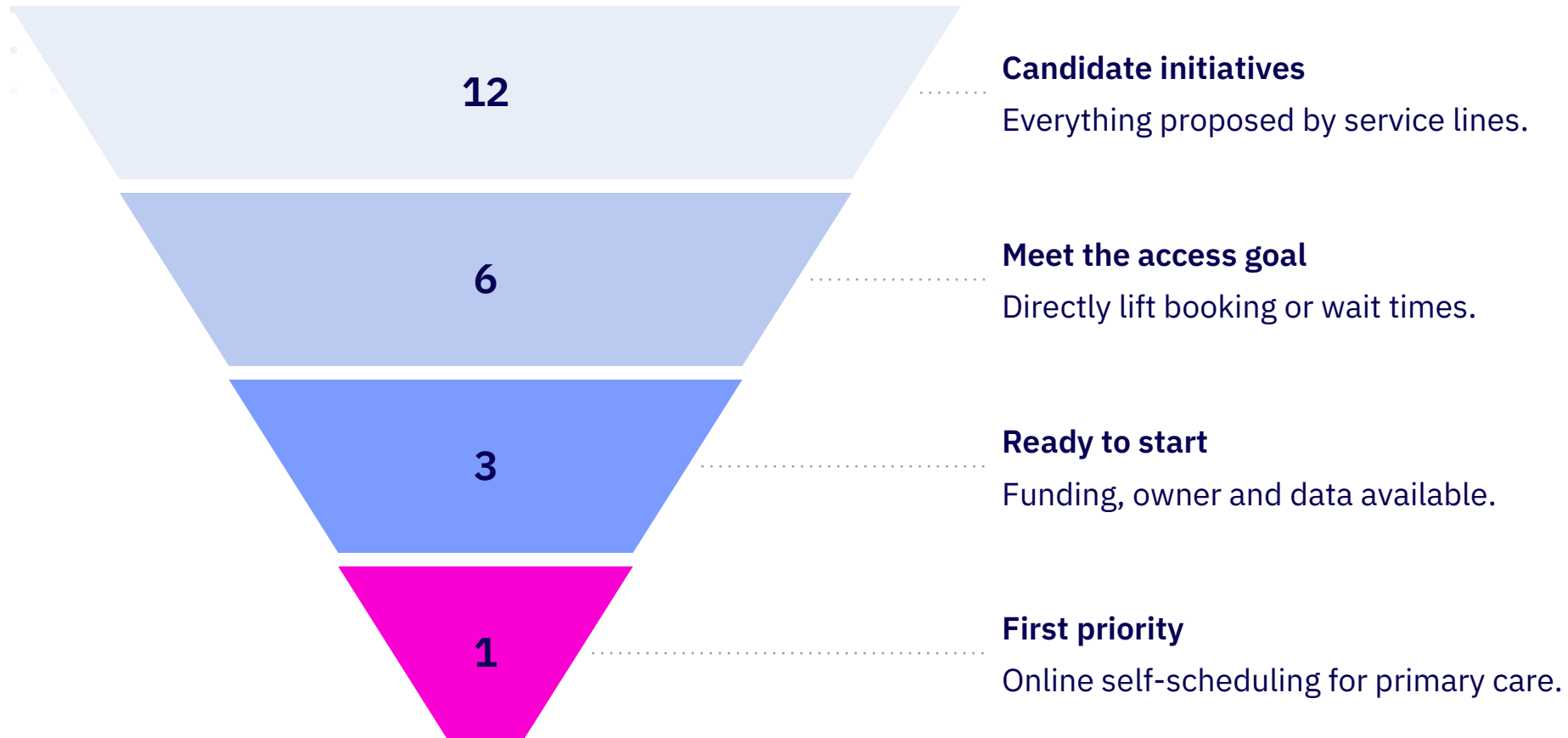
Source: Northfield Health System digital front door program, Sep 2026.

Strategy narrows from system goals to daily work



Source: Northfield Health System digital front door program, Sep 2026.

Narrow every list down to one priority



Source: Northfield Health System digital front door program, Sep 2026.

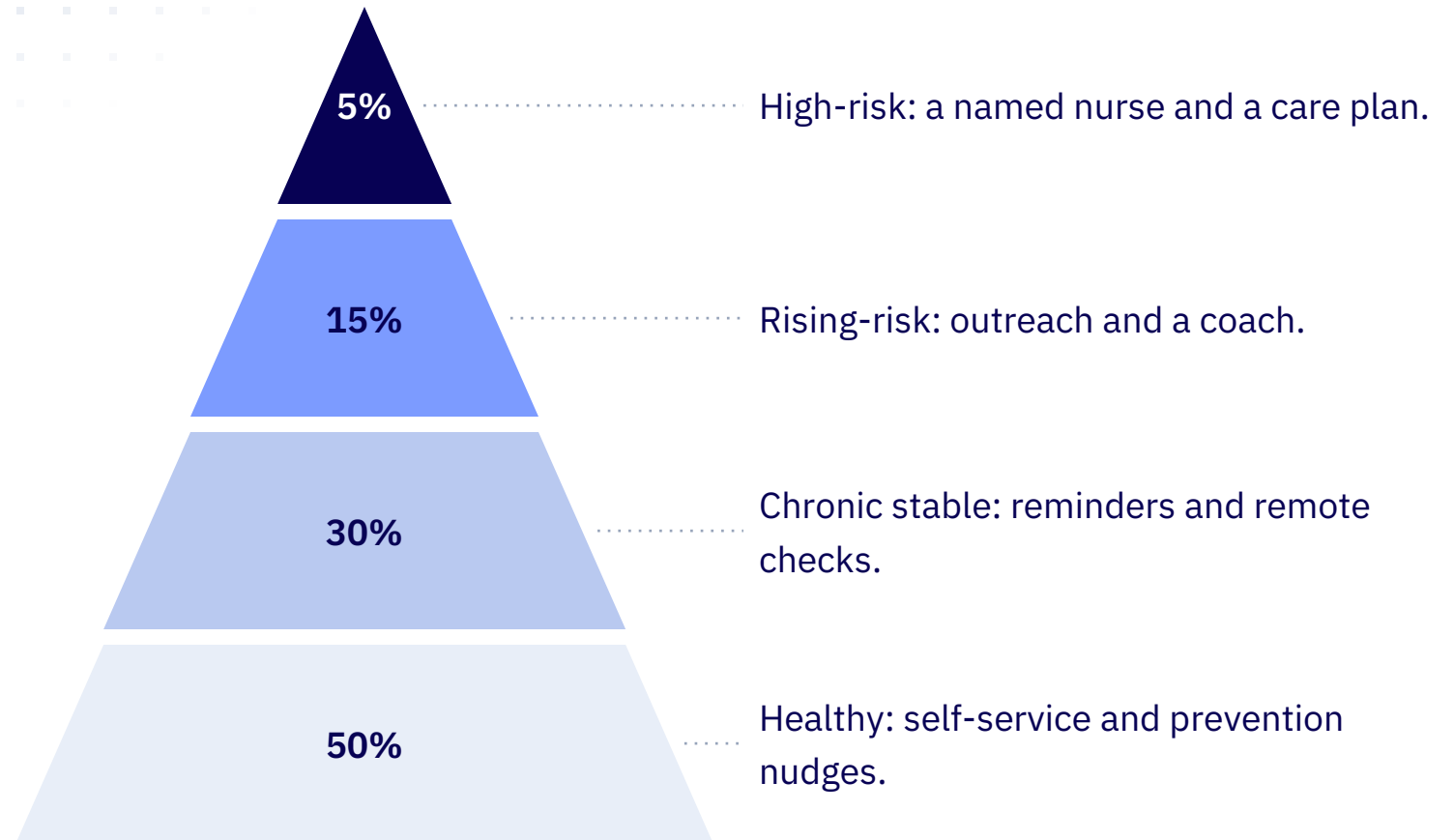
Match effort to need

Intensive care management is reserved for the few patients who drive most cost.

Digital tools carry the broad base so nurses can focus on the top two tiers.

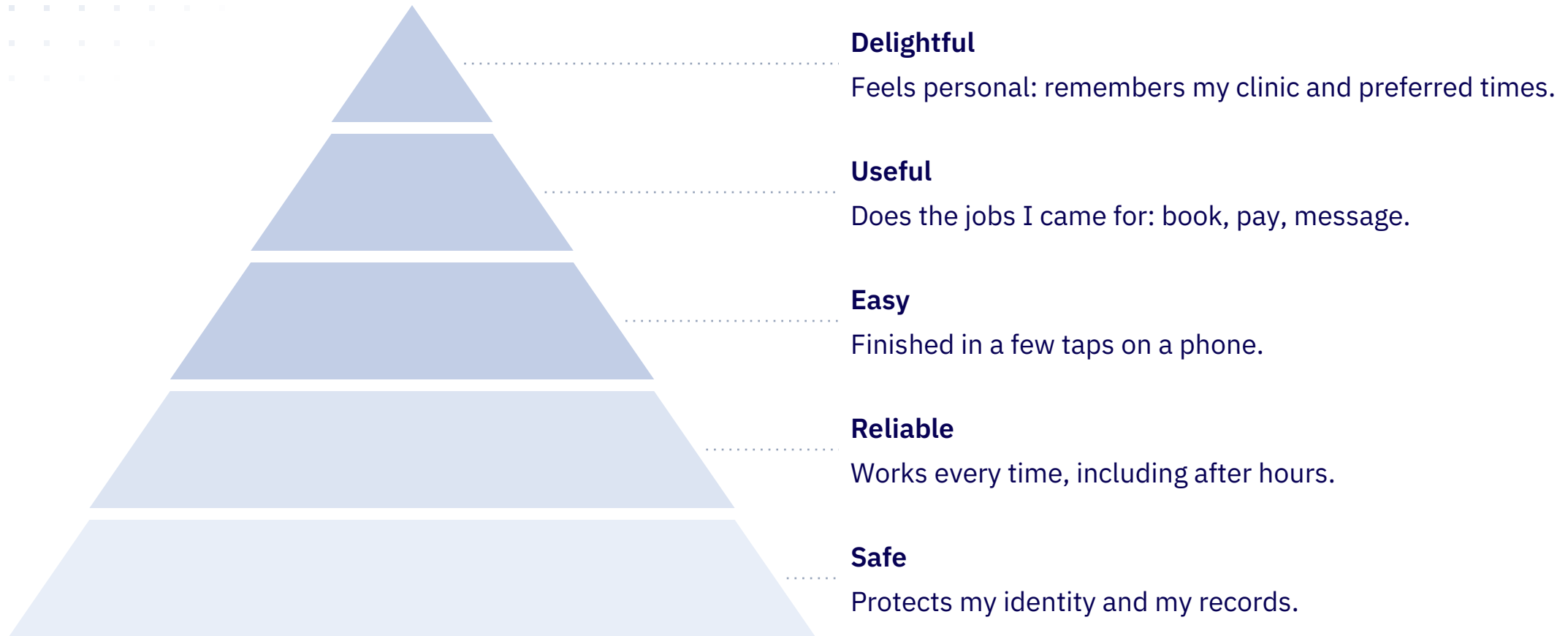
CARE MODEL

Most patients need light-touch care



Source: Sample figures for layout only; replace with current firm data before use.

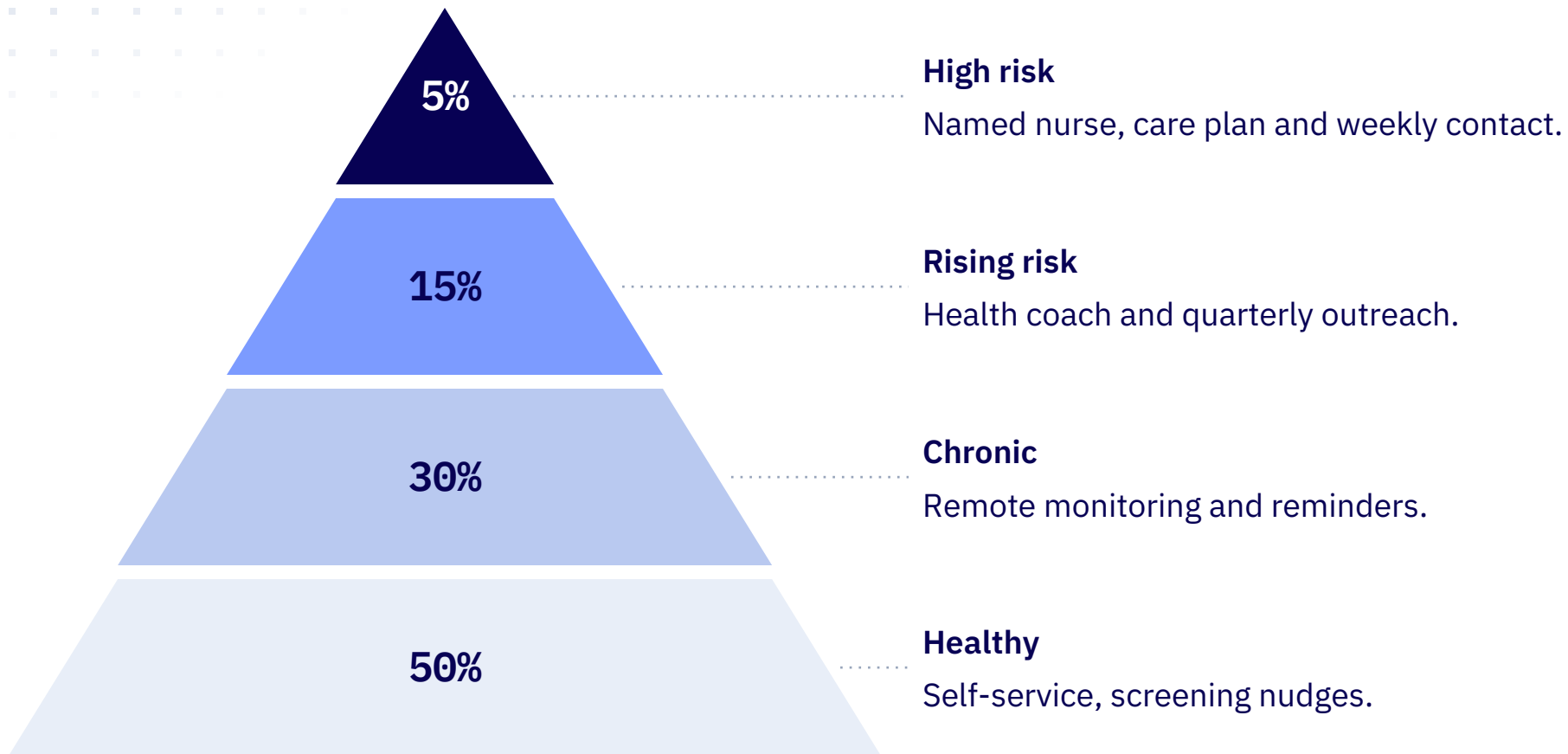
Fix the basics before adding delight



Source: Northfield Health System digital front door program, Sep 2026.

CARE MODEL

Four risk tiers decide how we reach each patient

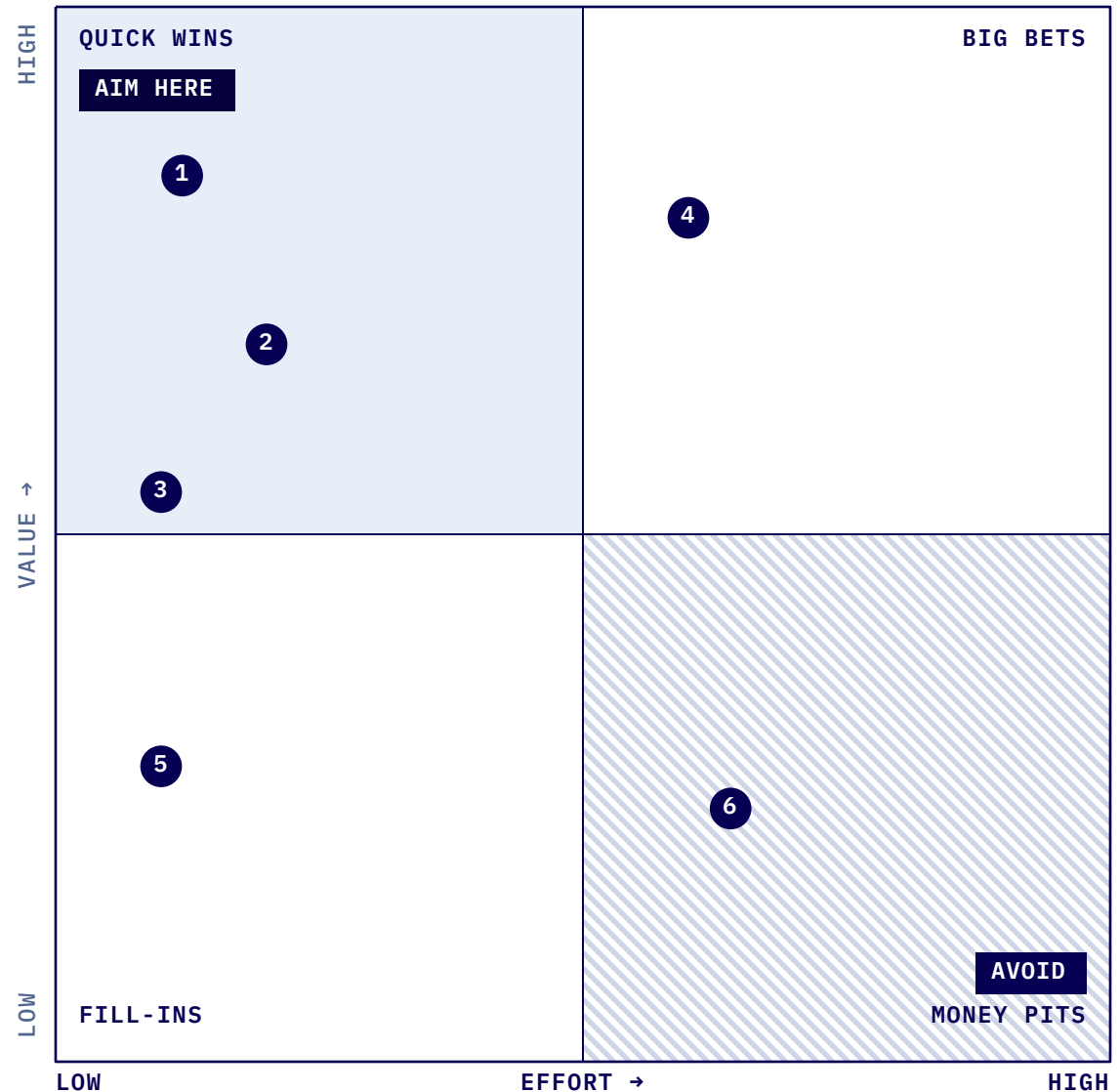


Source: Sample figures for layout only; replace with current firm data before use.

PRIORITIZATION

Six initiatives, ranked by value and effort

- 01 Reconciliations**
Automated matching, wave 1.
- 02 Rules-based review**
Replaces manual checks.
- 03 Close calendar**
One shared timetable.
- 04 Consolidation**
Highest value, most effort.
- 05 Account rationalization**
Fewer accounts to maintain.
- 06 Custom ERP reports**
Cost outruns the return.



Source: Northfield finance workshops, Sep 2026.

RISK

Out of the red zone by wave 3

STAY OUT

Labs sit in the red zone

Rules replace manual review.

STAY OUT

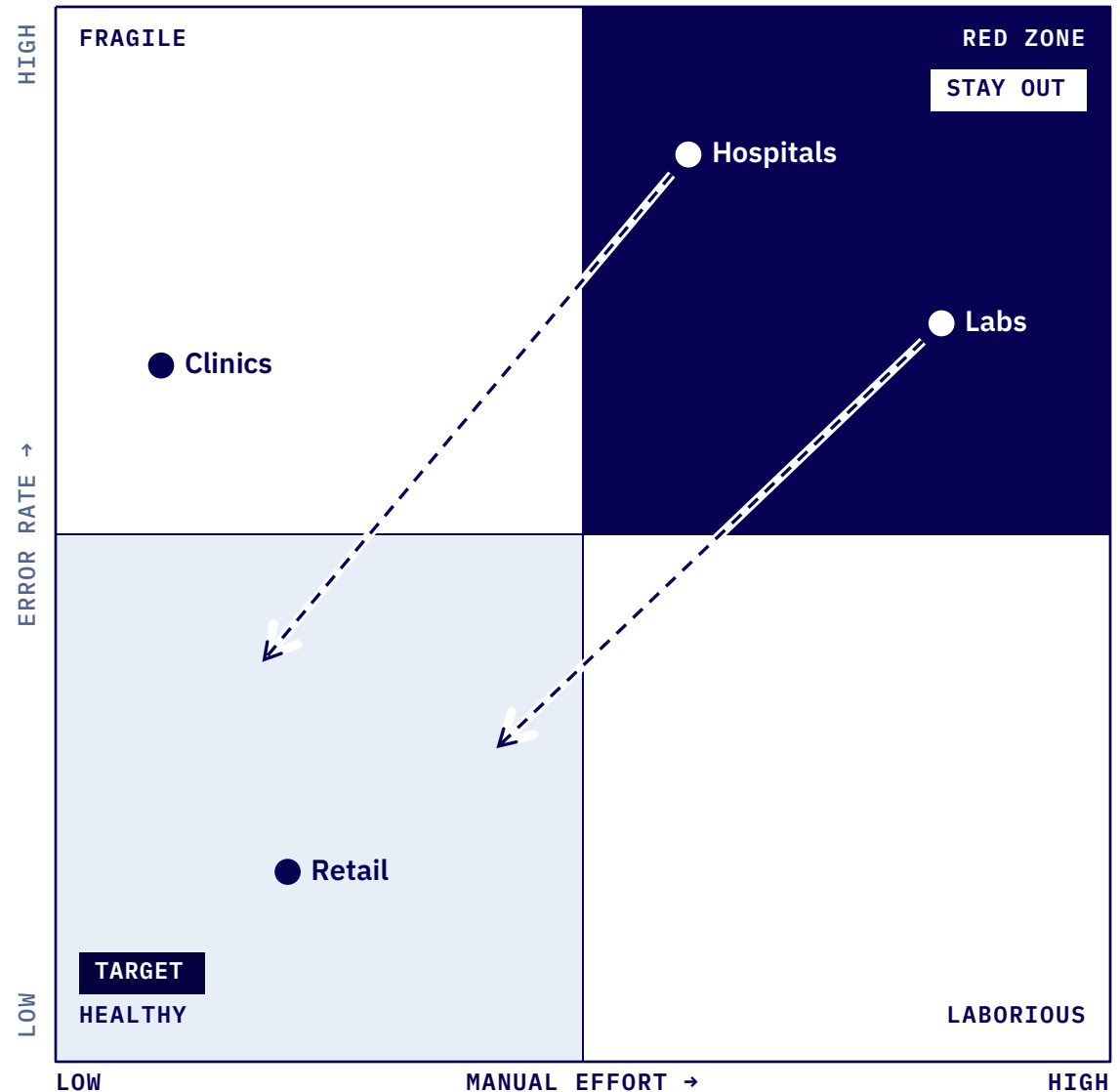
Hospitals follow next

Errors fall with earlier checks.

TARGET

Healthy by wave 3

Clinics and retail hold steady.



Source: Entity error review, 14 units, Aug 2026.

PRIORITIZATION

Quick wins first: high value, low effort

AIM HERE

Quick wins come first

Three items pay back inside wave 1.

PLAN CAREFULLY

Big bets need a sponsor

High value, but the most effort.

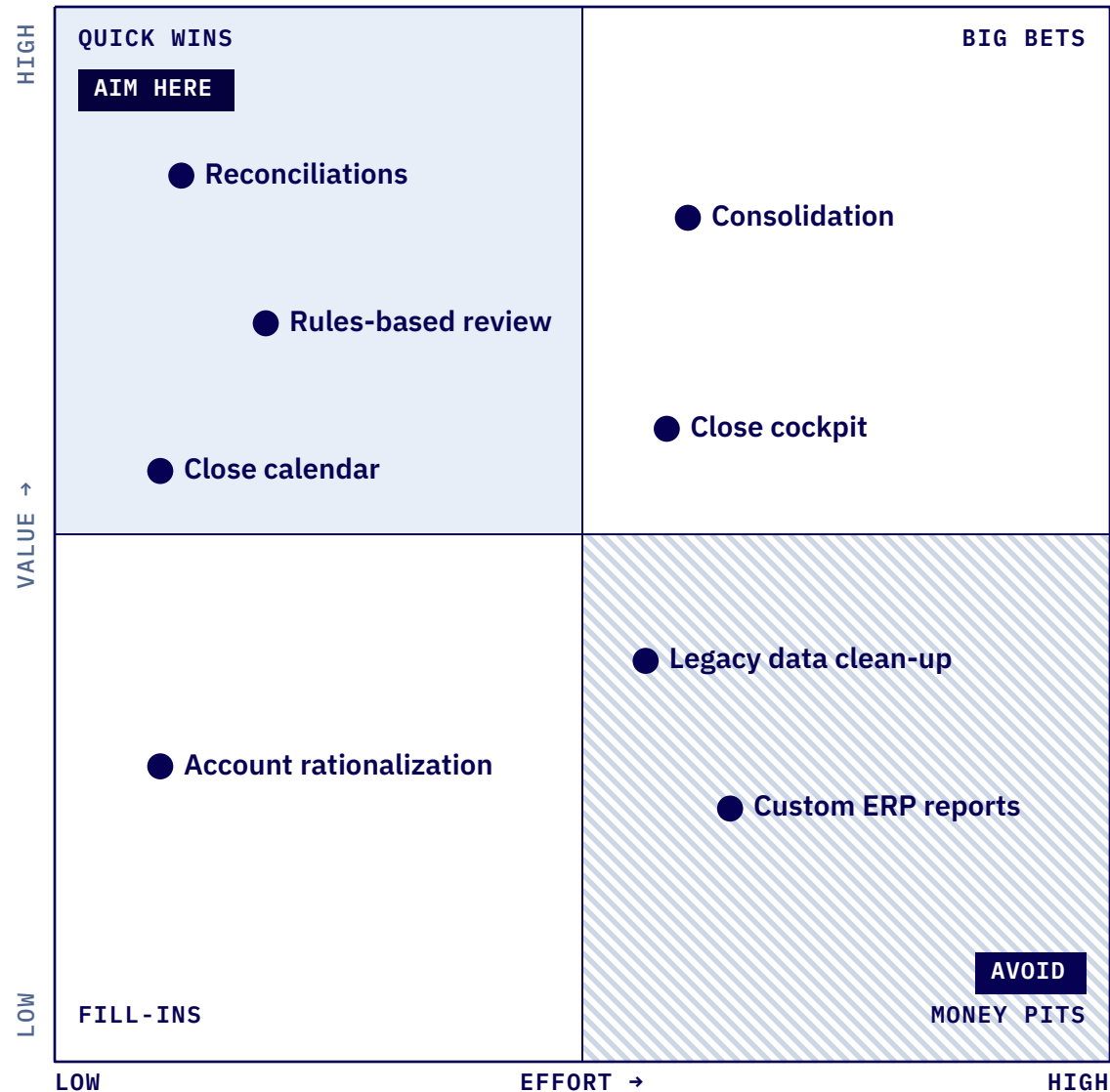
AVOID

Skip the money pits

Cost outruns return on both items.



Source: Northfield finance workshops, Sep 2026.



PRIORITIZATION

Start with quick wins, not money pits

AIM HERE

Quick wins come first

Three items pay back inside wave 1.

PLAN CAREFULLY

Big bets need a sponsor

High value, but the most effort.

AVOID

Skip the money pits

Cost outruns return on both items.

Source: Northfield finance workshops, Sep 2026.

One quote, four reasons behind it

“

We do not need faster spreadsheets. We need to stop finding problems on day nine.

CORPORATE CONTROLLER • NORTHFIELD
HEALTH SYSTEM

01 Day nine

Controllers first see most entries after the books are drafted.

02 Rework

Each late finding reopens work that was already signed off.

03 Spreadsheets

Eleven entities reconcile by email and spreadsheet.

04 Ownership

Tasks have no named owner or due time.

One quote, four reasons behind it

01 Day nine

Controllers first see most entries after the books are drafted.

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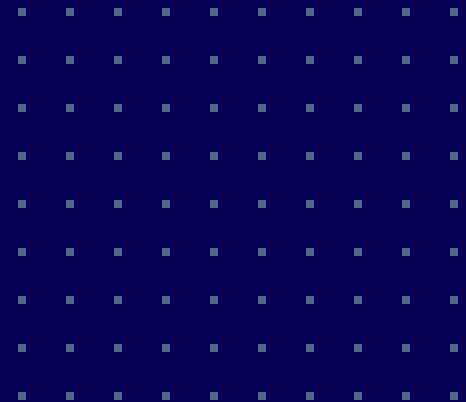
CORPORATE CONTROLLER • NORTHFIELD
HEALTH SYSTEM

WHAT WE HEARD

“

We do not need faster spreadsheets. We need to stop finding problems on day nine.

CORPORATE CONTROLLER • NORTHFIELD HEALTH SYSTEM



WHAT WE HEARD

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We do not need faster spreadsheets. We need to stop finding problems on day nine.

CORPORATE CONTROLLER • NORTHFIELD HEALTH SYSTEM





WHAT WE HEARD



**We do not need faster spreadsheets.
We need to stop finding problems on
day nine.**

CORPORATE CONTROLLER • NORTHFIELD HEALTH SYSTEM

WHY IT MATTERS

Day nine is when controllers first see most entries. By then the books are drafted, so every finding reopens work that was already signed off.

WHAT WE DID WITH IT

We moved review to the start of the close and set rules that check entries as they post.

Skills and sustainment are the widest readiness gaps

STAKEHOLDER GROUP	AWARENESS	BUY-IN	SKILLS	TOOLS	SUSTAIN
Executives	0	1	1	1	2
Physicians	1	3	2	2	3
Nurses	1	2	3	2	3
Front desk	1	1	3	3	2
Revenue cycle	2	2	3	2	2
Pharmacy	1	1	2	1	2
IT service desk	0	0	1	1	1

No gap Small Moderate Large Critical

Source: Northfield change readiness survey, Sep 2026.

WIDEST GAPS

Physicians and nurses

Both groups score 3 on buy-in or skills, the two levers that decide adoption.

SUSTAINMENT

No group is secure

Six of seven groups carry a gap of 2 or more once the team steps back.

STRENGTH

Leaders and IT

Executives and the service desk are ready to coach others.

CHANGE READINESS

Harbor Clinics are the least ready site

SITE	AWARENESS	BUY-IN	SKILLS	TOOLS	SUSTAIN
Main campus	0	1	1	1	1
Lakeside	1	1	2	2	1
Eastgate	1	2	2	2	2
Ridgeview	1	2	3	2	2
St. Anne's	1	1	2	3	2
Harbor Clinics	2	3	3	4	3
Westbrook	1	2	2	2	3
Northgate	2	2	3	3	2
Riverside	2	3	2	3	3

No gap Small Moderate Large Critical

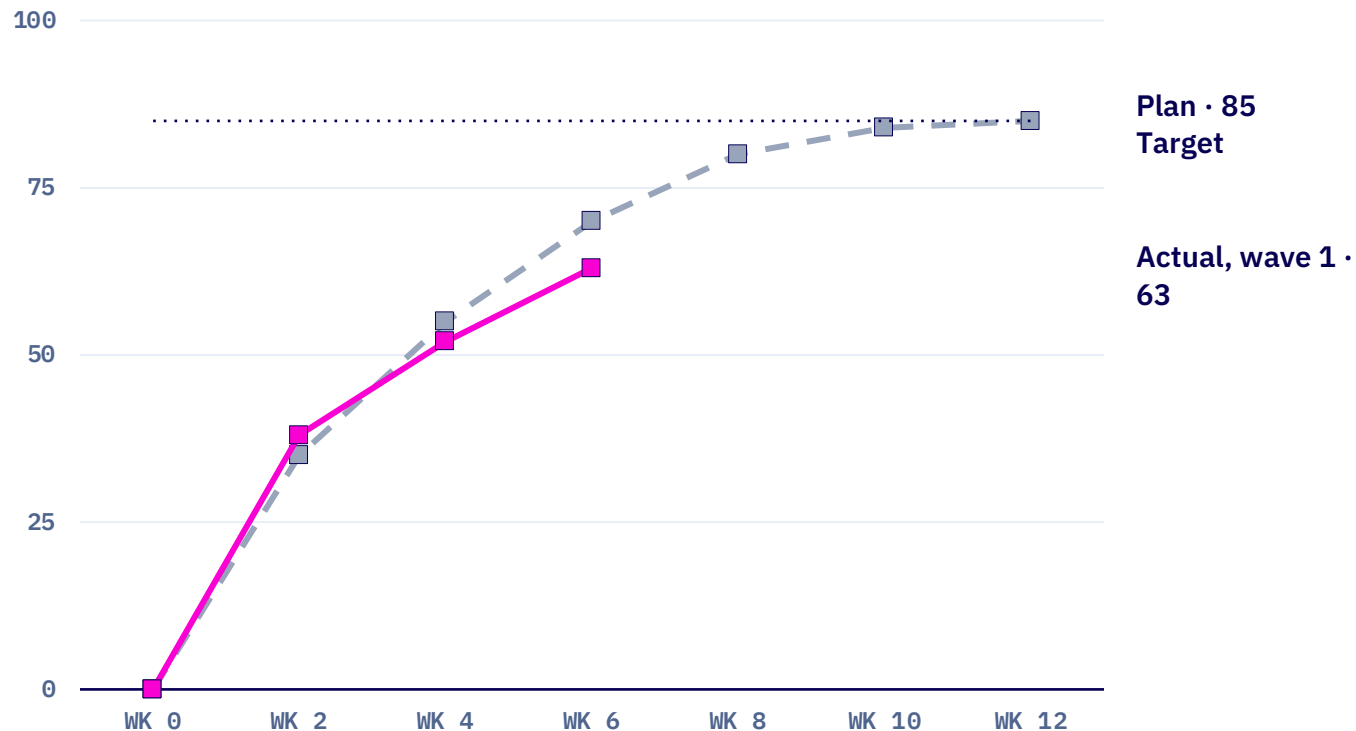
- **Harbor Clinics.** The only site with a critical gap, on tools, and a large gap on buy-in.
- **Riverside.** Urgent care staff cannot spare time for training.
- **Main campus.** No gap above small; use its super users to coach the others.

Source: Northfield change readiness survey, Sep 2026.

Adoption must reach 85% of bookings by week 12

Share of appointments booked in the new platform

PERCENT OF BOOKINGS, BY WEEK AFTER GO-LIVE



Source: Northfield Health System adoption plan; sample targets for layout only.

55%

adoption by week 4

70%

adoption by week 6

85%

adoption by week 12

Below plan for two straight checks triggers an intervention at that site.

Four sources of resistance, each with a planned response

01 Fear of job loss

WHAT WE HEAR

Rumors that automated scheduling cuts roles

RESPONSE

Publish role cards and a no-layoff statement

OWNER

HR director

02 Workload

WHAT WE HEAR

No time to train during busy clinic days

RESPONSE

Protect two hours per person; backfill coverage

OWNER

Practice managers

03 Tool distrust

WHAT WE HEAR

Past rollouts failed and were rolled back

RESPONSE

Sandbox, peer demos and a visible fix list

OWNER

Change lead

04 Loss of control

WHAT WE HEAR

Physicians lose their personal template rules

RESPONSE

Template sign-off by each clinic lead

OWNER

Medical director

Monitor: a two-question pulse survey every two weeks.

Source: Northfield Health System readiness interviews, Sep 2026.

Training attendance is the largest change risk

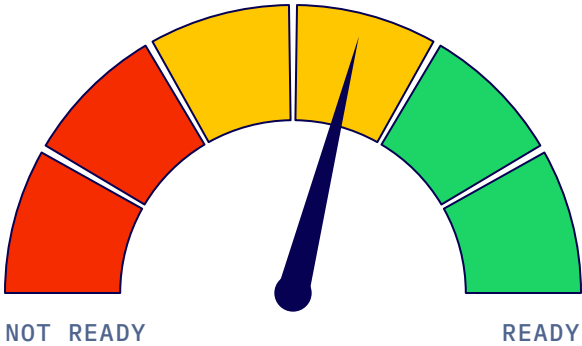
ID	RISK	CAUSE	RATING	MITIGATION
R1	Staff cannot attend training	Clinic coverage gaps	High	Extend sessions; add evening classes
R2	Schedulers revert to paper	No fallback habits retired	High	Floor-walkers; retire old screens at T+2
R3	Physician template rejection	Rules differ by clinic	Medium	Co-design templates with medical director
R4	Union grievance on role change	Role cards unclear	Medium	Joint briefing; clarify task changes
R5	Champion fatigue	Volunteers carry day jobs	Low	Cap hours; recognize publicly

Low Medium High

Source: Northfield Health System change risk register, Sep 2026.

Capacity and skills are the two dimensions below target

DIMENSION	SCORE OF 4	TARGET	STATUS
Leadership sponsorship	● ● ● ●	4	■ On track
Awareness of the change	● ● ● ●	4	■ On track
Skills and training	● ● ● ●	4	■ At risk
Local champions in place	● ● ● ●	4	■ At risk
Tools and sandbox access	● ● ● ●	4	■ At risk
Capacity to absorb change	● ● ● ●	3	■ Off track
Support model readiness	● ● ● ●	4	■ On track



2.6

Overall readiness (of 4)

3 of 7

at target; wave 1 needs five

Source: Northfield Health System readiness survey, Sep 2026.

The platform is generally available when Northfield can run it as one owned service

**Ready for GA when
Northfield can
confidently**

**say yes to the next
qualified workload**

and fulfill the request through one
standard enterprise service.

Architecture approved

- Role in the data estate is clear
- Placement criteria documented
- Approved patterns established

Automated and repeatable

- Environments provisioned consistently
- Policies embedded in automation
- Manual activities kept to a minimum

Adoptable and consumable

- Clear intake and onboarding
- Documentation and training ready
- Teams know when to use it

Secure and governed

- Identity and workspace segmentation
- Lineage and catalog integration
- Controls tested, remediation defined

Observable and supportable

- Health, usage and cost visible
- Alerting and runbooks in place
- Ownership and support established

Owned and sustainable

- Roles and decision rights clear
- Northfield teams can operate it
- Chargeback mechanisms in place

GA means Northfield runs it as one owned service

Ready for GA when Northfield can confidently say yes to the next qualified workload

and fulfill the request through one standard enterprise service.

Architecture approved

- Role in the data estate is clear
- Placement criteria documented
- Approved patterns established

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- Northfield teams can operate it
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RFP requirements map

REQUIREMENTS MAP • CONTINUED

RFP REF	REQUIREMENT	HOW WE MEET IT	SECTION
3.1.2	Name one accountable executive sponsor	- Sponsor named in week 1 - Monthly steering committee	Governance, p. 18
3.2.1	Reduce close to five days	- Rules at posting - Day 1 review	Approach, p. 7
3.2.4	Single source for close status	- Close cockpit - Daily status digest	Solution, p. 12
3.3.2	Audit-ready journal review	- Full audit trail - Exception routing	Controls, p. 15
3.4.1	Transfer skills to Northfield staff	- Paired delivery - Playbooks and training	Team, p. 22

Source: Northfield Health System RFP, section 3.

One risk is both certain and critical

LIKELIHOOD	IMPACT (1 MINOR TO 5 CRITICAL)				
	1	2	3	4	5
5 Almost certain			R9		R1
4 Likely		R7		R3	R10
3 Possible		R6	R2	R4	
2 Unlikely			R8		R5
1 Rare					

Low 1–3
Watch 4–9
Medium 10–15
High 16–21
Extreme 22–25

R1, SCORE 25

Interface failure at cutover

Owner: CIO. Mitigation: dual-run the lab and pharmacy interfaces for two weeks.

- R10 and R3 follow at scores of 20 and 16.
- Both share the same migration team and cutover window.
- After the dual run, R1 drops to a score of 15.

Source: Northfield program risk register, Sep 2026.

Mitigation clears the top two bands of the grid

BEFORE MITIGATION: LIKELIHOOD BY IMPACT

LIKELIHOOD	1	2	3	4	5
Certain			R9		R1
Likely		R7		R3	R10
Possible		R6	R2	R4	
Unlikely			R8		R5
Rare					

AFTER MITIGATION: LIKELIHOOD BY IMPACT

LIKELIHOOD	1	2	3	4	5
Certain					
Likely					
Possible			R9		R1
Unlikely	R7	R6	R2 R8	R3 R4	R10
Rare					R5

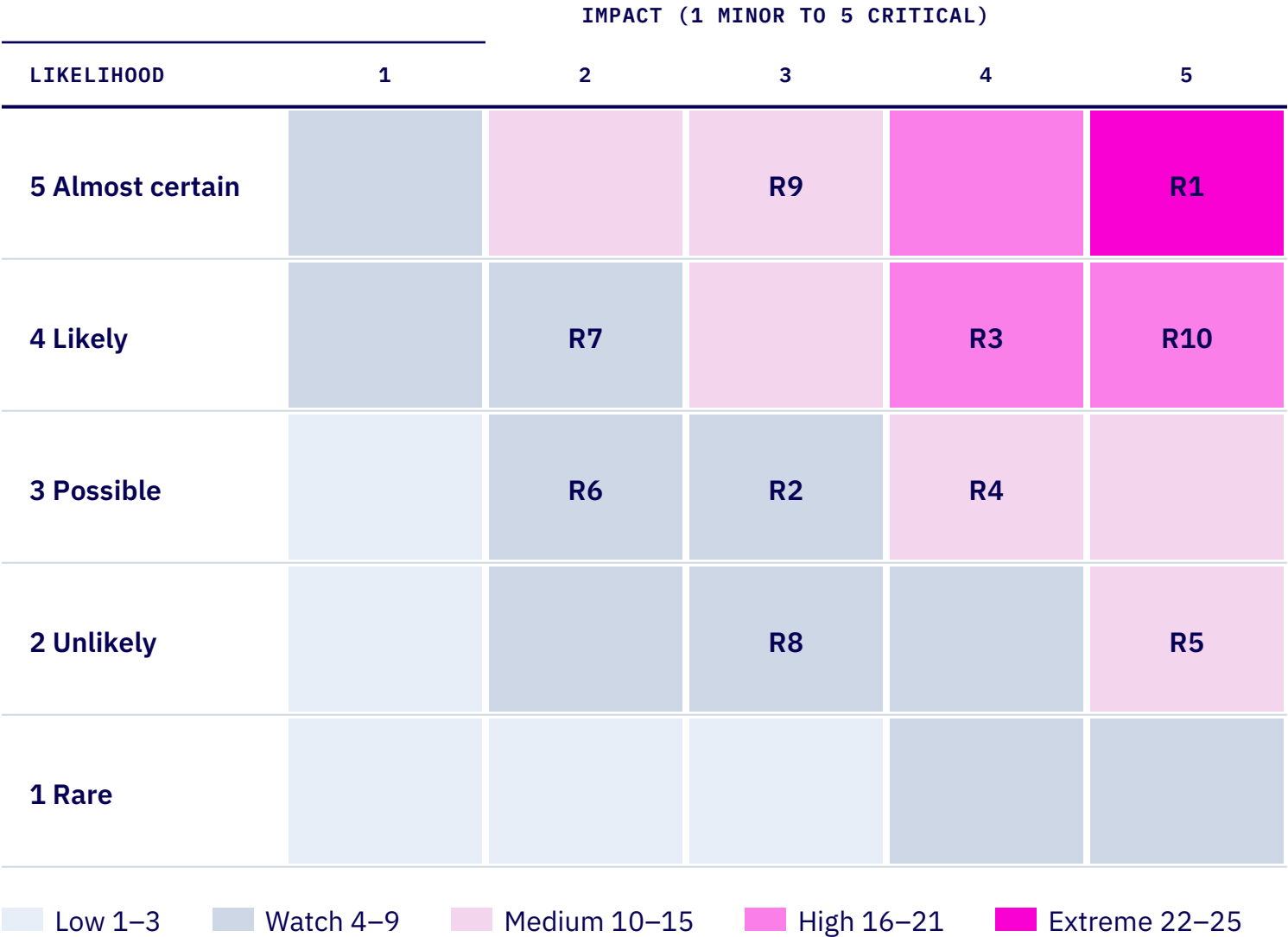
Low 1–3 Watch 4–9 Medium 10–15 High 16–21 Extreme 22–25

Source: Northfield program risk register, Sep 2026.

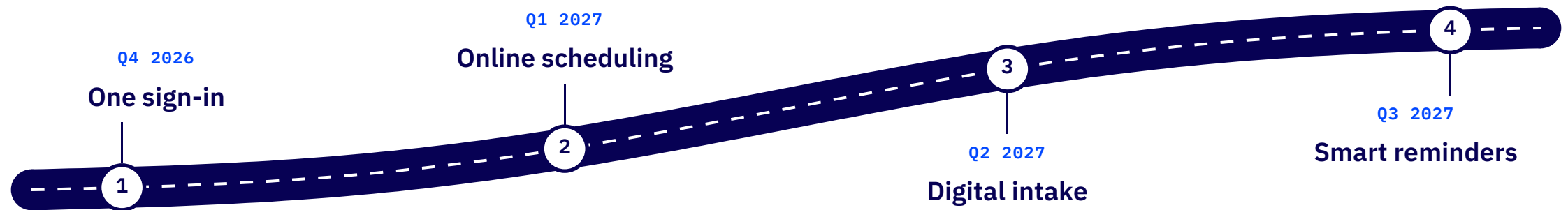
Three risks sit in the top two bands

ID	TOP RISKS, BY SCORE
R1	Interface failure at cutover
R10	Downtime at go-live
R3	Data migration errors
R9	Scope creep
R4	Vendor delivery slips
R2	Clinician champions leave
R5	Cybersecurity incident

Source: Northfield program risk register, Sep 2026.



The first four milestones land inside 12 months



01 Sign-in

Replaces three logins; 60% migrate in 90 days.

02 Self-booking

Primary care first, then specialty clinics.

03 Digital intake

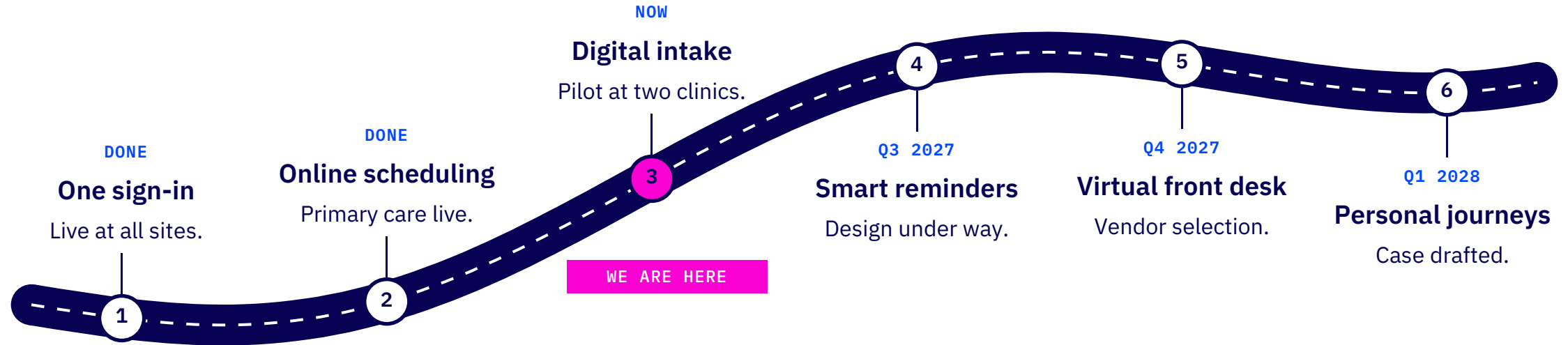
Forms and coverage checks before arrival.

04 Reminders

Two-way text cuts no-shows by a quarter.

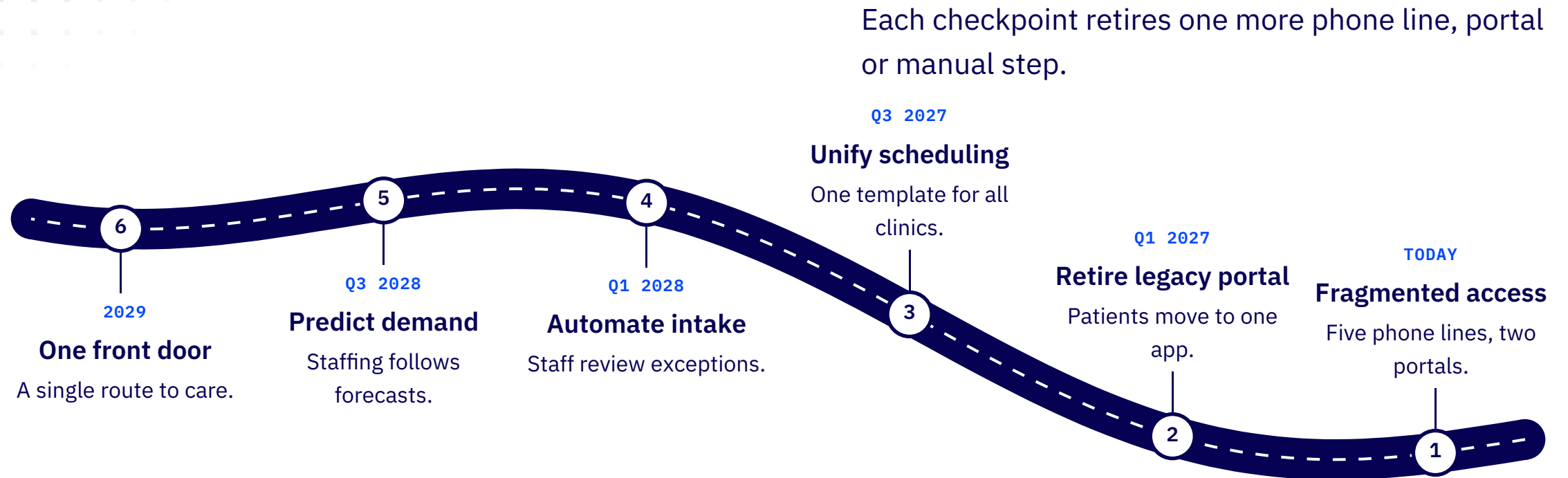
Source: Sample figures for layout only; replace with current firm data before use.

Digital intake is next: we are two milestones in



Source: Northfield Health System digital front door program, Sep 2026.

Six checkpoints lead back to one front door



Source: Northfield Health System digital front door program, Sep 2026.

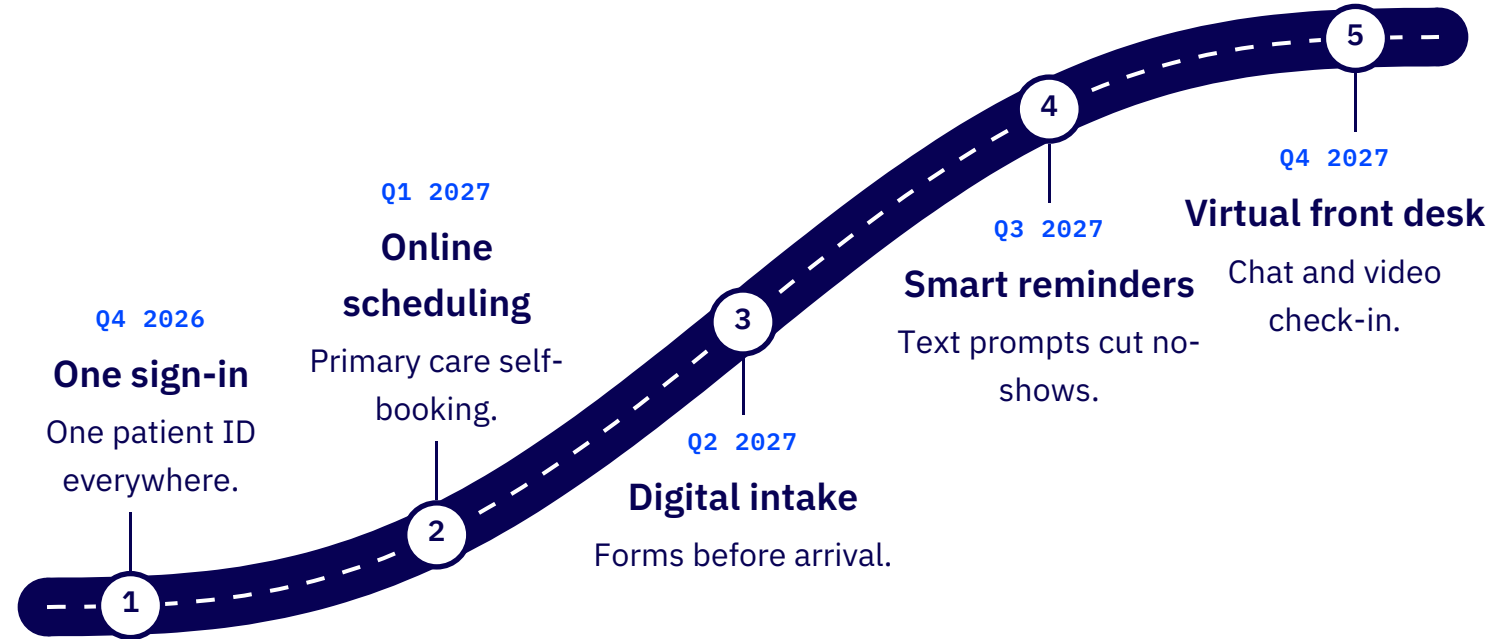
Five releases make the front door real

Each release removes a reason for patients to phone the access center.

- **Early.** Sign-in and booking build the habit.
- **Later.** Reminders and chat raise completion.

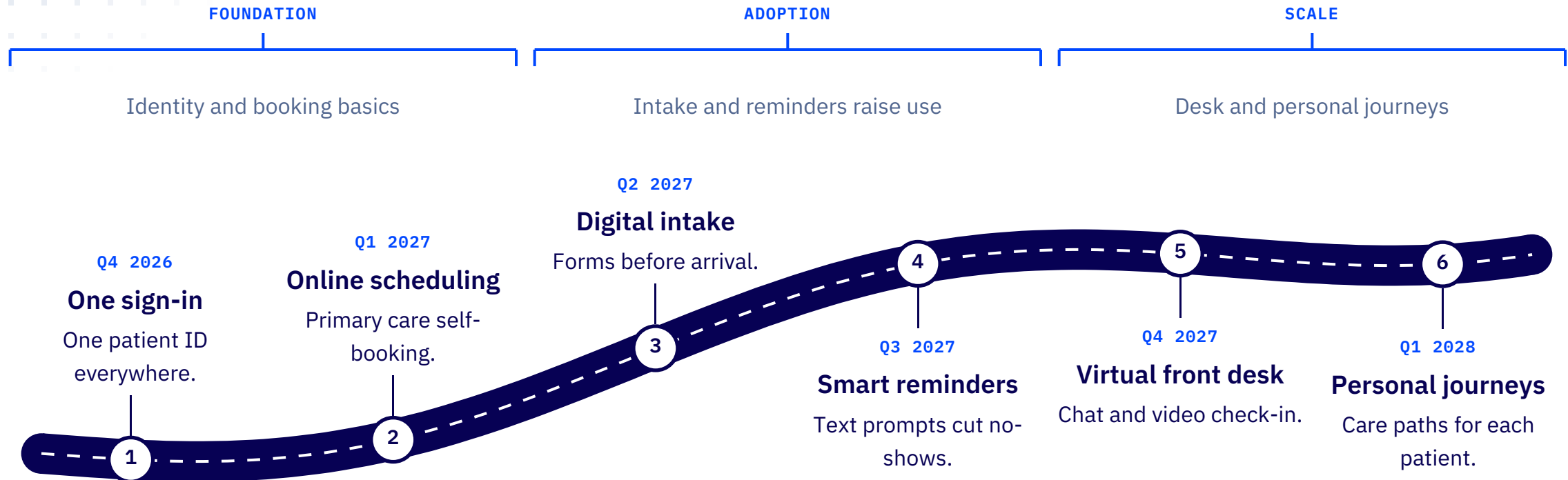
5

releases in 12 months



Source: Northfield digital front door program.

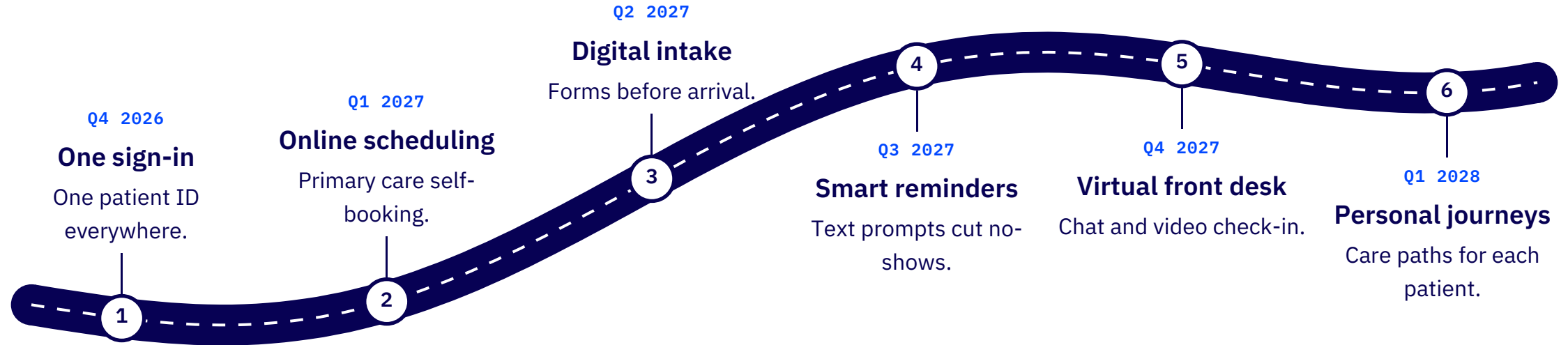
Three phases take sign-in to personal journeys



Source: Northfield Health System digital front door program, Sep 2026.

The digital front door ships in six steps over 18 months

Each release removes a reason for patients to phone the access center.



Source: Northfield Health System digital front door program, Sep 2026.

The roadmap in one view

Now	Next	Later
Self-scheduling	Care plans	Caregiver proxy
Waitlist	Remote monitoring	Multilingual booking
Messaging triage	Cost estimates	No-show outreach
Pay online	Pre-visit forms	Kiosk check-in
Payment plans	Console redesign	Data re-platform

IN BRIEF

Front door first, then care.

NOW

Remove friction from booking and billing.

NEXT

Extend care between visits.

LATER

Widen access to caregivers and communities.

Confidence falls as the horizon lengthens

Now

Committed: funded and staffed

RELEASE	CONFIDENCE
Self-scheduling	● ● ● ●
Waitlist	● ● ● ●
Messaging triage	● ● ● ●
Pay online	● ● ● ●

Next

Probable: scoped, not yet funded

RELEASE	CONFIDENCE
Care plans	● ● ● ●
Remote monitoring	● ● ● ●
Cost estimates	● ● ● ●
Console redesign	● ● ● ●

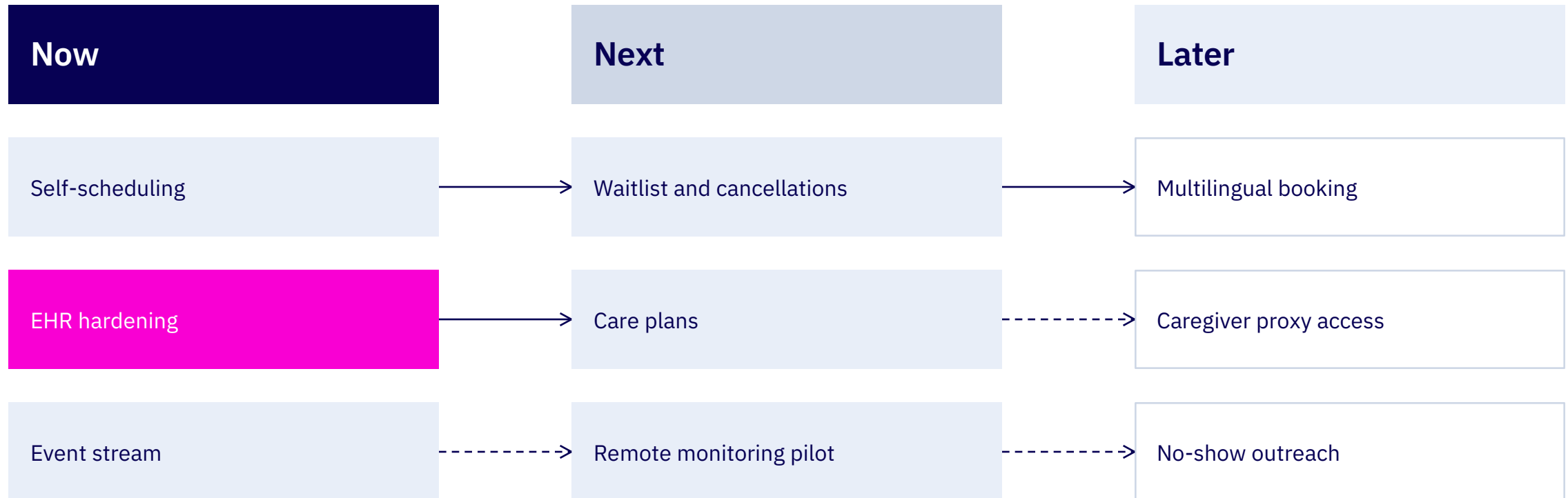
Later

Exploratory: needs discovery

RELEASE	CONFIDENCE
Caregiver proxy	● ● ● ●
Multilingual booking	● ● ● ●
No-show outreach	● ● ● ●
Data re-platform	● ● ● ●

Each Next and Later release is re-scored at the quarterly product council.

EHR hardening gates the later horizons



READING THE ARROWS

Solid arrows are hard dependencies; dashed are soft.

CRITICAL PATH

EHR hardening gates care plans, and a slip there moves caregiver access with it.

Each capability group moves through three horizons

	Access	Care	Billing	Platform
Now NEXT 90 DAYS	- Self-scheduling - Waitlist - Provider search	- Secure messaging triage - Lab alerts - Visit summaries	- Pay online - Payment plans - Statement redesign	- EHR hardening - Release pipeline - Crash reporting
Next TWO QUARTERS	- Pre-visit forms - Virtual visit v2 - Reminders v2	- Care plans - Remote monitoring - Risk flags	- Cost estimates - Card capture - Refund tracking	- Event stream - Observability - Test automation
Later BEYOND SIX MONTHS	- Multilingual booking - Kiosk check-in - Group visits	- Medication adherence - Proxy access - Wearable sync	- Price transparency API - Estimate accuracy - Coverage checks	- Data store re-platform - Zero-downtime release - Cost dashboards

Source: Northfield product council roadmap, Sep 2026.

Releases by capability group across three horizons

CAPABILITY GROUP	NOW · NEXT 90 DAYS	NEXT · TWO QUARTERS	LATER · BEYOND SIX MONTHS
Access	- Self-scheduling, primary care - Waitlist and cancellations	- Pre-visit forms - Virtual visit join v2	- Multilingual booking - Kiosk check-in
Care	- Secure messaging triage - Lab results alerts	- Care plans in the app - Remote monitoring pilot	- Medication adherence - Caregiver proxy access
Billing	- Pay online and statements - Payment plans	- Cost estimates - Insurance card capture	Price transparency API
Engagement	- Smart reminders - In-app feedback	- Health content library - Community programs	Predictive no-show outreach
Platform	- EHR integration hardening - Release pipeline	- Event stream - Observability	Data store re-platform

Three horizons take the patient app from booking to care

Now	Next	Later
NEXT 90 DAYS	FOLLOWING TWO QUARTERS	BEYOND SIX MONTHS
Remove friction from booking	Extend care between visits	Widen access and reach
Self-scheduling for primary care	Care plans in the app	Proxy access for caregivers
Waitlist and cancellations	Remote monitoring pilot	Community programs
Secure messaging triage	Cost estimates before visits	Predictive no-show outreach
Pay online and statements	Clinician console redesign	Multilingual experience

Each horizon ships one outcome patients can feel



Three horizons take the patient app from booking to care

- **Read across** to see what ships and when.
- **Read down** to see how the product matures.
- **Move items** only through the product council.

Horizons are planning commitments, not dates.

Now

NEXT 90 DAYS

- Self-scheduling
- Waitlist
- Messaging triage
- Pay online
- Payment plans
- EHR hardening

Outcome: bookings complete without a call

Next

TWO QUARTERS

- Care plans
- Remote monitoring
- Cost estimates
- Pre-visit forms
- Event stream
- Observability

Outcome: care continues between visits

Later

SIX MONTHS AND BEYOND

- Proxy access
- Multilingual booking
- Kiosk check-in
- Medication adherence
- No-show outreach
- Data store re-platform

Outcome: access reaches every community

WHY THIS ORDER

Patients give up at the front door before they reach care.

Fix booking first, extend care between visits next, then widen who the app reaches.

RULE OF THUMB

Each horizon names the problem, not the feature list.

PRODUCT ROADMAP

One patient problem per horizon

Now NEXT 90 DAYS

Patients give up on booking before they reach a visit

Measure: bookings completed without a call

Ships: self-scheduling, waitlist, messaging triage

Next TWO QUARTERS

Patients lose the thread between visits

Measure: care-plan tasks completed in the app

Ships: care plans, remote monitoring pilot, cost estimates

Later SIX MONTHS AND BEYOND

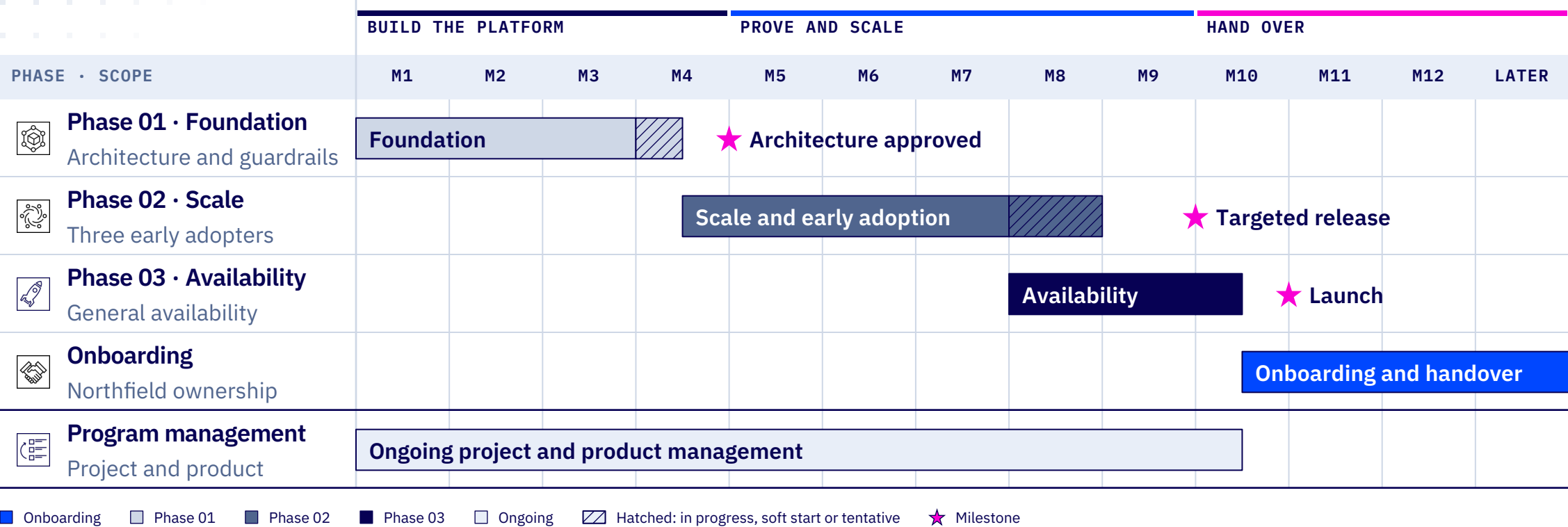
Families and caregivers are locked out of the app

Measure: share of patients with a proxy or language preference

Ships: proxy access, multilingual booking

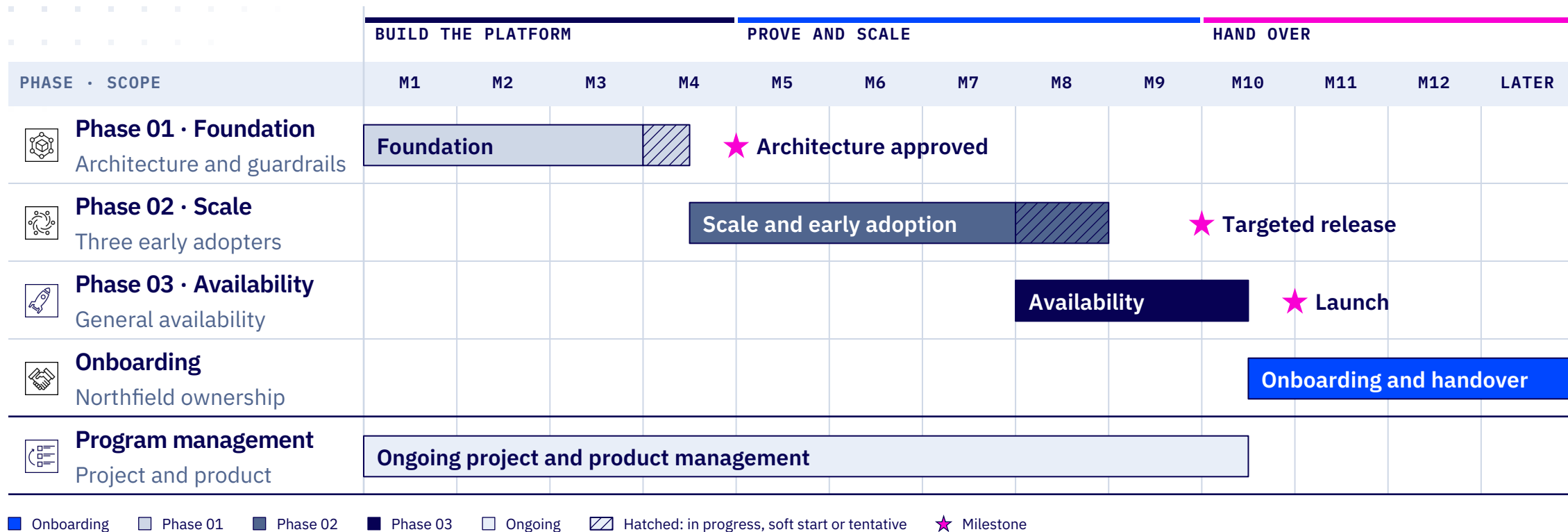
Horizons are planning commitments, not delivery dates.

Nine months from build to Northfield ownership



OUR APPROACH

Nine months from build to Northfield ownership



Now is mostly on track; messaging triage needs help

Now	Next	Later
Self-scheduling Access squad · live in May Waitlist Access squad · beta in Oct Messaging triage Care squad · staffing gap Pay online Billing squad · in test	Care plans Care squad · design done Remote monitoring Care squad · scoping Cost estimates Billing squad · discovery Console redesign Platform squad · planned	Caregiver proxy Care squad · unscheduled Multilingual booking Access squad · unscheduled No-show outreach Data squad · unscheduled Data re-platform Platform squad · unscheduled
Shipped On track At risk Not started		

Releases land lane by lane across three horizons

PRODUCT AREA	Now · 90 days	Next · two quarters	Later · six months+
Access	Self-scheduling		Multilingual booking
		Waitlist and cancellations	
Care	Messaging triage	Care plans in the app	
		Remote monitoring pilot	Caregiver proxy access
Billing	Pay online	Cost estimates	Insurance card capture
	Statements and plans		
Platform	EHR hardening	Event stream	
		Observability	Data store re-platform

Dark bars are committed, mid-blue planned, outlined exploratory. Magenta marks the launch commitment.

Three themes organise six bets across the horizons

THEME 1

Frictionless access

Patients book, pay and message without a call.

NOW • KEY BET

Self-scheduling

Primary care first, then specialties.

NEXT

Cost estimates

Prices shown before the visit is booked.

THEME 2

Care between visits

The app carries the care plan home.

NOW

Messaging triage

Nurses route every message in one queue.

NEXT • KEY BET

Remote monitoring

Pilot with chronic-care cohorts.

THEME 3

A platform that scales

One data and release foundation for every product.

NOW

EHR hardening

Two integrations stable at volume.

LATER

Data store re-platform

Move to the shared patient data store.

Roles and expectations are defined for every phase

RESPONSIBILITIES BY PHASE				
ROLE	PHASE 1 · PROVE	PHASE 2 · SCALE	PHASE 3 · LAUNCH	OWNER
Executive sponsor	Set direction, confirm success measures, clear escalations.	Champion early adopters, review risks, remove blockers.	Approve GA readiness and sponsor the launch.	WM
Engagement lead	Own outcomes, scope, quality and client relationships.	Oversee pod execution and manage escalations.	Ensure GA, transition and benefit realization.	WM
Enterprise architect	Define target architecture, standards and interoperability.	Set platform standards and review pod designs.	Define the evolution roadmap and governance model.	WM
Program manager	Mobilize, plan, run RAID, governance and communications.	Sequence adopters; manage capacity, budget and risks.	Manage GA readiness, handoff and program close.	JOINT
Data governance lead	Design policies, access model and metadata approach.	Implement controls for self-service onboarding.	N/A	WM
Lead data engineer	Define ingestion patterns and engineering standards.	Lead pods; direct builds, testing and releases.	Oversee hardening, reuse and knowledge transfer.	WM
Change and training lead	N/A	N/A	Lead readiness, training, office hours and adoption reporting.	JOINT

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Roles shift as the build hands over to Northfield

	RATIONALIZE	BUILD	SUSTAIN
 West Monroe	- Portfolio and architecture review - Business case and roadmap	- Onshore and nearshore delivery - Design, build, test, release	- Part-time expert support - AI oversight and validation
 Northfield team	- Priorities and target outcomes - Process and data context	- Validates workflows - Owns business trade-offs	- Owns the product roadmap - Accepts enhancements
 AI agents	- Code and dependency analysis - Candidate scoring	- Code generation and testing - Documentation at speed	- Routine sustainment automated - Expert-validated on call

Dot scale: five dots show relative team size in each phase; more filled dots means a larger share of the work.

Roles shift as the build hands over to Northfield

		RATIONALIZE		BUILD		SUSTAIN	
		SHARE	ROLE	SHARE	ROLE	SHARE	ROLE
	West Monroe		- Portfolio review - Business case, roadmap		- Nearshore delivery - Design, build, test		- Part-time experts - AI oversight
	Northfield team		- Priorities, outcomes - Process and data context		- Validates workflows - Owns trade-offs		- Owns the roadmap - Accepts enhancements
	AI agents		- Code analysis - Candidate scoring		- Code and test generation - Documentation		- Routine sustainment - Expert-validated

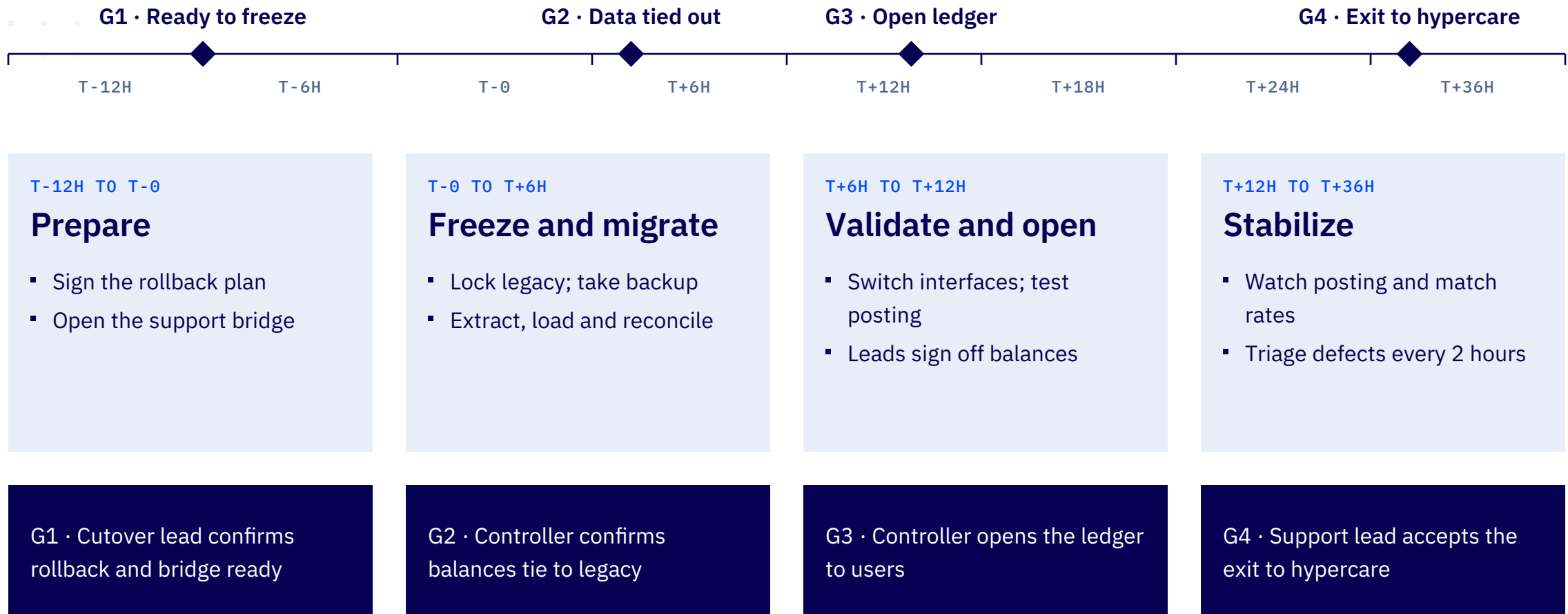
Harvey ball: relative share of the work in each phase, from empty (none) to full (most).

Six checkpoints keep users and leaders informed

CHECK	WHEN	AUDIENCE	MESSAGE	SENDER	SENT
C1	Fri 16:00	All finance users	Freeze starts at 18:00	Controller	☒
C2	Fri 18:00	Finance and IT leads	Freeze confirmed; cutover begins	Cutover lead	☒
C3	Sat 00:00	Steering committee	Midpoint status and any risks	Exec sponsor	☐
C4	Sat 03:30	Steering committee	Go or no-go decision	Cutover lead	☐
C5	Sat 06:00	All finance users	New ledger is open; where to get help	Comms lead	☐
C6	Sat 18:00	Business leaders	End-of-day status and next steps	Exec sponsor	☐

The full message plan, templates and approvals sit in the change communications plan.

Four go/no-go gates pace the weekend cutover

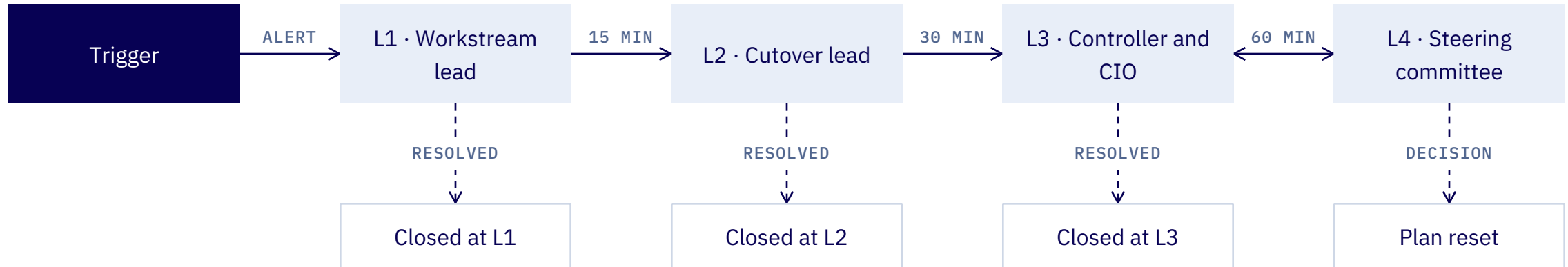


Four levels put every **issue** in front of a decision-maker

LEVEL	TRIGGER	TIME THRESHOLD	CONTACT OR OWNER	WHAT THIS LEVEL DECIDES
L1 · Workstream lead	A step runs late or a check fails	0–15 min	Workstream lead · bridge line	Retry the step or apply its rollback
L2 · Cutover lead	L1 cannot clear it, or the window slips	15–30 min	Program director · ext. 4410	Re-sequence steps; extend a window
L3 · Controller and CIO	Balances do not tie, or a gate is at risk	30–60 min	Controller and CIO · ext. 4420	Go or no-go; invoke full rollback
L4 · Steering committee	Rollback invoked or go-live date at risk	60+ min	Executive sponsor · ext. 4430	Reset the date; approve a new plan

Rule: the clock runs from the first alert. If a level has not acknowledged within its threshold, page the next level.

Each missed threshold moves an **issue** up one level



The first alert on the bridge line starts the clock.

Retry the step or apply its rollback.

Re-sequence steps or extend a window.

Go or no-go; invoke full rollback.

Reset the date and approve a new plan.

Rule: if a level has not acknowledged within its threshold, page the next level. Steering hears the outcome of every rollback.

Four levels put every issue in front of a decision-maker

Each level owns one class of decision. If a level has not acknowledged within its threshold, page the next level.

The clock runs from the first alert, not from the escalation.

L4 • STEERING COMMITTEE • 60+ MIN

- Trigger: rollback invoked or go-live date at risk
- Decides: reset the date; approve a new plan



L3 • CONTROLLER AND CIO • 30–60 MIN

- Trigger: balances do not tie, or a gate is at risk
- Decides: go or no-go; invoke full rollback



L2 • CUTOVER LEAD • 15–30 MIN

- Trigger: L1 cannot clear it, or the window slips
- Decides: re-sequence steps; extend a window



L1 • WORKSTREAM LEAD • 0–15 MIN

- Trigger: a step runs late or a check fails
- Decides: retry the step or apply its rollback

Wave 1 goes live only when every criterion is green

GO/NO-GO CRITERION	OWNER	EVIDENCE	STATUS	GO
Dress rehearsal passed with no open defects	Test lead	Rehearsal log, run 3	 On track	
Balances tie to the legacy trial balance	Controller	Reconciliation report	 On track	
Interfaces tested end to end	Integration lead	Interface test pack	 On track	
Rollback rehearsed within the window	Technical lead	Timed rollback record	 At risk	—
Users trained and access confirmed	Support lead	Training roster, 94%	 At risk	—
Support bridge staffed for 36 hours	Support lead	Rota signed by leads	 On track	
Audit sign-off on migration controls	Internal audit	Control walkthrough memo	 Off track	—

Decision rule: any red criterion is a no-go; an amber criterion needs a written mitigation from the Cutover lead.

Wave 1 needs every check green

GO/NO-GO CRITERION	OWNER	EVIDENCE	STATUS
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WHERE WE STAND

4 of 7

criteria are green

DECISION RULE

Any red criterion is a no-go. An amber criterion needs a written mitigation from the Cutover lead.

Wave 1 needs every check green

WHERE WE STAND

4 of 7

criteria are green

Any red criterion is a no-go. An amber one needs a written mitigation from the Cutover lead.

GO/NO-GO CRITERION	OWNER	STATUS
Dress rehearsal passed, no defects	Test lead	On track
Balances tie to legacy trial balance	Controller	On track
Interfaces tested end to end	Integration lead	On track
Rollback rehearsed in the window	Technical lead	At risk
Users trained, access confirmed	Support lead	At risk
Bridge staffed for 36 hours	Support lead	On track
Audit sign-off on migration controls	Internal audit	Off track

Hypercare starts every day with the same eight checks

DAILY HYPERCARE CHECK	OWNER	BY	RECORDED IN	DONE
Review overnight batch jobs	Data lead	08:00	Job monitor	☒
Check posting and period errors	Controller	08:30	Error log	☒
Confirm interface queues are clear	Integration lead	09:00	Queue dashboard	☒
Triage new tickets by severity	Support lead	09:30	Ticket board	☐
Review the top five user questions	Support lead	10:00	Support notes	☐
Compare performance with baseline	Technical lead	11:00	Monitoring pack	☐
Post the daily status to Steering	Cutover lead	16:00	Status page	☐
Log lessons for the retrospective	Test lead	17:00	Lessons log	☐

Hypercare day 9: tickets are falling in every stream

42

open tickets

▼ 18 SINCE MONDAY

3

Sev 1 and 2 open

▼ 2 SINCE MONDAY

93%

auto-match rate

▲ 3 POINTS

1,180

active users of 1,260

▲ 64 SINCE MONDAY

WORKSTREAM	OPEN TICKETS	SEV 1 AND 2	CHANGE IN WEEK	STATUS
Record to report	14	1	-6	On track
Procure to pay	11	1	-4	On track
Order to cash	9	1	-5	At risk
Treasury and cash	5	0	-2	On track
Reporting and close	3	0	-1	At risk

HYPERCARE EXIT

Day 9

of 14 in hypercare. 3 of 7 criteria are met.

EXIT RULE

Hypercare ends when every criterion is green on two consecutive days. Steering signs the exit.

HYPERCARE

Hypercare exits when seven criteria hold for two days

EXIT CRITERION	TARGET	CURRENT	STATUS
No Sev 1 or 2 defect open	5 days	5 days	 On track
Auto-match rate	92% or more	93%	 On track
Open tickets	Under 25	42	 At risk
Month-end close completed	1 close	In progress	 At risk
Ticket backlog age	Under 3 days	2.4 days	 On track
User satisfaction pulse	80% or more	76%	 At risk
Run-team handover signed	Signed	Pending	 Not started

One runbook governs every wave 1 cutover decision

PURPOSE

Move close and consolidation to the new ledger over one weekend, with a tested way back at every step.

SCOPE

- General ledger and subledgers
- Bank and intercompany interfaces
- Excludes planning and reporting

ROLE	HELD BY	ACCOUNTABLE FOR
Cutover lead	Program director	Runs the bridge; calls go or no-go
Ledger owner	Controller	Signs balances; opens the ledger
Technical lead	Finance IT	Runs loads and interface switch
Support lead	Shared services	User comms and hypercare

ENTRY CRITERIA

- Dress rehearsal signed off
- Rollback plan approved

EXIT CRITERIA

- Day 1 postings reconciled
- Hypercare handover accepted

ESCALATION PATH



CUTOVER RUNBOOK

One runbook governs every wave 1 cutover decision

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- Day 1 postings reconciled
- Hypercare handover accepted

ESCALATION PATH



Six roles hold every **cutover** decision, each with a backup

ROLE	HELD BY	ACCOUNTABLE FOR	DECISIONS AND AUTHORITY	BACKUP
Cutover lead	Program director	The runbook and the bridge	Calls go or no-go; invokes rollback	Deputy program director
Ledger owner	Controller	Balances and ledger opening	Signs balances; opens the ledger	Assistant controller
Technical lead	Finance IT	Loads, jobs and environments	Pauses a load; restores a snapshot	Platform engineer
Integration lead	Enterprise architecture	Interface switch and testing	Repoints interfaces to legacy	Integration engineer
Data lead	Data engineering	Extracts and reconciliation	Stops a load on a variance	Senior data analyst
Support lead	Shared services	User comms and hypercare	Sends notices; triages defects	Service desk manager

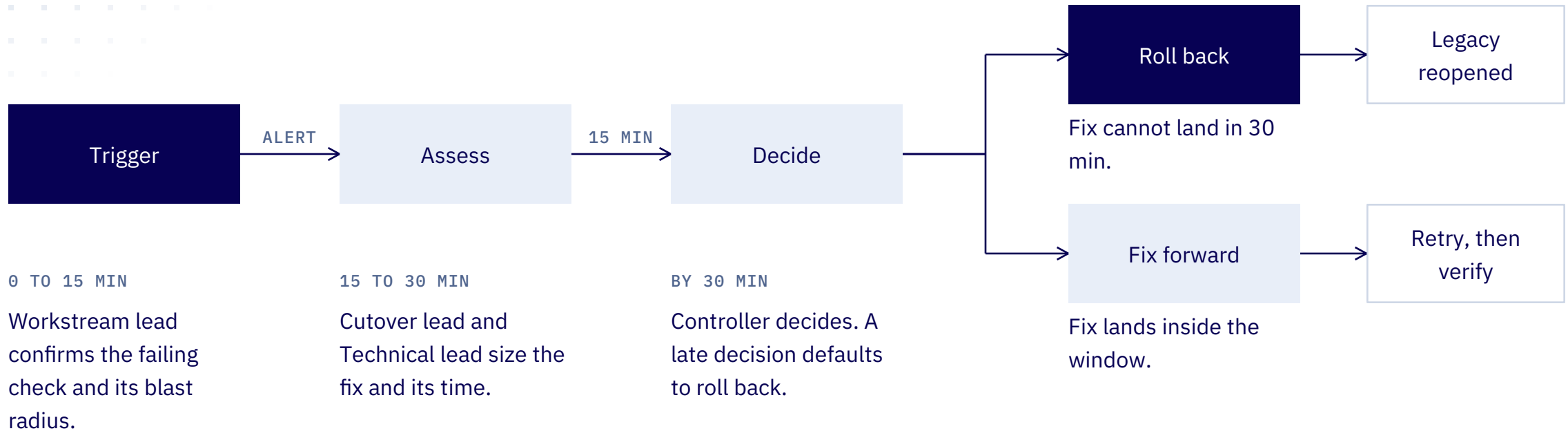
Every cutover step has exactly one accountable role

CUTOVER STEP	CUTOVER LEAD	LEDGER OWNER	TECHNICAL LEAD	INTEGRATION LEAD	DATA LEAD	SUPPORT LEAD
Freeze legacy postings	A	C	R	I	I	I
Extract and reconcile balances	I	A	C	I	R	I
Load balances to the new ledger	I	A	R	I	C	I
Switch interfaces to the new ledger	I	I	C	A R	C	I
Hold the go or no-go decision	A R	C	C	C	C	I
Notify users and open hypercare	A	I	I	I	I	R

R Responsible **A** Accountable **C** Consulted **I** Informed

Authority: only the accountable role signs a step off. The cutover lead's go or no-go call overrides any other sign-off.

A rollback is a decision with a clock on it



Point of no return: 04:30 Saturday. After it, the rollback cannot finish before users arrive.

Eight steps return the ledger to legacy in three hours

STEP	ROLLBACK ACTION	OWNER	BUDGET	VERIFICATION CHECK	DONE
1	Call the rollback on the bridge	Cutover lead	5 min	Call logged with time and reason	☒
2	Stop postings in the new ledger	Controller	10 min	Subledger lock report is clean	☒
3	Disable the new interfaces	Integration lead	15 min	Queues show zero in flight	☐
4	Repoint feeds to the legacy endpoints	Integration lead	20 min	Test postings reach legacy	☐
5	Restore the legacy snapshot	Finance IT	60 min	Checksums match the backup	☐
6	Tie legacy to the pre-cutover trial balance	Data lead	45 min	Zero variance by entity	☐
7	Reopen the legacy ledger to users	Controller	15 min	First posting accepted	☐
8	Notify users and Steering	Comms lead	10 min	Notice sent and acknowledged	☐

Six of eight smoke tests passed on the first run

SYSTEM	SMOKE TEST	OWNER	RUN AT	PASS	STATUS
General ledger	Post and reverse a journal	Controller	Fri 05:00	✓	■ Pass
Payables	Enter and approve an invoice	AP lead	Fri 05:15	✓	■ Pass
Receivables	Raise an invoice; apply cash	AR lead	Fri 05:30	✓	■ Pass
Treasury	Import one bank statement	Treasury	Fri 05:45	—	■ In progress
Reporting	Run the trial balance	Reporting lead	Fri 06:00	✓	■ Pass
Single sign-on	Log in as each of five roles	Security lead	Fri 06:15	✓	■ Pass
Integrations	Payroll journal arrives	Integration lead	Fri 06:30	—	■ Fail
Data warehouse	Nightly refresh completes	Data lead	Fri 06:45	✓	■ Pass

Step 1 freezes every legacy posting before any data moves

STEP 1

Freeze legacy postings

Stop every posting to the legacy ledger so the extract is one stable cut of the books.

OWNERS

- Controller · accountable
- Finance IT · executes
- Internal audit · observes

WINDOW

Fri 18:00 · 30 min

SUB-STEPS

1. Announce the freeze on the bridge
2. Close and post open batches
3. Lock subledgers and the calendar
4. Run the subledger lock report
5. Publish the freeze notice

RULES

- No manual journals after freeze
- Exceptions need the Cutover lead
- Late items are queued, not posted
- Record the freeze time on the log

GATE CLEARANCE

- Lock report shows no open batches
- Controller confirms the freeze on the log
- Cutover lead releases step 2

Tick every reconciliation before step 4 loads balances

STEP 4

Load balances to the new ledger

Load the extracted open balances by entity and prove they match the legacy books.

OWNERS

- Finance IT · executes
- Data lead · reconciles
- Controller · accountable

WINDOW

Fri 22:30 · 2 h

DONE	RECONCILIATION CHECK	LEGACY	TOLERANCE	NEW LEDGER
☐	Row count by entity	412,806	Zero	
☐	Debit and credit totals	\$1.284bn	\$0	
☐	Open items by entity	9,418	±0.1%	
☐	Intercompany balances	\$38.2m	±0.5%	

SUB-STEPS

- Load by entity, parents last
- Run the count and total checks
- Park rejected rows
- Log each variance

ACCEPTANCE

- Every check inside tolerance
- Variances explained and signed
- Data lead releases step 5

Tick every criterion before step 7 opens the ledger

STEP 7

Hold the go or no-go decision

Review the evidence for every criterion and decide whether the ledger opens to users.

OWNERS

- Cutover lead · decides
- Controller · signs balances
- CIO · endorses

WINDOW

Sat 03:30 · 30 min

GO	GO CRITERION	EVIDENCE	APPROVER
☐	Balances tie to legacy	Reconciliation report	Controller
☐	Interfaces pass end to end	Interface test pack	Integration lead
☐	Rollback fits the window	Timed rollback record	Technical lead
☐	Support bridge staffed	Signed rota	Support lead

IF GO

- Controller opens the ledger
- Support lead sends the notice
- Log the decision and time

IF NO-GO

- Cutover lead invokes rollback
- Escalate to level 3
- Reset the date at steering

Steps 1 to 4 lock the legacy ledger and move its balances

1 Freeze legacy postings

- Close open batches
- Lock subledgers
- Publish the freeze notice

Owner: Controller
Fri 18:00 · 30m

2 Take the final backup

- Snapshot the legacy ledger
- Verify checksums

Owner: Finance IT
Fri 18:30 · 1h

3 Extract open balances

- Run the extract jobs
- Tie to the trial balance
- Log variances

Owner: Data lead
Fri 19:30 · 3h

4 Load the new ledger

- Load by entity
- Check counts and totals
- Park rejected rows

Owner: Finance IT
Fri 22:30 · 2h

Step 4 · page 1 of 2 · Continued on next page

RUNBOOK PURPOSE

Lock the legacy ledger and move its balances to the new platform in one window.

RUNBOOK OWNER

Cutover lead

Window: Fri 18:00 to Sat 02:00

Step 4 · page 1 of 2 · continued on next page

CUTOVER RUNBOOK

Steps 1 to 4 lock the legacy ledger and move its balances

1 Freeze legacy postings

Controller · Fri 18:00 · 30m

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Finance IT · Fri 18:30 · 1h

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Finance IT · Fri 22:30 · 2h

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Steps 1 to 4 lock the legacy ledger and move its balances

Each step has an owner and a window. Work to the actions in order and log each one as it completes.

Step 4 · page 1 of 2 · Continued on next page

1 Freeze legacy postings

Owner: Controller
Fri 18:00 · 30m

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Owner: Finance IT
Fri 22:30 · 2h

- Load by entity
- Check counts and totals
- Park rejected rows

Eight steps take the ledger from freeze to go-live

STEP	ACTION	OWNER	WINDOW	VERIFICATION CHECK	ROLLBACK
1	Freeze legacy postings	Controller	Fri 18:00 · 30m	Subledger lock report shows no open batches	Lift the freeze flag
2	Take the final legacy backup	Finance IT	Fri 18:30 · 1h	Checksums match the source	Restore the snapshot
3	Extract open balances	Data lead	Fri 19:30 · 3h	Trial balance ties to the legacy ledger	Re-extract the data
4	Load balances to new ledger	Finance IT	Fri 22:30 · 2h	Row counts and totals reconcile	Purge target; reopen legacy
5	Run the reconciliation rules	Shared services	Sat 00:30 · 2h	Auto-match rate above 90%	Hold; rerun prior rules
6	Switch the interfaces	Integration lead	Sat 02:30 · 1h	Test postings flow end to end	Repoint to legacy endpoints
7	Hold the go or no-go decision	Cutover lead	Sat 03:30 · 30m	Every check is signed on the log	Invoke the full rollback
8	Open the ledger to users	Controller	Sat 06:00 · 1h	First live posting is accepted	Close the ledger; notify users

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Sixteen defects are open and none is critical

SEVERITY	NEW	TRIAGE	FIX	RETEST	CLOSED	FIX BY
Sev 1 · Critical	0	0	0	0	1	4 h
Sev 2 · High	0	1	1	1	6	1 day
Sev 3 · Medium	2	3	2	1	12	3 days
Sev 4 · Low	1	2	1	1	9	Release 2

TRIAGE RULES

- Triage twice a day at 09:00 and 16:00.
- Sev 1 and 2 go straight to the fix team.
- Sev 3 needs a workaround before go-live.
- Sev 4 moves to the post-go-live backlog.

Exit rule: no Sev 1 or 2 open, and every Sev 3 has an approved workaround.

16

open defects

1.8

days mean time to fix

3

awaiting retest

The test lead triages with the process owner. The Controller signs the exit.

UAT is 73% executed; reporting and treasury lag

180

scripts in scope

73%

executed to date

66%

passed first time

16

open defects

AREA	SCRIPTS	EXECUTED	PASSED	OPEN DEFECTS	STATUS
General ledger	46	95%	90%	2	On track
Payables	38	85%	80%	3	At risk
Receivables	42	70%	60%	5	At risk
Treasury	24	55%	50%	2	At risk
Reporting	30	40%	35%	4	Off track

Eight UAT scripts cover the finance month-end

SCRIPT	SCENARIO	STEPS SUMMARY	EXPECTED RESULT	STATUS
UAT-01	Post a manual journal	Create, route, approve and post	Posts to the correct period	Pass
UAT-02	Pay a vendor invoice	Enter, match, approve, run payment	Payment file made; ledger updated	Pass
UAT-03	Apply customer cash	Import remittance; clear exceptions	Cash applied; aging updated	In progress
UAT-04	Close a period	Accruals, reconcile, lock period	Period locked; no open items	Pass
UAT-05	Intercompany transfer	Raise, confirm and eliminate	Both entities match	In progress
UAT-06	Month-end report pack	Run the pack for all entities	Totals tie to the ledger	Fail
UAT-07	Role-based access	Log in as each of five roles	Access matches the matrix	Pass
UAT-08	Add a fixed asset	Capitalize, depreciate, dispose	Schedule and ledger agree	Fail

Five of eight reconciliations already tie

RECONCILIATION CHECK	OWNER	METHOD	THRESHOLD	RESULT
GL trial balance to legacy	Controller	Compare totals by entity and account	Zero variance	Pass
Open payables invoices	AP lead	Count and sum by vendor	Under \$1 per vendor	Pass
Open receivables invoices	AR lead	Tie aging buckets to legacy	Under \$1 per customer	In progress
Bank balances	Treasury	Statement balance to ledger	Zero variance	Pass
Vendor master records	Data lead	Record count plus 50-record sample	100% match	Pass
Customer master records	Data lead	Record count plus 50-record sample	100% match	Pass
Intercompany balances	Shared services	Pair offsets by entity	Nets to zero	In progress
Journal history	Data lead	Row counts and totals by period	100% match	Fail

The war room runs on named roles and one bridge

SHIFTS

Shift A runs Friday 18:00 to Saturday 06:00. Shift B takes over on the bridge at 06:00 and runs to 18:00.

HANDOVER

Outgoing leads brief their successor on the bridge and update the log before they leave.

Bridge: +1 555 0100 · access code 000000

ROLE	NAME	PHONE	SHIFT STARTS
Cutover lead	Maya Okafor	+1 555 0101	A · Fri 18:00
Technical lead	Daniel Reyes	+1 555 0102	A · Fri 18:00
Controller	Priya Raman	+1 555 0103	A · Fri 18:00
Data lead	Tomas Lindqvist	+1 555 0104	A · Fri 18:00
Integration lead	Hannah Cole	+1 555 0105	B · Sat 06:00
Test lead	Samir Haddad	+1 555 0106	B · Sat 06:00
Support lead	Lena Fischer	+1 555 0107	B · Sat 06:00
Comms lead	Owen Parker	+1 555 0108	B · Sat 06:00
Exec sponsor	Grace Whitfield	+1 555 0109	On call

The bridge checks in every 30 minutes

WHEN	FORUM	LEAD	PURPOSE
Every 30 min	Workstream roll call	Cutover lead	Status, blockers and next step
Every 2 hours	Step checkpoint	Technical lead	Verify and sign the step on the log
At each gate	Go or no-go call	Cutover lead	Decide with the Controller
06:00 / 18:00	Shift handover	Shift leads	Hand over open issues
Every 4 hours	Steering update	Exec sponsor	Send the one-page status
On demand	Swarm room	Issue owner	Fix one blocker; report back

Every check-in ends with one line on the log: who owns the next action and by when.

CHECK-IN CADENCE

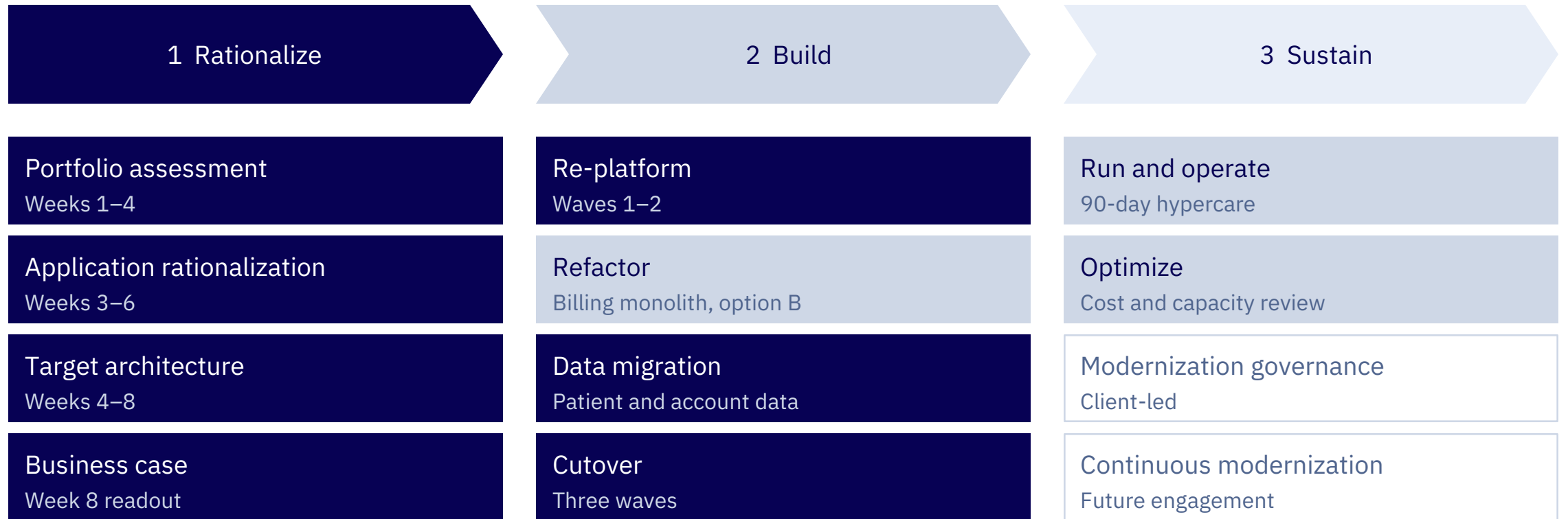
30 min

between workstream roll calls, all night

GROUND RULES

One voice at a time. Issues go on the log before they go to a side call. Decisions are read back.

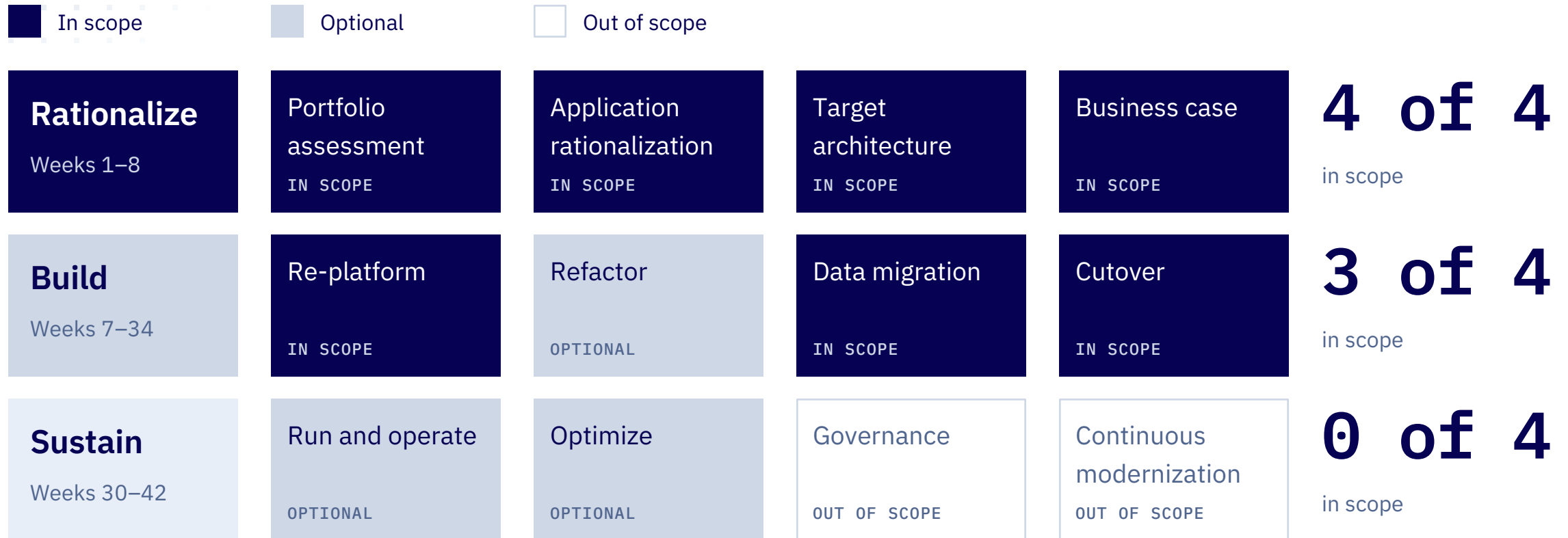
This proposal covers **seven of twelve** subtracks



In scope
 Optional, priced separately
 Out of scope

7 in scope · 3 optional · 2 out of scope

Northfield's engagement covers decide and deliver



Refactor, hypercare and the cost review are priced as options so the steering group can decide at the architecture gate.

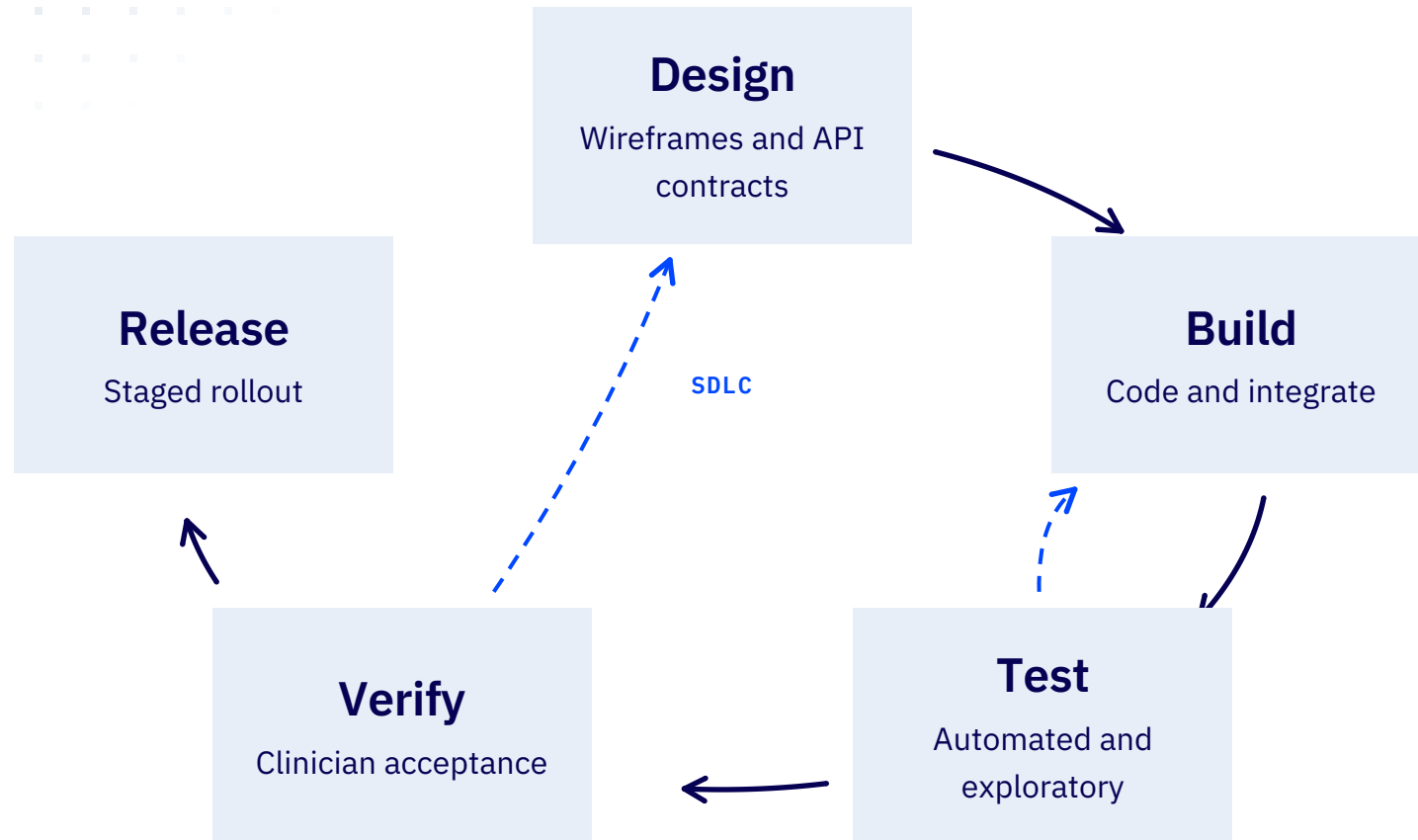
Optional scope can be added as demand grows

SERVICE	ADDITIONAL SCOPE	UNLOCKED VALUE	PRICING BASIS
Early-adopter expansion	- Assess and select three more adopter teams - Workload-specific design and implementation - Provisioning and operational validation - Workload hypercare and enhancements	- Twice the workloads onboarded - Higher value per workload - Faster user adoption	Fixed fee per added team
Change management ownership	- Lead end-to-end change for platform rollout - Readiness assessment and champion program - Reusable onboarding and adoption playbook - Program communications and socialization	- Stickier adoption - Less ongoing user support - Faster onboarding of future teams	Fixed fee by phase
Cloud engineering surge	- Continued cloud architecture and engineering - Post-deployment environment optimization - Ad hoc performance and scaling troubleshooting	- More capacity for workload growth - Faster cost optimization after launch	Team run-rate, monthly

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Five steps, and two ways work loops back



TEST TO BUILD

Failed tests return work

Defects go back to the sprint, not to a later phase.

VERIFY TO DESIGN

Gaps reopen the design

Workflow gaps change the design before release.

RELEASE

Hand-off to Optimize

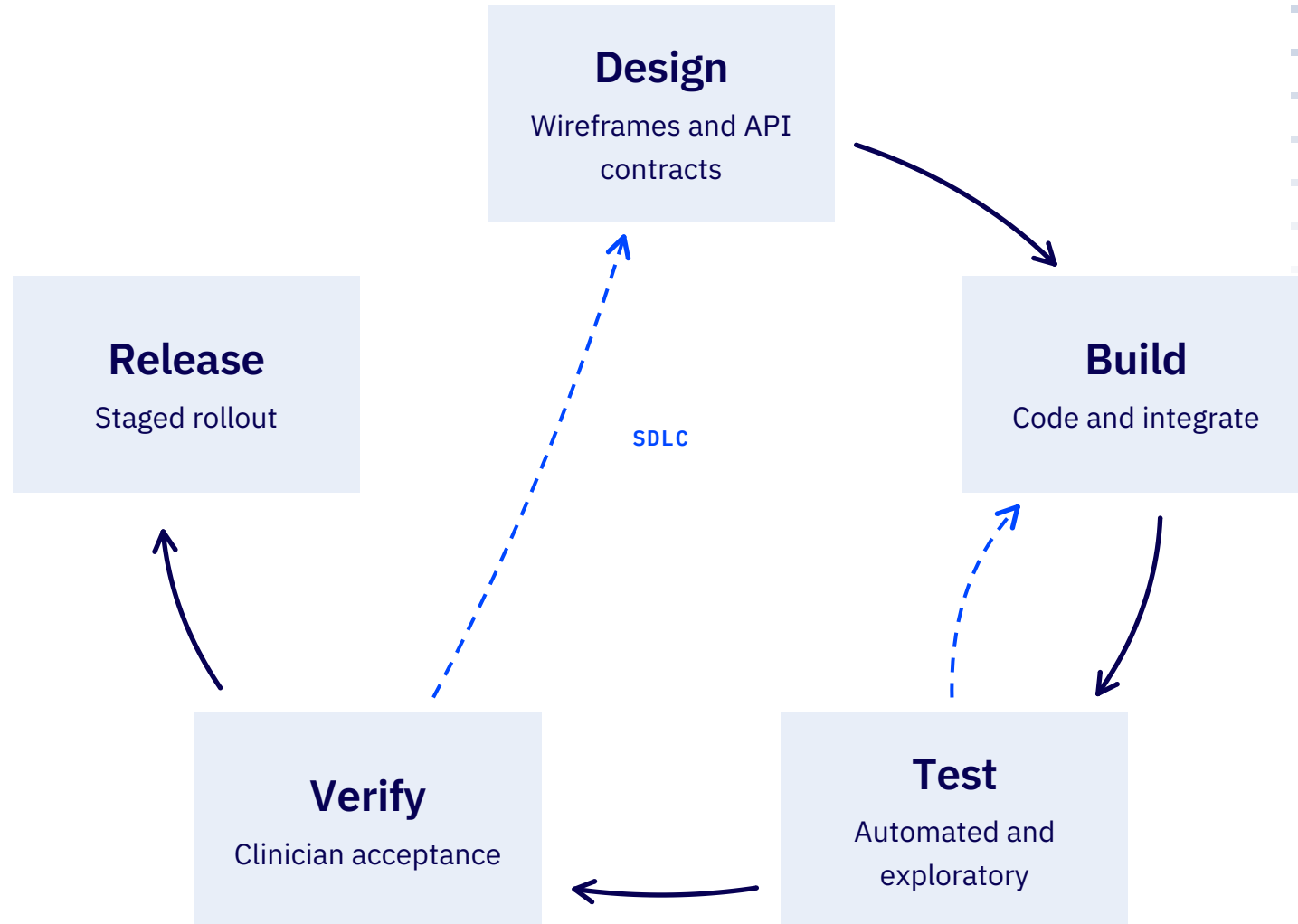
Live usage data starts the next PDLC loop.

Defects and gaps loop back inside the SDLC

- **Test to Build.** Defects return to the sprint.
- **Verify to Design.** Clinician gaps reopen the design.

HAND-OFF

Release feeds Optimize, where live usage data starts the next PDLC loop.



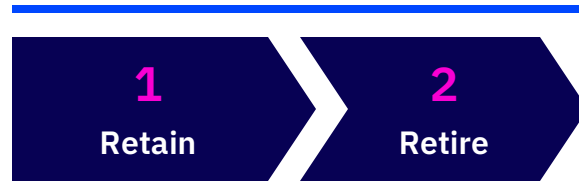
Three evidence streams become one validated model



¹ Observation requires consent and appropriate privacy, security and retention controls.

Seven options, one common decision language

KEEP OR RETIRE



Keep and manage deliberately

Remove obsolete software or work

MOVE AND IMPROVE



Move infrastructure; limit change

Change the runtime; keep behavior

Improve structure; keep function

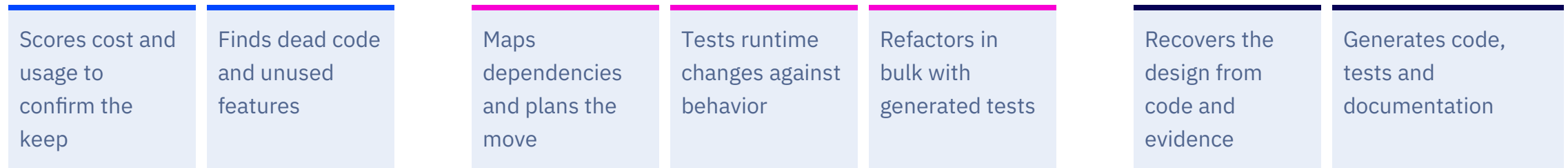
REBUILD THE DESIGN



Change structure and boundaries

Redesign and rewrite the capability

Where AI accelerates each option

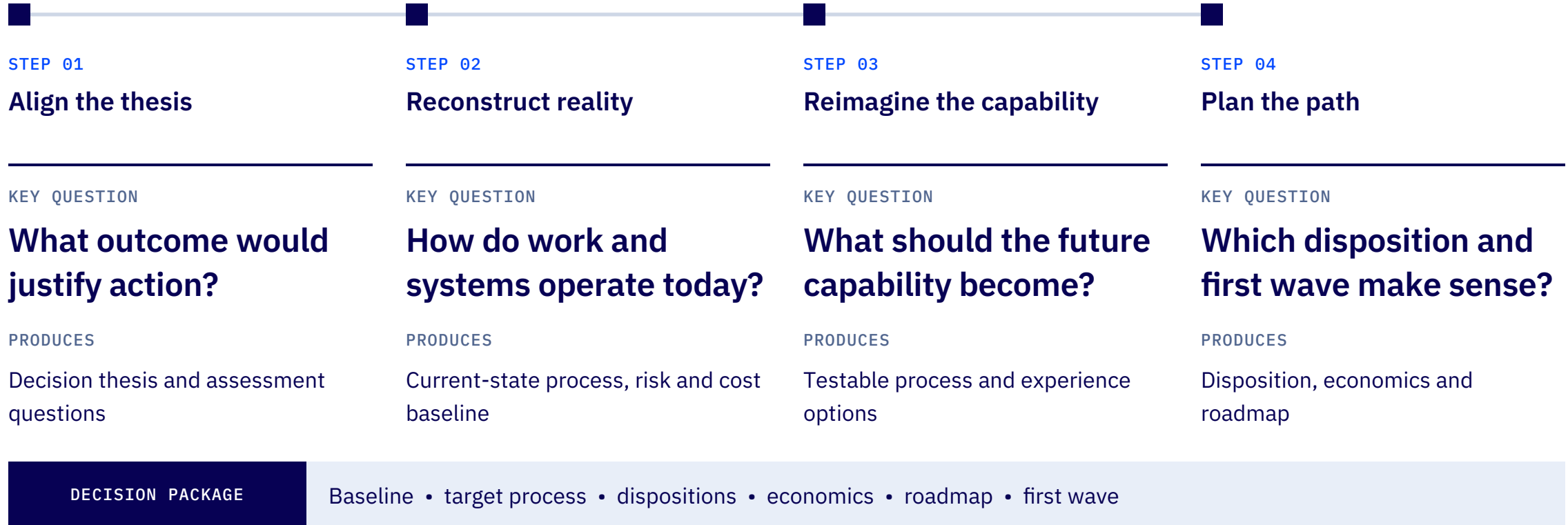


The framework still guides the decision; AI broadens the feasible set, and leaders remain accountable for the choice.

Four steps make the portfolio actionable

RATIONALIZE

Build the shared context that makes modernization actionable



Expect a facilitated build, not a passive demo

HOW THE SESSION RUNS

-  **Frame together** 15 MIN
Facilitators set the objective and decision criteria; teams bring their own context and constraints.
-  **Work in role** 30 MIN
Product and engineering build in focused tracks; facilitators keep time.
-  **Reconnect at gates** 30 MIN
Teams share artifacts, challenge ambiguity and agree what is ready before the next block.
-  **Iterate and close** 15 MIN
The group reviews evidence, records limits and turns learning into owned next actions.

HOW FACILITATION WORKS

Guide the process without taking over the work

- Short prompts, then participant work
- Named checkpoints and timeboxes
- Active coaching and unblocking
- Human approval at every gate
- Shared notes captured live
- Open risks logged for follow-up

What we need from participants: real context, active decisions, work in progress shared and candor about constraints.

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Review evidence and turn learning into owned actions.

Bring real context and share work in progress.

WHAT PARTICIPANTS WILL LEAVE WITH

Six outcomes define the session

Frame the right opportunity
Pick a feasible problem

Turn evidence into intent
Stories and criteria

Agents across the SDLC
Plan, code, test, release

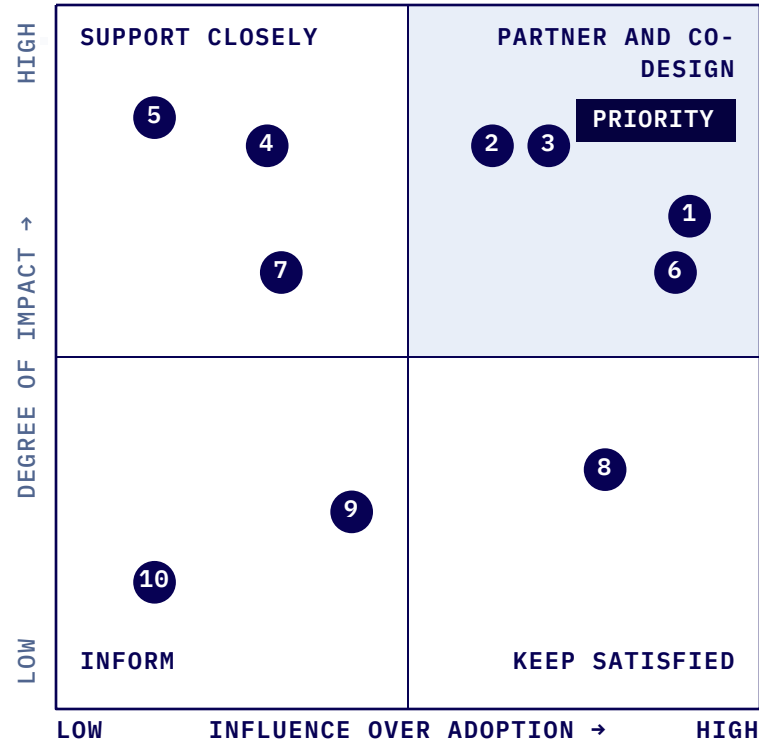
Product-to-R&D interface
Agree a demonstrable slice

Compress feedback loops
Prototype in parallel

Scale AI-enabled delivery
Adoption and governance

Seven stakeholders need active engagement before go-live

Influence over adoption and degree of impact



01

Partner

Chief medical officer, physician group and practice managers co-design the rollout.

02

Support

Schedulers and agents get coaching and a named champion.

03

Inform

Union and payer contacts get briefings ahead of each wave.

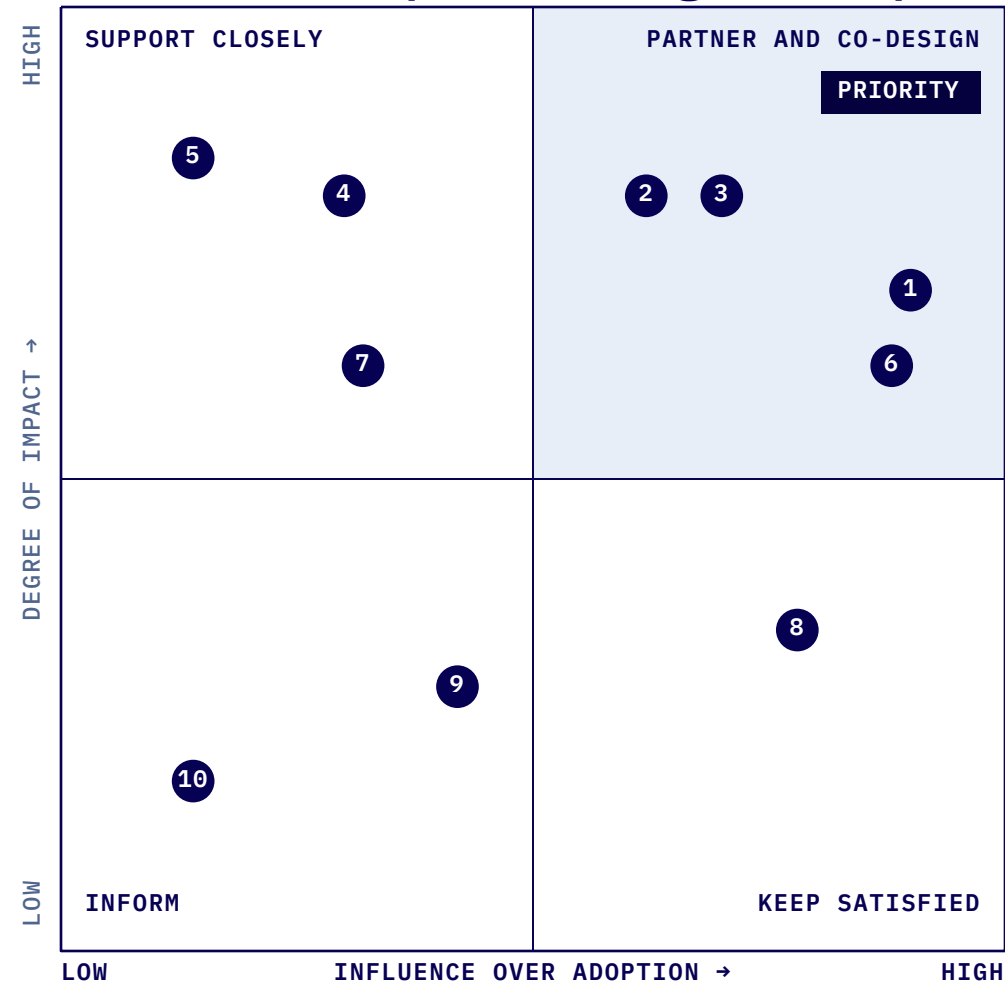
Source: Northfield Health System stakeholder interviews, Sep 2026.

Engage the top-right quadrant first

- 1 Chief medical officer
- 2 Clinic nurse leads
- 3 Practice managers
- 4 Call-center agents
- 5 Front-desk schedulers
- 6 Physician group
- 7 IT service desk
- 8 Union representatives
- 9 Patient advisory council
- 10 Payer relations

Partner with 1, 2, 3 and 6; support 4, 5 and 7; inform 8 to 10 monthly.

Influence over adoption and degree of impact



Source: Northfield stakeholder interviews, Sep 2026.

Every key stakeholder has an owner and a plan

STAKEHOLDER	ROLE IN ROLLOUT	IMPACT	INFLUENCE	STANCE	OWNER	ENGAGEMENT APPROACH
Chief medical officer	Sponsor	High	High	CHAMPION	S. Alvarez	Monthly steering briefing
Physician group	Primary users	Medium	High	NEUTRAL	Medical director	Peer demo, clinic huddles
Practice managers	Local leads	High	Medium	SUPPORTIVE	Change lead	Weekly working session
Call-center agents	Heavy users	High	Low	CONCERNED	Contact center VP	Floor-walkers, daily tips
Front-desk schedulers	Heavy users	High	Low	CONCERNED	Practice managers	Sandbox, buddy pairs
Union representatives	Labor relations	Medium	Medium	SKEPTICAL	HR director	Joint briefing before wave 1
IT service desk	Support	Medium	Low	SUPPORTIVE	IT operations	Runbook walk-through
Patient advisory council	Patients	Low	Low	NEUTRAL	Patient experience	Booking page preview

IMPACT  Low  Medium  High

INFLUENCE  Low  Medium  High

Source: Northfield Health System change team, Sep 2026.

The program pays for itself within 14 months

\$4.2M

annual run-rate savings

5 days

to close

12 days today

14 mo

to payback

14 business units

WHAT IT MEANS

Faster closes free finance capacity in the first two waves, and that capacity pays for the later work. Cost is recovered in 14 months across all business units.

Source: West Monroe analysis of the FY26 baseline; net of one-time cost.

Northfield closes in 5 days and saves \$4.2M a year

5 days

to close, down from 12

\$4.2M

annual run-rate savings

14

business units on one process

3 waves

to full rollout

WHERE THE SAVINGS COME FROM

Automated matching clears the intercompany balances that today take three people four days each month. Moving review to the point of posting means exceptions are fixed on day one instead of being found on day nine.

Together they remove about 11,000 hours of manual effort a year across the 14 business units, and let the controller sign off on day five without adding headcount.

Source: West Monroe analysis of the FY26 baseline; run-rate after wave 3.

BUSINESS CASE

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WHERE THE SAVINGS COME FROM

Automated matching clears the intercompany balances that take three people four days each month, and review at posting fixes exceptions on day one instead of day nine.

One schedule slip puts the program at risk

OVERALL STATUS

At risk

STATUS BY AREA

AREA	STATUS	THIS PERIOD
Scope	On track	Two change requests approved
Schedule	At risk	Wave 2 cutover slips six days
Budget	On track	\$0.2M under plan to date
Risks	At risk	Two high risks, both owned

COMPLETED THIS PERIOD

- Wave 1 entities live
- Journal rules signed off
- Close calendar published
- Training cohort one done

PLANNED NEXT PERIOD

- Wave 2 data migration
- User acceptance testing
- Cutover go or no-go
- Cockpit design review

RISK REGISTER

RISK	LIKELIHOOD	OWNER	MITIGATION	STATUS
Late data extracts	High	Finance IT	Daily extract check	At risk
Reviewer capacity	Medium	Controller	Add two reviewers	On track
Cutover freeze clash	Medium	PMO lead	Reset freeze date	At risk

DECISIONS NEEDED

- Approve new cutover date CFO, by 9 Oct
- Fund two extra reviewers Controller, by 16 Oct
- Confirm freeze window CIO, by 16 Oct
- Release Wave 3 budget CFO, by 30 Oct

Source: Northfield program office status report, 30 Sep 2026.

Wave 1 is on track; journal review is the one risk

7 days

to close, target 5

AT RISK

\$2.1M

run-rate savings of \$4.2M

ON TRACK

WAVE	ENTITIES	STATUS
Wave 1	5	On track
Wave 2	9	At risk
Wave 3	14	Off track

92%

of entries checked at posting

ON TRACK

Source: Program office status report, Sep 2026.

PROGRAM STATUS

Wave 1 is on track; journal review is the one risk

7 days

to close, target 5

AT RISK

\$2.1M

run-rate savings of \$4.2M

ON TRACK

WAVE	ENTITIES	STATUS
Wave 1	5	On track
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Wave 3	14	Off track

92%

of entries checked at posting

ON TRACK

Source: Program office status report, Sep 2026.

TAKEAWAY

On track, with one decision to make.

Add two journal reviewers by 16 Oct or Wave 2 cutover slips by six days.

Wave 1 is on track; journal review is the one risk

7 days

to close, target 5

AT RISK

\$2.1M

run-rate savings of \$4.2M

ON TRACK

WAVE	ENTITIES	STATUS
Wave 1	5	On track
Wave 2	9	At risk
Wave 3	14	Off track

92%

of entries checked at posting

ON TRACK

Decision needed: add two journal reviewers by 16 Oct, or Wave 2 cutover slips six days.

Source: Program office status report, Sep 2026.

Three of five workstreams are on track

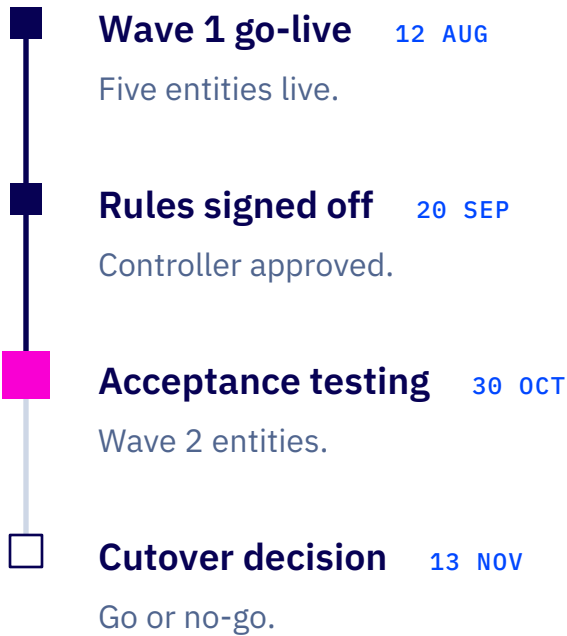
STATUS BY MONTH				
WORKSTREAM	JUL	AUG	SEP	OCT
Process design	On track	On track	On track	On track
Data migration	On track	On track	At risk	At risk
Controls	On track	On track	On track	On track
Change and training	On track	On track	On track	At risk
Close cockpit	On track	On track	On track	On track

NEXT STEPS

- Reset the Wave 2 data freeze with Finance IT
- Run the second training cohort in October
- Confirm the cutover go or no-go on 13 Nov

Source: Northfield program office status report, 30 Sep 2026.

MILESTONES



One issue needs escalation this week

TYPE	ITEM	OWNER	DUE	NEXT ACTION	STATUS
RISK	Late Wave 2 data extracts	Finance IT	9 Oct	Daily extract check	At risk
RISK	Reviewer capacity at quarter end	Controller	16 Oct	Add two reviewers	On track
ASSUMPTION	Wave 1 rules carry over	Program lead	2 Oct	Confirm in testing	On track
ASSUMPTION	Source ERP stays frozen	Finance IT	30 Oct	Hold change board	On track
ISSUE	Cutover overlaps year-end freeze	PMO lead	9 Oct	Escalate to CIO	Off track
ISSUE	Two entities miss mapping sign-off	Controller	6 Oct	Workshop on 4 Oct	At risk
DEPENDENCY	Payroll interface from HR systems	HR systems	23 Oct	Weekly sync	At risk
DEPENDENCY	Bank file format approval	Treasury	16 Oct	Vendor test file in	On track
DEPENDENCY	Security review of cockpit	CISO office	30 Oct	Book review slot	On track

Source: Northfield program office RAID log, 30 Sep 2026.

Program is at risk until Wave 2 cutover is reset

OVERALL STATUS

Wave 2

of 3, cutover date at risk

AT RISK

KEY MESSAGES

- Wave 1 is live in five entities
- Budget is \$0.2M under plan
- Wave 2 data extracts are late

Decisions requested

- Approve the new cutover date
- Fund two extra reviewers
- Confirm the year-end freeze window

TOP THREE RISKS

RISK	OWNER	DUE	STATUS
Late data extracts	Finance IT	9 Oct	At risk
Reviewer capacity	Controller	16 Oct	On track
Cutover freeze clash	PMO lead	16 Oct	At risk

NEXT MILESTONES

9 Oct Wave 2 data frozen

30 Oct Acceptance testing

13 Nov Cutover decision

ADR-014 replaces the 2024 choice of point-to-point feeds

SUPERSEDED • ADR-009 • MAR 2024

Point-to-point HL7 feeds

- Each system connects directly to the others
- Chosen when Northfield ran 6 systems
- Retired in 3 waves from Q1 2027



CURRENT • ADR-014 • SEP 2026

Shared event bus

- Systems publish and subscribe to events
- Fits 14 systems and growing
- Owned by the platform team

WHY IT CHANGED

The estate outgrew it

Systems grew from 6 to 14 and feeds from 9 to 61.

Incidents followed feeds

Most clinical interface incidents traced to one broken feed.

Cost per feed rose

Each new feed took a quarter and a test cycle across teams.

Time savings and workflow fit score lowest

STATEMENT	PHYSICIANS	NURSES	FRONT DESK	REV CYCLE	LEADERS	ALL
I understand why we are changing	4.1	3.9	3.6	3.4	4.6	3.9
I have the training I need	3.2	2.8	3.0	2.9	3.9	3.1
The new system saves me time	2.4	2.6	2.9	3.3	3.8	3.0
My leaders support the change	3.5	3.7	3.4	3.5	4.7	3.7
I know who to ask for help	3.3	3.8	3.1	3.4	4.2	3.6
Workflows fit how I work	2.1	2.5	3.2	3.0	3.5	2.8
Data in the system is trustworthy	3.1	3.2	3.0	2.7	4.0	3.2
I would recommend it to a peer	2.9	3.1	3.3	3.2	4.4	3.4

AVERAGE SCORE, 1 TO 5 4.5–5.0 3.5–4.4 2.5–3.4 1.5–2.4 Under 1.5

Source: Northfield go-live pulse survey, n = 412, Sep 2026.

Clinicians rate the new system lowest

- **Clinicians struggle most.** Physicians and nurses rate time savings, workflow fit and alerts lowest.
- **Leaders are upbeat.** Their scores run well above those of the people doing the work.
- **Act on the gap.** Fix alert noise and order-set layout first; both are configuration, not build.

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I would recommend it to a peer	2.9	3.1	3.3	4.4
Alerts help more than they interrupt	2.2	2.4	3.4	3.6
Reports meet my needs	3.0	3.3	2.8	4.1

4.5–5.0
3.5–4.4
2.5–3.4
1.5–2.4
Under 1.5

Source: Northfield go-live pulse survey, n = 412, Sep 2026.

BUSINESS CASE

The program funds itself by month 14

\$4.2M

run-rate savings from the first two waves

18%

lower cost to serve across 14 units

WHERE THE SAVINGS COME FROM

Automated matching clears intercompany balances every day instead of at period end, which removes most of the manual reconciliation effort. Earlier review moves controller checks to the point of posting, so errors are fixed while they are cheap rather than found on day nine. Together the two changes free controllers to analyze results instead of chasing status.

Source: West Monroe analysis of the FY26 baseline; net of one-time cost.

WHAT THIS MEANS

Start wave 1 in Q1 to reach payback in 2027

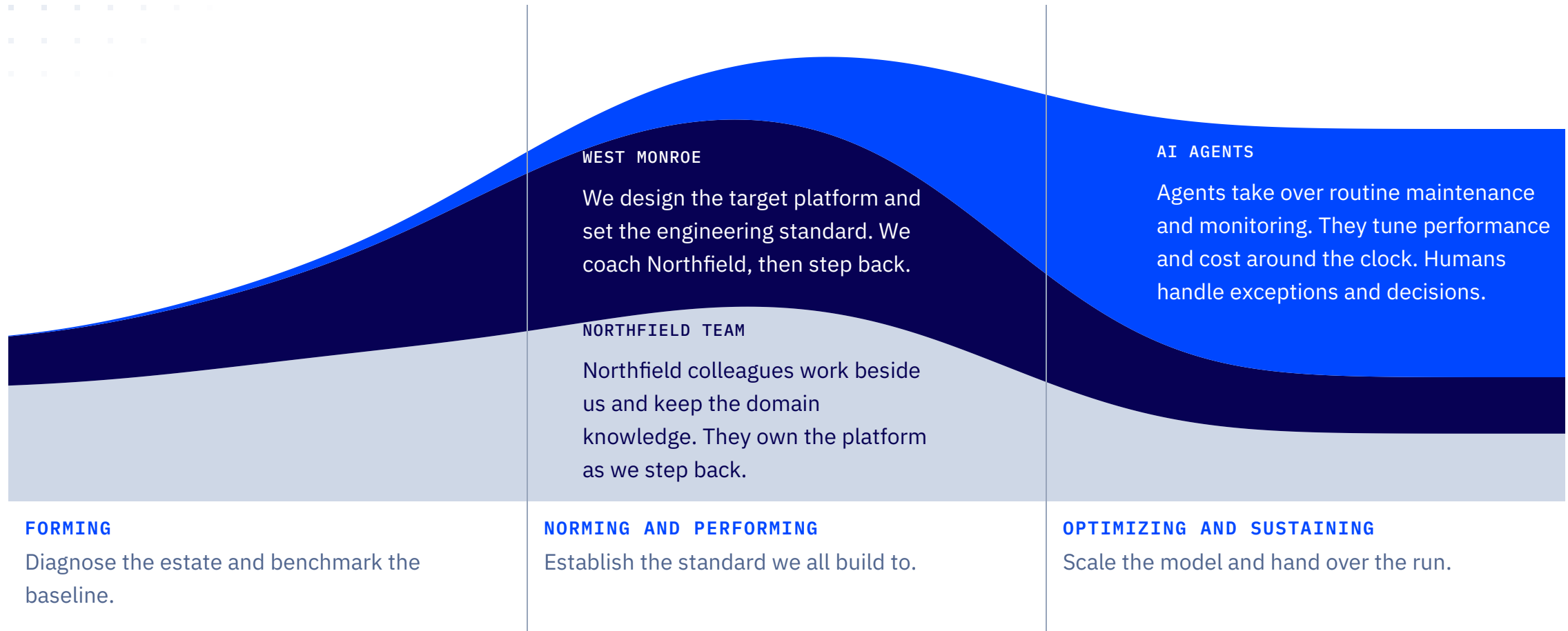
Each quarter of delay pushes payback out by about four months.

Agents cut our team and Northfield's maintenance

Our team and Northfield's work side by side. We diagnose the estate and benchmark the baseline. Agents are not yet in the loop.



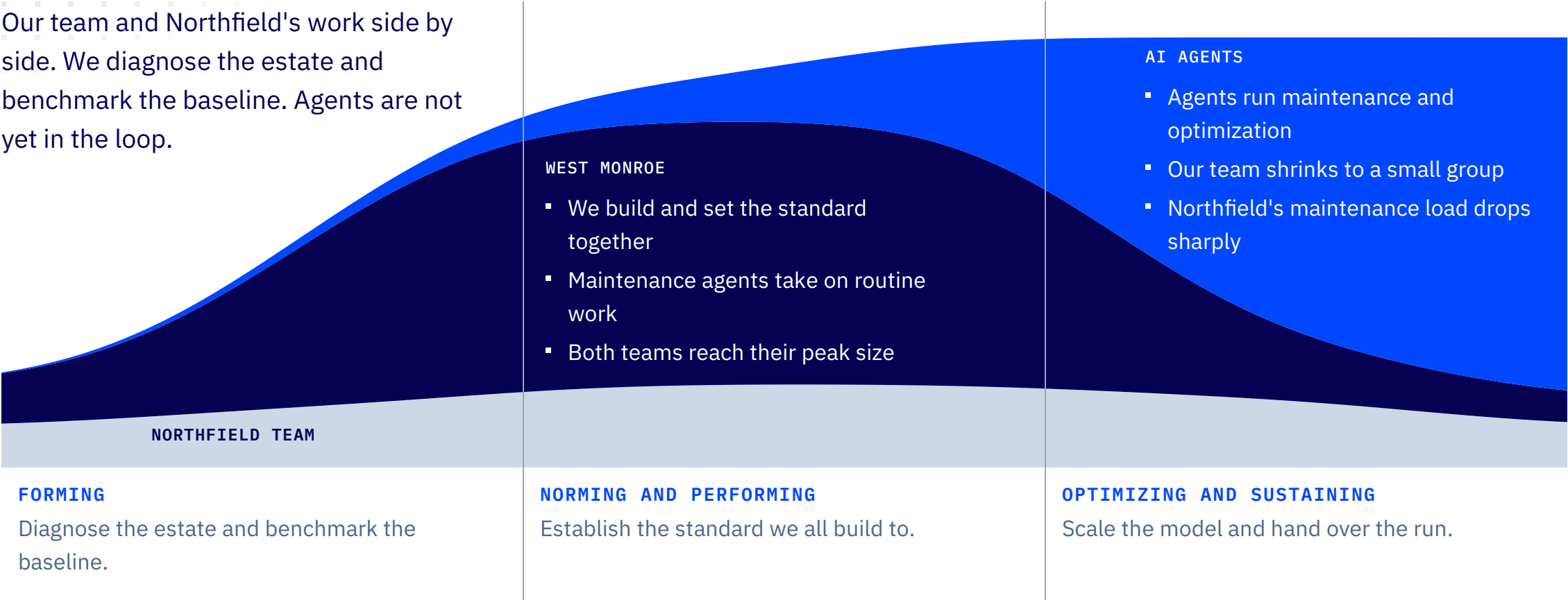
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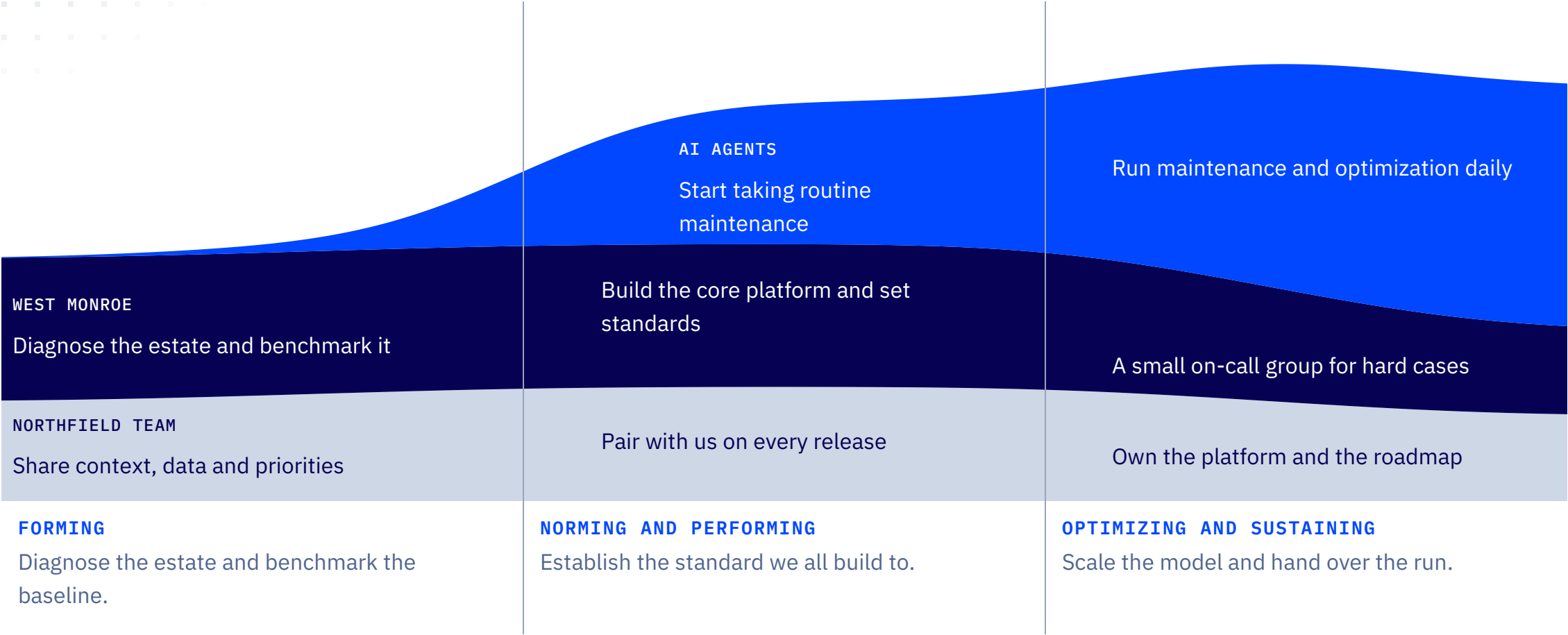
OUR APPROACH

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Agents cut our team and Northfield's maintenance



OUR APPROACH

Agents cut our team and Northfield's maintenance

NORTHFIELD TEAM

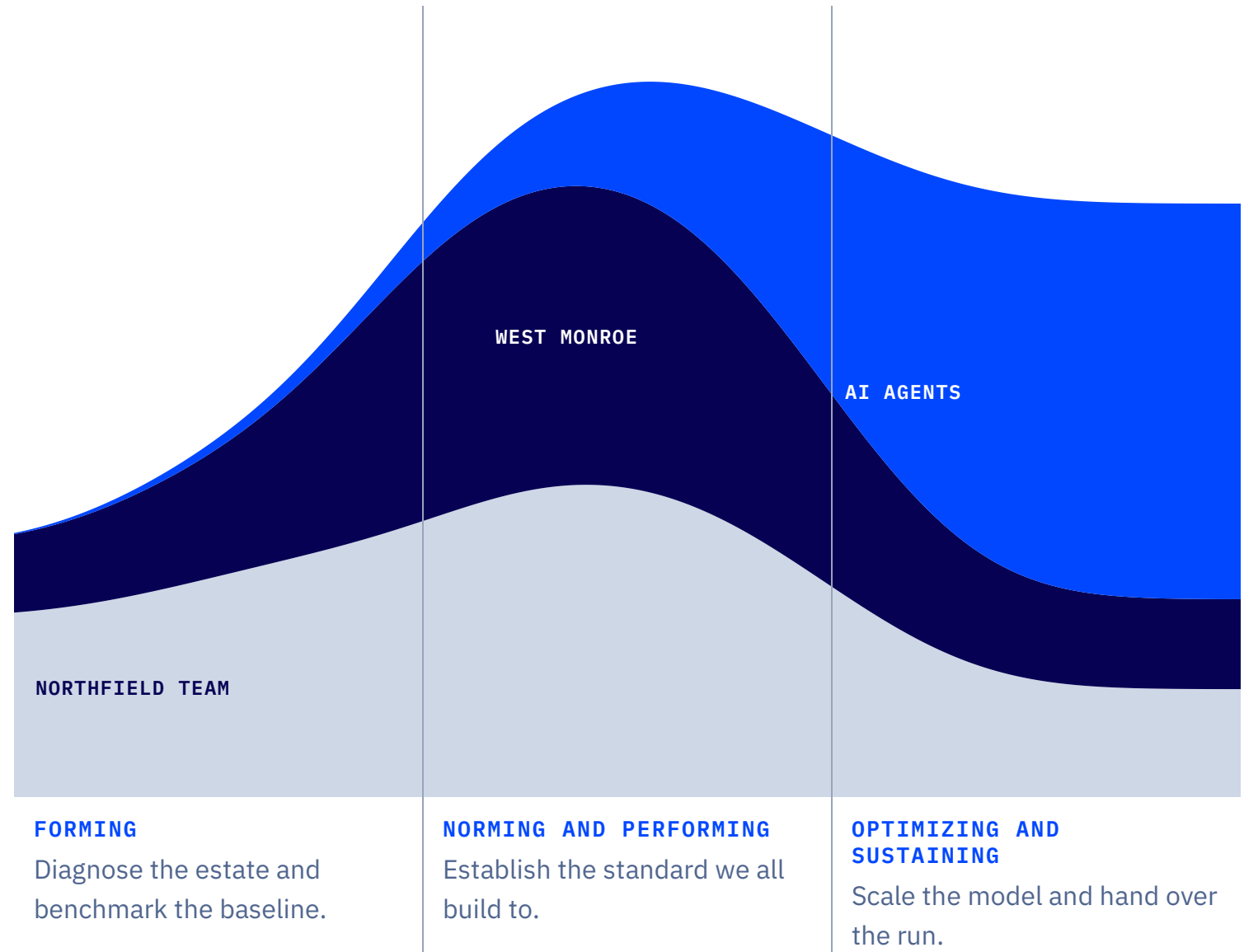
Domain experts who own the platform as we step back.

WEST MONROE

We set the engineering standard, then step back.

AI AGENTS

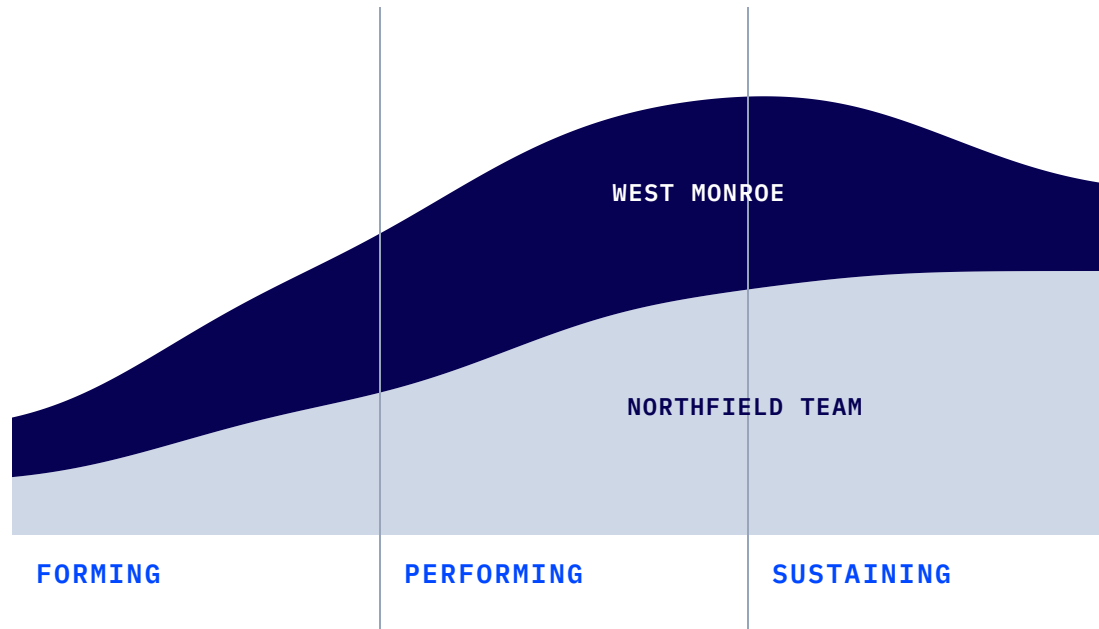
Agents run routine maintenance; humans decide exceptions.



Agents let a **smaller team** run the same platform

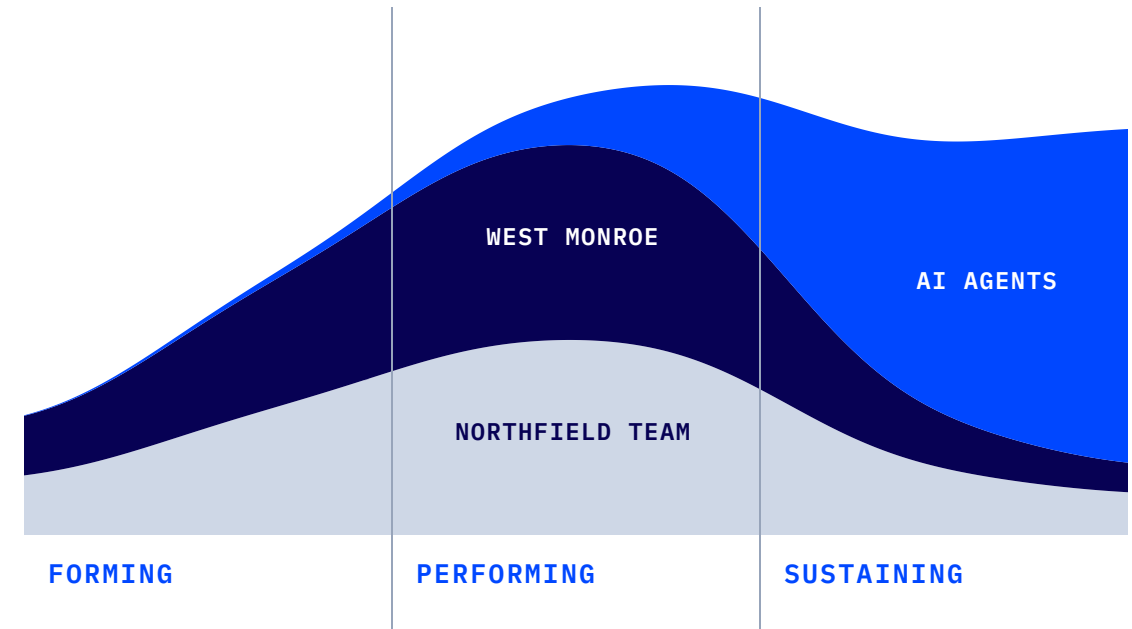
Traditional delivery

Humans only



Agent-enabled delivery

Humans plus agents



Same platform, same outcomes: agents take over maintenance, so both teams run lighter.

OUR APPROACH

We build together, then Northfield takes the lead

We lead the first sprints with a lean team. We stand up the architecture and delivery practices. Northfield colleagues join us from day one.

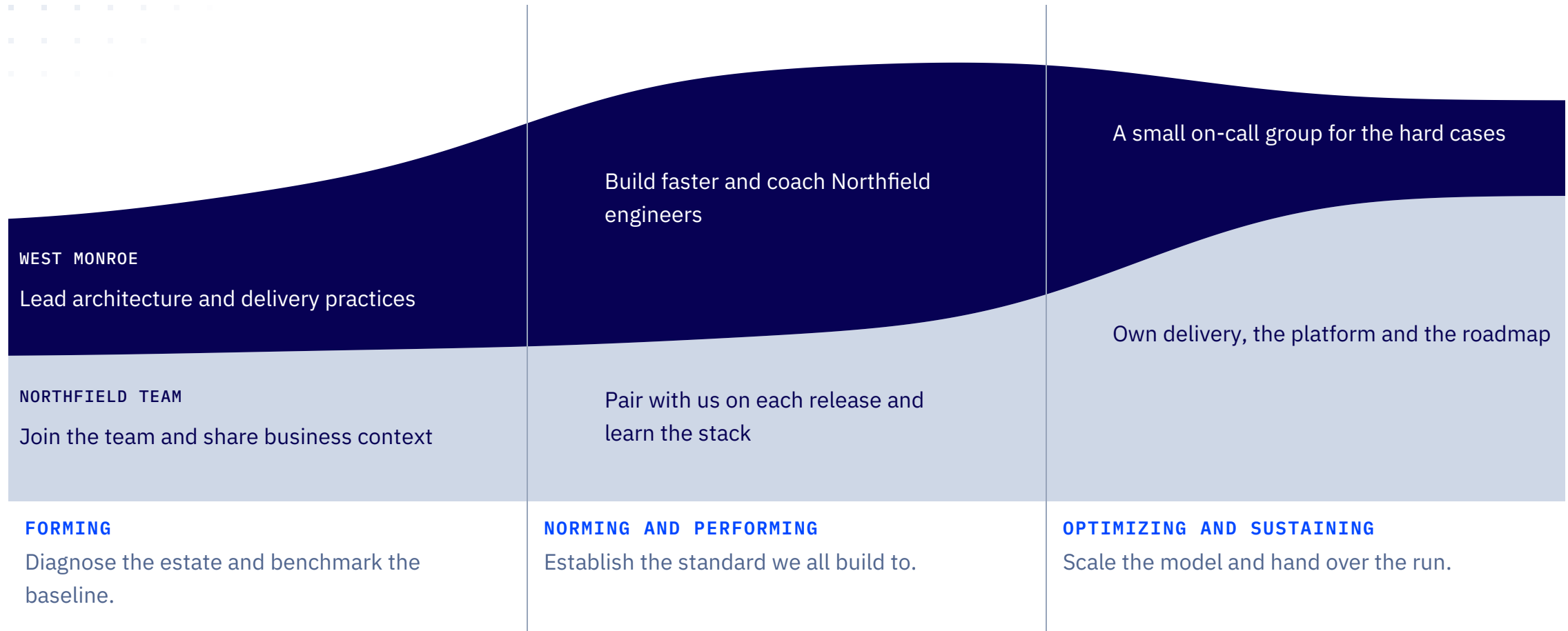


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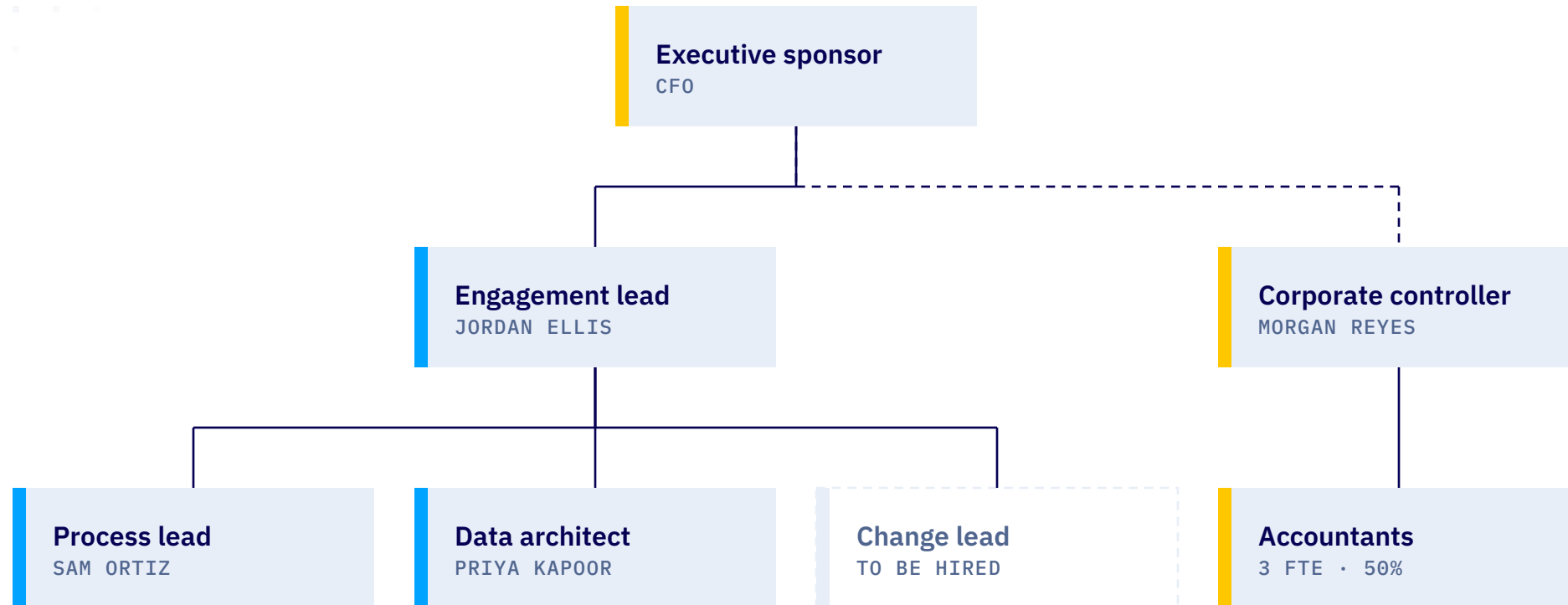


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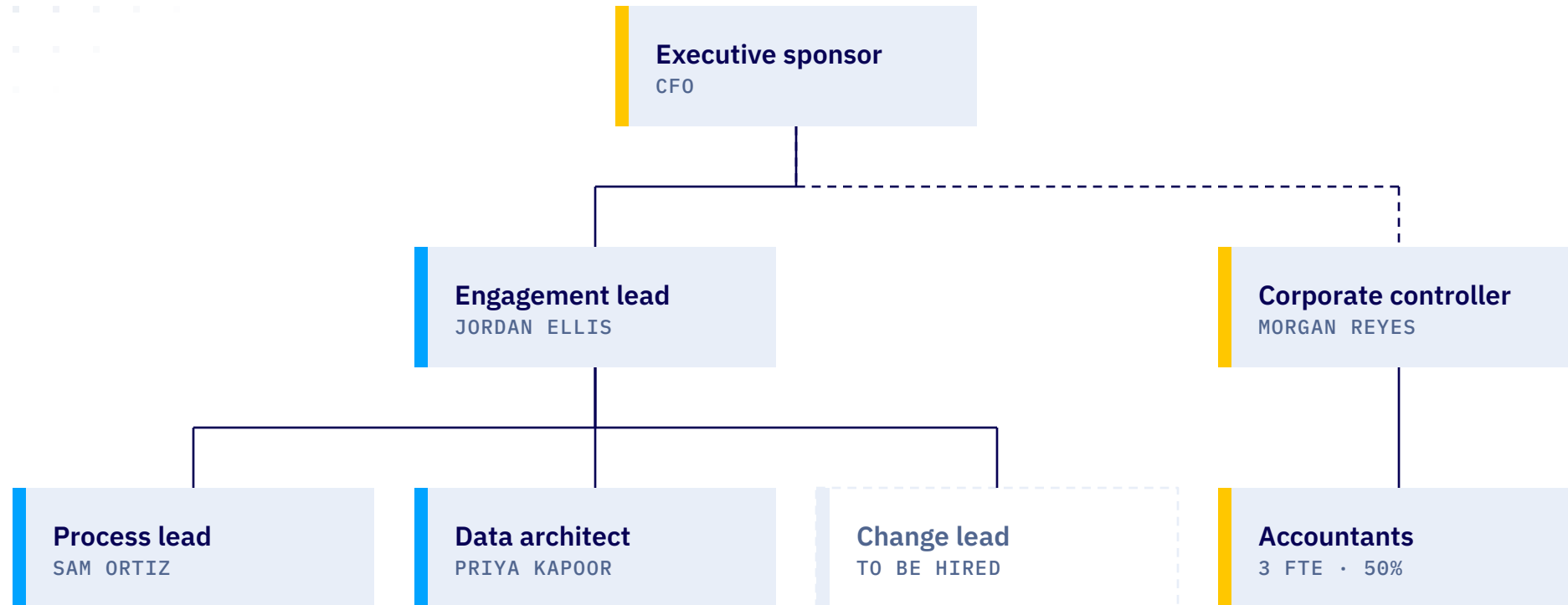


One joint team, led by one engagement lead



■ West Monroe ■ Northfield ■ To be hired — Dotted-line relationship

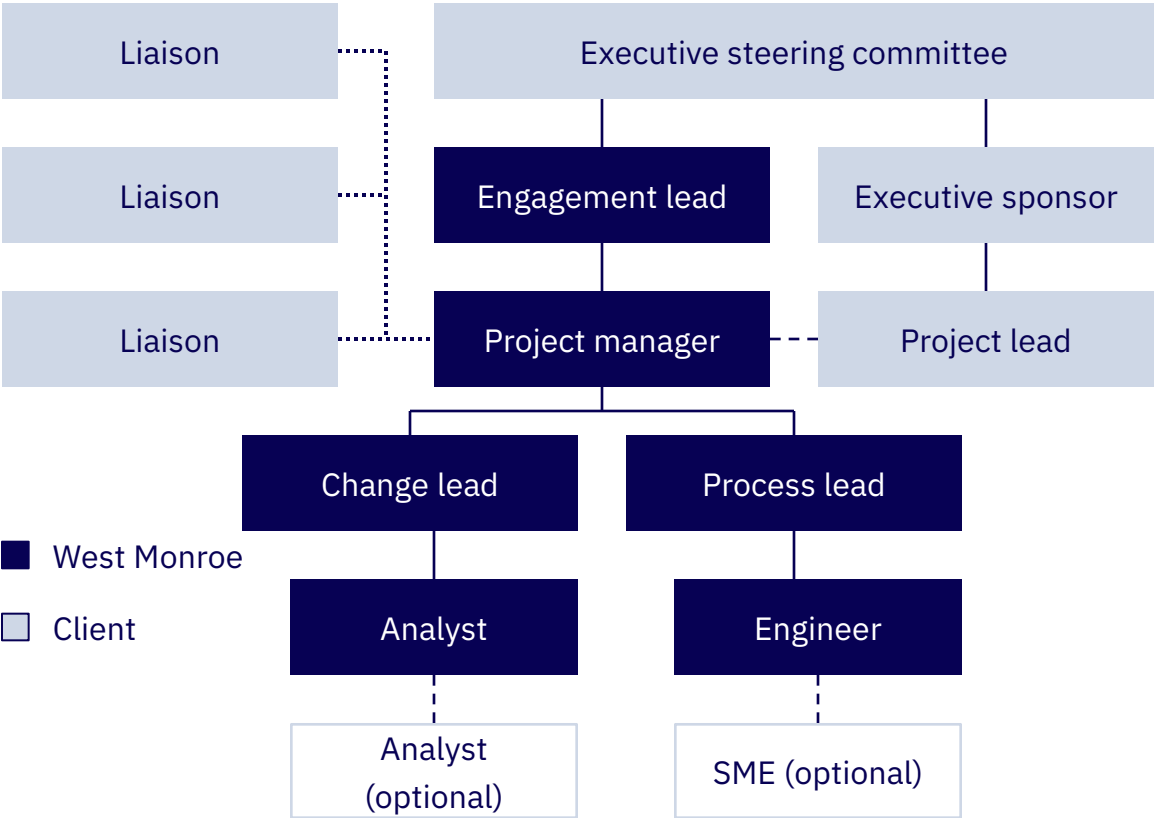
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One joint team, with clear roles on both sides

TEAM STRUCTURE



■ West Monroe

□ Client

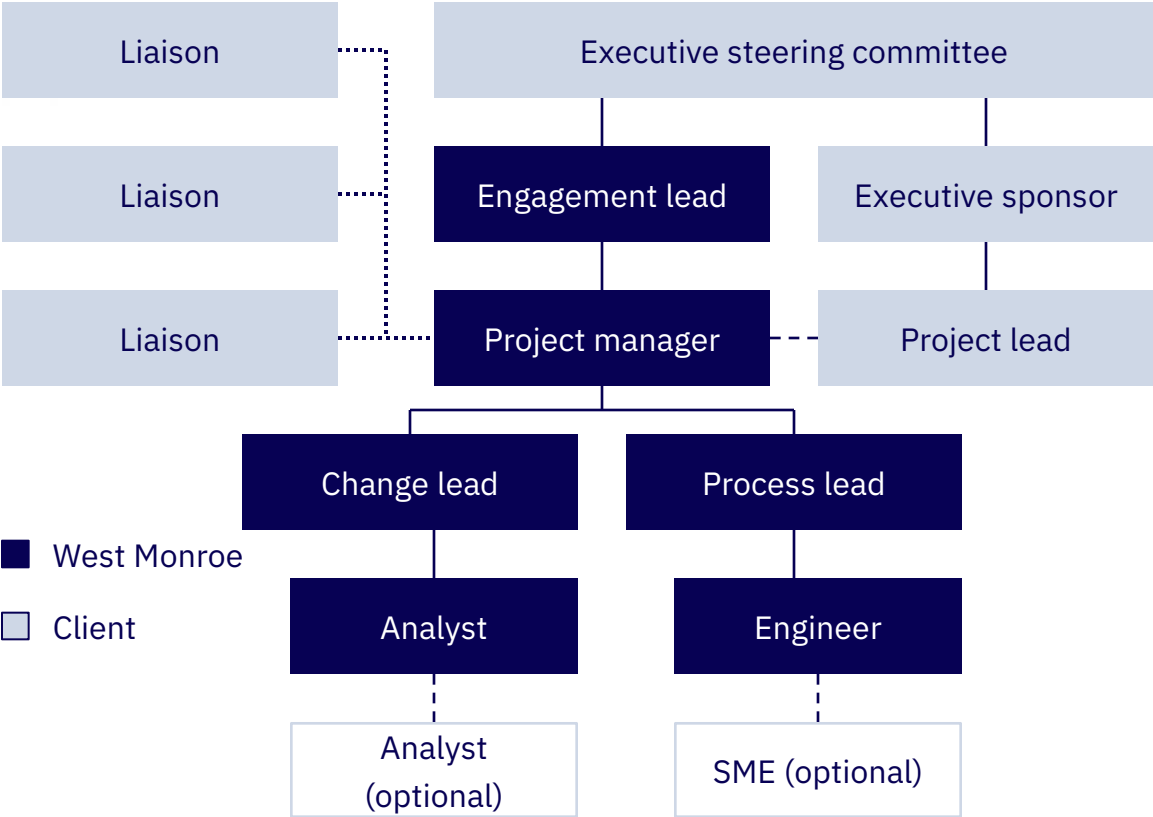
ROLES AND RESPONSIBILITIES

ROLE	RESPONSIBILITIES
Steering committee	- Provide oversight and direction - Make strategic decisions
Executive sponsor	- Remove barriers, approve major calls - Own quality and escalation
Liaisons	- Surface needs of each department - Support opportunity identification
Project lead	- Run day-to-day delivery - Serve as the point of contact
Delivery team	- Run workshops, gather requirements - Deliver the business case and roadmap
Business teams	- Join workshops and meetings as needed - Assess impact and change needs

TEAM STRUCTURE

One joint team, with clear roles on both sides

TEAM STRUCTURE



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Three pods work beside four Northfield roles

Close pod	Data pod	Change pod
Engagement lead	Data architect	Change lead
Process lead	2 × Data engineers	Trainer
2 × Accountants	Quality analyst	Communications

NORTHFIELD HEALTH SYSTEM

Executive sponsor

CFO · 0.1 FTE

Controller

0.5 FTE

Shared services lead

0.5 FTE

Accountants

3 FTE · 50%

Three pods work beside four Northfield roles

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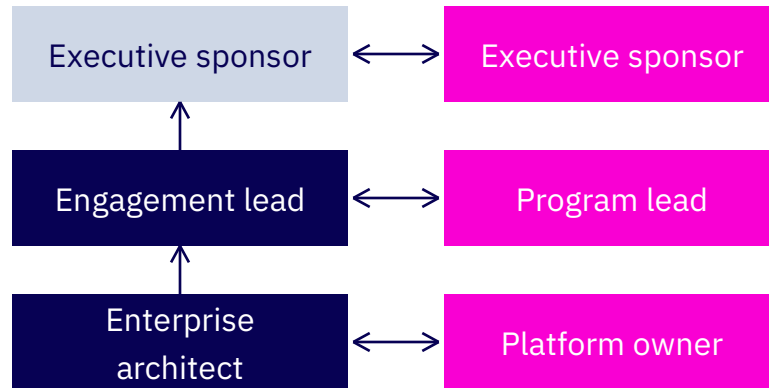
0.5 FTE

Accountants

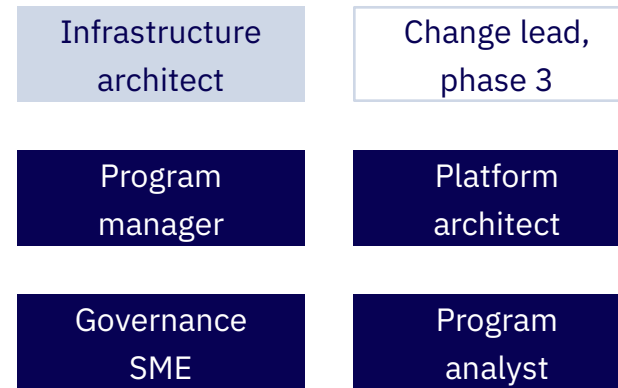
3 FTE · 50%

Delivery pods scale up and down by workstream

LEADERSHIP PAIRS



WORKSTREAM LEADS



Northfield leadership roles

- **Executive sponsor (5%).** Sets direction, removes blockers, approves major decisions.
- **Program lead (50–75%).** Planning, coordination, dependencies, approvals and RAID.
- **Platform owner (50–75%).** Priorities, backlog, acceptance and operating-model calls.

DELIVERY PODS



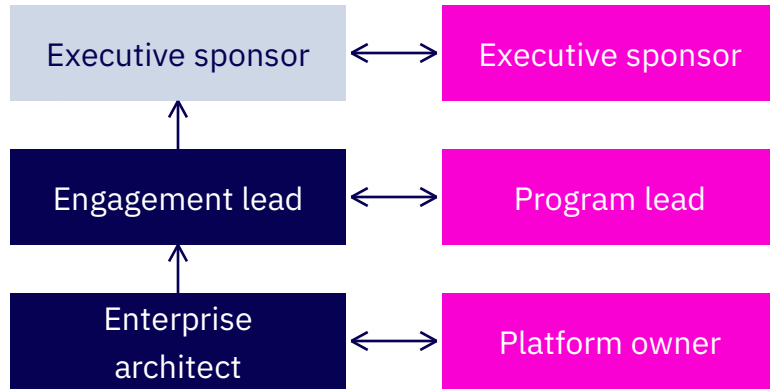
■ West Monroe full time

□ West Monroe part time

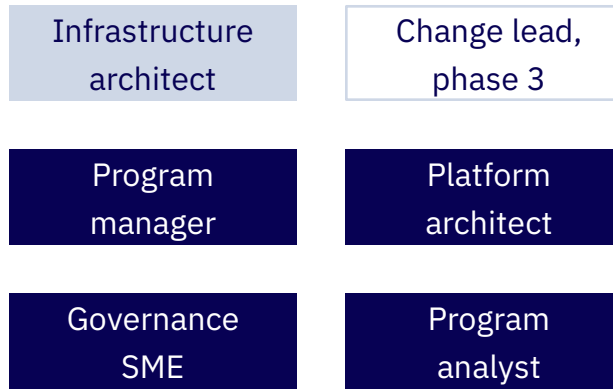
■ Northfield part time

Delivery pods scale up and down by workstream

LEADERSHIP PAIRS



WORKSTREAM LEADS



DELIVERY PODS



Northfield leadership roles

- **Executive sponsor (5%).** Sets direction, removes blockers, approves major decisions.
- **Program lead (50–75%).** Planning, coordination, dependencies, approvals and RAID.
- **Platform owner (50–75%).** Priorities, backlog, acceptance and operating-model calls.

- West Monroe full time
- West Monroe part time
- Northfield part time

A core team backed by matched specialists

Leadership and core team



Dana Whitfield
Executive sponsor

KM

Kiran Mehta
Executive sponsor

JA

Julia Ansel
Engagement lead

WHAT THE CORE TEAM DOES

- Own the sponsor relationship and quality
- Bring healthcare expertise to the work
- Align both teams and transfer knowledge

MO

Mike Odell
Platform architect

JS

Jag Soni
Enterprise architect

AR

Albert Reyes
Infrastructure lead

Subject matter experts and support specialists

SA

Scott Alder
SME, payer data

DC

Dan Calloway
SME, life sciences

BM

Brett Marlow
SME, provider

WHAT THE SPECIALISTS DO

- Advise on architecture and governance
- Assess processes and organization
- Lead training and knowledge transfer

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Anneke Groot
SME, governance

TZ

Tanner Zell
SME, design

GM

Greta Movsky
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Advise on architecture and governance, assess processes and lead training.

Four risks need an owner before wave one

RISK 1

Data quality

- Entity charts of accounts differ
- Mapping is done by hand
- Fix in wave one

RISK 2

Change load

- Close runs through every month
- Teams are already stretched
- Stage the rollout in waves

RISK 3

Tool fit

- Matching must handle 11 entities
- Vendor demos use clean data
- Pilot on real balances

RISK 4

Sponsorship

- Review moves earlier for controllers
- They need to trust the rules
- Controller owns the sign-off

Four risks need an owner before wave one

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- Pilot on real balances

RISK 4

Sponsorship

- Review moves earlier for controllers
- They need to trust the rules
- Controller owns the sign-off

Source: Northfield close readiness interviews, September 2026.

Three groups change how they work



CONTROLLERS

Review earlier

Controllers move from one review on day nine to rolling review from day one, and sign off on day five.

Time shifts from checking entries to judging exceptions.



ENTITY ACCOUNTANTS

Reconcile as you go

Accountants reconcile during the month with one template. Exceptions go to an owner the same day.

Month-end becomes a review of work already done.



SHARED SERVICES

Automate the matching

The shared-service team runs intercompany matching overnight and clears breaks each morning.

Fewer breaks reach the controller at all.

APPROACH

Sequence and ownership drive the gain

Most of the delay is waiting, so the fix is a change in sequence and ownership more than a change in tools.

SEQUENCE

Check as entries post

Today review happens after the books are drafted. We move rules to the point of entry, so a bad accrual is caught by the person who posted it, with the context still fresh.

OWNERSHIP

Name an owner per task

Every close task gets one owner, a due time and an escalation path. When a task slips, the controller sees it on day one rather than finding it on day nine.

The close moves into the team's week



BEFORE

Review on day nine

Controllers saw most entries for the first time on day nine, after the books were drafted. Every finding reopened work that had been signed off.

- One review, late in the close
- Findings reopen signed-off work
- Little time left to fix them

AFTER

Review as entries post

Rules check entries as they post. Controllers see only exceptions, review them from day one, and sign off on day five.

- Rolling review from day one
- Exceptions routed the same day
- Sign-off on day five

What changes for the controller

The controller's first morning

On day one the controller opens a single exceptions queue instead of a stack of spreadsheets. Each item shows the account, the amount, the likely cause and the person who owns the fix.

By the end of day two most items are cleared by the entity accountants. What remains are judgment calls: a reserve, a reclassification, a timing question. Those are the items a controller should spend time on.

Nothing waits for the end of the month to be looked at, so nothing arrives all at once.

NOTE

Pilot entities cleared exceptions in two days on average, against nine days before the pilot.

Where the savings come from

Most of the savings come from work that stops, not work that speeds up.

SUPPORTING DETAIL

Manual intercompany matching, duplicate review and rework after sign-off account for most of the \$4.2M run-rate saving. None of it needs a faster tool; it needs the work designed out of the process.

We size the saving from the close calendar for 14 business units, then validate each line with the entity controller before it enters the business case.

The script has **flipped** on legacy systems: what once looked too slow, too costly and too risky to change is now the fastest way to fund growth, cut risk and free the teams who run the business every day.

AI has cut the time and cost of complex modernization by changing how teams analyze, design, build and test.

WHAT CHANGES

- **Analysis.** Months of discovery compress to weeks.
- **Build.** Teams modernize and add value in one program.
- **Risk.** Dependencies surface early, not at cutover.

If modernizing cost one-tenth of what you expect, what would you unlock first?

Three groups change how they work

CORPORATE CONTROLLERS

Review earlier

Controllers move from a single review on day nine to rolling review from day one. They see exceptions, not every entry, and sign off on day five.

Time shifts from checking entries to judging exceptions.

ENTITY ACCOUNTANTS

Reconcile as you go

Accountants reconcile balance-sheet accounts during the month with one template. Exceptions go to an owner the same day, with the evidence attached.

Month-end becomes a review of work already done.

SHARED SERVICES

Automate the matching

The shared-service team runs intercompany matching overnight and clears breaks each morning, replacing email threads across eleven entities.

Fewer breaks reach the controller at all.

ROLES

Three groups change how they work

CORPORATE CONTROLLERS

Review earlier

Controllers move from a single review on day nine to rolling review from day one. They see exceptions, not every entry, and sign off on day five.

Time shifts from checking entries to judging exceptions.

ENTITY ACCOUNTANTS

Reconcile as you go

Accountants reconcile balance-sheet accounts during the month with one template. Exceptions go to an owner the same day, with the evidence attached.

Month-end becomes a review of work already done.

SHARED SERVICES

Automate the matching

The shared-service team runs intercompany matching overnight and clears breaks each morning, replacing email threads across eleven entities.

Fewer breaks reach the controller at all.

The close moves from waiting to flowing

TODAY

A close that waits

Close takes 12 days, and about seven of them are spent waiting. Balance-sheet accounts are reconciled after the month ends, eleven entities match intercompany balances by email, and controllers see most entries for the first time on day nine. Errors surface when they are most expensive to fix, and every late finding reopens work that was already signed off.

The calendar looks full, but most of it is waiting: for a reconciliation, for a match, for a reviewer.

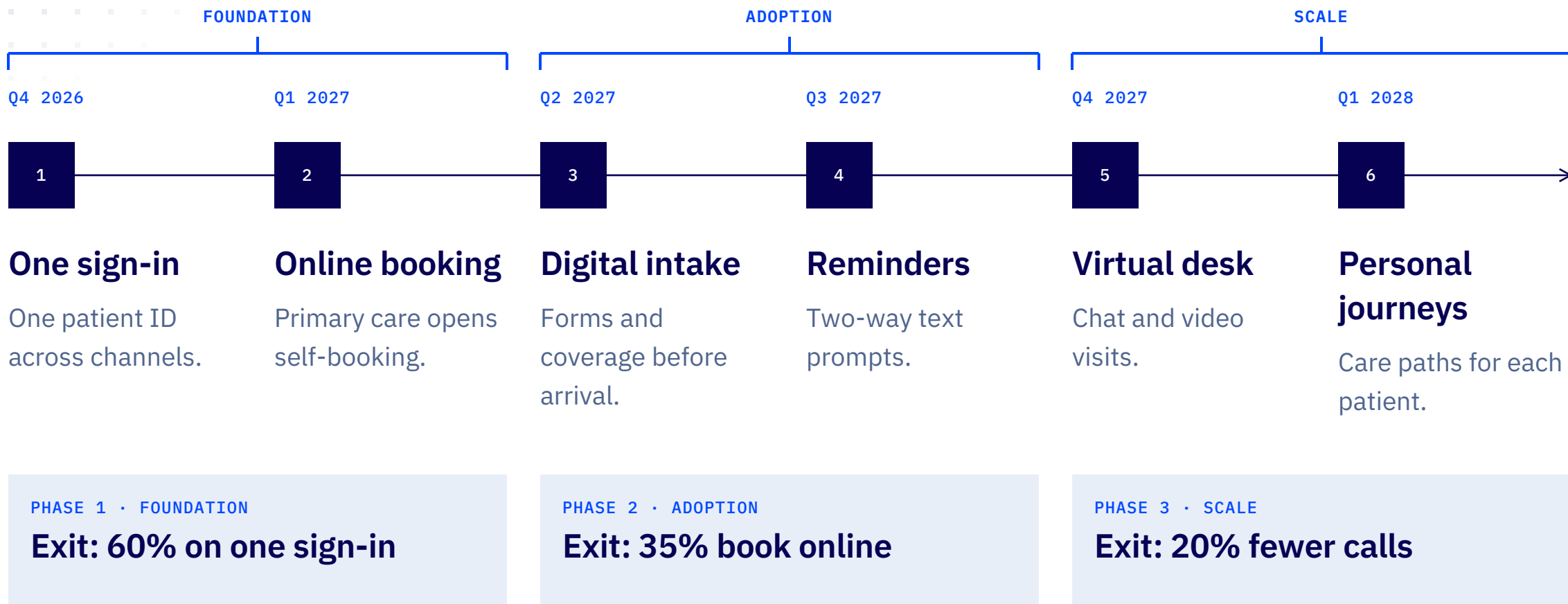
TARGET STATE

A close that flows

Reconciliations happen during the month, intercompany balances match overnight, and review rules check entries as they post. Exceptions reach a named owner on day one, so the days at the end are for sign-off rather than discovery. The controller signs off on day five, and the finance team gets its calendar back.

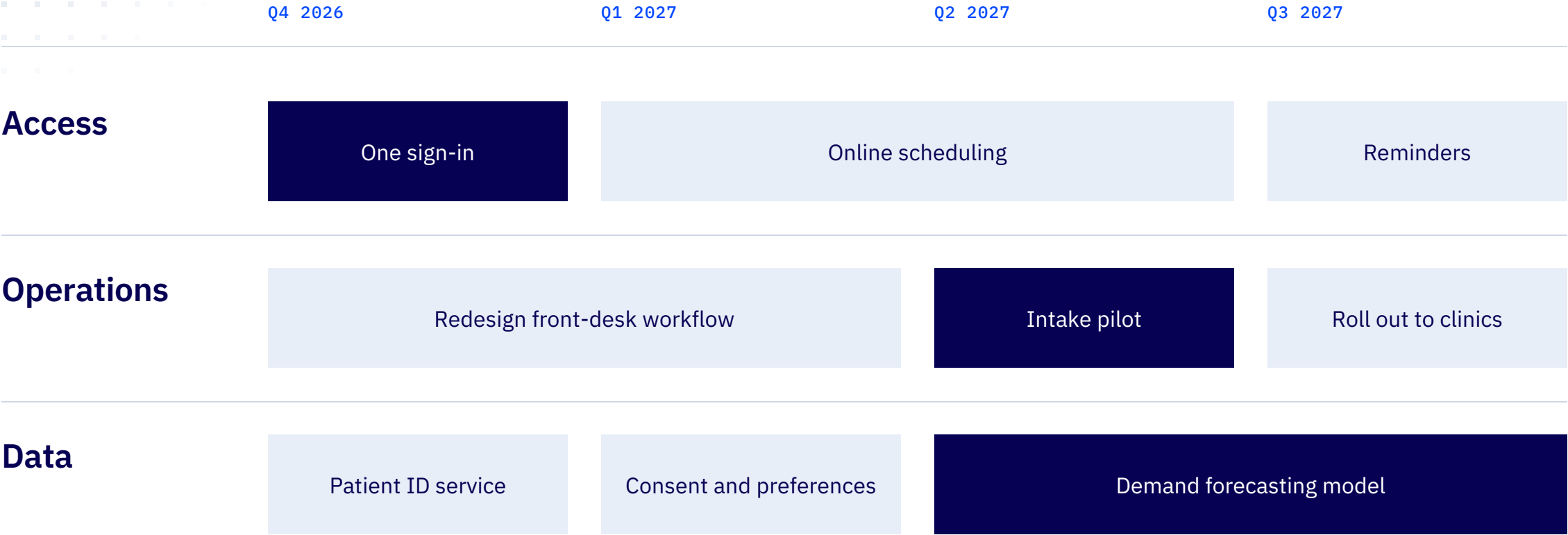
The calendar gets shorter because the waiting is designed out, not because anyone works faster.

Six milestones in three phases carry the program to 2028



Source: Sample figures for layout only; replace with current firm data before use.

Three lanes run in parallel and meet at the intake pilot



 Go-live milestone

Source: Sample figures for layout only; replace with current firm data before use.

Six milestones, two decisions to take now



Source: Sample figures for layout only; replace with current firm data before use.

DECISIONS NEEDED

Fund wave 2 by March

Intake and reminders need a named clinical owner at each site.

Choose the chat vendor

Selection closes in Q2 2027 to hold the desk release.

Six role-based curricula, 40 learner hours per site

ROLE	CURRICULUM	FORMAT	HOURS	WINDOW
Schedulers	Booking workflow, rules, waitlists	Hands-on lab	8	T-4 to T-1 wks
Call-center agents	Booking workflow, scripts, escalation	Hands-on lab	8	T-4 to T-1 wks
Nurses	Holds, referrals, clinic view	Virtual, 2 sessions	4	T-3 to T-1 wks
Physicians	Schedule view, templates	Peer-led huddle	2	T-3 to T-1 wks
Practice managers	Template builder, reports, coaching	Workshop	12	T-6 to T-2 wks
Service desk	Triage, runbook, known issues	Walk-through	6	T-3 to T-1 wks

Source: Northfield Health System training plan, Sep 2026.

TRAINING AT A GLANCE

1,840

staff trained across nine sites

8 hrs

for schedulers and agents, the
heaviest curriculum

96%

completion required before access
is granted



TRAINING PLAN

Four weeks to train 1,840 staff

ROLE	FORMAT	HOURS	WINDOW
Schedulers	Hands-on lab	8	T-4 to T-1 wks
Call-center agents	Hands-on lab	8	T-4 to T-1 wks
Nurses	Virtual, 2 sessions	4	T-3 to T-1 wks
Physicians	Peer-led huddle	2	T-3 to T-1 wks
Practice managers	Workshop	12	T-6 to T-2 wks
Service desk	Walk-through	6	T-3 to T-1 wks

Source: Northfield Health System training plan; sample figures for layout only.

Training closes one week before each site goes live

ROLE	W-6	W-5	W-4	W-3	W-2	W-1	GO-LIVE	W+1	W+2
------	-----	-----	-----	-----	-----	-----	---------	-----	-----

Practice managers

Template workshop

Schedulers and agents

Hands-on labs, 8 hrs

Floor-walkers

Nurses

Virtual sessions

Refreshers

Physicians

Peer huddles

Refreshers

Service desk

Runbook walk-through

Refreshers

Gate: no one receives platform access until their role curriculum is complete.

Source: Northfield Health System training plan, Sep 2026.

TARGET OPERATING MODEL

From a 12-day close to a 5-day close

TODAY

Entries reviewed on day 9

Intercompany matched by email

Controllers review every entry

Close takes 12 days



WITH THE NEW MODEL

Rules check entries as they post

92% AUTO-CHECKED

Matched automatically, daily

11 ENTITIES

Controllers review trends and exceptions

-70% REVIEW TIME

Close takes 5 days

-7 DAYS

Source: West Monroe analysis of the FY26 baseline.

Each pain point has a design answer

TODAY'S PAIN POINTS

Month-end close runs 12 days because reconciliations start after period end.

Eleven entities match intercompany balances by email and spreadsheet.

Three sources report different revenue and nobody owns the difference.

Controllers spend most of the cycle chasing status instead of analyzing.

DESIGN THEMES

Review at posting

Rules check entries at posting and surface errors early.

One matching engine

Balances match daily, with exceptions routed to an owner.

A governed data model

Finance and operations read one set of numbers.

A close cockpit

Every entity, task and blocker is visible in one view.

We removed these constraints by redesigning the process before the platform.

Each pain point is answered by several design themes

TODAY'S PAIN POINTS

Close work starts after period end, so errors surface late and the month-end close runs 12 days.

DESIGN THEMES

Review at posting

Rules check entries as they post.

One matching engine

Balances match daily; exceptions get an owner.

A close cockpit

Every entity, task and blocker in one view.

Finance and operations read different numbers, and nobody owns the difference between them.

Governed data model

One set of numbers for both teams.

Named data owners

Every metric has an accountable owner.

Shared definitions

Terms agreed once, reused everywhere.

Each pain point is answered by several design themes

TODAY'S PAIN POINTS

Close work starts after period end, so errors surface late and the month-end close runs 12 days.

Finance and operations read different numbers, and nobody owns the difference between them.

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One set of numbers for both teams.

Named data owners

Every metric has an accountable owner.

Shared definitions

Terms agreed once, reused everywhere.

Our understanding on one page

CONTEXT

- Seven hospitals, 64 clinics, three schedulers
- Three acquisitions in four years
- EHR go-live in Q3 next year
- 11% of denials start at registration
- Days in A/R run above peers

SCOPE

- In: scheduling, registration, estimates
- In: edits, claims, denial work queues
- In: EHR interface design, training
- Out: EHR build, documentation redesign
- To confirm: wave one, data access

OBJECTIVES

- One access experience at every site
- Fewer denials, faster cash
- Cleaner registration data
- Lower cost to collect
- A sustained benefits model

CONSTRAINTS

- EHR cutover date is fixed
- Phase one capped at FY27 capital
- Estimates and consent rules apply
- Two legacy schedulers retire
- Access and billing decide separately

STAKEHOLDERS

- J. Alvarez, VP Revenue Cycle
- R. Chen, Patient Access
- EHR program lead
- Compliance and privacy
- Finance, capital planning

SUCCESS MEASURES

- Days in A/R: 52 to 44
- Denial rate: 9.4% to 6.0%
- Access score: 68 to 80
- Cost to collect: 3.4% to 2.8%
- Adoption: 90% at six months

Source: Sample names and figures for layout only; replace with engagement stakeholders and data before use.

Four constraints decide how we answer the ask

TIMELINE

EHR go-live in Q3

Access workflows must be ready for cutover, which leaves about nine months to design and train.

BUDGET

Funded in two phases

Phase one is capped at the FY27 capital line; phase two depends on measured results.

THE ASK

One access and revenue cycle design

Join scheduling, registration, estimates, claims and denials into one design that Northfield can run, measure and extend.

- Seven hospitals
- 64 clinics
- Three scheduling tools

REGULATORY

No Surprises Act, HIPAA

Good faith estimates and consent must work in the new flow from the first day.

SYSTEMS

Three tools, one EHR

Two legacy schedulers retire; interfaces to the clearinghouse and EHR stay stable.

Source: Sample content for layout only; replace with the client's RFP and engagement facts before use.

Four constraints, and what each means for the answer

The ask: one access and revenue cycle design that Northfield can run, measure and extend across seven hospitals and 64 clinics.

TIMELINE

EHR go-live in Q3

- About nine months to design and train
- Sites go live in waves

IMPLICATION

Sequence waves around cutover.

BUDGET

Funded in two phases

- Phase one capped at the FY27 capital line
- Phase two depends on results

IMPLICATION

Prove value in phase one.

REGULATORY

Estimates and consent

- No Surprises Act and HIPAA apply
- Good faith estimates from day one

IMPLICATION

Build compliance into the flow.

SYSTEMS

Three tools, one EHR

- Two legacy schedulers retire
- Clearinghouse interfaces stay stable

IMPLICATION

Retire tools without breaking claims.

Source: Sample content for layout only; replace with the client's RFP and engagement facts before use.

THE ASK

One access and revenue cycle design

Join scheduling, registration, estimates, claims and denials into one design that Northfield can run, measure and extend.

- Seven hospitals
- 64 clinics
- Three scheduling tools

It must deliver

- One front door at every site
- Fewer front-end denials
- Faster cash

OUR UNDERSTANDING

Four constraints decide how we answer the ask

TIMELINE

EHR go-live in Q3

About nine months to design and train before cutover.

BUDGET

Funded in two phases

Phase one is capped at the FY27 capital line.

REGULATORY

Estimates and consent

No Surprises Act and HIPAA rules apply from day one.

SYSTEMS

Three tools, one EHR

Two legacy schedulers retire; interfaces stay stable.

Source: Sample content for layout only; replace with the client's RFP and engagement facts before use.

We read each of your three asks as a design choice

RFP REQUIREMENT 1

“Create a single patient access experience across every site.”



How we read it

We read this as one scheduling, registration and estimate flow, not one new tool.

RFP REQUIREMENT 2

“Reduce avoidable denials and days in accounts receivable.”



How we read it

We read this as fixing front-end data quality first, then denial work queues.

RFP REQUIREMENT 3

“Align the design to the planned EHR implementation.”



How we read it

We read this as sequencing waves around go-live and cutover, not running beside it.

Source: Sample RFP language for layout only; replace with the client's RFP text before use.

OUR UNDERSTANDING

Seven objectives, each with a measure you will judge us on

How we read success

The RFP lists objectives but few targets. We have proposed a baseline and target for each, to confirm in the first two weeks.

CASH IMPACT

8 days

fewer days in A/R is the measure most tied to the business case

Source: Sample figures for layout only; replace with current firm data before use.

OBJECTIVE	MEASURE	BASELINE	TARGET
Faster access	Days to third next available new patient visit	19	7
Cleaner front end	Registration error rate claims held at edit	11%	4%
Fewer denials	Initial denial rate front-end causes	9.4%	6.0%
Faster cash	Days in accounts receivable net of payer holds	52	44
Better experience	Patient access score survey, top box	68	80
Lower cost to collect	Cost to collect share of net revenue	3.4%	2.8%
Adoption	Staff on new workflows at six months	0%	90%

OUR UNDERSTANDING

Northfield is growing faster than its access model can follow

Growth by acquisition

Three recent acquisitions brought three scheduling tools and different registration practices.

Cash is leaking at the front end

Registration issues cause 11% of denials and push days in A/R above peers.

Patients repeat themselves

Patients give the same details at scheduling, check-in and billing, and errors follow.

The EHR date will not move

Cutover next Q3 is the fixed point every other decision must fit.

So what: the answer must fix the front end and fit one fixed date.

NORTHFIELD AT A GLANCE

7

hospitals

64

clinics

3.1M

annual patient visits

\$2.4B

net patient revenue

Source: Sample figures for layout only; replace with current Northfield data before use.

OUR UNDERSTANDING

We read the scope as the front end through to denials

IN SCOPE

What we will deliver

- Scheduling, registration, estimates
- Front-end edits and claims
- Denial work queues
- EHR interface design
- Training and adoption

OUT OF SCOPE

What stays with others

- EHR configuration and build
- Clinical documentation redesign
- Payer contract negotiation
- Network and hardware refresh

Boundary rule: any change to the EHR build is a change request.

TO CONFIRM

Assumptions

- Wave one covers four hospitals and 20 clinics
- We receive read access to all three schedulers
- One leader owns access and billing decisions
- The Q3 go-live date is fixed
- Phase one funding is approved
- Northfield names one executive sponsor

Source: Sample content for layout only; replace with the client's RFP and engagement facts before use.

OUR UNDERSTANDING

What is in scope, what is not and what we assume

IN SCOPE	OUT OF SCOPE	TO CONFIRM
Included in our proposal ▪ Scheduling, registration and estimates ▪ Front-end edits, claims and denials ▪ EHR integration design ▪ Training and adoption	Excluded or owned elsewhere ▪ EHR configuration and build ▪ Clinical documentation redesign ▪ Payer contract negotiation ▪ Hardware and network refresh	Assumptions we want to test ▪ Hospitals and clinics in wave one ▪ Data access to three scheduling tools ▪ Decision rights across access and billing ▪ Go-live date is fixed

If an assumption is wrong, this is how the plan changes

Wave one sites

Resize wave one; design holds.

Data access

Add two weeks to discovery.

Fixed go-live

Reorder waves around cutover.

Source: Sample content for layout only; replace with the client's RFP and engagement facts before use.

Three scheduling tools and one EHR date shape the ask

SITUATION

Where Northfield stands

- Seven hospitals and 64 clinics run three scheduling tools
- Registration quality varies by site
- A new EHR go-live is set for Q3 next year

COMPLICATION

What is getting in the way

- Front-end errors drive 11% of denials
- Patients repeat the same information at each step
- Billing and access teams answer to different leaders

WHAT YOU ASKED FOR

What the RFP requires

- A unified access and revenue cycle design
- A phased roadmap that fits the EHR date
- Named owners and measures for each benefit

Our reading: the complication is organizational as much as technical, so the answer has to join access, billing and the EHR plan.

Source: Sample content for layout only; replace with the client's RFP and engagement facts before use.

Three scheduling tools and one EHR date shape the ask

SITUATION

Where Northfield stands

- Seven hospitals and 64 clinics run three scheduling tools
- Registration quality varies by site
- Three acquisitions in four years

3

scheduling tools

71

sites to align

COMPLICATION

What is getting in the way

- Front-end errors reach the claim
- Patients repeat the same details at each step
- Access and billing answer to different leaders

11%

denials at intake

52

days in A/R

WHAT YOU ASKED FOR

What the RFP requires

- A unified access and revenue cycle design
- A phased roadmap that fits the EHR date
- Named owners and measures for each benefit

7

measures to track

Q3

EHR go-live

Our reading: the complication is organizational as much as technical, so the answer has to join access, billing and the EHR plan.

Source: Sample figures for layout only; replace with the client's RFP and engagement facts before use.

What sits behind the ask

SITUATION

Where Northfield stands

- Seven hospitals and 64 clinics run three scheduling tools
- Registration quality varies by site
- Three acquisitions in four years

COMPLICATION

What is in the way

- Front-end errors reach the claim
- Patients repeat the same details at each step
- Access and billing answer to different leaders

3

scheduling tools

71

sites to align

11%

denials at intake

52

days in A/R

Our reading: the complication is organizational as much as technical.

Source: Sample figures for layout only; replace with the client's RFP and engagement facts before use.

WHAT YOU ASKED FOR

One joined-up design

- A unified access and revenue cycle design
- A roadmap that fits the EHR date
- Named owners and measures
- Seven measures baselined in two weeks

FIXED POINT

EHR go-live in Q3

OUR UNDERSTANDING

Situation, complication and ask in one reading

How we read it

The complication is organizational as much as technical, so the ask has to join access, billing and the EHR plan.

WHERE CASH WAITS

52 days

in A/R, above peers, is the cost of the front-end gap

SITUATION

Where we start

- Seven hospitals, 64 clinics, three scheduling tools
- Registration varies by site
- Three acquisitions in four years

3

scheduling tools

COMPLICATION

In the way

- Front-end errors reach the claim
- Patients repeat the same details
- Access and billing split by leader

11%

of denials start at registration

THE ASK

What you need

- One access and revenue cycle design
- A roadmap that fits the EHR date
- Named owners and measures

7

measures to baseline

Source: Sample figures for layout only; replace with the client's RFP and engagement facts before use.

You asked for one front end, not another tool

We heard the same problem from four groups: patient access and billing run as separate programs that Northfield now wants run as one.

THEME	WHAT WE HEARD	WHY IT MATTERS	WHAT WE WILL DO
One front door Access	Three tools, three ways to register Operations, Patient Access	Patients repeat details and sites diverge on quality	One scheduling and registration flow at all 71 sites
One revenue cycle Billing	Front-end errors reach the claim Revenue Cycle, Finance	11% of denials start at registration; A/R runs above peers	Edits at entry and one denial work queue
One date EHR	Cutover next Q3 cannot move EHR program, IT	Training and workflows must be ready before go-live	Waves sequenced around cutover, not beside it
One owner Governance	Access and billing split by leader Executive team	Benefits fall between two budgets and no owner	A named owner and measure for each benefit

Source: Sample content for layout only; replace with the client's RFP and engagement facts before use.

OUR UNDERSTANDING

You asked us to modernize patient access and the revenue cycle on one platform

Northfield wants one partner to redesign how patients are scheduled, registered and billed, and to carry the change through to measurable results.

01 One front door

One experience across seven hospitals and 64 clinics, where three scheduling tools run today.

02 One revenue cycle

Common front-end, claims and denial work to cut the 11% of denials that start at registration.

03 One owner

A partner accountable from design through adoption, not only for delivery.

RFP SECTION 1.2

“We want a partner who owns the outcome, not the work plan.”

Source: Sample RFP language and figures for layout only; replace with the client's RFP text and data before use.

RFP SECTION 1.2

“We want a partner who owns the outcome, not the work plan.”

NORTHFIELD TODAY

- Seven hospitals, 64 clinics
- Three scheduling tools
- 11% of denials start at registration
- EHR go-live in Q3 next year

OUR UNDERSTANDING

You asked us to modernize patient access and the revenue cycle

Northfield wants one partner to redesign how patients are scheduled, registered and billed, and to own the result.

01 One front door

A single scheduling and registration experience across all seven hospitals and 64 clinics.

02 One revenue cycle

Common front-end, claims and denial processes that cut days in A/R and rework.

03 One owner

A partner accountable from design through adoption.

Source: Sample RFP language and figures for layout only; replace before use.

OUR UNDERSTANDING

You asked us to unify patient access and the revenue cycle

Northfield wants one partner to redesign how patients are scheduled, registered and billed, and to own the result through adoption.

01 One door

One scheduling and registration flow at all 71 sites.

02 One cycle

Common front-end, claims and denial work that cuts days in A/R.

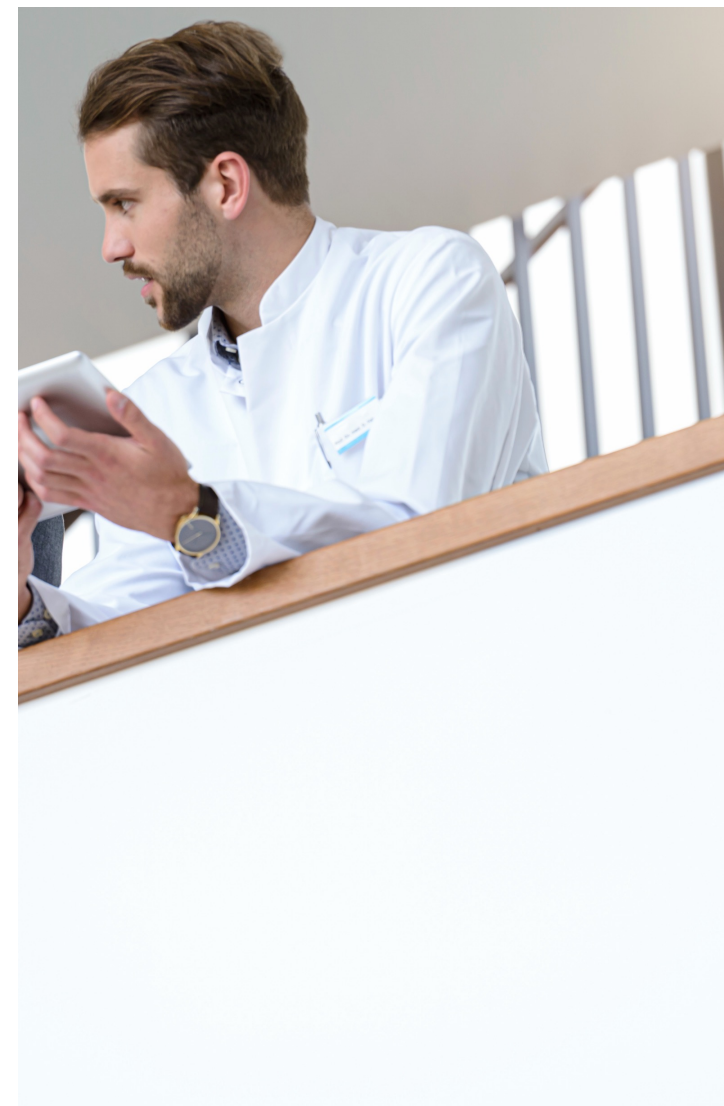
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Source: Sample content for layout only; replace with the client's RFP and engagement facts before use.



You asked us to modernize patient access and the revenue cycle on one platform

Northfield's RFP asks for one partner to redesign how patients are scheduled, registered and billed, and to carry the change through to measurable cash and experience results.

01 One front door

A single scheduling and registration experience across all seven hospitals and 64 clinics.

02 One revenue cycle

Common front-end, claims and denial processes that cut days in A/R and rework.

03 One owner

A partner accountable from design through adoption, not only for delivery.

RFP SECTION 1.2

“We want a partner who owns the outcome, not the work plan.”

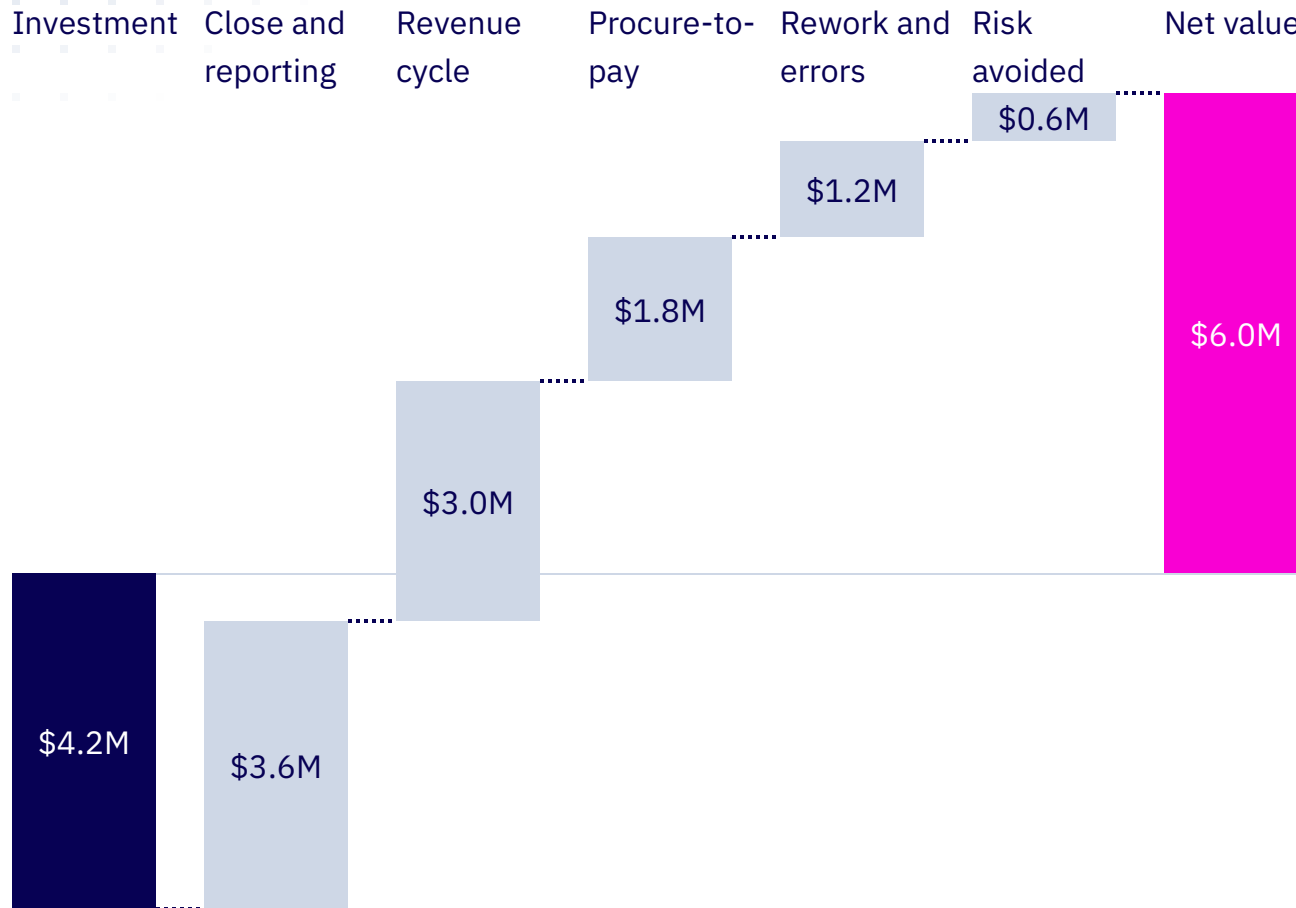
Source: Sample RFP language for layout only; replace with the client's RFP text before use.

Every RFP requirement maps to a section of this response

REF	RFP REQUIREMENT	OUR UNDERSTANDING	SECTION	PAGE	COVERAGE
R1.1	Single access experience	One scheduling and registration flow across all sites	Approach	14	Covered
R1.2	Online self-scheduling	Primary care and imaging scheduling in wave one	Approach	16	Covered
R2.1	Price estimates	Good faith estimates at scheduling, tied to eligibility	Solution	21	Covered
R2.3	Denial reduction	Front-end edits and a queue for top five denial causes	Solution	23	Covered
R3.1	EHR alignment	Waves sequenced around cutover; interfaces in scope	Roadmap	28	Partial
R3.4	Data migration	Legacy scheduler data retired; build is Northfield-led	Assumptions	31	Partial
R4.2	Change and training	Role-based training and super-user network	Adoption	36	Covered
R5.1	Benefits tracking	Baseline and monthly tracking of seven measures	Value	40	Covered

Source: Sample content for layout only; replace with the client's RFP and engagement facts before use.

Five levers turn \$4.2M of spend into \$6.0M net



READING THE BRIDGE

- **Close and reporting** 35% of five-year value
- **Revenue cycle** 29%, the fastest to start
- **Procure-to-pay** 18%
- **Rework and risk** 18% together

Bars show five-year value in \$M. The investment hangs below the zero line.

2.4x 143%

value per dollar

five-year ROI

Source: Sample figures for layout only; replace with current program data before use.

INVESTMENT AND VALUE

Five levers turn \$4.2M of spend into \$6.0M net

SHARE OF FIVE-YEAR VALUE

- Close and reporting 35%
- Revenue cycle 29%
- Procure-to-pay 18%
- Rework and risk 18%

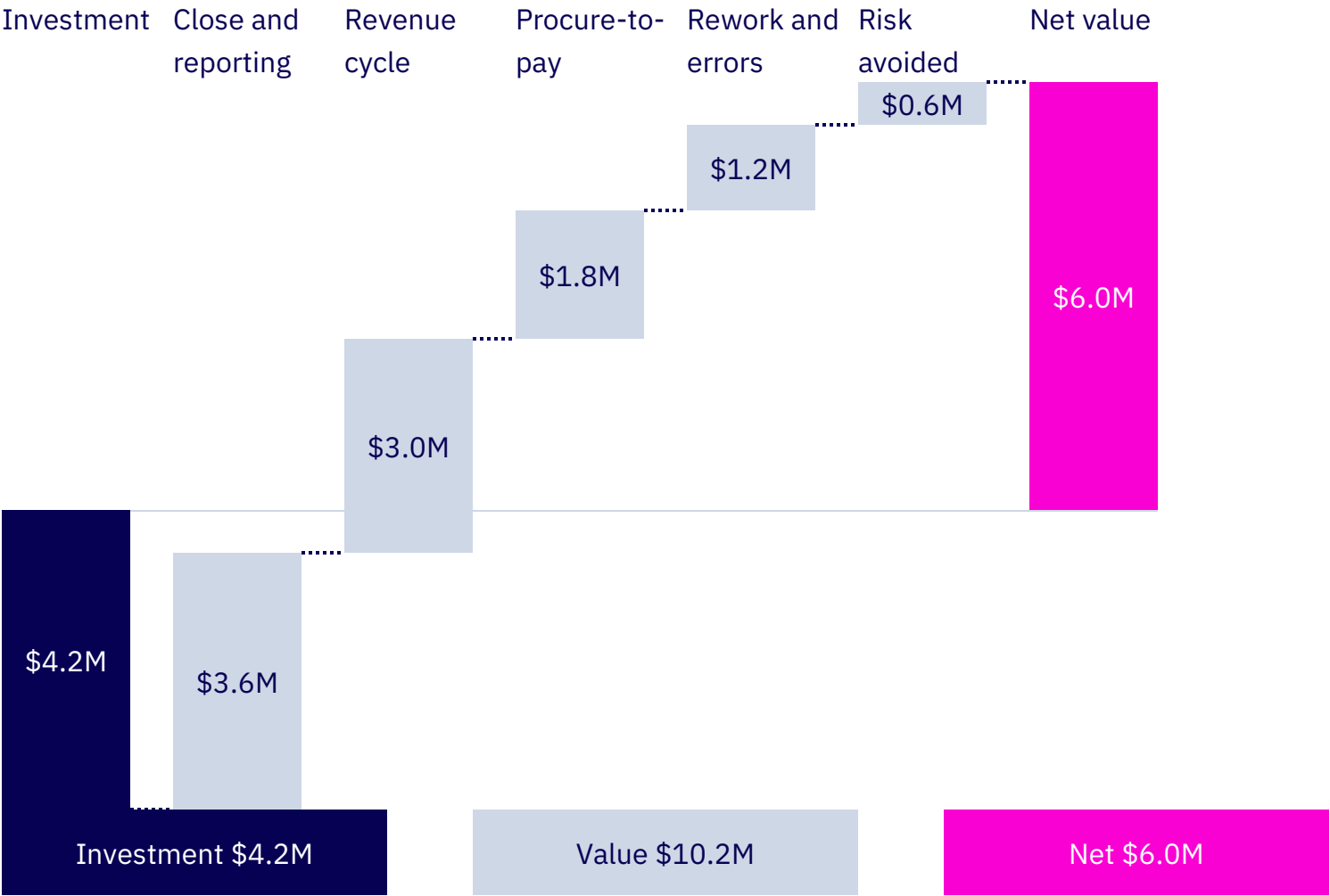
Value per dollar 2.4x

Five-year ROI 143%

Payback month 31

NPV at 8% \$4.1M

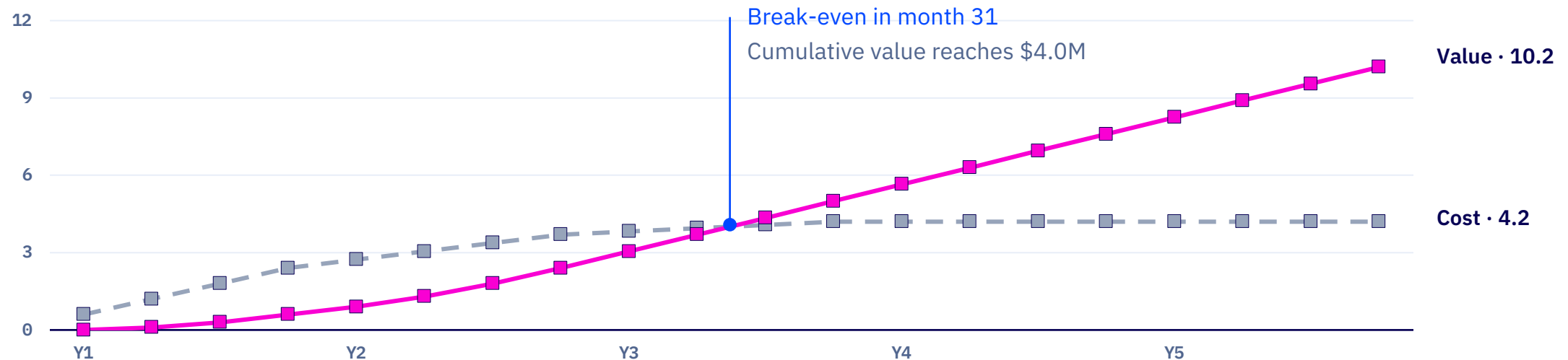
Source: Sample figures for layout only; replace with current firm data before use.



Cumulative value overtakes cost in month 31

Cumulative cost and value

\$M, BY QUARTER, FIVE YEARS



YEARS 1 TO 2 • INVEST

Spend runs at \$0.6M a quarter in year one while value is still ramping.

YEAR 3 • BREAK-EVEN

Cumulative value passes cumulative cost and net value turns positive.

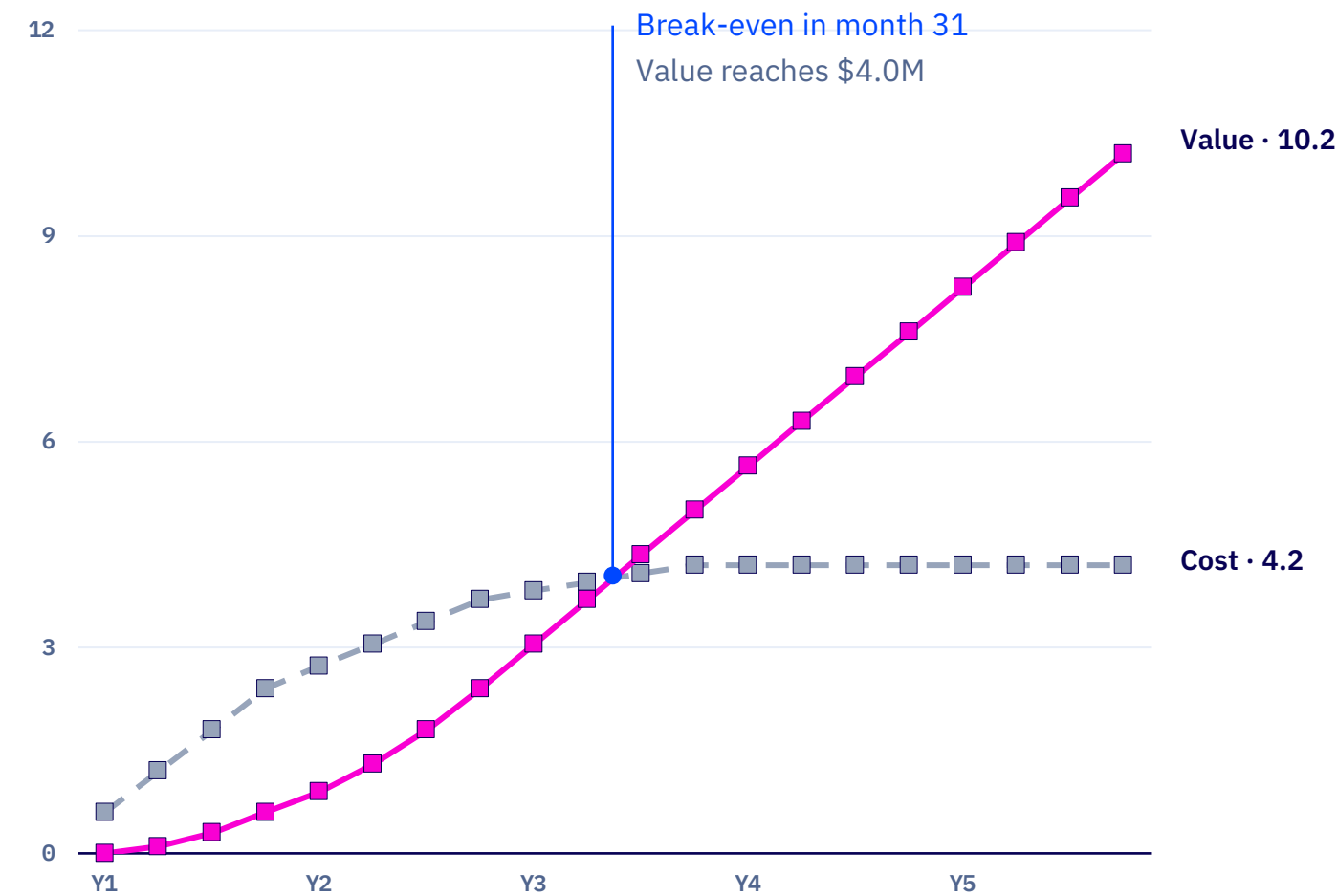
YEARS 4 TO 5 • COMPOUND

Spend stops and \$2.6M a year drops straight to net value.

Source: Sample figures for layout only; replace with current program data before use.

Cumulative cost and value

\$M, BY QUARTER, FIVE YEARS



INVESTMENT AND VALUE

Value overtakes cost in month 31

READING THE CURVES

- Spend peaks in year one at \$0.6M a quarter
- Value ramps from \$0.6M to \$2.6M a year
- Spend stops after year three

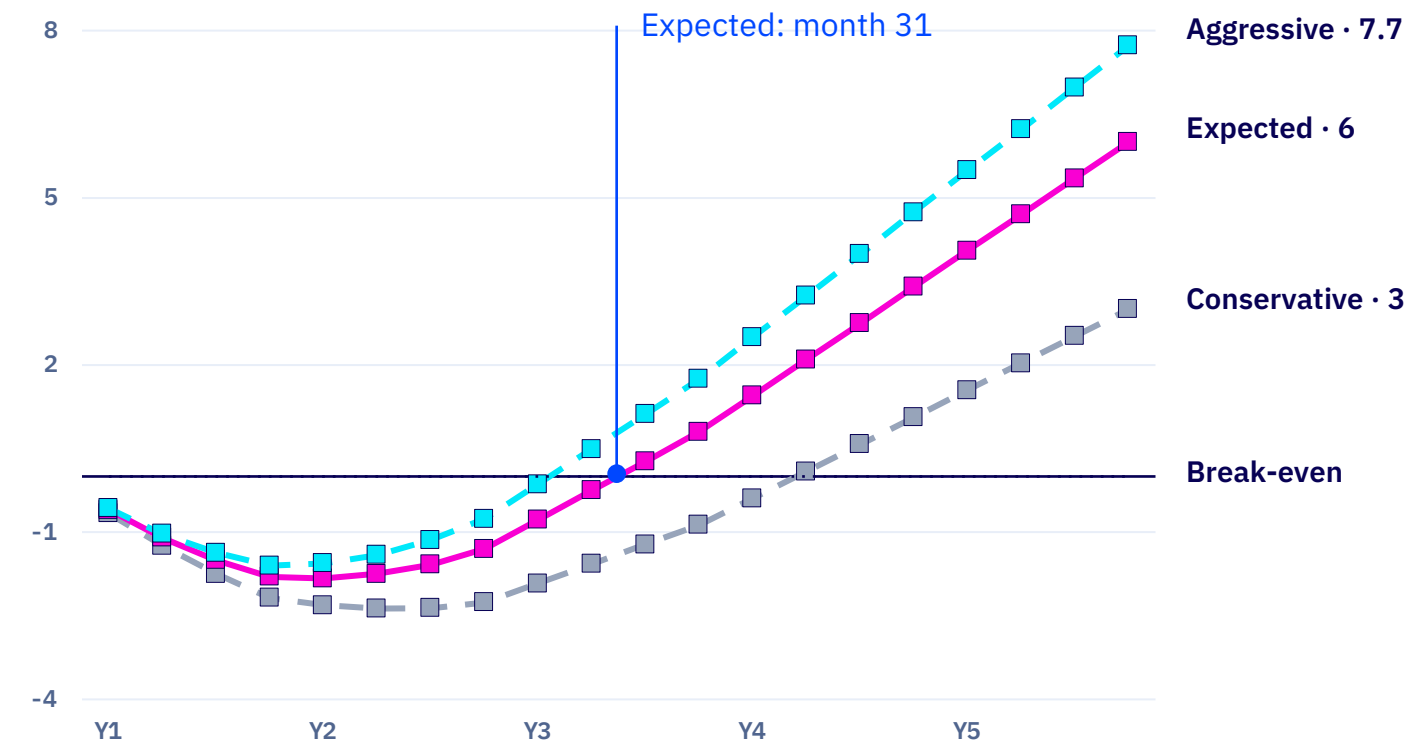
Total cost	\$4.2M
Five-year value	\$10.2M
Net value	\$6.0M
Payback	month 31

Source: Sample figures for layout only; replace with current firm data before use.

Break-even lands between month 28 and month 41

Cumulative net value by scenario

\$M, BY QUARTER, FIVE YEARS



Source: Sample figures for layout only; replace with current program data before use.

CONSERVATIVE

month 41

ROI 65% • NPV \$1.8M

EXPECTED

month 31

ROI 143% • NPV \$4.1M

AGGRESSIVE

month 28

ROI 193% • NPV \$5.5M

Each lever has an owner, a baseline and a target

LEVER	METRIC	BASELINE	TARGET	ANNUAL VALUE	FULL BY	CONFIDENCE
Close and reporting Controller	Hours per close cycle	4,200	2,400	\$1.0M	Year 3	■■■■■
Revenue cycle VP Revenue Cycle	Denial rate	9.1%	6.4%	\$0.8M	Year 3	■■■■□
Procure-to-pay Supply Chain Director	Spend under contract	54%	72%	\$0.4M	Year 2	■■■■■
Rework and errors Controller	Corrections a month	310	120	\$0.3M	Year 2	■■■■□
Risk avoided Compliance Officer	Open audit findings	14	4	\$0.1M	Year 3	■■□□
Annual value at run-rate				\$2.6M		

Source: Sample figures for layout only; replace with current program data before use.

Five levers add up to \$2.6M of annual value

VALUE DRIVER	LEVER	METRIC, BASELINE TO TARGET	ANNUAL VALUE
Productivity	Close and reporting	Hours per close: 4,200 to 2,400	\$1.0M
	Rework and errors	Corrections a month: 310 to 120	\$0.3M
Revenue	Revenue cycle	Denial rate: 9.1% to 6.4%	\$0.8M
Cost	Procure-to-pay	Spend under contract: 54% to 72%	\$0.4M
Risk	Risk avoided	Open audit findings: 14 to 4	\$0.1M
Five levers, annual value at run-rate			\$2.6M a year

Source: Sample figures for layout only; replace with current program data before use.

From wave two, new value funds the next wave

WAVE 1 • MONTHS 1 TO 12

Foundation

Data platform, close automation in three entities

Investment \$2.4M



Value in the wave \$0.6M



NET IN THE WAVE

-\$1.8M

Budget-funded

WAVE 2 • MONTHS 13 TO 24

Scale

Revenue cycle and procure-to-pay across all entities

Investment \$1.3M



Value in the wave \$1.8M



NET IN THE WAVE

+\$0.5M

Self-funding

WAVE 3 • MONTHS 25 TO 36

Optimize

Controls, analytics and run handover

Investment \$0.5M



Value in the wave \$2.6M



NET IN THE WAVE

+\$2.1M

Self-funding

Source: Sample figures for layout only; replace with current program data before use.

Waves two and three pay for themselves

WAVE	WHAT IT DELIVERS	INVESTMENT	VALUE	NET	CUMULATIVE	FUNDING
Wave 1 Months 1 to 12	Data platform; close automation in three entities	\$2.4M	\$0.6M	-\$1.8M	-\$1.8M	BUDGET
Wave 2 Months 13 to 24	Revenue cycle and procure-to-pay in all entities	\$1.3M	\$1.8M	\$0.5M	-\$1.3M	SELF-FUNDING
Wave 3 Months 25 to 36	Controls, analytics and run handover	\$0.5M	\$2.6M	\$2.1M	\$0.8M	SELF-FUNDING
Years 4 and 5	Run state; no new investment	–	\$5.2M	\$5.2M	\$6.0M	

Wave 1 is funded from the program budget: \$2.4M committed up front.

Wave 2 spend is covered by its own value and by wave 1 run-rate.

Cumulative net turns positive in wave 3, at month 31.

Source: Sample figures for layout only; replace with current program data before use.

Payback ranges from month 28 to month 41

MEASURE	CONSERVATIVE	EXPECTED	AGGRESSIVE
Share of planned value realized	75%	100%	115%
Adoption at the end of year two	70%	85%	95%
Investment	\$4.6M	\$4.2M	\$4.0M
Five-year value	\$7.6M	\$10.2M	\$11.7M
Net value	\$3.0M	\$6.0M	\$7.7M
Five-year ROI	65%	143%	193%
Payback	month 41	month 31	month 28
NPV at 8%	\$1.8M	\$4.1M	\$5.5M

Source: Sample figures for layout only; replace with current program data before use.

Even the conservative case pays back in month 41

Conservative		Expected		Aggressive	
Investment	\$4.6M	Investment	\$4.2M	Investment	\$4.0M
Five-year value	\$7.6M	Five-year value	\$10.2M	Five-year value	\$11.7M
Net value	\$3.0M	Net value	\$6.0M	Net value	\$7.7M
Five-year ROI	65%	Five-year ROI	143%	Five-year ROI	193%
Payback	month 41	Payback	month 31	Payback	month 28
NPV at 8%	\$1.8M	NPV at 8%	\$4.1M	NPV at 8%	\$5.5M
75% of planned value realized; cost 10% over plan		Planned value and planned cost		115% of planned value realized; cost 5% under plan	

Source: Sample figures for layout only; replace with current program data before use.

INVESTMENT AND VALUE

A \$4.2M investment returns \$6.0M net in five years

ONE-TIME INVESTMENT

\$4.2M

over three years of spend

ANNUAL VALUE AT RUN-RATE

→ \$2.6M

from year three

VALUE PER DOLLAR

2.4x

five-year value per \$1 spent

143%

five-year ROI

31

months to payback

\$4.1M

net present value at 8%

\$10.2M

five-year value

BASIS

Value ramps from \$0.6M in year one to \$2.6M in year three.

NOT INCLUDED

Strategic benefits and any value beyond year five.

Source: Sample figures for layout only; replace with current program data before use.

Cumulative net value turns positive in year three

\$M	YEAR 1	YEAR 2	YEAR 3	YEAR 4	YEAR 5	FIVE-YEAR TOTAL
Investment	2.4	1.3	0.5	–	–	4.2
Value	0.6	1.8	2.6	2.6	2.6	10.2
Net value	-1.8	0.5	2.1	2.6	2.6	6.0
Cumulative net	-1.8	-1.3	0.8	3.4	6.0	6.0
Discounted net, 8%	-1.7	0.4	1.7	1.9	1.8	4.1

143%

five-year ROI

31

months to payback

\$4.1M

net present value at 8%

2.4x

value per dollar

Source: Sample figures for layout only; replace with current program data before use.

INVESTMENT AND VALUE

Every dollar invested returns \$2.40 in value

FIVE-YEAR NET VALUE

\$6.0M

\$10.2M of value on \$4.2M of investment

143%

five-year ROI

31

months to payback

The investment is recovered in month 31; value then compounds at \$2.6M a year.

HOW IT ADDS UP

Investment, three years	\$4.2M
-------------------------	---------------

Annual value at run-rate	\$2.6M
--------------------------	---------------

Five-year value	\$10.2M
-----------------	----------------

Net present value at 8%	\$4.1M
-------------------------	---------------

Value per dollar invested	2.4x
---------------------------	-------------

Source: Sample figures for layout only; replace with current program data before use.

ANNUAL VALUE AT RUN-RATE

\$2.6M

against a one-time investment
of \$4.2M

FIVE-YEAR ROI

143%

\$6.0M net on \$4.2M invested

INVESTMENT AND VALUE

Value passes spend in year two

\$4.2M

total investment

31

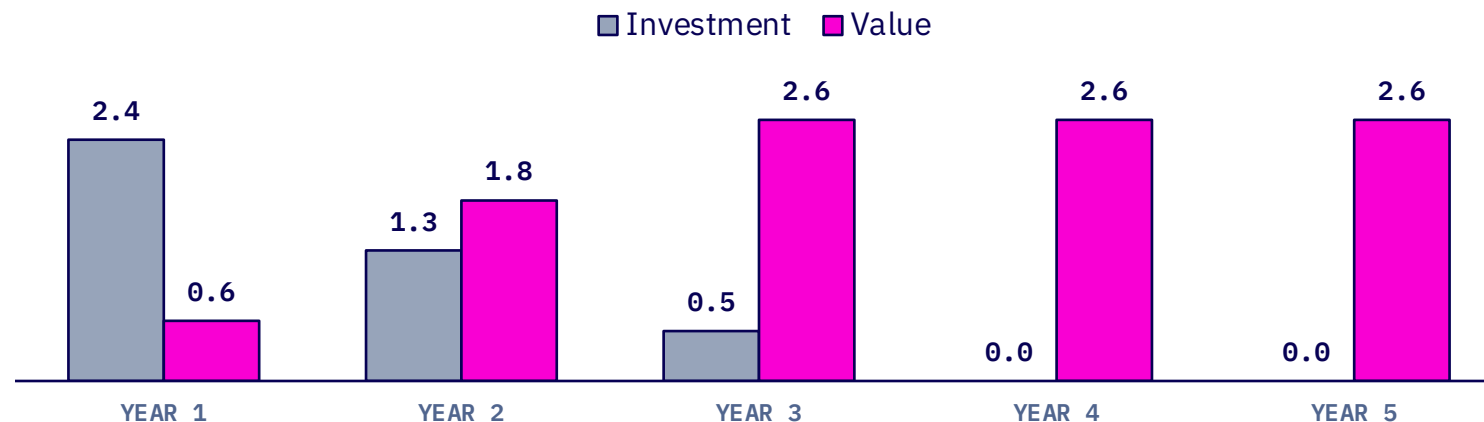
months to payback

\$4.1M

net present value

Investment and value by year

\$M



Source: Sample figures for layout only; replace with current program data before use.

INVESTMENT AND VALUE

Every value KPI has a named owner and a cadence

KPI	OWNER	BASELINE	TARGET	CURRENT	STATUS	REVIEW
Hours per close cycle	Controller	4,200	2,400	3,100	On track	Monthly
Denial rate	VP Revenue Cycle	9.1%	6.4%	8.2%	At risk	Monthly
Spend under contract	Supply Chain Director	54%	72%	66%	On track	Quarterly
Corrections a month	Controller	310	120	190	On track	Monthly
Open audit findings	Compliance Officer	14	4	9	On track	Quarterly

HOW VALUE IS CONFIRMED

Finance validates every realized dollar against the ledger.

The CFO chairs a monthly value review; the PMO owns the tracker.

Source: Sample figures for layout only; replace with current program data before use.

Realized value is 85% of plan after six quarters

\$1.3M

planned to date

\$1.1M

realized to date

85%

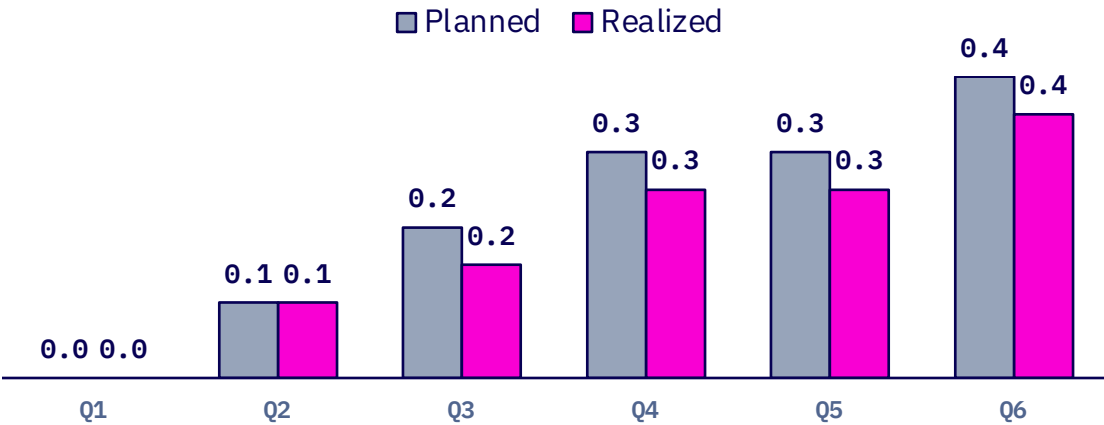
of plan

4 of 5

levers on plan

Value by quarter

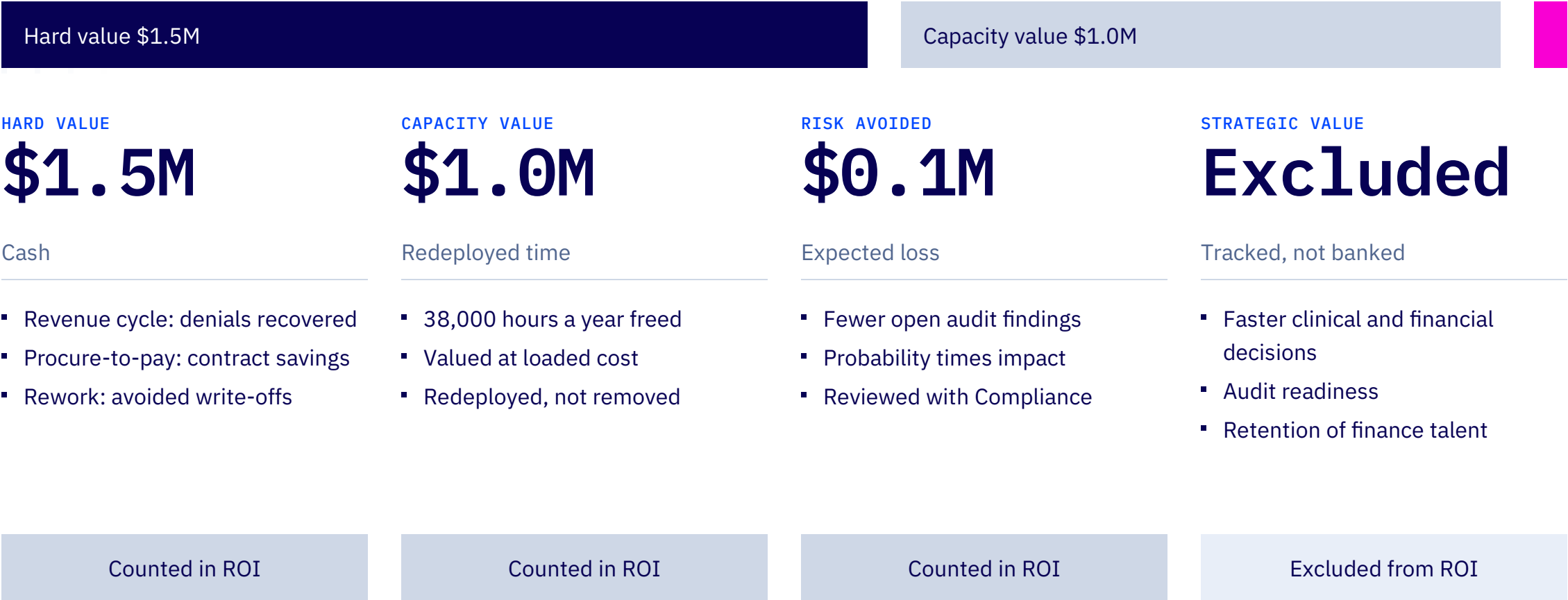
\$M



Source: Sample figures for layout only; replace with current program data before use.

LEVER	PLANNED	REALIZED	STATUS
Close and reporting	\$0.45M	\$0.42M	On track
Revenue cycle	\$0.35M	\$0.22M	At risk
Procure-to-pay	\$0.25M	\$0.24M	On track
Rework and errors	\$0.15M	\$0.14M	On track
Risk avoided	\$0.10M	\$0.08M	On track

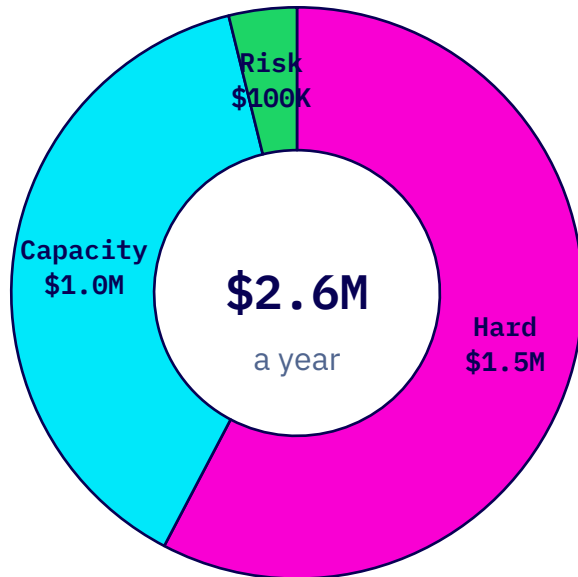
Hard and capacity value make up 96% of the case



Source: Sample figures for layout only; replace with current program data before use.

Cash and capacity are 96% of value

Annual value by type



WHAT EACH TYPE MEANS

Hard value shows up in the ledger: collected revenue, contract savings and avoided write-offs.

Capacity value is time released, valued at loaded cost and redeployed.

Risk avoided is expected loss: probability times impact.

58%

hard value

38%

capacity value

TREATMENT

Finance validates against the ledger; Compliance reviews the risk estimate yearly.

Source: Sample figures for layout only; replace with current program data before use.

NOT IN THE CASE

- Faster clinical and financial decisions
- Audit readiness
- Retention of finance talent
- A platform for later waves

Tracked every quarter, never counted toward ROI or payback.

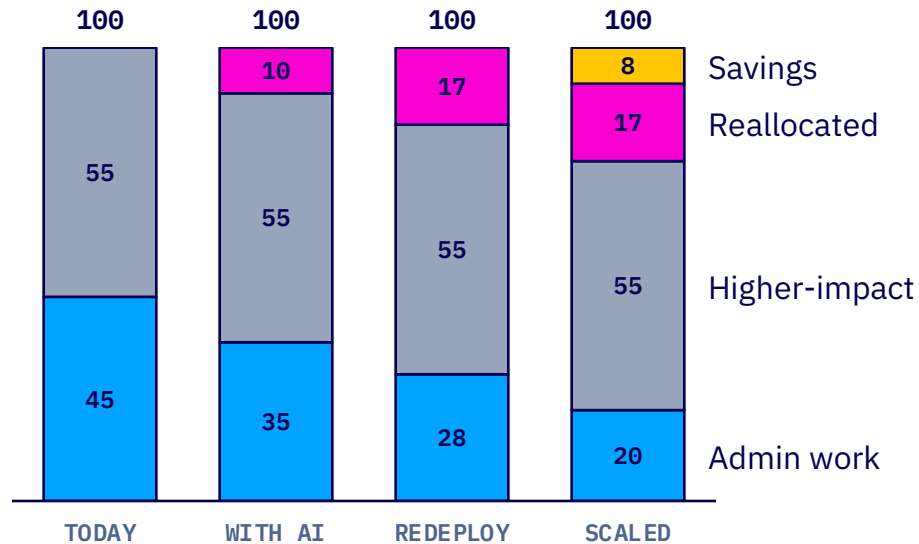
HOW IT IS TRACKED

A quarterly pulse survey and decision-cycle times, reported beside the value tracker.

AI creates capacity, but change management turns it into adopted behavior and Finance-recognized value

Capacity becomes value only when banked

SHARE OF TEAM EFFORT, ILLUSTRATIVE



Value is banked only when an owner changes cost

- 01 Baseline the work and cost**
Volumes, effort, loaded cost, behavior change.
- 02 Verify the work removed**
Touch time, cycle time and exceptions after use.
- 03 Protect operating outcomes**
Quality, compliance and service guardrails hold.
- 04 Execute an owned workforce action**
Absorb growth, avoid hiring or redeploy.
- 05 Validate and track the value**
Finance approves timing and banked savings.

Three of six vendors clear the bar for Northfield

CRITERION	VENDOR A	VENDOR B	VENDOR C	VENDOR D	VENDOR E	VENDOR F
Functional fit	●●●●	●●●●	●●●●	●●●●	●●●●	●●●●
Integration effort	●●●●	●●●●	●●●●	●●●●	●●●●	●●●●
Security and compliance	●●●●	●●●●	●●●●	●●●●	●●●●	●●●●
Total cost of ownership	●●●●	●●●●	●●●●	●●●●	●●●●	●●●●
Delivery support	●●●●	●●●●	●●●●	●●●●	●●●●	●●●●
VERDICT	SHORTLIST	SHORTLIST	RESERVE	DROP	RESERVE	SHORTLIST

Vendor B is the strongest fit for Northfield

Vendor B leads the weighted scorecard

CRITERION	WEIGHT	VENDOR A	VENDOR B	VENDOR C
Functional fit	25%	4	5	3
Integration effort	20%	3	4	5
Security and compliance	20%	4	4	3
Total cost of ownership	15%	3	3	5
Vendor viability	10%	4	4	3
Delivery support	10%	2	4	3
Weighted total	100%	3.45	4.10	3.70

RECOMMENDED

RECOMMENDATION

Select Vendor B

Vendor B scores highest overall and leads on functional fit and integration.

- Confirm security controls before contract
- Negotiate the cost of ownership terms
- Pilot with one Northfield business unit

4.10

Vendor B total, out of 5

Scores run 1 to 5, with 5 best. Weights were agreed with the steering committee and total 100%.

Option B scores highest across the weighted criteria

CRITERION	WEIGHT	OPTION A	OPTION B	OPTION C	OPTION D
Fit to requirements	25%	4	5	3	4
Delivery effort	20%	3	4	5	4
Risk and compliance	20%	4	4	3	2
Total cost	15%	3	3	5	4
Long-term viability	10%	4	4	3	5
Support and enablement	10%	2	4	3	3
Weighted total	100%	3.45	4.10	3.70	3.60

Scores run 1 to 5, with 5 best. Weights were agreed with the working group and total 100%.

Three steps narrowed twelve vendors to one



Each step tightens the test: must-haves first, then fit, then total value.

Three vendors, each with a clear trade-off

VENDOR A LOGO

Established suite with broad coverage

STRENGTHS

- Wide module coverage
- Large partner network

WATCH-OUTS

- Heavy configuration effort
- Slower release cadence

FIT	INDICATIVE COST
■■■□	\$\$ · 9 months

VENDOR B LOGO

RECOMMENDED

Modern platform built for fast integration

STRENGTHS

- Strong ERP integration
- Quick time to value

WATCH-OUTS

- Thinner security depth
- Smaller support team

FIT	INDICATIVE COST
■■■■	\$\$ · 6 months

VENDOR C LOGO

Low-cost option for focused needs

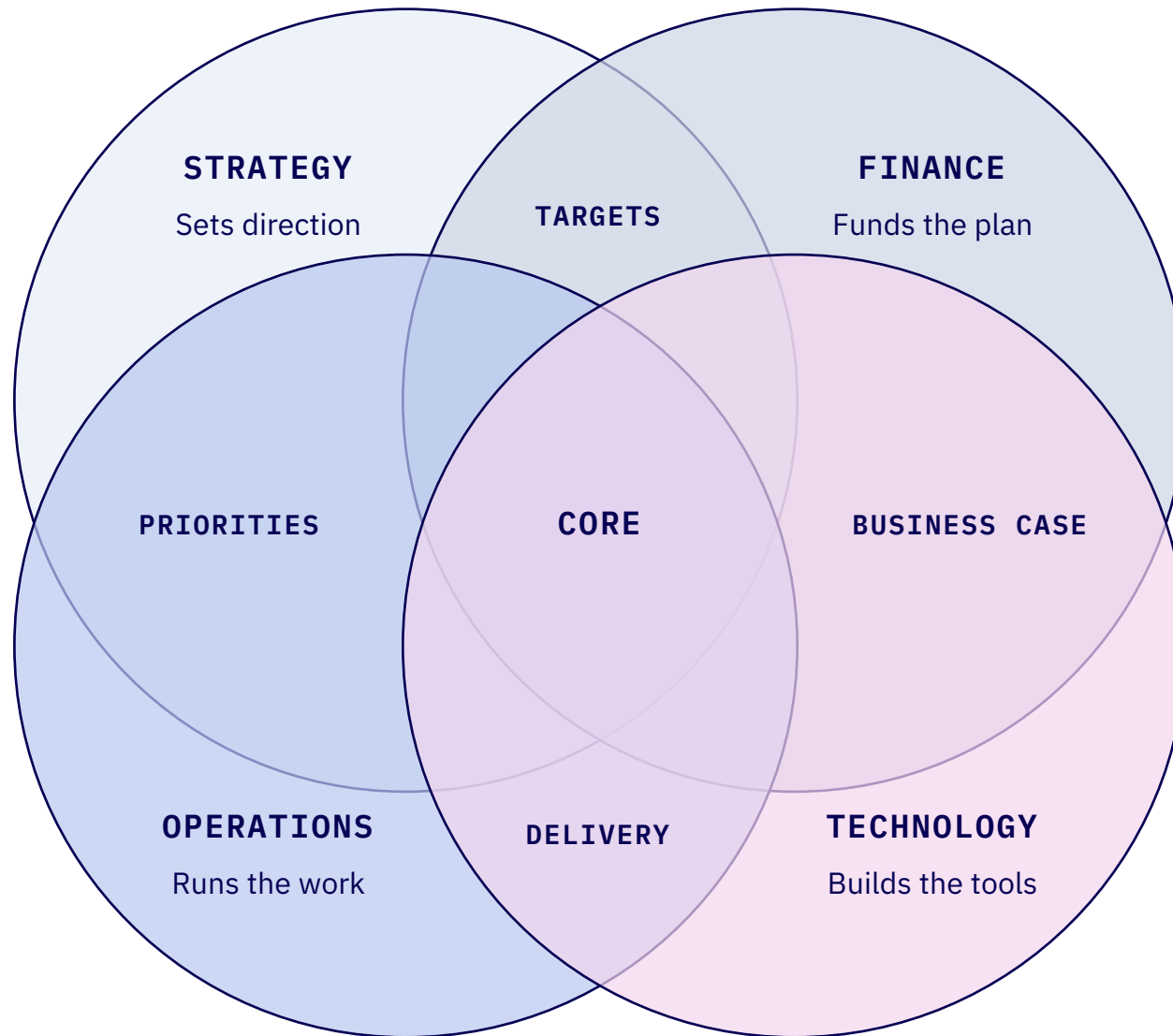
STRENGTHS

- Lowest license cost
- Simple to administer

WATCH-OUTS

- Limited workflow depth
- Few healthcare references

FIT	INDICATIVE COST
■■□□	\$ · 5 months



OPERATING MODEL

The centre is where the four teams align

CENTRE

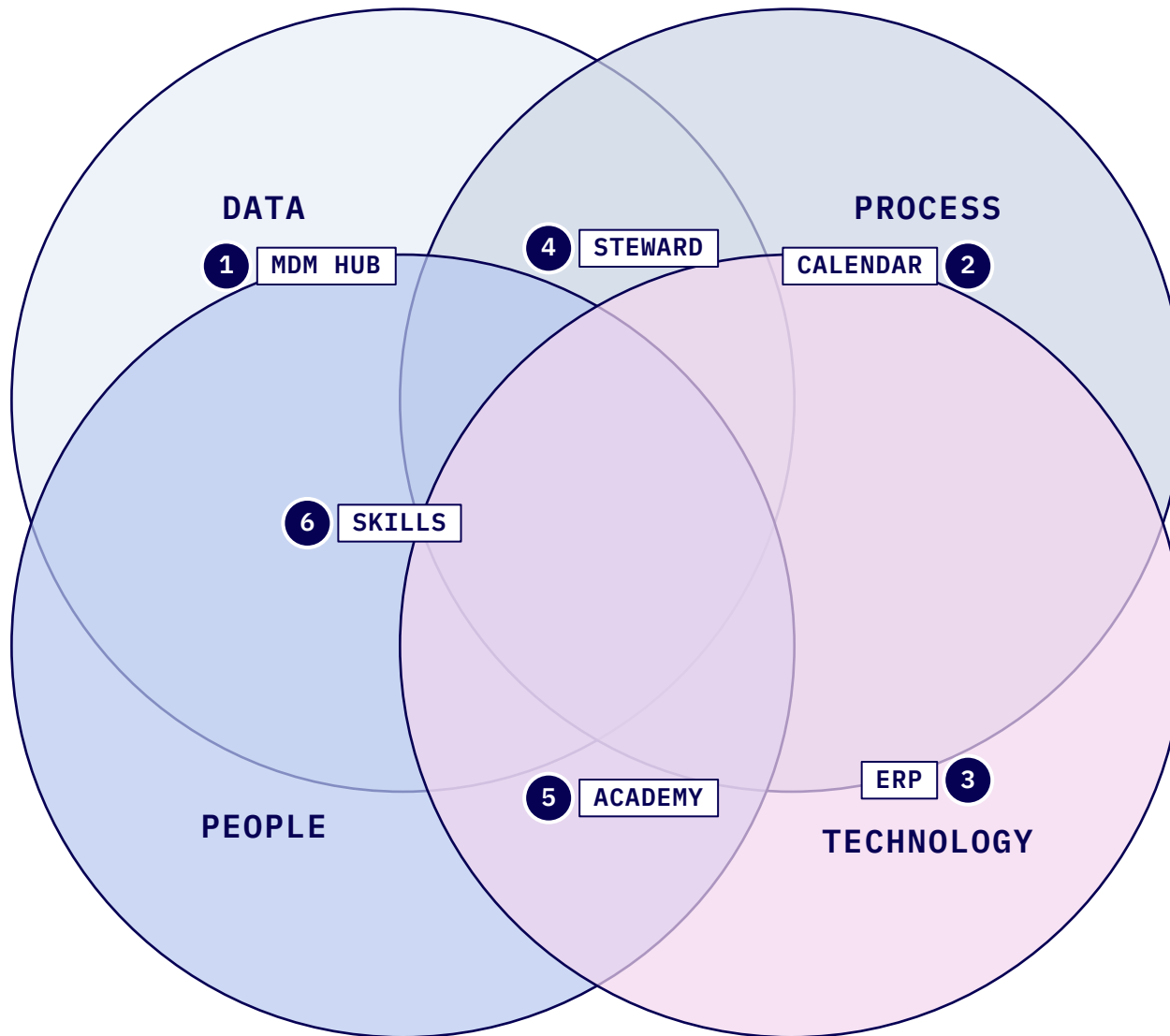
Core

Where strategy, funding, delivery and tooling agree on the same targets.

- **Targets.** Strategy and finance.
- **Delivery.** Operations and technology.

Source: Northfield finance and operations workshops, Sep 2026.

Six initiatives across four capabilities



- 01 Master data hub**
Data only.
- 02 Close calendar**
Process only.
- 03 ERP upgrade**
Technology only.
- 04 Data stewardship**
Data and process.
- 05 Finance academy**
People and technology.
- 06 Skills inventory**
Data and people.

Source: Northfield finance and operations workshops, Sep 2026.

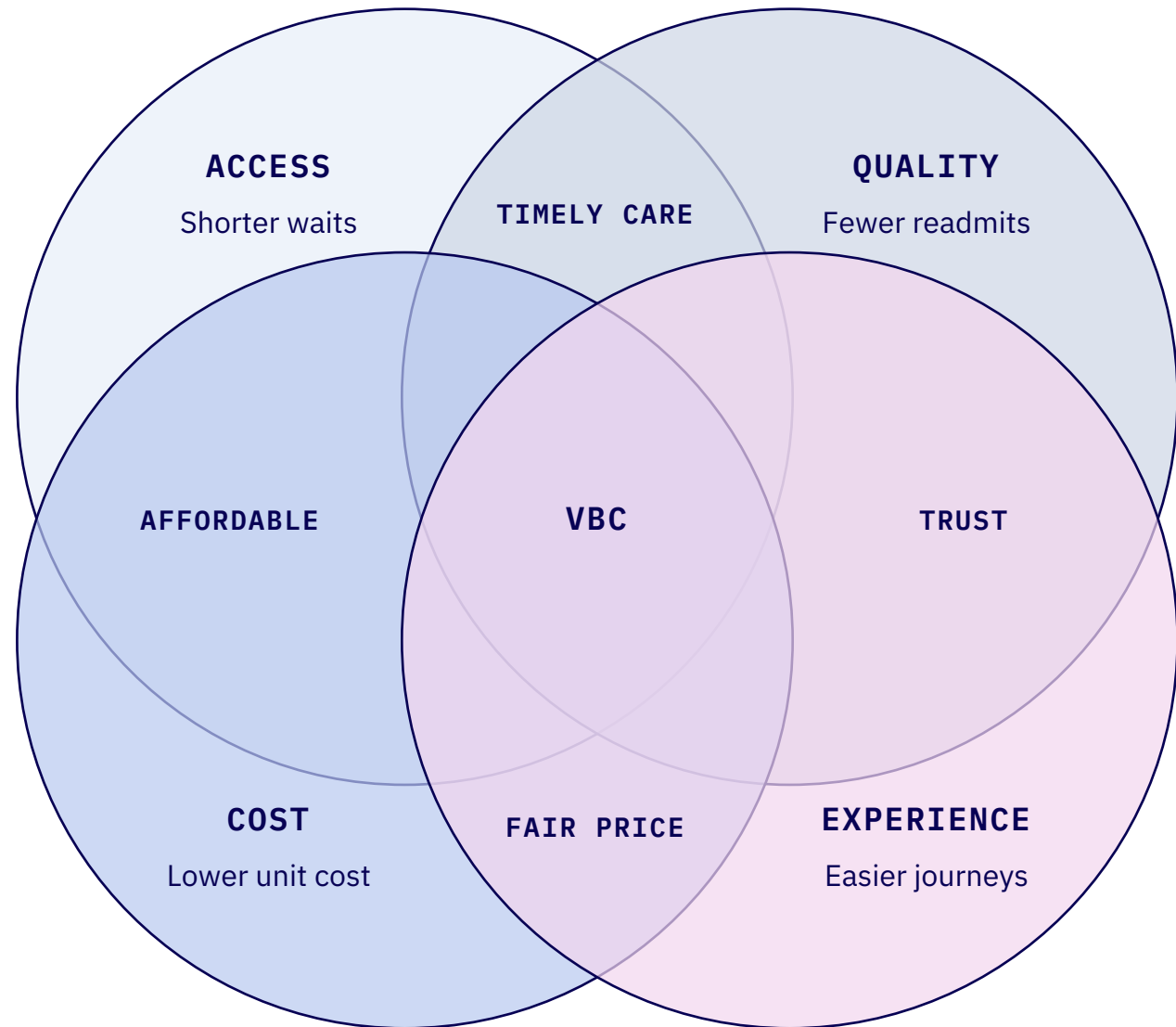
VALUE DRIVERS

Value-based care needs all four drivers

Four drivers, one aim

Gains in one driver often cost another. Value-based care (VBC) shows up only where all four hold.

VBC: small, and worth protecting.



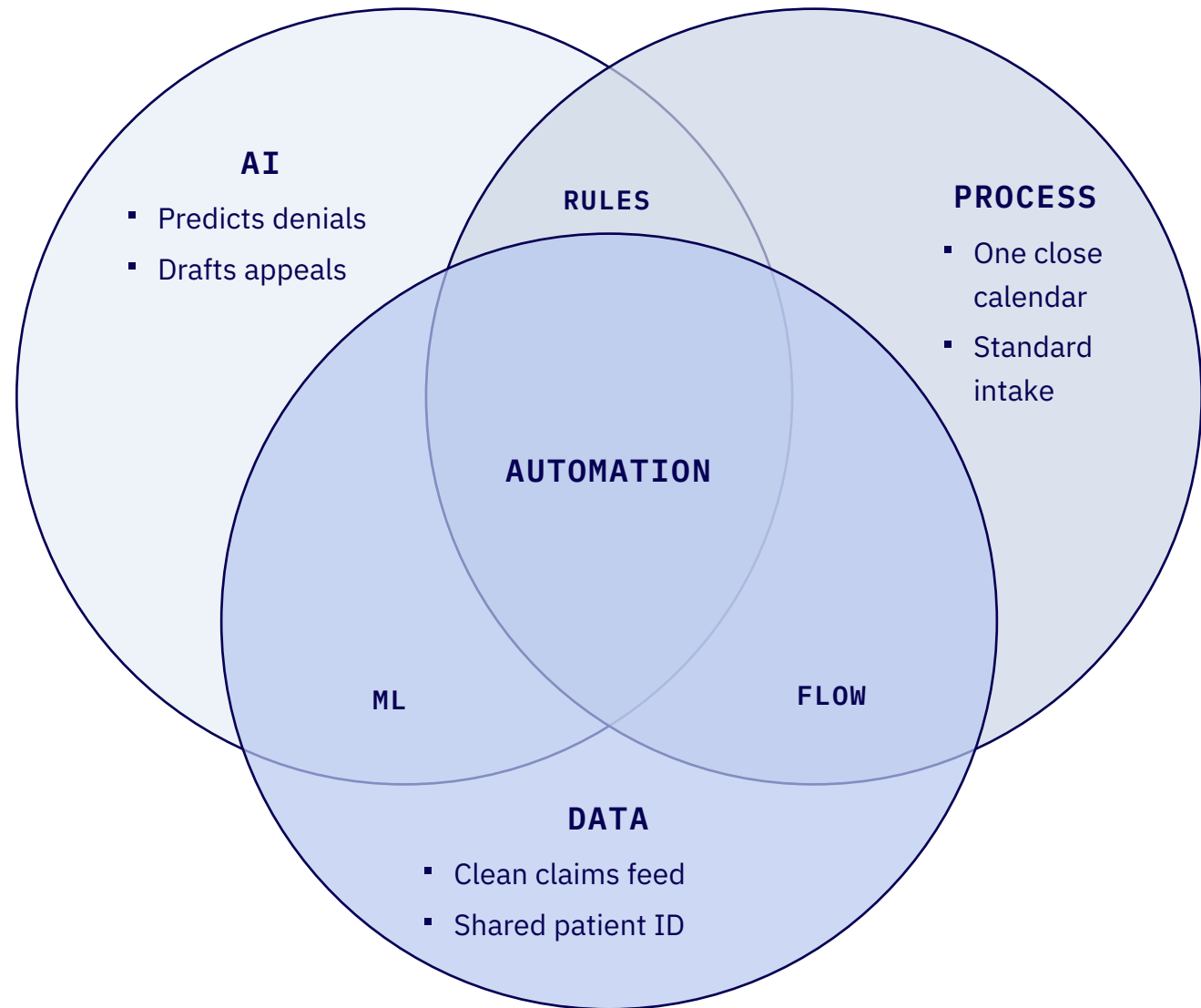
Source: Northfield finance and operations workshops, Sep 2026.

CAPABILITIES

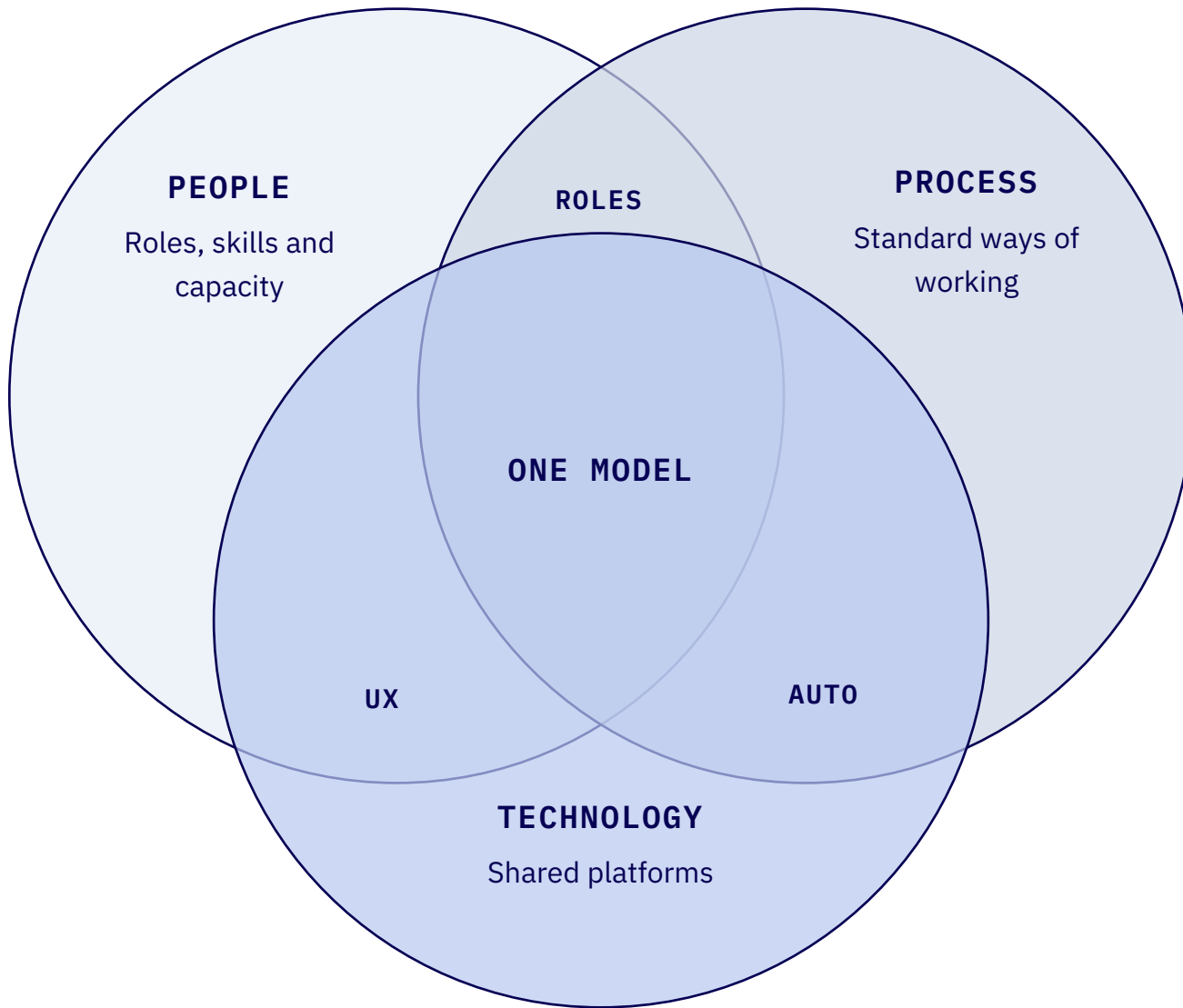
AI pays off only on shared process and data

Three capabilities, one result

Each capability stalls alone. Models need clean data, and clean data needs a standard process to keep it that way.



Source: Northfield finance and operations workshops, Sep 2026.



OPERATING MODEL

Three teams share one operating model

What each team brings

- **People.** Hire and train for the model.
- **Process.** Document it once.
- **Technology.** Automate the standard.

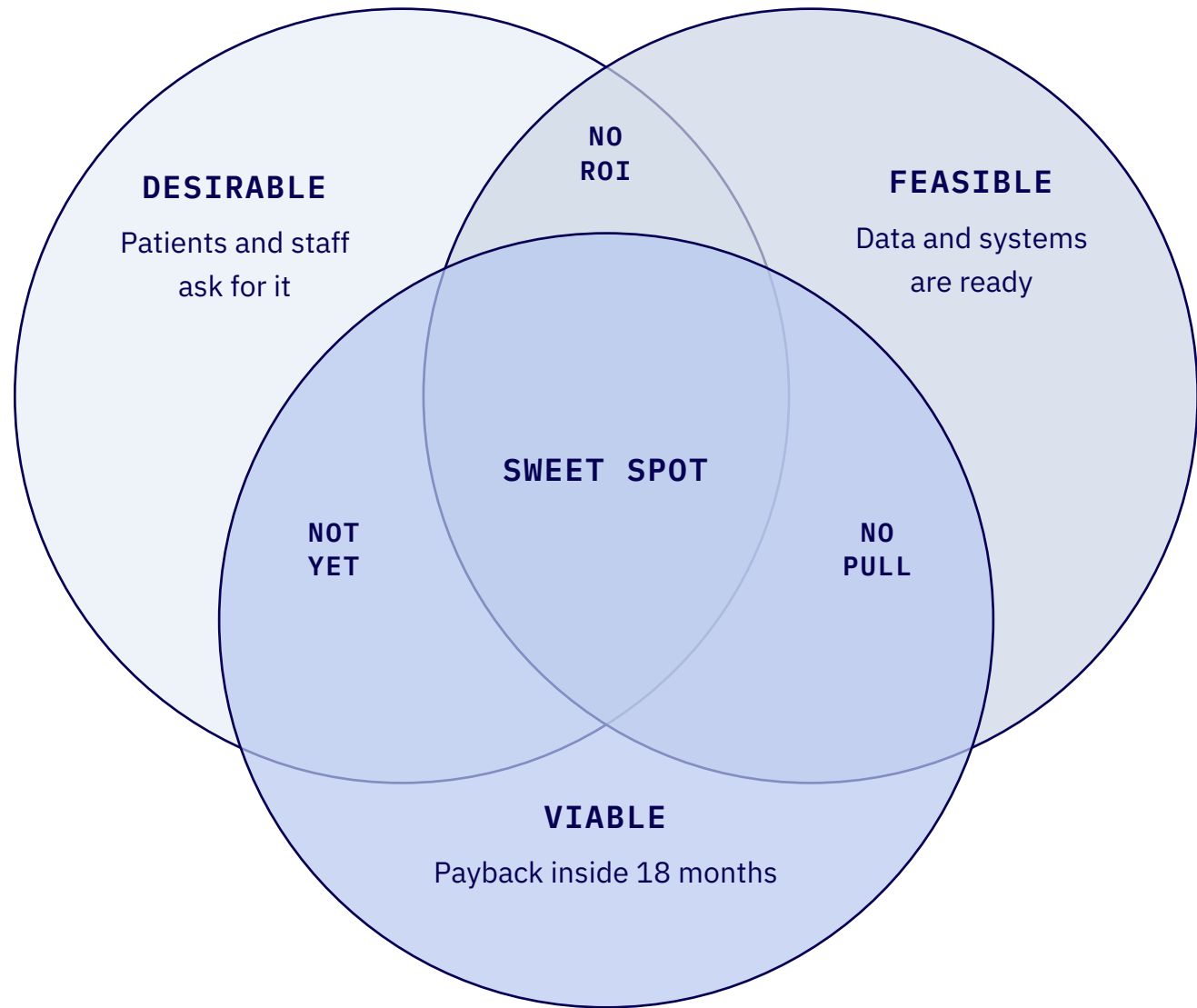
Source: Northfield finance and operations workshops, Sep 2026.

OPTIONS

Options that clear all three tests

Screening result

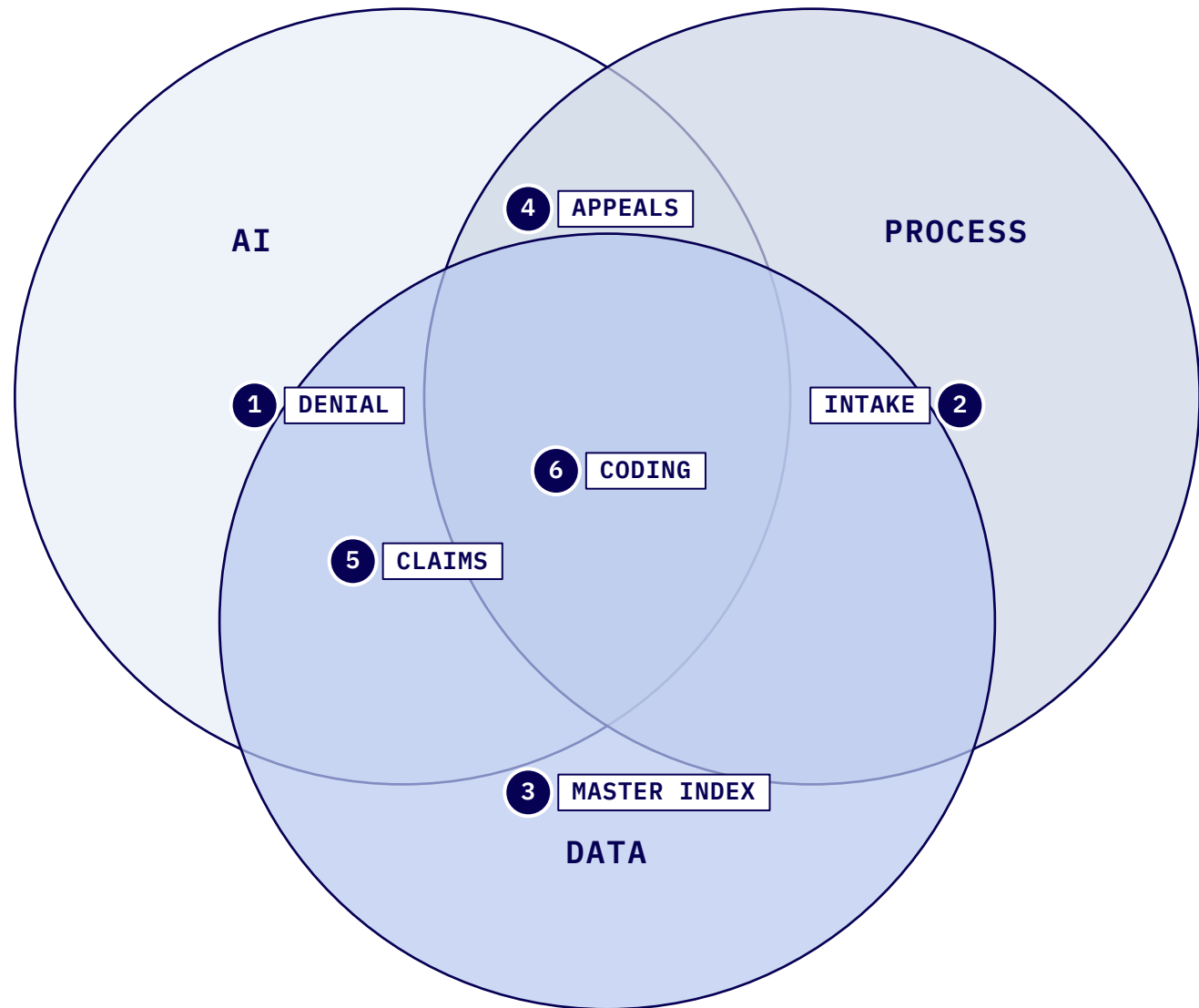
- Two options clear all three tests
- Four clear two and need a fix



Source: Northfield finance and operations workshops, Sep 2026.

Six initiatives mapped to three capabilities

- 01 Denial prediction**
AI only.
- 02 Intake redesign**
Process only.
- 03 Patient master index**
Data only.
- 04 Appeal drafting**
AI and process.
- 05 Claims analytics**
AI and data.
- 06 Autonomous coding**
All three.



Source: Northfield finance and operations workshops, Sep 2026.

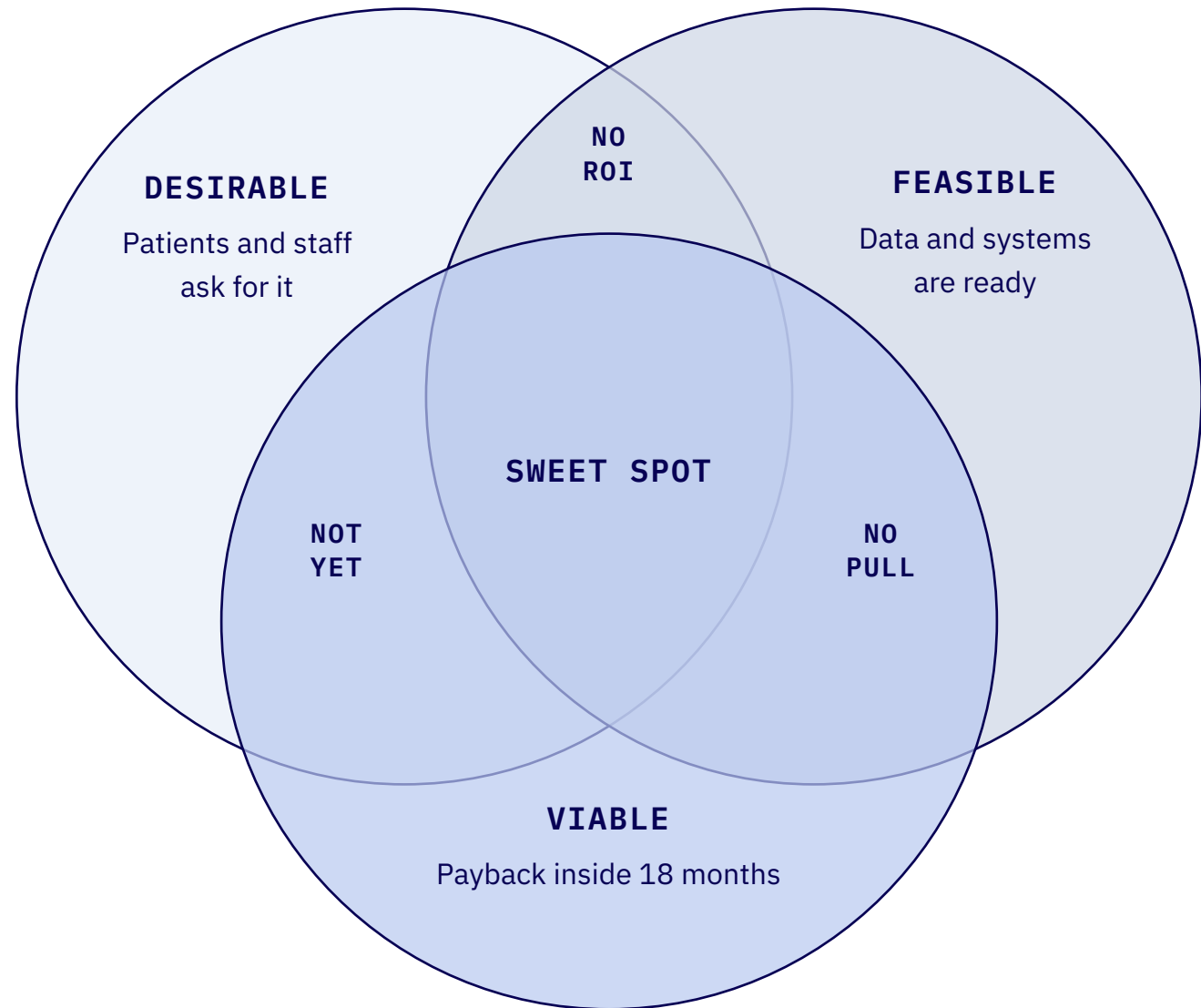
PRIORITIZATION

Fund what passes all three tests

Three ways to miss

- **Unfunded.** Wanted and buildable, no payback.
- **Not yet.** Wanted and pays back, not buildable.
- **No demand.** Buildable and pays back, unwanted.

Fund the sweet spot first, then fix the gap in each overlap.

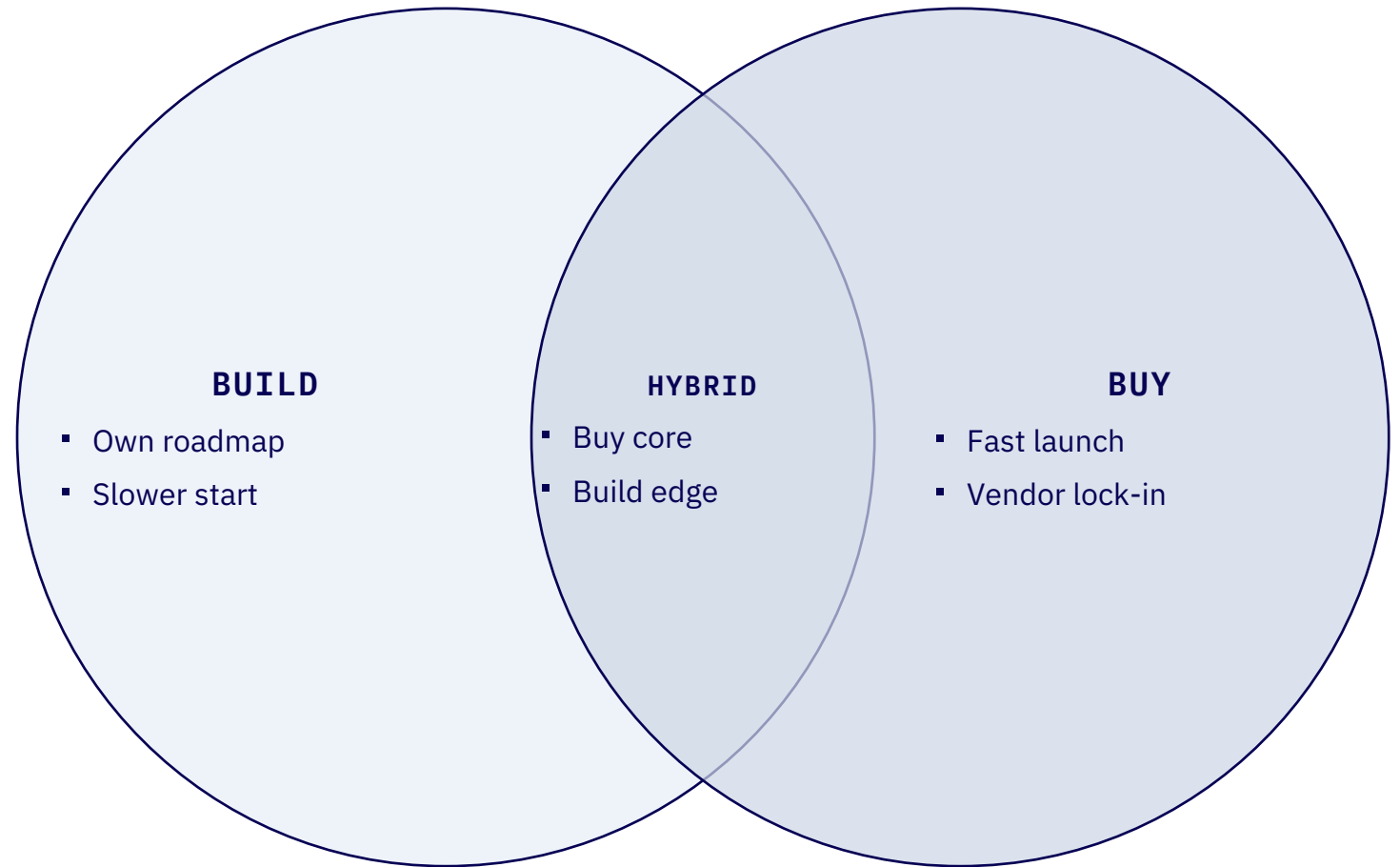


Source: Northfield finance and operations workshops, Sep 2026.

Build and buy overlap on shared data

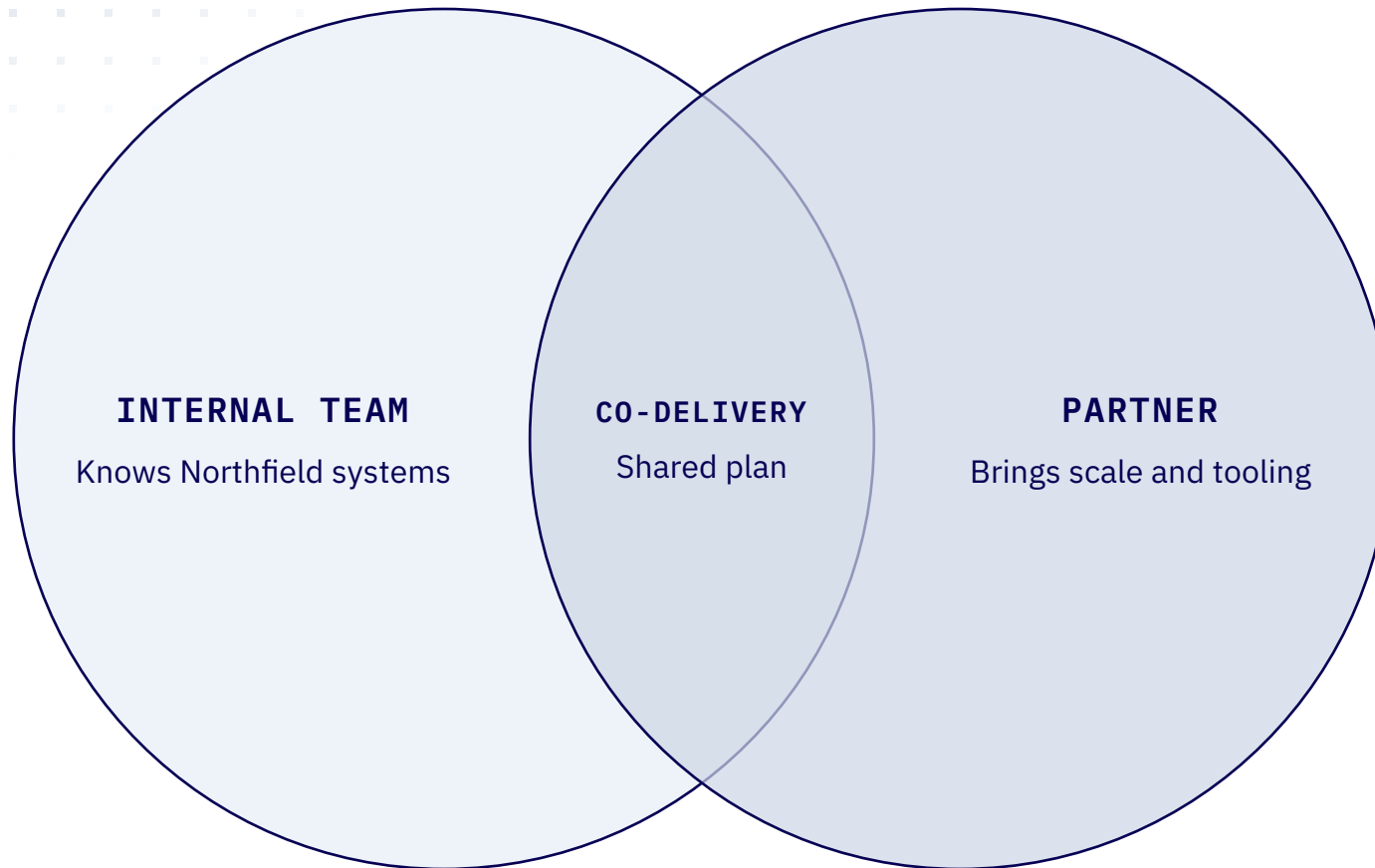
Choose by differentiation

- **Build.** Where we compete.
- **Buy.** Where we do not.
- **Hybrid.** Where both help.



Source: Northfield finance and operations workshops, Sep 2026.

Co-delivery blends internal and partner strengths



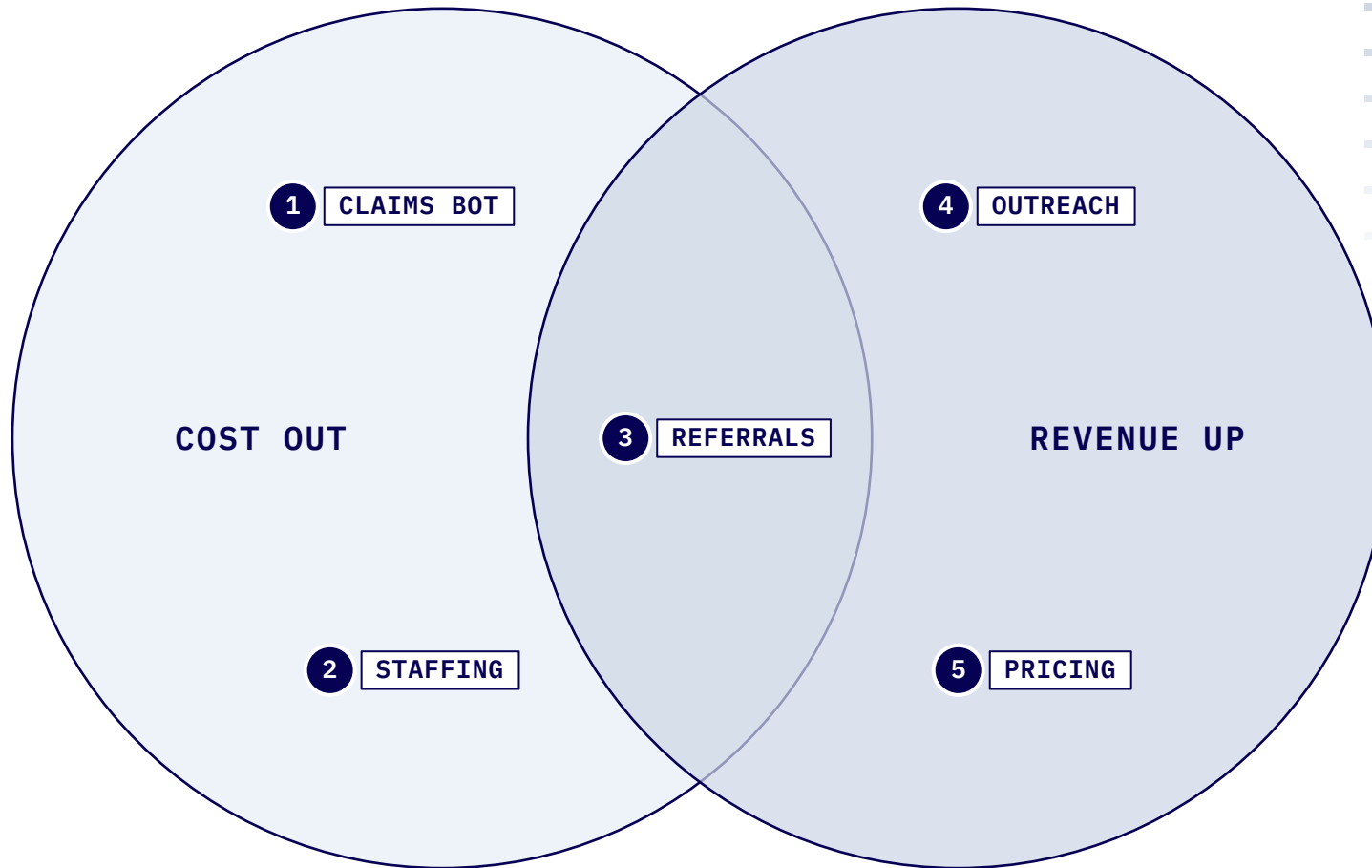
Why co-deliver

- **Internal.** Keeps context and ownership.
- **Partner.** Adds capacity and speed.
- **Together.** Faster, with knowledge retained.

Blend the two teams on every workstream, not only at handoffs.

Source: Northfield finance and operations workshops, Sep 2026.

Most AI initiatives cut cost, few grow revenue



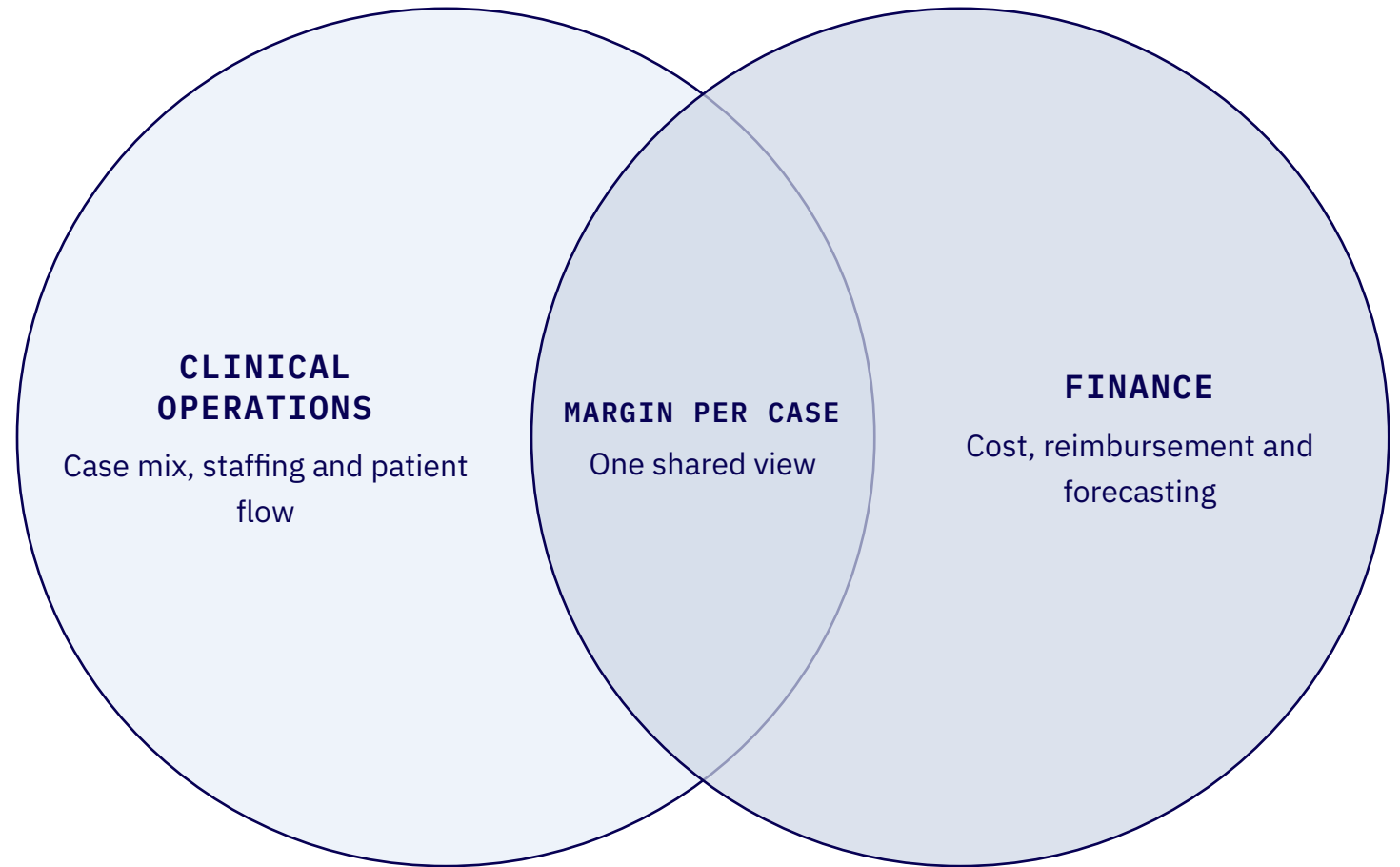
- 01 Claims status bot**
Cost out only.
- 02 Staffing forecast**
Mostly cost out.
- 03 Referral matching**
Both, at the overlap.
- 04 Outreach scoring**
Mostly revenue.
- 05 Pricing analytics**
Revenue up only.

Source: Northfield finance and operations workshops, Sep 2026.

Margin gains sit where two teams meet

Why the overlap matters

- **Clinical.** Owns volume and throughput.
- **Finance.** Owns cost and payment.
- **Overlap.** One shared margin view per case.



Source: Northfield finance and operations workshops, Sep 2026.

Five workstreams take the platform from project to service

01 Define the service catalog

Name who the platform serves and what it offers, so every team knows what is central and what is theirs.

KEY RISKS

- The platform becomes an IT silo
- Catalog goes stale within months

02 Set the standards

Agree the patterns, workspace rules and data sharing standards that every environment follows.

KEY RISKS

- Standards are ignored in practice
- Exceptions outnumber patterns

03 Industrialize delivery

Automate provisioning, release and monitoring so a new workload goes live without a project.

KEY RISKS

- It stays a project, not a service
- Manual steps slow every launch

04 Drive adoption and ownership

Open intake, training and a self-service path, then hand day-to-day ownership to Northfield teams.

KEY RISKS

- Adoption stalls at capacity limits
- Owners are never named

05 Prove value and cost

Baseline what workloads cost today, track savings against plan and report value to sponsors.

KEY RISKS

- Costs rise faster than usage
- Value is never shown to funders

Design general availability in from the start

01 Define the service catalog	02 Set the standards	03 Industrialize delivery	04 Drive adoption and ownership	05 Prove value and cost
ACTIVITIES ▪ Name the workloads served ▪ Split central from team services OUTPUT Published catalog	ACTIVITIES ▪ Approve patterns per environment ▪ Agree workload placement rules OUTPUT Signed-off standards	ACTIVITIES ▪ Automate provisioning ▪ Build CI/CD promotion paths OUTPUT Release pipeline	ACTIVITIES ▪ Open intake and onboarding ▪ Define the self-service path OUTPUT Trained owner teams	ACTIVITIES ▪ Baseline cost per workload ▪ Track savings against plan OUTPUT Value report
KEY RISKS IF THE WORK IS NOT DONE RIGHT				
The platform becomes an IT silo	It stays a project, not a service	Costs rise faster than usage	Adoption stalls at capacity limits	Value is never shown to funders